North East London
Integrated Care Board

NHS North East London ICB board

30 September 2026 13.00 – 16.00; Vicarage Lane Health Centre

AGENDA

	Item	Time	Lead	Attached/ verbal	Action required
1.0	**Welcome, introductions and apologies**	13.00	Chair	Verbal	Note
1.1	Declaration of conflicts of interest			Attached	Note
1.2	Minutes of the meeting held on 29 July 2026			Attached	Approve
1.3	Matters arising: • Previous resident story			Attached	Note
1.4	Actions log			Attached	Note
2.0	**Questions from the public**	13.05	Chair	Verbal	Note
3.0	**Resident story**	13.25		Verbal	Note
4.0	**Chair and chief executive report**				
4.1	Chair and chief executive officer's report	13.45	Chair/NCO	Attached	Note
4.2	Update on policy and public affairs	13.55	NCO	Attached	Note
5.0	**Quality**				
5.1	North east London Winter Plan 2026/27	14.05	CP	Attached	Approve
5.2	North east London maternity and neonatal service demand and capacity stock take	14.20	PG	Attached	Approve
6.0	**Strategy**				
6.1	Once for London ICB collaborative commissioning	14.35	HB	Attached	Approve
6.2	Neighbourhood health: progress, self-assessment learning and next steps	14.45	CP	Attached	Note
7.0	**Finance and performance**				
7.1	Financial overview	14.55	HB	Attached	Note
7.2	Performance reporting				
	• Provider quality and performance report	15.05	RC/ PG	Attached	Note
	• Place-based performance reporting	15.15	RC	Attached	Discuss
8.0	**Governance**				
8.1	Annual report and accounts	15.25	HB	Attached	Note
8.2	Governance update	15.35	NCO	Attached	Approve
8.3	Committee exception reports • Audit and Risk Committee • Remuneration Committee • Finance and Investment Committee • Strategic Commissioning Committee	15.45	Chair	Attached	Note

	Item	Time	Lead	Attached/ verbal	Action required
	• Quality, Safety and Improvement Committee • Transition Committee				
9.0	**Board forward plan**	15.50	Chair	Attached	Note
10.0	**Emerging risks**	15.55	Chair	Verbal	Discuss
11.0	**Any other business**	16.00	Chair	Verbal	Note
Date of next meeting: 25 November 2026					

NHS
North East London
Integrated Care Board

ICB Board members and attendees

Member	Role
Dame Marie Gabriel	Chair, NHS North East London and North East London Health & Care Partnership
Dr Nnenna Osuji	Chief executive officer
Diane Herbert	Non-executive member
Imelda Redmond	Non-executive member
Dr Kevin Cleary	Non-executive member
Lakh Jemmett	Non-executive member
Bidesh Sarkar	Non-executive member
Lorraine Sunduza	NHS trust partner member
Shane DeGaris	NHS trust partner member
Cllr Maureen Worby	Local authority partner member
Dr Jagan John	Primary care partner member
Dr Janakan Crofton	Primary care partner member
Najnin Islam	VCFSE partner member
Henry Black	Chief finance officer
Dr Paul Gilluley	Chief clinical and quality commissioning officer
Charlotte Pomery	Chief strategic commissioning officer
Ralph Coulbeck	Chief strategy officer
Participant	**Role**
Andrew Blake-Herbert	Local authority executive participant
Sally Beaven	Healthwatch representative
Anne-Marie Keliris	Head of governance and company secretary

Glossary of terms and abbreviations

Terms and abbreviations will also be explained within each report.

Term	Explanation
A&E	Accident and Emergency department
B&D	Barking and Dagenham
BAF	Board Assurance Framework
BH	Barts Health NHS Trust
BHRUT	Barking, Havering and Redbridge University Hospitals NHS Trust
BCYP	Babies, Children and Young People
C&H	City and Hackney
CAMHS	Children and Young People Mental Health Services
CCQCO	Chief Clinical and Quality Commissioning Officer
CEO	Chief Executive Officer
CFO	Chief Finance Officer
CHC	Continuing Healthcare
CIP	Cost Improvement Plan
CSO	Chief Strategy Officer
CSCO	Chief Strategic Commissioning Officer
DHSC	Department of Health and Social Care
ELFT	East London NHS Foundation Trust
FPIC	Finance, Performance and Investment Committee
ICB	Integrated Care Board
ICP	Integrated Care Partnership
ICS	Integrated Care System
INA	Integrated Needs Assessment
LA	Local Authority
LTCs	Long Term Conditions
MHLDA	Mental Health, Learning Disabilities and Autism
NEL	North East London
NELFT	North East London NHS Foundation Trust
NEM	Non-executive Member
NHSE	NHS England
NOF	National Oversight Framework
OPIC	Office for Pan-ICB Commissioning
PbP	Place-based Partnership
PELC	Partnership of East London Co-operatives
PCN	Primary Care Network
PHIC	Population Health and Integration Committee
PTL	Patient Tracking List
RTT	Referral to Treatment
QSIC	Quality, Safety and Improvement Committee
UEC	Urgent and Emergency Care
UTC	Urgent Treatment Centre
VCFSE	Voluntary, Community, Faith and Social Enterprise

Purpose, priorities, aims and our decision-making principles

Our agreed ambition, which is also that of North East London Health and Care Partnership which we are part of, is that "**We will work with and for all the people of north east London to create meaningful improvements in health, wellbeing and equity**".

To help guide our work, together partners have agreed **four priorities, or joint action areas,** where we want to create measurable change, which will create key outcomes for our system and place strategies. These are:

1. **Employment and workforce** – to work together to create meaningful work opportunities and employment for people in north east London now and in the future.
2. **Long term conditions** – to support everyone living with a long-term condition in north east London to live a longer, healthier life and to work to prevent conditions occurring for other members of our community.
3. **Children and young people** – to make north east London the best place to grow up, through early support when it is needed and the delivery of accessible and responsive services.
4. **Mental health** – to transform accessibility to, experience of and outcomes from mental health services and well-being support for the people of north east London.

Partners also agreed the following design or operating principles for our system:

Improving quality and outcomes: Individually and together, we will continuously improve access, experience and outcomes for and with our residents, with a specific focus on delivering integrated care in the neighbourhoods where our residents live and work. We will seek to learn together and from international best practice to continuously improve quality, to reinvent our ways of working and better secure our outcomes.

Securing greater equity: We will resolutely tackle inequality in outcomes and experience for our residents and staff, harnessing the diversity of our north east London experience to create better and more responsive solutions and utilising our combined resources to tackle the causes of inequality. We embrace the right of our residents to meaningfully participate, as an equal part of our team, benefiting from the strengths that they bring as individuals and communities.

Creating value: We will transparently work with our residents and staff to secure the maximum, sustainable benefit from our physical, digital and financial resources, repurposing what we have, reducing waste and taking care of our environment. Critically we will support and enable our most important resource, our staff, to reach their potential, enjoy work and be able to effectively contribute to our vision.

Deepening collaboration: We will work in meaningful partnership towards shared goals, holding each other to account for the commitments we have made to each other and to our residents. We will set resident interest and the common good as our

defining success measure and we will support our staff to lead and deliver across organisational boundaries. Our key collaboration will be with our residents, who will drive and co-deliver and evaluate the outcomes of our partnership

The four aims of our integrated care system

- To improve outcomes in population health and healthcare
- To tackle inequalities in outcomes, experience and access
- To enhance productivity and value for money
- To support broader social and economic development

Our decision-making principles

ICB board members have agreed a set of principles for decision making as follows:

- Always put the best interests of all the residents of north east London first within a culture where our residents are our partners and co- production is universally applied
- Proactively tackle health inequities in access, experience and outcomes. Demonstrably consider the equality, diversity and inclusion implications of the decisions we make
- Bring our experience and sector perspective, rather than representing the individual interests of any member organisation or place over those of another.
- Be open and transparent, including when we have challenges, and ensure our communities can hold us to account for delivery. Though this provide constructive challenge, but always remain 'solution-focused'
- Create a culture of creativity, innovation, improvement and inspiration, enabling transformation for better outcomes with our people and communities
- Be brave and ambitious for our communities, while ensuring we are grounded and realistic. In doing this consider risks and mitigations carefully, but not be risk averse where we believe we can make improvements for local people
- Support distributed leadership and decision making – close to people – being outcome focused whilst assuring performance.
- Demonstrate and enable collaboration, mutual accountability, shared learning, embedding of best practice and joint development.
- Secure the best value and benefit from our collective resources, maximising productivity.

North East London Integrated Care Board Register of Interests

- Declared Interests as at 22/09/2026

Name	Position/Relationship with ICB	Committees	Declared Interest	Name of the organisation/business	Nature of interest	Valid From	Valid To	Action taken to mitigate risk
Bidesh Sarkar	Non-Executive Member and Chair of Audit & Risk Committee	ICB Audit and Risk Committee ICB Board ICB Finance & Investment Committee Strategic Commissioning Committee	Non-Financial Professional Interest	St. Georges, Epsom & St. Helier University Hospitals & Health Group	Non-Executive Member and Chair of the Finance & Performance Committee	2025-10-13	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Professional Interest	Notting Hill Genesis	Non-Executive Director and Chair of the Audit & Risk Committee	2026-06-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	MLC Consulting	Entered a partnership with MLC Consulting to develop their public sector consulting business. MLC Partners have supplied recruitment services within the NEL system.	2026-06-01	Ongoing	Excuse myself from any involvement in tenders and contracts between NEL (and its system) and MLC Consulting. Declare interests at the beginning of meetings where there is or could be a perceived conflict on the agenda
Charlotte Pomery	Chief Strategic Commissioning Officer	ICB Audit and Risk Committee ICB Board ICB Finance & Investment Committee ICP Committee ICS Executive Committee Patient Choice Panel Strategic Commissioning Committee	Non-Financial Personal Interest	Pomery McGregor Consultancy Limited	Director of consultancy company, with husband who is also a director of the company. There are no employees and I have not carried out work through the company since 2011 and have never carried out any work in north east London	2009-06-01	Ongoing	No action required as no conflicts declared.
Henry Black	Chief Finance Officer	ICB Audit and Risk Committee ICB Board ICB Finance & Investment Committee ICS Executive Committee	Indirect Interest	Great Ormond Street Hospital NHS Trust	Wife is Deputy CFO	2026-01-01	Ongoing	Declarations to be made at the beginning of meetings
			Indirect Interest	GSTT NHS Trust	Daughter employed as a graduate trainee	2023-09-01	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Professional Interest	Phoenix Futures Charity	Trustee of charity	2025-01-01	Ongoing	Declarations to be made at the beginning of meetings
Imelda Redmond	Non-Executive Member	ICB Audit and Risk Committee ICB Board ICB Quality, Safety & Improvement Committee ICB Remuneration Committee Strategic Commissioning Committee	Non-Financial Professional Interest	Age UK East London	Chair of Trustees	2024-02-18	Ongoing	Declarations to be made at the beginning of meetings
			Financial interest	Intuitive	Member of an Advisory Board	2024-06-12	Ongoing	Declarations to be made at the beginning of meetings
Kevin Cleary	Non Executive Member	ICB Board ICB Finance & Investment Committee ICB Quality, Safety & Improvement Committee ICB Remuneration Committee Strategic Commissioning Committee	Financial Interest	Nottinghamshire Healthcare NHS Foundation Trust	Undertaking review of actions taken in relation to staff	2026-06-28	Ongoing	Declarations to be made at the beginning of meetings
Lakh Jemmett	Non-Executive Member	ICB Audit and Risk Committee ICB Board ICB Finance, Performance & Investment Committee ICB Quality, Safety & Improvement Committee	Non-Financial Professional Interest	NHS Property Services Ltd	Non Executive Director	2023-07-01	Ongoing	Declarations to be made at the beginning of meetings

Name	Position/Relationship with ICB	Committees	Declared Interest	Name of the organisation/business	Nature of interest	Valid From	Valid To	Action taken to mitigate risk
Dr Jagan John	Primary Care ICB Board representative	ICB Board ICB Quality, Safety & Improvement Committee	Financial Interest	Aurora Medcare (previously known as King Edward Medical Group)	GP Partner	2020-01-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Parkview Medical Centre	GP Partner	2020-05-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Together First CIC (GP Federation)	Practice is a shareholder	2014-01-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Harley Fitzrovia Health Limited	Director and shareholder	2018-01-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Diagnostics4u Ltd (previously Monifieth Ltd)	Director and Shareholder	2020-10-01	Ongoing	Declarations to be made at the beginning of meetings
			Indirect Interest	Aurora Medcare (previously known as King Edward Medical Group)	Other GPs are family members	2020-01-01	Ongoing	Declarations to be made at the beginning of meetings
			Indirect Interest	New West Primary Care Network	Brother / GP Partner is the Clinical Director	2020-11-01	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Professional Interest	NHS England - London Region	Personalised Care Clinical Director	2017-05-01	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Professional Interest	North East London Foundation Trust – Barking and Dagenham Community Cardiology Service	GPWSI in Cardiology	2011-08-01	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Professional Interest	New West PCN health inequalities programme	Co lead for health inequalities, and run by the federation (Together First CIC) on behalf of NEL ICB	2023-04-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Faircross Medical Care	GP partner and family member (brother) is also a partner	2024-04-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Protecting Forever Ltd	One of the directors of the company with my brother and 2 others who are not related	2025-04-30	Ongoing	No action required as no conflicts declared.
			Financial Interest	Prestige Wrap Services Ltd	Brother is co director and there are two other who are not related. I am a director of this company	2025-05-02	Ongoing	No action required as no conflicts declared.
			Financial Interest	Tivutah Ltd	Property and medical consultancy	2025-12-20	Ongoing	No action required as no conflicts declared.
			Financial Interest	South Neighbourhood Clinical Lead (Barking and Dagenham)	As part of the national neighbourhood pilot, I am the clinical lead for the South Neighbourhood. My practice (aurora medcare) is part of this where I am a partner of the practice, my other partner of this practice is my brother Dr Jason John (who is also the clinical lead for New West PCN). The Neighbourhood has 2 PCN involved, one of them is New West.	2026-03-02	Ongoing	Declarations to be made at the beginning of meetings

Name	Position/Relationship with ICB	Committees	Declared Interest	Name of the organisation/business	Nature of interest	Valid From	Valid To	Action taken to mitigate risk
Dr Janakan Crofton	NEL ICB Primary Care Partner Member	ICB Board	Financial Interest	Addison Road Medical Practice	GP Partner	2020-07-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Francis Road Medical Centre	GP Partner	2023-10-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	The Firs Medical Centre	GP Partner	2022-08-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Forest Surgery	GP Partner	2023-04-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Beam Park Medical Practice	GP Partner	2023-04-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	St James Medical Practice	GP Partner	2025-10-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Thamesmead Health Centre	GP Partner	2025-04-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Riverview Health LLP	Thamesmead Health Centre PCN member practice	2025-04-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Theodore Health Limited	Healthcare consultancy service	2024-04-02	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Caritas Health Ltd	Healthcare consultancy service	2024-03-15	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Melio Health Solutions Ltd	Medical Director	2026-08-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Walthamstow Central PCN	Dr Mathumai Kanthasamy (Spouse) is the PCN Clinical Director of Walthamstow Central PCN (Waltham Forest) and fellow GP Partner at GP Practices named above	2020-07-01	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Professional Interest	RCGP NEL Faculty	RCGP NEL Faculty Provost	2024-10-01	Ongoing	Declarations to be made at the beginning of meetings

Name	Position/Relationship with ICB	Committees	Declared Interest	Name of the organisation/business	Nature of interest	Valid From	Valid To	Action taken to mitigate risk
Dame Marie Gabriel OBE	ICB and ICP Chair	ICB Board ICB Finance & Investment Committee ICB Quality, Safety & Improvement Committee ICB Remuneration Committee ICP Committee NEM Remuneration Committee Strategic Commissioning Committee	Non-Financial Personal Interest	West Ham United Foundation Trust	Trustee	2020-04-01	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Personal Interest	East London Business Alliance	Trustee	2020-04-01	Ongoing	Declarations to be made at the beginning of meetings
			Financial Interest	Race and Health Observatory	Chair of the Race and Health Observatory, (paid). The Race and Health Observatory are now considering the potential to enter into contracts with NHS organisations to support their work to tackle racial and ethnic health inequalities.	2020-07-23	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Professional Interest	UK Health Security Agency	Associate NED, (paid), UKHSA works with health and care organisations to ensure health security for the UK population	2022-04-25	Ongoing	Declarations to be made at the beginning of meetings
Dr Paul Francis Gilluley	Chief Clinical and Quality Commissioning Officer	ICB Audit and Risk Committee Clinical Advisory Group ICB Board ICB Quality, Safety & Improvement Committee ICP Committee ICS Executive Committee Primary care contracts sub-committee	Non-Financial Professional Interest	British Medical Association	I am a member of the +organisation	2022-07-01	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Professional Interest	Royal College of Psychiatrists	Fellow of the College	2022-07-01	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Professional Interest	Medical Defence Union	Member	2022-07-01	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Professional Interest	General Medical Council	Member	2022-07-01	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Personal Interest	Stonewall	Member	2022-07-01	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Personal Interest	National Opera Studio	Trustee on the Board	2023-08-01	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Professional Interest	University of East London	Health Fellowship	2024-10-01	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Professional Interest	Greater London Authority	Appointed to the Mayoral Cultural Leadership Board	2025-02-27	Ongoing	Declarations to be made at the beginning of meetings

Name	Position/Relationship with ICB	Committees	Declared Interest	Name of the organisation/business	Nature of interest	Valid From	Valid To	Action taken to mitigate risk
Dr Nnenna Osuji	Chief Executive Officer	ICB Audit and Risk Committee ICB Board ICB Remuneration Committee ICP Committee ICS Executive Committee Strategic Commissioning Committee	Indirect Interest	GS1 UK Advisory Board	GS-1 is a not-for-profit organisation which promotes use of standards and is agnostic of product. I sit on the advisory board	2023-10-01	Ongoing	Declarations to be made at the beginning of meetings
			Indirect Interest	Dr. Nnenna Osuji Consulting Limited	Provision of independent sector specialist clinical services. No direct competition or impact with NEL or impact on/benefits from commissioning decisions. Current clinical practice at Nuffield Health and Circlehealth facilities and medicolegal reviews.	2006-10-01	Ongoing	Declarations to be made at the beginning of meetings
			Indirect Interest	Tottenham Spurs Foundation	Trustee member of the board. No direct impact in NEL.	2023-07-01	Ongoing	Declarations to be made at the beginning of meetings
			Indirect Interest	Tottenham Hotspur OSC - Health branch transitioning into a CIC	Work to leverage benefits of football to promote health in terms of heart, head and blood. May overlap with pan London United work and work with West Ham. No financial or other personal benefits	2025-06-01	Ongoing	Declarations to be made at the beginning of meetings
			Indirect Interest	Sickle cell Retinopathy Network	International partnership of clinicians focused on improving retinopathy care for patients with Sickle cell disease. No personal or financial benefits.	2025-01-01	Ongoing	Declarations to be made at the beginning of meetings
			Indirect Interest	Nuffield Trust	Trustee of Nuffield Trust. No personal benefit. No direct conflict. ICB may commission work from Nuffield Trust as with other think tanks. I will recuse myself from any such discussions.	2023-12-01	Ongoing	Declarations to be made at the beginning of meetings
			Indirect Interest	NHS England (London)	Co-Chair, London People Board Co chair London EDI Steering Committee HLRO appraiser	2021-06-01	Ongoing	Declarations to be made at the beginning of meetings
			Indirect Interest	University College London - Global business school for health	Honorary Professor support delivery of Executive programmes. Work to develop programmes	2025-02-01	Ongoing	Declarations to be made at the beginning of meetings
Ralph Coulbeck	Chief Strategy Officer	ICB Board ICB Quality, Safety & Improvement Committee ICS Executive Committee Strategic Commissioning Committee	Non-Financial Professional Interest	Princess Alexandra NHS Trust	I am an associate non-executive director at Princess Alexandra NHS Trust and attend the board and Quality and Safety Committee	2024-05-17	Ongoing	Declarations to be made at the beginning of meetings
			Indirect Interest	Barts Health	My spouse is a consultant haematologist at BBH	2018-04-01	Ongoing	Declarations to be made at the beginning of meetings
Shane Degaris	ICB partner member	ICB Board ICS Executive Committee	Non-Financial Professional Interest	University College London Partners	Board member (interim Chair 01/10/25-28/02/26)	2025-10-01	Ongoing	Declarations to be made at the beginning of meetings
			Non-Financial Professional Interest	NHS Spurs OSC	steering committee member	2025-04-01	Ongoing	Declarations to be made at the beginning of meetings

- Nil Interests Declared as of 22/09/2026

Name	Position/Relationship with ICB	Committees	Declared Interest
Andrew Blake-Herbert	London Borough of Havering	ICB Board ICS Executive Committee	Indicated No Conflicts To Declare.
Diane Herbert	Non Executive Member	ICB Board ICB Quality, Safety & Improvement Committee ICB Remuneration Committee	Indicated No Conflicts To Declare.
Lorraine Sunduza	ICB Board Member	ICB Board ICS Executive Committee	Indicated No Conflicts To Declare.
Maureen Worby	Member of Committee	ICB Board ICB Quality, Safety & Improvement Committee ICP Committee Strategic Commissioning Committee	Indicated No Conflicts To Declare.
Najnin Islam	Member of Committee	ICB Board ICP Committee Strategic Commissioning Committee	Indicated No Conflicts To Declare.
Sally Beaven	ICB Board Member	ICB Board ICP Committee	Indicated No Conflicts To Declare.

North East London
Integrated Care Board

Minutes of the NHS North East London ICB board

29 July 2026, 1.00pm – 4.00pm, MS Teams

Members:	
Dame Marie Gabriel (MG)	Chair, NHS North East London and North East London Health and Care Partnership
Dr Nnenna Osuji (NCO)	Chief executive officer
Diane Herbert (DH)	Non-executive member
Dr Kevin Cleary (KC)	Non-executive member
Lakh Jemmett (LJ)	Non-executive member
Henry Black (HB)	Chief finance officer
Dr Paul Gilluley (PG)	Chief clinical and quality commissioning officer
Charlotte Pomery (CP)	Chief strategic commissioning officer
Ralph Coulbeck (RC)	Chief strategy officer
Cllr Maureen Worby (MW)	Local authority partner member
Lorraine Sunduza (LS)	NHS trust partner member
Najnin Islam (NI)	VCFSE partner member
Attendees:	
Andrew Blake-Herbert (ABH)	Local authority executive participant
Sally Beaven (SB)	Healthwatch participant
Diane Jones (DJ)	Director of clinical and quality commissioning
Phillipa Cox (PC)	Clinical Head of local maternity and neonatal programmes
Anne-Marie Keliris (AMK)	Head of governance and company secretary
Katie McDonald (KMc)	Governance lead
Apologies:	
Bidesh Sarkar (BS)	Non-executive member
Imelda Redmond (IR)	Non-executive member
Shane DeGaris (SD)	NHS trust partner member
Dr Jagan John (JJ)	Primary care partner member

1.0	**Welcome, introductions and apologies**
	The Chair welcomed everyone to the meeting including members of the public who had joined the board meeting to observe. The Chair welcomed Lorraine Sunduza, Chief Executive of East London NHS Foundation Trust, as the new NHS Trust partner member to her first ICB Board meeting in public. The Chair advised people of housekeeping matters before proceeding.
1.1	Declaration of conflicts of interest
	The Chair reminded members of their obligation to declare any interest they may have on any issues arising at the meeting which might conflict with the business of the Integrated Care Board (ICB). No additional conflicts were declared. Declarations made by members of the ICB Board are listed on the ICB's register of interests. The register is available either via the governance team or on the ICB's website.

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1.2	Minutes of the last meeting
	The minutes of the meeting held on 11 June 2026 were agreed as a correct record.
1.3	Matters arising
1.3.1	Outputs and learning from the last resident's story
	CP presented the report which provided an update on actions taken in response to the resident story presented at the last meeting, which had focused on the experiences of parent carers supporting children and young people with Special Educational Needs and Disabilities (SEND).and outlined the following points: • The resident story had highlighted three principal themes: ○ The importance of engaging parent carers from the earliest stages of service design and commissioning. ○ The need for improved communication and support following the diagnosis of a child with SEND. ○ The importance of emotional, psychological and practical support for families navigating SEND pathways. • In response, the ICB and local authority partners have incorporated these themes into the newly submitted local SEND Reform Plans. These plans explicitly recognised the role of Parent Carer Forums and strengthened commitments to coproduction and resident involvement. All north east London places have submitted their plans to the Department for Education in accordance with national requirements. • Further work is underway to review access to talking therapies and mental health support for parents of newly diagnosed children, where gaps in provision had been identified. Additionally, work being undertaken through commissioning reviews and development of care frameworks to strengthen recognition and support for parent carers across the system. Members welcomed the report, and points of discussion included the following: • It is important we demonstrate that resident stories result in visible action and system learning. • The resident story programme should continue to influence strategic commissioning decisions and remain an important mechanism for maintaining a resident-led approach. The ICB board noted the report.
1.3.2	Assurance statement to NHS England
	NCO provided a verbal update and points included the following: • At the last ICB Board meeting, the Board delegated authority to the Chair, chief executive officer, chief finance officer and chair of the Finance, Performance and Investment Committee to submit an assurance statement to NHS England (NHSE). The assurance statement related to NHSE's expectations regarding ICB running cost reductions and compliance with the nationally prescribed capitation funding model of approximately £19 per head of the population. • The submission set out the ICB's current position and confirmed that the emerging organisational configuration was expected to meet national requirements. However, the submission also acknowledged the need to continue testing and evaluating the sustainability of arrangements over time as the organisation completes its transition programme. • Progress will continue to be monitored through the Transition Committee as part of the final phase of organisational redesign and implementation. Members discussed the update and points included the following:

	• It would be helpful to have assurance that oversight arrangements will remain in place through the final stages of implementation and that organisational effectiveness will continue to be reviewed alongside financial compliance. In response, it was confirmed that organisational performance, workforce implications and delivery capability will continue to be monitored and reported through established governance arrangements, including the Transition Committee. The ICB board noted the update,
1.4	Actions log
	The ICB board noted the actions taken since the last meeting.
2.0	**Questions from the public**
	The Chair explained that a question had been submitted for the meeting on 11 June 2026, but that as the executive lead had sent apologies, the question would be carried over to today's meeting. In the interim, a written response was provided to the person who submitted the questions. The questions submitted were received from Mary Burnett on behalf of 'NEL NHS Save Our NHS' regarding Health Navigator, and a copy of the questions and responses can be found on our website at the following link https://northeastlondon.icb.nhs.uk/about-us/about-nhs-north-east-london/our-board/questions-from-members-of-the-public/. Since providing a written response, a series of follow up questions have been submitted. Due to the technical nature of the questions, Dr Paul Gilluley confirmed that he would be seeking to meet with Mary to discuss these in detail to ensure the concerns are fully understood. The ICB board noted the update.
3.0	**Resident story**
	DJ presented the resident's story concerning a mother (anonymised as 'P') whose baby had been born prematurely at 24 weeks gestation following complications associated with placenta praevia. The resident had been unable to attend in person because her baby remains in neonatal care, and therefore her story was shared on her behalf. Key points arising from P's story included the following: • Positive experiences of maternity triage and hospital care. • Effective communication and clinical management. • Anxiety associated with transfers between Level 3 and Level 2 neonatal units. • The need for improved psychological support during neonatal care. • The importance of communication regarding ward and cot moves. • Difficulties accessing postnatal care while spending prolonged periods in neonatal units. Members thanked P for consenting for her story to be shared, and points of discussion included the following: • P's courage and openness was highlighted, particularly given the difficult period she and her family are experiencing. There was universal recognition that the story provided important insight into how maternity and neonatal pathways are experienced from a resident perspective. • A number of emerging actions have already been identified. These include improving preparation for anticipated neonatal transfers, strengthening communication throughout the neonatal pathway, reviewing psychological support arrangements, strengthening transitional and rooming-in arrangements,

	and considering how postnatal care can be delivered more flexibly for mothers whose babies remain in hospital. • It is important to recognise the complexity of care journeys when pregnancies become high-risk, and P's experience illustrates the challenges of coordinating care across multiple providers and settings. The ICB is committed to being resident-led and actions will be picked up regarding the preparation of families for intensive neonatal care and the psychological support required before and during such experiences. • We should recognise the distinction between commissioning additional psychological services and ensuring that compassionate, person-centred care is embedded within provider cultures. There is a risk that emotional support could become viewed as a separate service rather than an integral responsibility of all staff involved in delivering care. High-quality care should naturally recognise and respond to patient and family anxiety as part of routine practice. • Members discussed the balance between provider responsibilities and commissioner responsibilities in addressing these issues. It was acknowledged that whilst commissioners could specify expectations and promote service improvements, providers also had a direct responsibility to ensure that emotional and psychological wellbeing form part of holistic care. • Consideration should be given to the potential role of parents with lived experience in supporting other families navigating neonatal pathways, with recognition that peer support could be a valuable component of future service developments. • P's story demonstrated both examples of excellent care and opportunities for improvement. It is important to ensure that learning from resident experiences is translated into measurable service change and ongoing improvements in maternity and neonatal pathways. The ICB board noted the resident's story.
4.0	**Chair and chief executive reports**
4.1	Chair and chief executive officer's report
	The Chair and NCO presented the report which provided an update on the most significant activities undertaken by the chair and chief executive since the last ICB board meeting. The following key areas were highlighted: • At its last meeting, the Integrated Care Partnership (ICP) agreed three priority areas for collective action: fair employment and good work, giving every child the best start in life, and improving mental health through prevention and tackling inequalities. These priorities reflect wider system ambitions and reinforce the importance of focusing on the wider determinants of health alongside traditional health services. • The Mayor of London's new Health Improvement Plan has strong alignment in its themes with north east London's existing strategic priorities, particularly in relation to children and young people, mental health and cardiovascular health. • The Chair's annual appraisal objectives have been agreed, and the annual Fit and Proper Person Test reviews have been completed for Board members. • Extensive organisational change is taking place across the ICB, and it is important we recognise the significant contribution made by staff that are leaving the organisation through the redesign process. Approximately 95% of the redesign process has now been completed. • There have been significant national political developments since the last meeting, including changes in government leadership and emerging policy priorities, namely the increased focus on social care reform. It will be important for us to understand the implications of these national changes for the future direction of integrated care.

	• Staff and local residents who had received recognition through the King's Honours list were celebrated, and members welcomed the opportunity to celebrate achievements across north east London. • Tribute was paid to Ralph Coulbeck, Chief Strategy Officer and his contribution as Interim Chief Executive, who will be leaving the organisation to take up an exciting national opportunity at NHS Alliance. His contribution to strategic commissioning and system development was acknowledged by members Members discussed the report and key points included the following: • Given the scale of organisational change in terms of staffing numbers as well as functions, it was suggested that more could be done to sufficiently communicate these changes with residents and stakeholders to describe the ICB's responsibilities. In response, it was agreed that public understanding of the ICB remains limited and it was confirmed that the Executive Management Team have already started to consider how to strengthen strategic communications and improve public awareness of the organisation's role, neighbourhood developments and resident-led approaches. • Assurance was sought regarding sufficient human resources (HR) and workforce capacity to support the organisation through transition. It was then confirmed that workforce capacity remained under active review. Although the people function remains relatively well resourced compared with the overall size of the organisation, reductions in staff could create pressure so options are being explored to maximise collaboration across workforce and people functions. • A query was raised regarding how progress on digital transformation and implementation of NHS Online is being measured and benchmarked against other systems. In response it was explained that north east London performs relatively well on a range of digital measures, including NHS App utilisation and electronic patient record coverage, but a more comprehensive report on digital maturity and progress would be beneficial. It was agreed that this should be considered through the Strategic Commissioning Committee. **ACTION:** Report to be presented to the Strategic Commissioning Committee regarding digital transformation, including measures of digital maturity. The ICB board noted the report.
4.2	Headline update on key current affairs
	CP presented the report and explained the following points: • The report provides an update on newly appointed elected members and political leadership positions following recent local government elections and leadership appointments. • A series of significant partnership submissions have been made jointly with local authorities, including Better Care Fund returns and SEND Reform Plans. Considerable collaborative work has been undertaken across north east London to meet national deadlines and the importance of maintaining productive relationships with local government partners was highlighted. • Preparations are underway for winter planning; however, planning is increasingly being reframed as "seasonal planning" to reflect the health impacts associated with both cold weather and extreme heat events. Heat-related pressures are contributing to increased demand within urgent and acute services and need to be considered alongside traditional winter pressures. Members discussed the report and points included the following: • The evolution from traditional winter planning to a broader seasonal planning approach is welcomed, as health and care services are increasingly

	experiencing operational pressures associated with heatwaves and extreme weather events. Planning arrangements should reflect the changing nature of population health risks and service demand. • Members should note that Cllr Susan Masters has now been appointed as Chair of London Borough of Newham's Health and Wellbeing Board. The ICB board noted the report.
5.0	**Quality**
5.1	Summary of the Ockenden and Amos reports, with immediate actions undertaken by NHS England and NHS North East London ICB
	PG, DJ and PC presented the report, and key points included the following: • The Ockenden Review represents a detailed investigation into maternity services in Nottingham, involving over 2,000 cases and extensive engagement with families. The Amos Review provides a broader national assessment of maternity services, incorporating evidence from families, staff and service users across England. Both reports identified recurring themes previously highlighted in earlier investigations, including failures to listen to women and families, poor organisational culture, workforce pressures, discrimination and inequalities, and weaknesses in governance and accountability. • These findings are not entirely new and mirror issues emerging from previous reviews of maternity services nationally, including Morecambe Bay, Shrewsbury and Telford and East Kent. The persistence of common themes indicate that systemic change remains necessary. • NHS England has now launched a 100-day implementation programme based on ten immediate actions. These cover: o Anti-discrimination and equity initiatives. o Enhanced listening and engagement arrangements. o Strengthened responses to incidents and safety concerns. o Introduction of Martha's Rule into maternity services. o Workforce and leadership improvements. o Greater accountability and assurance mechanisms. • Within north east London, progress is already underway and includes: o Appointment of a dedicated lead commissioner for maternity and neonatal services. o Recruitment of additional clinical leadership arrangements. o Ongoing demand and capacity reviews. o Strengthening the Local Maternity and Neonatal System (LMNS). o Development of maternity triage improvements, neonatal pathway redesign and home birth service reviews. Members welcomed the report, and points of discussion included the following: • Concerns were shared that women and families in north east London have experienced periods where maternity services have failed to meet expected standards. Whilst recognising progress has been made, it is important to refrain from treating the issues identified nationally as remote or historical. Proposed actions will need to result in demonstrable improvements and measurable outcomes. • Several of the commitments are framed as aspirations rather than deliverable actions. We will need specificity regarding milestones, timescales, target outcomes and methods for measuring success. Future updates should include explicit delivery trajectories and evidence demonstrating whether improvements are being achieved. • Many of the findings relate to deeply embedded cultural behaviours rather than isolated operational issues. The system will need to sufficiently understand the scale of cultural change required and distinguish between commissioning

	additional services and ensuring providers deliver compassionate, respectful and responsive care as a standard expectation. • Concerns were raised regarding capacity given the significant workforce reduction and increasing commissioning responsibilities, and whether the ICB has the resource to oversee large-scale maternity improvement programmes whilst maintaining oversight of broader quality agendas. In response, it was explained that steps are being taken to strengthen maternity commissioning arrangements, including dedicated leadership responsibilities and participation in national commissioning support programmes designed to build expertise and capability. • Thought should be given to how we can be assured that women feel listened to, particularly women from communities experiencing inequalities or discrimination. In response, the importance of lived experience data was highlighted and that maternity voice partnerships and systematic resident engagement are being used alongside performance measures. • A query was raised as to whether existing reporting arrangements are sufficiently granular to identify disparities affecting specific communities. In response it was explained that there are limitations within national data classifications and that work is underway to disaggregate data wherever possible and complement quantitative data with local intelligence gathered through community engagement. • Proposals to repurpose underused maternity facilities were discussed, including opportunities to develop women's health services in communities where birthing services are no longer provided. In response it was explained that meaningful engagement with communities will need to take place before any decisions are taken. • There are opportunities arising from the ICB's strategic commissioning role, including consideration of how patient experience and family feedback could become core commissioned outcomes rather than supplementary information. Cultural improvement, resident experience and outcomes should increasingly shape how services are commissioned and assessed. • A further report containing measurable actions, defined milestones, delivery timescales and clear indicators of success should be presented back to the Board. It was agreed that detailed scrutiny should be undertaken through the Quality, Safety and Improvement Committee before returning to Board. **ACTION:** Report regarding how north east London will respond to the Ockenden and Amos reviews to be presented to the Quality, Safety and Improvement Committee, including a delivery plan, before returning to the ICB Board. The ICB board: • Noted the report findings and immediate actions required • **Endorsed** the recommended considerations for commissioners as a body of work for us to progress, noting that future updates will have deliverable actions, explicit delivery trajectories, target outcomes, methods for measuring success and demonstrate where improvements are being achieved • Considered what more is needed/can be done further for north east London. • Noted the transition to the new model from Local Maternity and Neonatal System (LMNS) to a Maternity and Neonatal Quality Improvement System (MNQIS) • **Supported** the plan for MNQIS to remain part of the ICB (rather than transfer to a provider organisation).
6.0	**Strategy**
6.1	Board Assurance Framework
	CP presented the report and outlined the following points:

	• The revised Board Assurance Framework (BAF) has been developed following extensive Board development sessions, discussions with Board committees and feedback from members. • The aim has been to create a framework that more accurately reflects the ICB's role as a strategic commissioner whilst retaining oversight of the wider system risks for which we remain accountable. • The revised BAF has been streamlined to improve clarity and focused more explicitly on strategic objectives, principal risks, mitigating actions, sources of assurance, risk appetite considerations, and progress monitoring. • The revised framework will support a more dynamic approach to risk management and enable improved oversight through Board committees and the Audit and Risk Committee. It is intended that committees will undertake detailed scrutiny of relevant risks before providing assurance back to the Board. Members discussed the report and points included the following: • Members welcomed the revised format, describing it as significantly clearer and more accessible than previous versions. The risks, controls and mitigating actions are presented in a more coherent manner, and the revised structure provides improved visibility of strategic issues facing the organisation. A message of thanks was extended to the governance team for their work on this. • There are uncertainties associated with proposed legislative reforms, including the NHS Modernisation Bill, so it will be important to have the ability to promptly include any emerging strategic risks into the BAF as they arise. In response, it was explained how the new BAF has been designed to be a living document that can evolve and adapt as new risks emerge. • A query was raised as to whether risk appetite had been applied consistently across all strategic risks and that mitigating actions are aligned appropriately with the level of risk the organisation is willing to tolerate. It was suggested that committees should further test these issues and report their findings back to Board. • It will be important to articulate a credible "path to green" for each risk area. Some strategic ambitions will require sustained effort over a number of years; however, clearer interim milestones will allow progress to be monitored more effectively. In response it was explained that longer target dates reflect the complexity of some risks, particularly those associated with health inequalities and wider population health outcomes. However, clearer intermediate indicators would improve transparency and assurance. **ACTION:** Committees of the Board to consider the BAF risks they hold responsibility for and suggestions to be incorporated into the next iteration of the document. The ICB board: • **Approved** the refreshed Board Assurance Framework. • **Approved** the closure of the previous Board Assurance Framework. • **Endorsed** the strategic risk architecture, risk ownership arrangements and committee oversight structure contained within the refreshed framework. • **Agreed** the proposal for regular dynamic review of the BAF, including routine review by risk owners, committee scrutiny, exception reporting and quarterly Audit and Risk Committee and Board oversight. • Noted the arrangements for ongoing review, monitoring and reporting of the BAF through Board committees and the Audit and Risk Committee.
6.2	Integrated Needs Assessment
	RC presented the report and highlighted the following points: • The Integrated Needs Assessment (INA) is one of the key building blocks supporting the ICB's role as a strategic commissioner. The INA brings together

	population, demographic, activity, expenditure and health outcome data alongside intelligence obtained from engagement with residents and communities. • The intention is to create a single evidence base that can support commissioning decisions, resource allocation and strategic planning across north east London. • The INA will be an evolving resource that will be updated and refined as new information became available. It has been developed in collaboration with directors of public health across north east London and is intended to support both system-wide and place-based planning. The data could increasingly be analysed at neighbourhood level, supporting the wider neighbourhood health agenda. • Around 50% of health and care expenditure is being used to support approximately 3% of residents with the highest and most complex needs. This finding reinforces the importance of proactive care, prevention and more effective support for residents with multiple long-term conditions and complex care requirements. Members welcomed the report and viewed it as a significant step forward in supporting evidence-based commissioning. Points of discussion included the following: • Stronger links will be required between the INA and existing Joint Strategic Needs Assessments (JSNAs) produced by local authorities. The INA should be viewed as complementary to, rather than separate from, established local intelligence and public health assessments. • Having a clearer narrative connecting regional and local needs assessments will strengthen confidence in the analysis. • It was suggested that some long-standing allocation patterns may have contributed to health inequalities between places so it would be helpful to understand whether there is a relationship between spending decisions and outcomes. • It would be helpful to understand whether the dataset could be used to identify emerging quality concerns and adverse trends in a similar way to performance reporting, such as mortality indicators, maternity outcomes and areas where service quality concerns might not be immediately visible through traditional performance measures alone. • The INA will support difficult prioritisation questions as emerging data may result in increased investment in some areas and reduced focus in others. In response it was explained that the purpose of the INA is to inform the full commissioning cycle, helping commissioners identify where resources should be focused, where additional intervention might be required and where existing investment may require review. The ICB board noted and discussed the findings of the Integrated Needs Assessment and the implications for the ICB's priorities.
6.3	Neighbourhood health: next steps
	CP presented the report and explained the following points: • Significant infrastructure has now been developed, including place-based governance arrangements, multidisciplinary team models, data and intelligence tools such as the INA, neighbourhood-level population health analysis, emerging care closer to home programmes, and increasing collaboration between providers, commissioners and local government. • The next phase of work will focus less on structures and more on delivery and outcomes. Emphasis will be placed on proactive care for residents whose needs are beginning to increase, intensive and coordinated support for residents with

	complex needs, reducing reliance on crisis-driven services, and improving integration across organisational boundaries. • Progress is being made in developing neighbourhood integrator functions. These arrangements are intended to provide leadership, coordination and partnership across neighbourhoods rather than creating new statutory organisations. Discussions are ongoing with local partners regarding how these functions might operate and which organisations might fulfil integrator roles. Members discussed the report and points included the following: • A request was made to provide flexibility to the proposed timeframes for establishing integrator arrangements given recent changes in political leadership across several boroughs. It is important that local authorities are given sufficient time to engage fully in discussions and help shape future arrangements. In response, it was agreed that there is some flexibility and that this would be discussed further outside of the meeting. • Members sought clarification about the implications for Primary Care Networks and whether future contracting arrangements might replace or restructure existing partnership models. In response it was explained that a national consultation is underway and that many of the detailed implications remain under development. • It will be important to have a consistent set of outcome measures across north east London whilst retaining flexibility for places to focus on local priorities. Having common measures could support the reduction of unwarranted variation and enable clearer comparison of progress across neighbourhoods. • Clarity was sought regarding the difference between care closer to home and proactive care. In response it was explained that proactive care is one of the strands within the wider care closer to home programme. • A query was raised regarding whether the establishment of neighbourhood integrators would result in the transfer of contracts or budgets or any longer-term implications. In response it was explained that any future delegation of responsibilities would be subject to appropriate governance arrangements, maturity assessments and accountability mechanisms. The ICB will retain overall accountability and continue to hold lead contracts where delegation occurred. • As neighbourhood plans emerge, these should be presented to the Strategic Commissioning Committee for oversight. **ACTION:** Develop neighbourhood health outcome measures, including a core set of north east London metrics. **ACTION:** Update on the care closer to home programme to be presented to the Strategic Commissioning Committee, including delivery progress and outcomes. The ICB board noted: • The vision and direction of travel for neighbourhood health in north east London • The progress in delivering against the national Neighbourhood Health Framework and NHS England oversight expectations • The development of the integrator function in north east London • The refreshed governance arrangements for the Community and Neighbourhoods Steering Group.
7.0	**Finance and performance**
7.1	Financial overview
	HB presented the report and outlined the following points: • The ICB is reporting a broadly breakeven position at Month 3, with a small year to date surplus of £15k which is £0.8m favourable variance to plan. The year

	end forecast remains breakeven, with run rate pressures in acute and mental health services being mitigated through a range of actions. • The north east London system is facing a range of financial pressures at Month 3, reporting a £15.6m adverse year to date variance, which is a £39.3m deficit against a planned £23.8m deficit. The variance is driven by providers, which are £16.3m adverse to plan, partially offset by the ICB's £0.8m favourable position. • The report highlights key financial risks including ADHD and autism assessment pressures, continuing healthcare expenditure growth, section 117 mental health aftercare costs, out-of-area mental health placements, delivery of efficiency and savings targets, and emerging prescribing cost pressures linked to national pricing and supply issues. • Spending has significantly exceeded the Mental Health Investment Standard, reflecting both recognised population need and a strategic decision to increase investment in mental health services. Approximately £20m of additional investment has been directed towards mental health initiatives. Members discussed the report and points included the following: • There is a significant challenge to achieve the shift of investment away from hospital-based care towards community and neighbourhood-based services, and evidence of the resource shift is not yet readily visible within financial reporting. • A query was raised as to how emerging findings from the INA will influence future resource allocation and whether funding decisions relating to areas such as continuing healthcare would increasingly be informed by population need and prevention goals. In response, it was explained that the INA is intended to become a central component of future resource allocation approaches and will increasingly support commissioning and budget-setting decisions. The creation of transformation funding has been designed to support investment in community, primary care and preventative models. • Assurance was sought that recovery actions being implemented by NHSE to Barking, Havering and Redbridge University Hospitals Trust (BHRUT) would not undermine commissioning priorities or neighbourhood health ambitions. In response it was explained that the ICB continues to work closely with both NHSE and provider partners to ensure that recovery plans remain aligned to wider strategic objectives. **ACTION:** Future reporting to reflect how resources are shifting from acute services into community and primary care services. The ICB board noted the Month 3 financial position and associated risks.
7.2	Performance report
	RC and PG presented the report and highlighted the following points: • The report is being redesigned to better reflect the role of the ICB as a strategic commissioner and to provide greater focus on place-based outcomes, population health and commissioning interventions. • Key points of current system performance include stronger performance against referral to treatment standards, ongoing challenges relating to diagnostics, some deterioration in aspects of cancer performance, continuing pressures within urgent and emergency care, and challenges within mental health access and talking therapies performance. • Recent operational challenges at BHRUT, including issues with the implementation of electronic patient records, continue to influence a number of system performance measures. • Place-based performance reporting is emerging and designed to support neighbourhood health ambitions and provide greater understanding of variation

	across localities. The report includes an initial prototype illustrating how performance, activity and population health measures could be brought together at borough level. • Work is underway to strengthen quality reporting, and a wider programme is in development for a quality management system and more comprehensive quality dashboards linking performance, quality and outcomes. Members welcomed the refreshed report, and key points of discussion included the following: • Neighbourhood and borough-level reporting will support a better understanding of variation and help local partnerships take ownership of performance and outcomes. • It will be important to maintain a manageable number of key performance indicators rather than allowing performance frameworks to become excessively complex. A smaller set of meaningful indicators will support more effective Board scrutiny and assurance. • It is positive to see the improved emphasis on commissioner actions and that the refreshed report addresses questions previously raised by Board members regarding oversight, accountability and strategic focus. Alongside areas of challenge the report also demonstrates examples of improvement across services, including primary care and mental health. **ACTION:** Smaller set of key performance indicators to be developed to support more effective Board scrutiny and assurance. The ICB board noted the report and the programme of work underway to evolve performance reporting in the longer-term.
8.0	**Governance**
8.1	Governance update
	CP presented the report and outlined the following points: • The report forms part of a wider programme of governance development with the objective of ensuring that governance arrangements are aligned with the ICB's evolving role as a strategic commissioner and reflective of the changes already approved through organisational redesign and committee development work. • The revised terms of reference have been developed collaboratively with committee chairs and members, with the intention of strengthening consistency across committees whilst ensuring each committee retained a clear and distinct purpose. Particular attention had been paid to: o Clarifying committee responsibilities and delegated authority o Strengthening links between committees and the Board Assurance Framework o Improving consistency regarding membership, quoracy and reporting o Ensuring effective routes for escalation and assurance to the Board o Supporting oversight of strategic commissioning responsibilities. • The report also sets out a proposed framework of principles to guide future amendments to the Scheme of Reservation and Delegation (SORD), enabling decisions regarding future delegation to be assessed against a consistent set of governance principles. Members discussed the proposal, and points included the following: • The significant progress made in modernising the governance framework was welcomed as the revised arrangements better reflect the ICB's future role. • In relation to Terms of Reference, it was suggested that minor amendments are made to reflect that committees provide scrutiny and assurance "to" rather than

	"on behalf of" the Board and that consideration is given to reflecting the importance of residents, communities and citizen engagement within the documents. • We should ensure that committee terms of reference remain broadly consistent, particularly around membership provisions and quorum arrangements within the Audit and Risk Committee. • References to the management of risks within the Finance, Performance and Investment Committee, should be aligned appropriately with the oversight responsibilities of the Audit and Risk Committee. • The revised committee framework provides greater clarity and improved visibility to partners in understanding where key discussions and decisions will take place. • The framework should include consideration of committee effectiveness measures, enabling committees to demonstrate how they are contributing to assurance, challenge and organisational improvement. **ACTION:** Amend committee Terms of Reference to reflect Board comments. Subject to the amendments outlined above, the ICB board: • **Approved** the refreshed Terms of Reference for the Committees of the Board. • Noted the proposed governance principles for future updates to the Scheme of Reservation and Delegation. • Noted how the revised governance framework aligns with the refreshed Board Assurance Framework and supports clearer assurance flows to the Board. • Noted the continued role of Provider Collaboratives, Place-based Partnerships, Health and Wellbeing Boards and the Integrated Care Partnership in supporting collaboration, engagement, transformation and delivery outside the formal ICB decision-making structure. • Noted the proposed approach to implementation through to September 2026, supported by transitional arrangements where required.
8.2	Committee exception reports for information
	The Chairs and executive leads of the committees of the board each presented an exception report which highlighted the work undertaken by its members since the last meeting. The reports included updates from: <u>Audit and Risk Committee</u> It was highlighted that external auditors had identified significant weakness in the governance around procurement processes, particularly relating to the use of procurement waivers and contract extensions. Whilst some exceptions had arisen for legitimate operational reasons, the overall volume of non-compliant procurement activity exceeded the level expected by auditors. <u>Remuneration Committee</u> There were no additional points to note other than those in the report. <u>Finance, Performance and Investment Committee</u> There were no additional points to note other than those in the report. <u>Strategic Commissioning Committee</u> The emerging VCFSE strategy represents an important component of future neighbourhood and prevention ambitions. It was suggested that once final development work had concluded, the strategy should return to the Board for formal discussion and endorsement. <u>Quality, Safety and Improvement Committee</u>

	There are ongoing concerns regarding elevated Hospital Standardised Mortality Ratios (HSMR) within Barts Health. The Committee received explanations regarding the underlying data and performance position but has sought further assurance before determining whether the concerns have been satisfactorily addressed. <u>Transition Committee</u> The committee discussed concerns regarding nursing leadership, quality oversight arrangements and organisational capability. Additional work is underway and further assurance will be sought at the committee's final meeting in September before responsibilities are formally transferred into other governance arrangements. **ACTION:** VCFSE strategy to be added to the Board forward plan. The ICB board noted the exception reports.
9.0	**Board forward plan**
	The Chair explained that work is underway to align future Board business with the newly agreed governance framework and committee structure. As governance arrangements mature, the forward plan will increasingly reflect the strategic priorities, assurance requirements and commissioning responsibilities of the Board. The ICB board noted the forward plan.
10.0	**Emerging risks**
	The ICB board noted the emerging risks during the discussions held at the meeting.
11.0	**Any other business and close**
	There was no other business to note.
Date of next meeting – 30 September 2026	

NHS
North East London
Integrated Care Board

Matters arising: Outputs from the resident's story

Below is a summary of the outputs and work being taken forward following the resident's story received at the last ICB board meeting.

Date of ICB board meeting	29 July 2026
Name of resident	'P'
Chief officer lead	Dr Paul Gilluley, chief clinical and quality commissioning officer
Themes of the story and experiences	Diane Jones, Director of Clinical and Quality Commissioning, shared a mother's story (anonymised as 'P') whose baby had been born prematurely at 24 weeks gestation following complications associated with placenta praevia. The resident had been unable to attend in person because her baby was in neonatal care. The key themes raised from P's experience included: • Positive experiences of maternity triage and hospital care. • Effective communication and clinical management. • Anxiety associated with transfers between Level 3 and Level 2 neonatal units. • The need for improved psychological support during neonatal care. • The importance of communication regarding ward and cot moves. • Difficulties accessing postnatal care while spending prolonged periods in neonatal units. Board members extended a message of thanks to P for sharing her story, and in discussion the following points were brought out: • Resident experience and learning: P's story provided valuable insight into the maternity and neonatal care pathway from a resident perspective, highlighting both positive aspects of care and areas requiring improvement. • Improving neonatal pathways: Members discussed opportunities to strengthen preparation for neonatal transfers, communication with families, psychological support, transitional care arrangements, and the flexibility of postnatal support for mothers whose babies remain in hospital. • Supporting families through complex care journeys: The discussion recognised the challenges faced by families navigating high-risk pregnancies and care across multiple providers and reinforced the importance of preparing families for intensive neonatal care and associated emotional impacts. • Compassionate care and psychological wellbeing: Members emphasised that emotional support should be embedded within everyday clinical practice and organisational culture, rather than being viewed solely as a specialist service offer. • Shared responsibilities for improvement: Discussion focused on the respective roles of commissioners and providers in driving improvements, with agreement that providers have a direct responsibility to deliver holistic, person-centred care while commissioners can support improvement through commissioning expectations.

	• Value of peer support: Consideration was given to the potential contribution of parents with lived experience in supporting other families navigating neonatal pathways.
Actions arising and follow up	A number of actions have since been agreed and taken forward since the Board discussion, including the following: • Whilst north east London has demonstrated overall improvement in in utero transfer (IUT) rates over the last three years, progress has recently plateaued. In response, an in utero transfer (IUT) task and finish group has now been established to oversee improvement of neonatal pathways across north east London. The group has strengthened the prioritisation and coordination of timely, clinically appropriate IUTs; embedding agreed pathways consistently across providers; and resolving identified operational and pathway-related issues. Delivery of the agreed actions and associated outcomes are monitored through the established maternity and neonatal governance arrangements. • Holistic care: The IUT task and finish group have discussed and agreed an approach that focuses on the wider needs of women and families when progressing pathway improvements, including communication, continuity of care and the impact of transfers across maternity, postnatal and neonatal services. This is a standing item of the Maternity and Neonatal Collaborative. • Service-user feedback and lived experience: north east London maternity providers will continue to work with neonatal Maternity and Neonatal Voices Partnership (MNVP) leads to ensure that service-user feedback and the lived experiences of women and families affected by preterm birth inform ongoing pathway development and improvement. It has been included in the ICB commissioning intentions for 2027/28. In addition, Tower Hamlets, Waltham Forest and Newham MNVP have held a maternity and neonatal listening session with insights used to support the development of more inclusive and equitable care and to further strengthen opportunities for peer support. • The ICB is in the process of finalising it's approach to quality which focuses on safety, effectiveness and experience which has been included in the commissioning cycle and commissioning intentions.

NHS
North East London
Integrated Care Board

ICB board – action log

OPEN ACTIONS						
Agenda item	**Meeting date**	**Action required**	**Lead**	**Required by/ RAG**	**Status**	**RAG**
5.1 A focus on children and young people's mental health as a strategic commissioner (2)	31.03.26	The new 0-25 strategic commissioning strategy to be shared with the MHLDA Provider Collaborative and presented at a future meeting, outlining clear timelines, milestones, areas of prioritisation and measurable outcomes.	CP	Nov 2026	A draft will be presented at several fora once developed in the autumn. Item scheduled on ICB Board forward plan for November 2026.	Scheduled
4.2 Headline update on key current affairs (1)	20.05.26	Review the political manifestos to better understand local priorities and align health discussions accordingly.	DHo	Nov 2026	This action has been reassigned following changing portfolios and an update will be brought to the November meeting following the party conferences.	Not due
6.4 Delivering the vision for integrated neighbourhood working in north east London (2)	20.05.26	Neighbourhood delivery to be scheduled at the next board development session.	CP	Feb 2027	Item scheduled for February's development session.	Scheduled
5.0 Mental health, learning disabilities and autism (MHLDA) out of area placements	11.06.26	Report regarding MHLDA out of area placements to be discussed at the Quality, Safety and Improvement Committee, including an overview of the data through a health inequalities lens.	CP	Oct 2026	Report scheduled for the Quality, Safety and Improvement Committee meeting in October.	Scheduled
4.1 Chair and chief executive officer's report	29.07.26	Report to be presented to the Strategic Commissioning Committee regarding digital	RC	Oct 2026	Report scheduled for the meeting on 14 October.	Scheduled

OPEN ACTIONS						
Agenda item	**Meeting date**	**Action required**	**Lead**	**Required by/ RAG**	**Status**	**RAG**
		transformation, including measures of digital maturity.				
5.1 Ockenden and Amos reports	29.07.26	Report regarding how north east London will respond to the Ockenden and Amos reviews to be presented to the Quality, Safety and Improvement Committee, including a delivery plan, before returning to the ICB Board.	PG/ DJ	Nov 2026	Report scheduled to be presented to the Quality, Safety and Improvement Committee in October and will be reported back to the Board in November.	Scheduled
6.1 Board Assurance Framework	29.07.26	Committees of the Board to consider the BAF risks they hold responsibility for and suggestions to be incorporated into the next iteration of the document.	CP	Sep 2026	Complete. BAF risks presented to the responsible committees and an updated BAF is included in the committee exception reports.	Complete
6.3 Neighbourhood health: next steps (1)	29.07.26	Develop neighbourhood health outcome measures, including a core set of north east London metrics.	CP	Feb 2027 - To be fed back as part of the neighbour-hood developme nt session	Work is underway to develop local outcome metrics, both those for north east London and those specific to place. We continue to await the neighbourhood health planning guidance while working with colleagues across the NHS and local government to develop our approach.	Not due and in progress
6.3 Neighbourhood health: next steps (2)	29.07.26	Update on the care closer to home programme to be presented to the Strategic Commissioning Committee, including delivery progress and outcomes.	CP	Sep 2026	Complete. Report presented on 9 September and information is included in the committee exception report.	Complete
7.1 Financial overview	29.07.26	Future reporting to reflect how resources are shifting from acute services into community and primary care services.	HB	Sep 2026	Complete. Update report is included on the agenda.	Complete
7.2 Performance report	29.07.26	Smaller set of key performance indicators to be developed to support more effective Board scrutiny and assurance.	RC	Nov 2026	The performance report has been further developed to include place-based measures in addition to the focus on key performance and quality indicators across our major providers. A small suite of key metrics will be developed as part	In progress

OPEN ACTIONS						
Agenda item	**Meeting date**	**Action required**	**Lead**	**Required by/ RAG**	**Status**	**RAG**
					of the planning process to measure progress on the ICB's agreed headline ambitions.	
8.1 Governance update	29.07.26	Amend committee Terms of Reference to reflect Board comments.	CP	Sep 2026	Complete. Final Terms of Reference have been published here https://northeastlondon.icb.nhs.uk/about-us/about-nhs-north-east-london/our-governance/	Complete
8.2 Committee exception reports	29.07.26	VCFSE strategy to be added to the Board forward plan.	CP	Nov 2026	Item added to the Board forward plan for November 2026.	Not due

North East London
Integrated Care Board

NHS North East London ICB board

30 September 2026

Title of report	Chair and Chief Executive Officer's Report
Author	Dame Marie Gabriel CBE, Chair Dr Nnenna Osuji, Chief Executive Officer
Presented by	Dame Marie Gabriel CBE, Chair Dr Nnenna Osuji, Chief Executive Officer
Contact for further information	Dan Hodgson Daniel.Hodgson12@nhs.net Chief of Staff
Executive summary	This report updates the Board on significant developments since it last met on 29 July 2026. It covers national proposals on devolution and the future NHS operating model; the emerging London Regional Operating Framework; the transition of NHS North East London Integrated Care Board into its strategic commissioning role, including culture and belonging; progress on neighbourhood health and integrated care coordination; and digital transformation, including NHS Online and new national guidance on ambient voice technology.
Action required	The Board is asked to note the contents of this report
Previous reporting	N/A
Next steps/ onward reporting	N/A
Conflicts of interest	No conflicts of interest have been identified.
Strategic fit	The report aligns to our strategic purpose, priorities and objectives of the ICB and ICS: • To improve outcomes in population health and healthcare • To tackle inequalities in outcomes, experience and access • To enhance productivity and value for money • To support broader social and economic development
Impact on local people, health inequalities and sustainability	The ICB operates to improve health outcomes, tackle inequalities and support our local residents. The contents of this report relate directly to the delivery of this.
Impact on finance, performance and quality	N/A
Risks	N/A

1.0 Introduction and context

1.1 We start with words of welcome to our new Board member, Dr Janakan Crofton, who has been appointed as a representative of Primary Care providers. Dr Crofton is a GP and managing partner of one of the largest at-scale primary care providers in

north east London, serving approximately 75,000 residents. He brings over 15 years' experience in primary care, system leadership and healthcare transformation, with expertise spanning health inequalities, population health management, integrated neighbourhood working and strategic commissioning. Dr Crofton currently serves as Provost of the North East London Faculty of the Royal College of General Practitioners and brings extensive board-level experience and a strong commitment to improving outcomes, reducing inequalities and supporting sustainable healthcare across north east London.

1.2 We are awaiting finalisation of the arrangements for the Local Authority representatives and will provide a verbal update on this at the Board meeting.

1.3 Since the Board last met on 29 July 2026, the national and London context has continued to change. Alongside national discussion about devolution and public service reform, work has progressed on a clearer operating model for the NHS in London. Within north east London, we have maintained our focus on strategic commissioning, neighbourhood health, digital development and the safe completion of organisational transition.

1.4 At the 3 September meeting, the Integrated Care Partnership (ICP) Steering Group agreed to establish a priority focus on employment for young people, with a specific focus on care leavers. This will be the priority focus for the next Integrated Care Partnership meeting. A different approach to the ICP meeting itself was also agreed. The focus will now be on inviting others with expertise into the meeting, asking partners to share their work, what works well and what is missing, concluding with a discussion on the specific value we can add to improving outcomes and synergy through wider collaboration.

1.5 This has remained a demanding period for our staff. We recognise the professionalism and commitment colleagues have continued to show while moving into new structures, supporting those leaving the organisation and maintaining delivery of our statutory functions and strategic priorities. Our emphasis remains on transparency, belonging, anti-racism, neurodiversity, kindness and psychological safety.

2.0 Significant developments

2.1 <u>National reform and the future NHS operating model</u>

2.1.1 During August, we continued to consider the Government's wider public service reform and devolution agenda, including proposals to align Integrated Care Board footprints with strategic authority arrangements. London has distinct geography and governance, and the implications remain under discussion. We will continue to engage constructively while maintaining our focus on the needs of north east London residents and the relationships that support their care.

2.1.2 The Department of Health and Social Care (DHSC) published the NHS Public Health Functions Agreement for 2026/27 on 20 August. It confirms the arrangements through which the Secretary of State delegates specified public health functions to NHS England and reinforces the three shifts of hospital to community, analogue to digital and sickness to prevention. It also emphasises improving vaccination uptake and access for underserved communities. These priorities are closely aligned with our work on prevention, neighbourhood health and reducing inequalities.

2.1.3 NHS England (NHSE) has also launched the NHS College of Leadership and Management, providing a nationally recognised centre of excellence for standards,

development, talent and accreditation. In its nascence, the College is welcoming input into what staff, leaders, managers and the wider community believe is important, as the College creates its business plan for the next three years and its offer to the service.

2.1.4 NHS England issued the Maternity and Neonatal 10 Point Plan assurance process on 12 August. Trusts are expected to submit a baseline assessment by 25 September and a progress update by 31 December, with trust board approval and escalation of significant concerns through Integrated Care Board and regional routes. This is a substantive implementation update since the July Board considered the national reviews, and reinforces the importance of visible system leadership, rigorous provider assurance and learning from women, babies, families and staff.

2.1.5 Following the releases of the Ockenden and Baroness Amos review findings, we have more recently seen the release of the Thirlwall Inquiry report into neonatal deaths at the Countess of Chester Hospital, which included 17 recommendations for change which will likely have implications for all ICBs, trusts and others. DHSC and NHSE have issued recommendations to trusts and ICBs which at their heart call for urgency, curiosity and openness and reinforce key protocols for care and training of all staff. Our thoughts go out to the families impacted by the loss of babies. We will review our services against these recommendations and report back to the board.

2.1.6 In August we saw a government announcement on the expansion of community mental health support across England, which included 159 new mental health centres to be opened to support prevention and early intervention in neighbourhoods, and dedicated mental health emergency care. For north east London this has supported

- Barking and Dagenham 24/7 Community Mental Health Hub
- London Borough of Barking and Dagenham Children and Young People and Learning Disabilities and Autism Crisis Accommodation
- Raybould Centre / Homerton Hospital Mental Health Crisis Assessment Service (MHCAS)
- 23 Primrose Square 24/7 Neighbourhood Hub
- Newham General Hospital MHCAS
- Flying Angel 24/7 Neighbourhood Hub
- Coborn Centre Children and Young People and Learning Disabilities and Autism Crisis Accommodation
- Tower Hamlets / The Royal London MHCAS
- Waltham Forest / Whipps Cross MHCAS
- The Chapel, South Forest Centre 24/7 Community Mental Health Hub

2.1.7 We welcome the announcement which links with our existing neighbourhood approach, with proposals to come through our commissioning routes in due course.

2.2 <u>London-wide strategy and strategic commissioning</u>

2.2.1 We have contributed to the development of an emerging NHS England London Regional Operating Framework. Its purpose is to make it clearer what should be done once for London, what should remain local and how organisations should work together where collective action adds value.

2.2.2 The current model proposes a focused London portfolio of transformation programmes and strategic enabling agendas, supported by more consistent programme oversight.

2.2.3 I (Marie) attended this month, the first meeting of the London Transformation Advisory Board, which has oversight of the London transformation programmes. We received presentations on London wide work on Neighbourhood Health, Digital access, Million Hearts and Minds Prevention Programme (the three key programmes within the London 10 year plan) and Life Sciences and Innovation Zone. We will continue to update the board as this new structure progresses. As agreed hosts of Office of Pan-ICB Commissioning (OPIC), we continue to develop the operating model with our regional partner ICBs and in line with national requirements, and will bring a paper through the respective ICB Boards over the coming period.

2.2.4 The Greater London Authority is consulting on a draft London Plan, providing the strategic framework for how London should grow and develop, covering the period 2026/27 to 2046/47 with plans for 558,000 homes in the first ten years, given the potential population growth of 902,000. For north east London, housing growth remains highest in Newham and Tower Hamlets; targets rise in Havering, Redbridge and Waltham Forest; existing Opportunity Areas remain a focus for growth; and Green Belt release is proposed in Barking and Dagenham, Havering and Redbridge. While we welcome the stronger recognition of healthcare infrastructure in the draft, there is an opportunity for greater parity of prominence, policy protection and delivery support as with housing, transport and economic growth. Our response will therefore build on our earlier "Towards a New London Plan" submission, which highlighted that healthcare infrastructure must be treated as essential infrastructure which is planned, funded and delivered alongside housing and population growth. It additionally focuses on the need for earlier priorities of health in all policies, health-ready housing growth, stronger health impact assessment, reform of infrastructure funding, healthy placemaking and measurable action on health inequalities

2.3 <u>Neighbourhood health and commissioning transformation</u>

2.3.1 Work has continued to develop the North East London Single Point of Access and Integrated Care Coordination models. The purpose is to support safer and more equitable assessment of need, improve access to alternative pathways and help residents receive the right care closer to home. I (Nnenna) spent an afternoon visiting the Integrated Care Coordination (ICC) Hub, where I was hugely impressed by the team who juggled multiple screens and gave expertise to paramedics on the scene caring for people in their homes. This approach, through the hybrid of both the Barts Health based REACH model and the more recent ICC model, reduce conveyancing in this cohort in 50 percent of contacts. This model also provides paramedics in the field with an important lifeline of reassurance and peer support.

2.3.2 Commissioning work has linked Integrated Care Coordination and Single Point of Access with Same Day Access, so service design, digital interoperability and outcomes are considered together rather than through separate initiatives. The developing model shows increasing maturity, stronger clinical leadership and collaboration, and we are swiftly developing our approach to recurrent commissioning and benefits realisation.

2.3.3 Our approach is now to move beyond individual pilots toward clear commissioning models, measurable benefits and greater consistency, while allowing appropriate adaptation to local communities. This includes demonstrating impact on access, resident experience, urgent care use and inequalities, and recognising differences in partnership maturity between places.

2.3.4 NHS England's consultation on the multi-neighbourhood provider (MNP) and single neighbourhood provider (SNP) contracting models closed on 10 September. In our

response to the consultation, we reaffirmed that the contracting model should create genuine opportunities for a range of partners to shape priorities, influence decisions, receive funding and deliver services directly not just through an intermediary or sub-contracting model, and highlighted that the proposals are yet to fully recognise the complexity of the current landscape and the need to prioritise the primacy of relationships, build new entities and develop innovation. We highlighted that the role of the ICB as strategic commissioner is fundamental and should be reflected throughout any new approach. Care is needed to minimise over-reliance on contractual reform, excessive procurement burdens – particularly under the Provider Selection Regime – and barriers for general practice and smaller community organisations. Overall, our response calls for greater local flexibility, national procurement reform and clarity on funding, implementation and articulating the additional benefits of the proposed model.

2.4 Digital transformation and NHS Online

2.4.1 Engagement with NHS Online has provided further information about its developing model and ambition to begin seeing patients during 2027. We were pleased to see the new chief executive officer of NHS Online has come from our patch. We congratulate Dame Caroline Clarke on her appointment and look forward to working with her in this new role.

2.4.2 Work is needed to understand how national digital routes will interact with existing local services, the NHS App, commissioning plans and future demand assumptions.

2.4.3 In other digital news, NHS England and the Medicines and Healthcare products Regulatory Agency issued updated guidance on Ambient Voice Technology. The guidance distinguishes products used for transcription and summarisation from those intended to support diagnosis, treatment or automated clinical action. It sets expectations for board oversight, clinical safety, information governance, staff training, procurement, interoperability and risk management. We welcome the potential for digital tools used safely, to reduce administrative burden and improve access and productivity, and will continue to consider how these are built into our commissioning programmes in a way that ensures that adoption is clinically safe, evidence-led and inclusive by design. We embrace the safe use of artificial intelligence to assist, augment and automate processes particularly within closed loop administrative functions under human supervision and accountability. The increased footprint of these digital developments creates an increased cybersecurity risk which is being reviewed in parallel.

2.5 Organisational transition, culture and belonging

2.5.1 Our organisational change programme is moving from implementation towards assurance, and transition to business as usual. As we move toward the tail end of the recruitment process, we sadly move toward larger numbers of our staff leaving the organisation – they do so with our gratitude and best wishes for the future.

2.5.2 We remain conscious that the change does not end here; it must be followed by sustained work to build effective teams, clarify responsibilities and ensure that staff are supported to succeed. As staff take on new roles and new teams form, this will now accelerate. We have curated a Reflect, Reset, Rebuild framework as we chart our next steps as an organisation. Our Reflect event held in July, provided helpful insight into what we can continue to do to support staff and what we can improve; we

take that feedback into the Reset event which is taking place over the week of this ICB Board meeting.

2.5.3 As we move out of reorganisation and into a new business as usual pattern, we would like to thank Board members involved in the Transition Committee set up to oversee the reorganisation. The Committee has received the assurances it needs and has agreed that it can be stood down with further oversight and refinement absorbed through named existing governance.

3.0 Performance

3.1 As we move into October, NHS England has commenced its mid-year reviews under the NHS Oversight Framework. Of the four segments, all north east London Mental Health providers have been rated in segment 2 (‘Above Average’), while all north east London Acute providers have all been rated in segment 3 (‘Below Average and/or Financial Deficit’). While NHS England is now responsible for provider oversight, the ICB continues regular engagement with providers to discuss, support and facilitate individual and collaborative improvement as appropriate.

3.2 The equivalent framework for ICBs is emerging although formal assessment and rating of ICBs, is yet to be finalised.

4.0 Events

4.1 New Secretary of State for Health and Social Care, Yvette Cooper, visited Homerton Hospital recently where teams to talked about the work they do every day, the challenges facing health and care services, and the difference that joined-up hospital and community care can make for local people. We are delighted that a visit to north east London and our Homerton colleagues, was one of the first to be made by the New Secretary of State for Health and Social care, giving opportunity for us to showcase the diversity of north east London and the community orientated approach of the Homerton.

4.2 I (Nnenna) attended the opening of the new wing and refurbishments at St Barts Hospital, alongside Dame Caroline Clarke. We heard of the amazing contributions and innovations in cardiovascular care and oncology and saw the impressive new facilities dedicated to breast surgical care. It was equally inspiring to meet some of the staff who spoke with a genuine spirit of patient care and innovation.

4.3 In August, NHS North East London hosted a Health Equity event with partners across the system to reflect on the impact of the North East London's Health Inequalities Programme and help shape what comes next. The event showcased examples of real collaboration delivering positive change for local people, while sharing learning on what works and how successful approaches can be scaled further. The insights gathered will directly inform future investment decisions, the ongoing development of neighbourhood health plans and delivery of the North East London Outcomes and Equity Framework.

4.4 Barts Health, supported by Barts Charity, delivered an event in the next phase of the Healthcare Horizons programme, helping young people build the skills, confidence and real-world experience to kick-start a career in healthcare. This is timely given the recent Milburn report highlighting the number of young people not in education, employment or training. One of our four priorities as an ICB remains work and employment, wealth being a key determinant of health.

5.0 Achievements

5.1 Congratulations to City and Hackney Primary Care colleagues, who have been named finalists in four categories at the Nursing Times Awards 2026, recognising the outstanding work of colleagues, and highlighting the passion, dedication and innovation of colleagues working across primary care to improve services and outcomes for local communities.

5.2 Congratulations also to our colleagues at Barts Health NHS Trust, East London NHS Foundation trust Digital team and North East London NHS Foundation Trust on their HSJ Awards nominations – a fantastic achievement and well-deserved recognition of their dedication, innovation, and commitment to improving care for people across north east London.

5.3 Finally, congratulations to Richmond Road Medical Centre, which recently received an 'outstanding' rating – the highest recognition the Care Quality Commission (CQC) can award – reflecting excellence across every aspect of general practice. This brings us to five GP practices in north east London with an outstanding rating, with Jubilee Street Practice and St Andrews Health Centre both achieving this on two successive occasions, a rare feat.

6.0 Conclusion and recommendation

6.1 The period since the July Board has reinforced the scale of change across national, London and local health and care arrangements. North east London has made important progress in developing its strategic commissioning role, embedding organisational transition and advancing neighbourhood and digital models. The next phase must remain focused on outcomes, equity, evidence and the practical experience of residents and staff.

6.2 The Board is asked to note the contents of this report.

Dame Marie Gabriel CBE
Dr Nnenna Osuji

North East London
Integrated Care Board

NHS North East London ICB board

30 September 2026

Title of report	Update on policy and public affairs
Author	Melissa Hoskins, Head of Communications
Presented by	Dr Nnenna Osuji, Chief Executive Officer
Contact for further information	Melissa Hoskins melissa.hoskins@nhs.net
Executive summary	This paper sets out a summary of key events and announcements since July 2026 that will have significance for the ICB and its work with partners and stakeholders.
Action / recommendation	The Board is asked to note this report
Previous reporting	n/a
Next steps/ onward reporting	n/a
Conflicts of interest	There are no conflicts of interest
Strategic fit	The ICS aims this paper aligns with are: • To improve outcomes in population health and healthcare • To tackle inequalities in outcomes, experience and access • To support broader social and economic development
Impact on local people, health inequalities and sustainability	Not applicable for this report
Has an Equalities Impact Assessment been carried out?	No, not applicable for this report
Impact on finance, performance and quality	There are no additional resource implications/revenue or capitals costs arising from this report.
Risks	No risks associated with this report

1.0 Introduction

1.1 This paper sets out a summary of key events and announcements since July 2026 with significance for the ICB and work with partners and stakeholders. This paper should read alongside the Chair and Chief Executive's report

1.2 The Board is asked to note the contents of the report

2.0 Key update – national context

2.1 The following announcements and developments will be of interest to Board members:

2.1.1 Social care reform

In August, the Prime Minister, Andy Burnham, set out his commitment to reform adult social care and deliver a National Care Service. Key to reforming the adult social care system will be the findings of the Independent Commission led by Baroness Louise Casey, which was set up last year to look at both shorter-term improvements to the social care system and the bigger questions that will lead to a National Care Service. The Prime Minister announced that the Commission's work will now be accelerated, delivering its report by summer 2027. The Prime Minister also announced that he will hold cross party talks with the leaders of the Liberal Democrat and Conservative parties.

The Prime Minister also made clear that the government will not wait until next summer to continue improving the social care system. He and the Secretary of State for Health, Yvette Cooper, talked about a new group bringing together all relevant ministers from across government, including the Chancellor and the Secretary of State for Housing, Communities and Local Government, to drive progress. This will include looking at ways to broaden the Fair Pay Agreement into a bigger reform to bring the workforce closer to NHS standards in terms of pay and progression routes; and ensuring tangible progress is made on the recommendations Baroness Casey has made already on improving support for those with dementia and motor neurone disease and strengthening adult safeguarding.

Baroness Casey also launched the 'Big Conversation on Care' in August. This will run until April 2027. This extensive engagement exercise presents a major opportunity for people with lived experience, families, carers and the wider public, to give their views on the future of adult social care and the big questions that underpin its reform.

2.1.2 **National rough sleeper pledge**

Everyone sleeping rough is to be offered a route off the streets this winter if they need it through the first major milestone in the Prime Minister's commitment to end rough sleeping

Under a £442 million package, areas with the greatest need will receive the largest share of support, to get people out of the cold this winter and give people facing long-term homelessness a settled home and the support to keep it.

New accommodation and support will be rolled out across England this winter, learning from innovative approaches put into place during the pandemic. Alongside emergency accommodation to end the need for rough sleeping, the programme will offer practical help with housing, health and other needs, helping people build a more stable future.

The Prime Minister also announced a first of its kind national summit on tackling rough sleeping, which is due to take place in the autumn.

2.1.3 **Expansion of Medical specialty training posts from February 2027**

In August, more than 350 new additional postgraduate training places were made available in England to start in February 2027. This goes further than the 250 places originally promised as part of the agreement reached with the British Medical Association's Resident Doctors Committee (RDC) on 29 June 2026. It forms part of a wider commitment to create up to 4,500 new postgraduate training places over the next three years. These posts are in addition to vacancies already scheduled for advertisement and recruitment

2.1.4 **Ministerial appointment to focus on social care and mental health**
Since the last Board meeting, Alison McGovern MP was appointed as Minister of State for Social Care and Mental Health in the Department of Health and Social Care.

Her portfolio covers: adult social care

- hospital discharge
- frailty and dementia
- end of life and palliative care
- disabilities and SEND (special educational needs and disabilities)
- mental health
- work and health

2.1.5 **Your Voice: building a better future for maternity and neonatal care**
More than 9,000 NHS staff members contributed their views to the Independent National Maternity and Neonatal Investigation led by Baroness Amos – and our concerns were heard.

In response, NHS England is now launching Your Voice to help staff voices shape the future and drive real and sustainable change in your services. This includes staff working in ICBs and not just those who work in maternity and neonatal strategic or commissioning roles.

Your Voice is an NHS-wide opportunity for an open conversation to determine what happens next for all maternity and neonatal services including those in north east London. The NHS want this to be the largest single conversation ever held about an NHS service, bringing together staff voices from across the country to help turn the Independent National Maternity and Neonatal Investigation led by Baroness Amos into meaningful action for the women, babies and families.

We are encouraging staff in the ICB, primary care and the wider community to share their views. This will help build the clearest possible picture of what needs to change locally and nationally. The insight will help inform the National Maternity and Neonatal Taskforce's action plan and provide valuable evidence for the Modern Service Framework, so that future decisions are grounded in the realities of staff, services and families. It will also help out our Trusts in north east London to test, shape and improve their local action plans.

2.1.6 **Martha's Rule extended to all hospital A&Es and waiting rooms**
The NHS has expanded Martha's Rule to all hospital A&E departments and waiting rooms to further boost patient safety for some of the most vulnerable patients.

This initiative will see patients, loved ones and staff in busy emergency departments able to call a dedicated Martha's Rule phone number to trigger an urgent review if they spot that a patient's condition is deteriorating, to ensure concerns are acted upon.

This follows a pilot at seven Trusts to test whether the escalation process could work safely and effectively in busy and fast-paced A&E environments. This found that Martha's Rule could work alongside existing clinical procedures to better spot the early signs of acute deterioration patients, without requiring additional clinical staff.

During the A&E pilot, 69 calls were made to dedicated Martha's Rule phone numbers, including some calls related to acute deterioration following an urgent clinical review. In some cases, this led to a patient having urgent surgery or being transferred to intensive care.

While Martha's Rule has to date been used to flag concerns for those already admitted to hospital, the landmark expansion provides additional safeguards in A&E waiting rooms should any patient begin experiencing acute deterioration while they are in the department.

3.0 Local news

3.1 The following announcements and developments will be of interest to Board members:

Chai and Chat bring residents together in Newham

Our Chai and Chat event in Forest Gate showed what is possible when health services, local government, voluntary and community organisations, and residents come together with a shared purpose. Delivered through partnership working between NHS North East London, NHS England, London Borough of Newham, Subco Trust, Blossom Group and Hawaa Network the event created a welcoming space for open conversations about health, wellbeing and vaccine confidence.

More than 200 residents joined us for an evening shaped by creativity, culture and lived experience. Through drama, poetry, music and storytelling in Hindi, Urdu, Punjabi, Bengali, Sylheti and English, community members shared powerful messages in ways that felt relevant, familiar and accessible. Alongside the performances, local services including Live Well Newham, Woodgrange Medical Practice and Vaccination UK Ltd offered health checks, advice and information.

Events like this matter because they help build trust, strengthen relationships and support more meaningful conversations. By working with communities, not just for communities, we can take practical steps to reduce health inequalities and improve access to care.

North east London residents receive outstanding care

We are delighted to share that Richmond Road Medical Centre in Hackney has been rated 'Outstanding' by the Care Quality Commission (CQC) – the highest inspection rating a GP practice can achieve.

Richmond Road is one of a number of our practices rated as outstanding, including the Jubilee Street practice in Tower Hamlets which was the first practice in England to achieve a 2nd outstanding rating from the CQC in December 2025.

4.0 Risks

4.1 No risks are identified as part of this update

5.0 Recommendation

5.1 Members are asked to note this report

6.0 End

6.1 Melissa Hoskins, Head of Communications
30 September 2026

North East London
Integrated Care Board

NHS North East London ICB board

30 September 2026

Title of report	North east London Winter Plan 2026/27
Author	Kelvin Hankins, Carla Morgan, Gemma Amin (NEL Proactive Care Strategic Commissioning Team)
Presented by	Charlotte Pomery, Chief Strategic Commissioning Officer (and NEL Winter Executive Sponsor)
Contact for further information	Kelvin Hankins, kelvin.hankins@nhs.net
Executive summary	The North East London (NEL) Winter Plan 2026/27 sets out a coordinated, whole-system approach to maintaining safe, timely and equitable care during increased winter demand. Developed with providers, Places, local authorities, primary care, ambulance, community, mental health and voluntary, community, faith and social enterprise (VCFSE) partners, it aligns with NHS England requirements and the new operating model, under which ICBs commission, providers deliver and regions oversee. The plan prioritises prevention, vaccination and proactive neighbourhood care; timely access to community and mental health alternatives; improved ambulance and hospital flow; and safe, earlier discharge. Demand and capacity modelling, additional winter schemes, tested escalation arrangements and System Coordination Centre oversight will support resilience.
Action / recommendation	The ICB Board is asked to approve and assure the NEL Winter Plan 2026/27 ahead of submission to NHS England.
Previous reporting	A first draft of the Winter Plan paper was reviewed by the executive management team on 7 September. All feedback has been addressed in the final paper.
Next steps/ onward reporting	The next step following ICB Board approval is submission to NHS England on 30 September 2026. Onward risk management, reporting and delivery oversight will be managed by the Urgent and Emergency Care Board, providing ongoing assurance to the executive management team and ICB Board.
Conflicts of interest	None identified.
Strategic fit	The ICS aims this report aligns with are: • To improve outcomes in population health and healthcare • To tackle inequalities in outcomes, experience and access • To enhance productivity and value for money
Impact on local people, health inequalities and sustainability	The Winter Plan is expected to benefit local people by improving access to timely, safe and coordinated care, with greater emphasis on

	prevention, vaccination, neighbourhood support and alternatives to hospital. Targeted outreach and population health approaches will prioritise protected, vulnerable and underserved groups, helping to reduce avoidable deterioration and unequal access. Sustainability will be supported by shifting care closer to home, reducing avoidable hospital use and making better use of system resources.
Has an Equalities Impact Assessment been carried out?	An Equality, Quality and Inequality Impact Assessment (EQIA) has been developed to inform the Winter Plan and has been endorsed by the Equality, Diversity and Inclusion (EDI), Quality, and Health Equity and Inequalities teams (Appendix 4). The EQIA concluded that the Winter Plan is expected to have an overall positive impact on equality by targeting support towards protected and underserved groups, with continued monitoring and accessible, non-digital alternatives required to mitigate potential barriers and prevent inequalities from widening.
Impact on finance, performance and quality	**Finance:** The ICB Board is asked to note that a business case has been developed to be taken through ICB governance by the Proactive Care Strategic Commissioning Team for the proposed use of Physical Capacity Funding for Winter 2026/27. The ICB, Neighbourhoods and Places are developing coordinated NEL-wide and local winter schemes, including acute respiratory infection (ARI) hubs, intermediate capacity and urgent community response capacity, aligned with Care Closer to Home, mental health, NEL and NHS England priorities, to improve outcomes and value for money, with commissioning and mobilisation intended before December 2026. **Performance:** Planned interventions aim to support the delivery of A&E 4-hour performance, ambulance response and handover times, hospital flow and discharge throughout periods of increased demand, while reducing avoidable conveyance, attendance and admission. **Quality:** The plan is expected to safe, timely and equitable care by strengthening prevention, vaccination, community alternatives, mental health crisis support and discharge pathways.
Risks	Key risks are articulated in Appendix 5 and include intermediate and mental health capacity, workforce pressures, corridor care, discharge delays and inconsistent use of alternative pathways. Delivery, quality, inequalities, performance and emerging risks will be monitored through established governance, with mitigations adjusted collaboratively.

1. Introduction

1.1 The full Winter Plan for north east London is attached as an appendix to this report. It has been developed with Provider Trusts, place teams local authorities, primary care, ambulance services and Voluntary, Community, Faith and Social Enterprise Sector (VCFSE) partners.

1.2 The Plan is organised around the patient journey, with neighbourhood health and prevention at its foundation. Multidisciplinary neighbourhood teams will use population health data to identify people most vulnerable to winter deterioration and provide earlier, coordinated support

across primary care, community services, mental health, social care and the VCFSE sector. Delivery of proactive care closer to home will reduce avoidable demand for ambulance and hospital services, ensuring acute capacity is used for the sickest patients. The system commitment to focus on safe, earlier discharge will improve hospital flow and help patients to return home quickly and safely. Together, these services will support people to stay well, access care within their own home and avoid preventable ambulance conveyance, emergency department attendance and hospital admission.

1.3 In this cover report, we highlight four areas of priority action to accent and accelerate impact and delivery over the winter period. In drafting the Plan, all partners have commented on the current high levels of demand and activity across all strands of urgent and emergency care, necessitating a focus on doing more in some areas in anticipation of further peaks in demand.

1.4 Also attached to this report is a set of Board Assurance Statements as required by NHS England (NHSE) to be signed off by the Board at today's meeting. These have been completed in light of work across the system to demonstrate preparedness for winter.

2. Winter Plan Overview

2.1 To support residents throughout the winter and strengthen service resilience, the ICB will prioritise four areas:

- enhanced discharge and support for care homes – so keeping people at home wherever possible
- improved access to primary and community care within neighbourhoods – so proactively working with communities
- increased uptake of immunisation and vaccination programmes – so maintaining a focus on prevention
- targeted communications and engagement – so enabling our community to keep themselves well

2.2 Together, these measures will strengthen proactive care, enable earlier intervention, reduce avoidable escalation and ensure residents receive timely, appropriate support during periods of increased winter demand. They provide a plan on a page of the areas where we want to see additional focus and emphasis in preparation for and response to winter pressures.

Enhanced discharged improvement and support for care homes

- Strengthening the Home First approach to accelerate complex discharges, with earlier escalation of residents at risk of delay to proactively address barriers, reduce length of stay, and support timely transfers of care back into the community.

- All care homes across North East London receive primary care support through the Care Home Local Incentive Scheme (LIS). In addition, a number of targeted initiatives are being tested to further enhance care for residents and reduce unnecessary hospital attendance and admissions:

  - **Redbridge:** Piloting an enhanced care home support model aimed at reducing London Ambulance Service (LAS) conveyances to hospital by providing additional support and intervention within care home settings.

  - **Havering:** Testing a dedicated NHS 111 Care Home Navigation Tool, designed to provide care homes with faster access to clinical advice and appropriate care pathways, helping residents receive the right care in the right setting and reducing avoidable hospital presentations.

Increased primary and community care support within neighbourhoods

- In addition to existing arrangements, a number of targeted initiatives are being implemented to strengthen community-based care, improve access to timely support, and reduce avoidable hospital attendances and admissions:
  - **Hackney and Waltham Forest:** Expanding Rapid Response services to increase the complexity of residents who can be safely supported within Rapid Response and Frailty Virtual Ward pathways, including the provision of intravenous (IV) therapy in community settings.
  - **North East London-wide:** Mobilising **Acute Respiratory Illness (ARI) Hubs** from November 2026, delivered through primary care across all seven boroughs, providing same-day assessment and treatment for residents with acute respiratory conditions and helping to reduce pressure on urgent and emergency care services.
  - **Barking, Havering and Redbridge (BHR):** Launching the Hospice Response Service from early December 2026, providing rapid palliative and end-of-life support in community settings, helping residents remain in their preferred place of care and reducing unnecessary hospital admissions.
  - **Children and Young People:** Strengthening post-attendance follow-up for children and young people presenting to A&E with an asthma exacerbation, ensuring a primary care review and updated personalised asthma care plan is provided within 48 hours of attendance to support ongoing management and reduce the risk of further episodes.
  - **North East London-wide:** Continuing to embed and enhance the Single Point of Access (SPoA), supporting the London Ambulance Service (LAS) to access appropriate community-based alternatives to hospital conveyance, ensuring residents receive care in the most appropriate setting for their needs.
  - Support high risk residents under existing proactive care models to transition to a Universal Care Plan.

Immunisation and vaccination programmes	Targeted communications and engagement
• **Increasing uptake of childhood immunisations** through targeted engagement and enhanced call and recall programmes. • **Delivering the RSV vaccination programme** for eligible older adults, care home residents and pregnant women. • **Mobilising the Autumn/Winter COVID-19 and flu vaccination campaigns** through primary care, pharmacies and outreach services. • **Reducing vaccine hesitancy** through PCN incentives, community pharmacy advice services and VCFSE-led engagement. • **Improving frontline staff vaccination rates** through workplace campaigns, peer vaccinators and pop-up clinics. • Supporting ICB staff to receive their flu vaccination through workplace campaigns, peer-led vaccination initiatives, and regular communications across staff networks and channels.	• **Delivering a coordinated winter communications and engagement campaign** focused on prevention, self-care and supporting residents to stay well throughout the winter period. • **Promoting uptake of seasonal vaccinations**, including flu, COVID-19 and RSV, through targeted communications and community outreach. • **Providing winter wellbeing advice and early intervention messaging** to help residents manage their health and seek support before conditions escalate. • **Improving awareness of local health and care services** by signposting residents to the most appropriate support, including primary care, community services and neighbourhood-based offers. • **Targeting communications and engagement activity at priority populations**, including older people, those with long-term conditions, pregnant women, children and families, using trusted community, primary care, digital and partner channels. • **Working with local authorities, VCFSE organisations, primary care and community partners** to co-design and deliver culturally appropriate campaigns that reduce health inequalities and avoidable demand on urgent and emergency care services. • Developing a targeted communications and engagement programme for residents with **serious mental illness** (SMI), promoting medication adherence, supporting self-management, and encouraging appropriate use of NHS 111 Select Mental Health (Option 2) to access timely advice and support and prevent crisis presentations.

3. Conclusion

3.1 It is important to recognise the high levels of collaboration which the Winter Plan presented here reflects. Each Provider Trust has developed their own winter plan, which aligns and enables the NEL Winter Plan and which requires partners to work together to deliver the additional focus required.

3.2 The Board is asked to consider the Plan and to approve the Board Assurance Statements which are presented, as required by NHSE colleagues.

Appendix 1

North East London
Integrated Care Board

North East London Winter Plan 2026/27 Board Summary

Executive Summary

The North East London Winter Plan 2026/27 sets out a coordinated whole-system approach to maintaining safe, timely and equitable care during periods of increased demand. It reflects the new operating model in the Model ICB and Model Region Blueprints: ICBs commission, providers deliver and regions oversee. As strategic commissioner, the ICB will align forecast demand with sufficient capacity across primary, community, mental health, intermediate and social care services, while coordinating operational escalation through established system arrangements. Providers remain accountable for delivery, workforce and flow, with regional oversight providing assurance.

Strategic approach for winter 2026/27

The plan is organised around the patient journey, with neighbourhood health and prevention at its foundation. Multidisciplinary neighbourhood teams will use population health data to identify people most vulnerable to winter deterioration and provide earlier, coordinated support across primary care, community services, mental health, social care and the voluntary, community, faith and social enterprise (VCFSE) sector. Delivery of proactive care closer to home will reduce avoidable demand for ambulance and hospital services, ensuring acute capacity is used for the sickest patients. The system commitment to focus on safe, earlier discharge will improve hospital flow and help patients to return home quickly and safely. Together, these services will support people to stay well, access care within their own home and avoid preventable ambulance conveyance, emergency department attendance and hospital admission.

Prevention and vaccination

Prevention activity includes seasonal wellbeing communications, anticipatory care, targeted health checks, medicines optimisation, falls prevention and targeted support for vulnerable cohorts. Increased vaccination uptake is a critical to winter resilience which will be delivered through a coordinated approach from general practice, community pharmacy, outreach services and locally tailored communications, with a particular focus on communities with lower participation. This combined approach will improve equitable access, target vaccine hesitancy and increase protection for eligible residents and frontline and ICB staff before and throughout winter.

Learning, partnership and next steps

Winter 2025/26 demonstrated that early planning, committed staff and strong partnership working across providers, neighbourhoods, Places, local authorities, ambulance services and the VCFSE sector can protect performance and productivity despite sustained pressure. The 2026/27 plan builds on that learning by prioritising earlier integrated planning, mobilisation of proven interventions, clearer governance, shared intelligence and coordinated escalation rather than creating new structures during peak demand. Immediate next steps are to finalise assurance actions, commission and mobilise winter schemes, and monitor delivery and emerging risks through established system governance which is being refreshed following the NEL ICB restructure.

North East London Winter Plan 2026/27

1. Introduction

This paper sets out the North East London (NEL) Integrated Care Board (ICB) Winter Plan 2026/27, developed in collaboration with neighbourhood and place teams, providers and system partners. The plan reflects the requirements and expectations set out in the NHS England (NHSE) Winter Planning 2026/27 Letter (Appendix 1) and addresses the wider system challenges and priorities set out in the NHSE 10 Year Health Plan for England and Neighbourhood Health Framework. Our approach aligns with operational plans, lessons learned from previous years, knowledge of our local population and national best practice.

2. Context and priorities for winter 2026/27

Last winter demonstrated the strength of partnership working across NEL. Against a backdrop of sustained demand, workforce pressures and increasing complexity, partners from across acute providers, community and mental health providers, primary care, community services, local authorities, ambulance services and the voluntary, community, faith and social enterprise (VCFSE) sector worked together to maintain services, support patient flow and improve access to care for residents.

Responsibilities for winter reflect the new operating model set out in the Model ICB and Model Region Blueprints: ICBs commission, providers deliver, and regions oversee. As strategic commissioners, ICBs must lead prevention now – vaccination uptake (particularly maximising uptake in children to help reduce spread), and proactive support for people most at risk from winter viruses and cold weather – and commission the right level of activity for winter so that people are treated closer to home wherever possible. That means sufficient commissioned capacity across primary care, community services, mental health and intermediate care, and infection prevention and control. ICBs must also coordinate the winter response day-to-day through their coordination centre, operating 7 days a week and using the OPEL framework, with tested escalation, surge and mutual aid arrangements agreed by all partners.

For 2026/27, UEC ambitions remain focused on recovery and improvement across the whole pathway: achieving sustained improvement in all type A&E 4-hour performance (to 82% in March 2027), reducing Category 2 ambulance response times (30 min average in 2026/27), reducing ambulance handover delays, making progress towards eliminating corridor care, accelerating discharge through stronger hospital-to-community pathways, and improving access to urgent mental health assessment and support. These objectives align with the broader neighbourhood health ambition of reducing avoidable emergency attendances and admissions, improving patient experience, and ensuring people receive timely care in the most appropriate setting. These priorities require the system to maintain capacity gains, improve productivity across services and expand alternatives to acute care.

The performance and quality metrics linked to these priorities and in line with 2026/27 operating plan submissions will continue to be monitored throughout the year to proactively manage performance against the agreed operational planning trajectories.

3. Development of the NEL Winter Plan

The NEL winter plan consolidates system, provider, place and neighbourhood initiatives and aims to provide a holistic view of demand, capacity and resilience - with a focus on prevention and neighbourhood health.

The NEL approach to winter planning has been developed in collaboration with system partners, places and providers - with active participation from acute providers, community and mental health providers, primary care, community services, local authorities, ambulance services and the VCFSE sector. We have mirrored the

approach used in 2025/26 to ensure consistency for all partners and ensure we are building on the successes and learning from last winter.

The following steps have been undertaken to develop a plan for the system:

- Submission of operating plan in Q1 2026/27 that forecasted for an increase in demand for the winter period and factored in population growth.
- The NEL Winter Wash-Up 2025/26 presentation pack shared performance data and learning from last winter, followed by a Winter Survey that captured successes from 2025/26 and priorities for 2026/27. Key messages from the survey were:
    - Winter 2025/26 was successfully managed despite significant operational pressures because of strong partnership working, workforce commitment and service resilience.
    - Strongest priorities for Winter 2026/27 are earlier integrated planning and funding certainty, increased primary care and Same Day Access capacity (particularly for respiratory conditions), better shared data and analytics, and improved mental health support and system flow.
    - Focus on mobilising known priorities earlier and whole system resilience, with clearer governance and better shared intelligence, rather than standing up new services or models of care.
- Place Partnerships held local Winter Planning events in August. Events were used to agree a locally owned winter plan, co-designed by all Neighbourhood partners, that reflects local priorities and risks and addresses NHSE requirements. The Place winter plans have been incorporated, along with Provider plans, into this NEL system-wide plan.
- System partners have provided assurance against national winter ambitions and actions as well as a rating (red, amber and green) related to confidence in plans to deliver key ambitions and actions (summarised in Appendix 3).
- An Equality, Quality and Inequality Impact Assessment (EQIA) has been developed to inform the NEL Winter Plan and has been endorsed by the EDI, Quality, and Health Equity and Inequalities Teams (Appendix 4). The EQIA concluded that the Winter Plan is expected to have an overall positive impact on equality by targeting support towards protected and underserved groups, with continued monitoring and accessible, non-digital alternatives required to mitigate potential barriers and prevent inequalities from widening.
- A regional scenario testing exercise in September will provide further opportunities to derisk the system will be discussed and explored. Any final adaptations to the plan will be shared with relevant Boards following this event.

National timelines and provider sign-off

National submission timelines are the same for ICBs and NHS Trusts. As a result, the Trust information included in this paper remains subject to change.

Trusts will complete their internal review and obtain approval through their respective governing bodies during September. The NEL winter plan will be updated as final, approved information becomes available, including final Board Assurance Statements in Appendix 2.

Physical Capacity Funding (Winter Funding)

The ICB is in the process of designing NEL-wide winter schemes where a system-wide approach will be beneficial and provide a more strategic and coordinated response to known winter pressures, such as ARI

Hubs, intermediate care beds and UCR capacity. This process will consider alignment with Care Closer to Home priorities and ensure that Mental Health requirements are considered. Neighbourhoods and Places are also identifying local schemes that support winter pressures, ensure alignment to NEL and NHSE winter priorities, create positive impact for our residents, and value for money. The intention is for schemes to be commissioned and in place before December.

System risks and winter governance

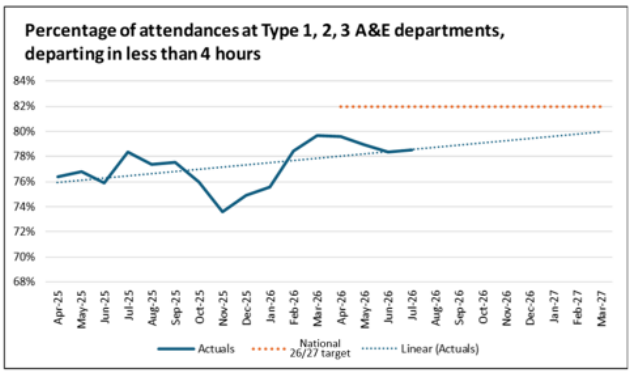
The whole-system approach to winter planning supports the mitigation of risks that arise from sustained periods of pressure on our services and workforce throughout winter. Despite steady progress and improvement, assurance matrix returns and system planning discussions highlight pressures in intermediate capacity, mental health capacity, and discharge delays affecting hospital flow. Maximising usage of community alternative care pathways, frailty and paediatric pathways and workforce pressures continue to be raised as ongoing challenges that require sustained, system-wide focus throughout winter. There is an added layer of risk this winter following the ICB restructure which has led to significantly reduced capacity, loss of institutional memory, and changes to local and regional structures that may impact the system's ability to respond to winter pressures compared to previous years.

A summary of key risks and mitigating actions is provided in Appendix 5. Responsibility for signing off the NEL Winter Plan and assurance sits with the NEL ICB Executive Management Team (EMT) and NEL ICB Board. The NEL UEC Board will oversee the delivery of the NEL Winter Plan 2026/27. Its broad membership includes chief operating and medical officers, adult and children's social care representatives, primary care and the VCFSE sector. Drawing on local Trust and Place operational delivery, the NEL UEC Board will collectively review system-wide winter risks and mitigations and provide assurance to NEL ICB EMT and NEL ICB Board.

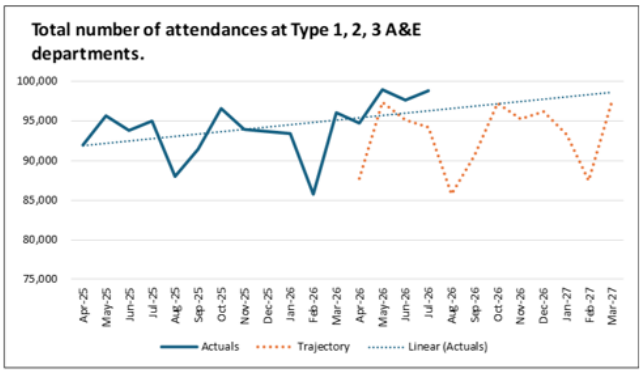
4. NEL performance against key winter metrics

Data is a key enabler to support our interventions over the winter period. Focusing on the key metrics that NEL ICB is accountable for delivering during the winter period enables us to better target interventions and priorities.

Figure 1: NEL All Types 4-hour A&E Performance and NEL All Types Total Attendances (excluding standalone UTCs)



	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	78.4%	78.4%	78.6%	82.0%	82.0%
Barts	73.4%	73.3%	73.6%	79.0%	82.1%
BHRUT	79.9%	80.2%	81.3%	82.0%	82.0%
Homerton	81.1%	80.3%	79.1%	83.9%	84.0%



	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	94,895	97,539	98,725	76,059	97,349
Barts	45,493	46,420	47,580	36,583	47,430
BHRUT	30,627	32,019	32,315	29,709	38,479
Homerton	11,437	11,549	11,507	9,767	11,440

Source: Published A&E data, NHS England

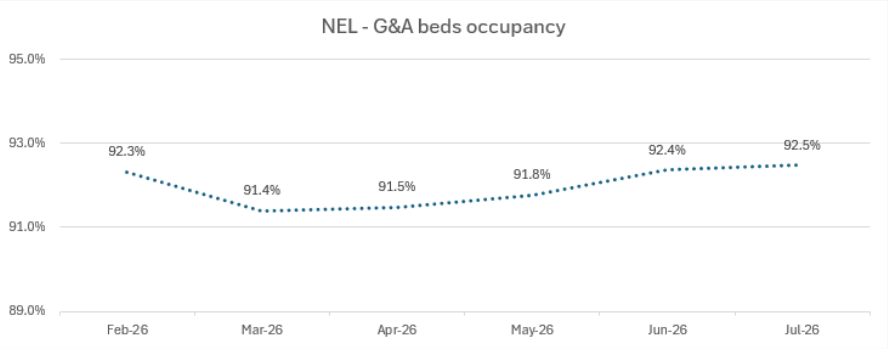
Acute Trusts set operating plan trajectories for 2026/27 in March 2026 with plans to meet or exceed the national 4-hour A&E performance of 82% by March 2027. It is noted that NHS England did not require ICBs to submit system-wide A&E performance trajectories for 2026/27 in line with the wider NHS reorganisation, so A&E performance trajectories are only provided at Acute Trust level. Figure 1 therefore includes the 82% national target which the ICB is expected to achieve by March 2027. It is also noted that UTC standalone sites

are not included in Figure 1 as operating plan trajectories were not submitted for these sites, but performance of these sites (Harold Wood and Barking) remains consistently strong, in line with the national target for Type 3 performance.

Figure 1 indicates that NEL performance has been consistently over 78% throughout 2026/27 despite higher than planned UEC attendances. In July 2026 (the latest published data available at the time of writing), NEL A&E performance was 78.6% which is considered stable within NHSE control limits. All Trusts are currently operating below their planned performance for July 2026 due to the severe and prolonged heatwave conditions experienced in the UK. It is therefore important for the system to consider the impact of this prolonged pressure on our UEC services and workforce in the lead up to the winter period.

We will need to ensure that performance levels continue to improve in line with the planned trajectory as a core feature of local winter plans through continued focus on prevention, proactive care and attendance and admission avoidance initiatives.

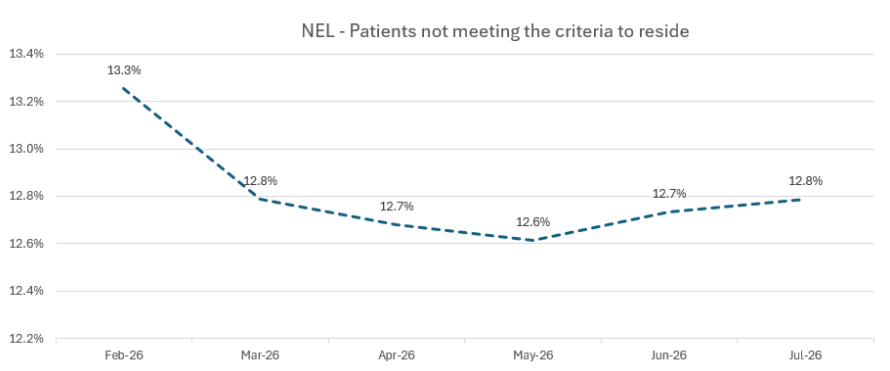
Figure 2: NEL G&A bed occupancy



Source: Published data, NHS England

Figure 2 indicates that current levels of bed occupancy are operating at between 91% - 92.5%, higher than the same period last year (89 - 91%). Barts and Homerton have regularly had lower than planned G&A bed occupancy this year, whereas BHRUT are consistently higher than plan (recording 100% occupancy in June, July and August). BHRUT are using Model Hospital data to provide length of stay (LOS) opportunities as part of their bed modelling exercise, setting optimum LOS improvement targets by speciality which will be managed in a clinically-led UEC improvement programme for LOS. Across the whole system, there continue to be a range of targeted schemes to improve hospital flow, discharge planning and intermediate care pathways and capacity to support NEL to maintain this core metric.

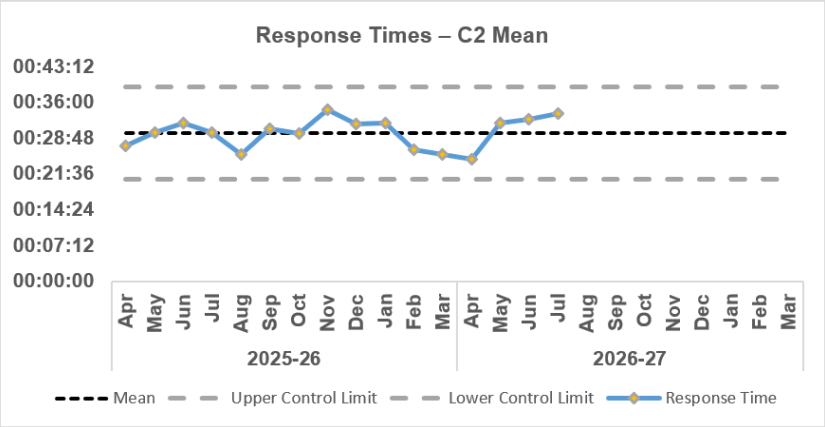
Figure 3: NEL patients not meeting the criteria to reside



Source: Published data, NHS England

Figure 3 shows a fairly stable position in the percentage of adult G&A beds that are occupied with patients who no longer meet criteria to reside, and a similar position to the same time last year (12.3% in July 2025). Daily reporting at Trusts using the Medically Fit For Discharge (MFFD) / No Criteria To Reside (NCTR) flag for action, escalation and oversight at bed meetings aims to reduce the number of patients with no criteria to reside and improve timely discharge.

Figure 4: Ambulance Category 2 response times



Source: London Ambulance Service 999 Dashboard

Figure 4 shows an improvement in Category 2 ambulance response times compared to 2025/26, however response times have increased in May-July 2026 due to the additional pressure on ambulance services during the severe hot weather. This reiterates the importance of strong system-wide planning ahead of the winter pressures. The London Ambulance Service (LAS) winter plan highlights a series of pan-London interventions to improve ambulance availability and response times over winter such as increasing the number of vehicles on the road, reducing the amount of vehicles sent to some incidents where it is safe to do so and focusing on Category 2 segmentation to respond rapidly.

5. Winter resilience and preparedness

To support a whole-system approach, the plan has been organised around the end-to-end patient journey. This reflects the ambition to intervene early through prevention and proactive neighbourhood care, provide alternatives to attendance and admission where appropriate, optimise ambulance and acute pathways for those requiring hospital-based treatment, and ensure timely discharge with coordinated follow-up support. Together, these priorities support improved patient outcomes, better experience of care and more sustainable and equitable use of health and care resources throughout the winter period.

There is a strong focus on meeting the needs of vulnerable populations, specifically babies, children, young people (BCYP) and their families, recognising the specific needs of this cohort over the winter and ensuring timely access to good information and advice, including to families and carers.

5.1 Prevention, proactive care and vulnerable populations

Neighbourhood health and population stratification

Through the neighbourhood health model, multidisciplinary teams (MDTs) bring together primary care, community services, mental health, social care and VCFSE sector partners to provide targeted and proactive support focused on prevention, early intervention and admission avoidance. These approaches are informed by local population health data, enabling resources and capacity to be directed towards communities and patient groups most likely to experience winter-related pressures. NEL has a data-led approach in place to identify and support individuals at increased risk of winter-related deterioration, emergency admissions and poor health outcomes, including care home residents, housebound patients, people living with long-term

conditions, those with multiple co-morbidities and other clinically vulnerable groups who may be more susceptible to complications arising through winter.

Examples of local schemes that are in place to provide proactive support and reduce the risk of avoidable hospital admission include:

- The Supplementary Care Home Service delivers proactive care to care home residents, including personalised care planning, regular MDT reviews, aspects of the Comprehensive Geriatric Assessments (CGAs), medication reviews, falls assessments and immunisation support. This scheme also includes a two-hour response standard during core hours for residents requiring urgent primary care intervention, enabling timely assessment and management in the community.
- The Long-Term Conditions Local Improvement Scheme (LTC LIS) supports people living with conditions associated with increased risk of winter deterioration including respiratory conditions, severe mental illness and cardiovascular disease. Practices are commissioned to adopt a Year of Care approach from 2027, ensuring proactive identification of patient need, regular reviews, personalised care planning and MDT working to support self-management, optimise treatment and identify deterioration at an early stage.
- The Dedicated Housebound Checks and Winter Vaccinations Scheme provides proactive outreach to housebound patients who are unable to attend their GP practice. The scheme aims to identify risks prior to the winter period, ensure patients are up to date with flu, COVID-19 and pneumococcal vaccinations, review wider health and wellbeing needs, and implement early interventions where required. This targeted approach supports some of the most vulnerable residents and is designed to reduce unnecessary and inappropriate use of emergency and secondary care services during periods of increased winter pressure.
- Expansion of Universal Care Plans (UCPs) across frailty cohorts, residents in proactive care schemes, end of life residents and care home populations. UCP visibility for NHS 111 and LAS will support alternatives to conveyance and improved decision making.
- Falls prevention services support the identification of residents at risk of falling and support onward referrals into relevant services that will reduce the likelihood and/or impact of falling. The Falls Proactive Care Model initiated by London Borough of Barking and Dagenham in 2025 links residents to appropriate services in the community to improve health and wellbeing and reduce future demand on health and care services.
- The Health Navigator programme uses a clinically validated AI algorithm to identify patients at rising risk of experiencing an extended stay in hospital and offer person-centred clinical coaching to help people take control of their health and stay out of hospital.
- End of life care transformation plans are in place to enable more people to remain in the place they call home and be cared for within their community at the end of life. Through ensuring effective support is in place to increase use of the Universal Care Plan and to embed the delivery of end life care into neighbourhood working and increasing the reach and capacity of established successful Hospice at Home services - inappropriate ambulance conveyances, ED attendances and emergency admissions will be reduced.
- In partnership with the British Red Cross, we plan to introduce a new approach for people with complex needs who frequently use urgent and emergency care. The implementation plan is being developed, with launch targeted for December 2026.

Collectively, these schemes provide a programme of risk stratification, preventative care, personalised support and early intervention, ensuring that vulnerable populations are identified and supported ahead of and throughout the winter period to reduce avoidable deterioration and hospital admissions.

Community-wide vaccination programme (children, RSV, flu, COVID)

Vaccination programmes across all the priority areas are designed to reduce complacency, build confidence, and maximise convenience. Priority programmes include childhood vaccinations, RSV vaccination for pregnant women and older adults and the annual winter flu and covid vaccination campaigns.

- **Childhood immunisations:** NHSE and UKHSA have delivered webinars and supporting materials to promote the importance of childhood immunisations and the changes to the schedule. Assurance is overseen by the London Immunisations Board (LIB) chaired by NHS England (London), with representation from each of London's ICBs, local authority representatives, UKHSA, academia and a range of other subject matter experts.
- **RSV:** NHS North East London ICB has established arrangements to support delivery of the RSV vaccination programme in accordance with NHS England and UKHSA guidance. Eligible cohorts, including older adults, residents of care homes for older adults, pregnant women from 28 weeks gestation, and any newly eligible groups announced for autumn/winter 2026/27, are identified and offered vaccination through commissioned providers. The system operates a proactive call and recall approach through general practice in line with Vaccination and Immunisation Standards, supplemented by NHS England national invitation campaigns where applicable. Vaccination is available through primary care providers and commissioned community pharmacy sites, with access arrangements in place for care home residents and housebound patients. Uptake and provider performance are monitored through established programme governance arrangements, and delivery plans are in place to maximise protection ahead of the winter
- **COVID-19 and influenza vaccinations:** NHS North East London ICB has arrangements in place to deliver the Autumn/Winter 2026/27 COVID-19 and influenza vaccination programmes in line with NHSE requirements. A mixed provider model will offer equitable access for eligible populations through general practice, community pharmacy and targeted outreach. Proactive call and recall, national invitations, the National Booking Service and local outreach will support uptake and reduce inequalities. Provider sign-up demonstrates strong system capacity: 223 community pharmacies and 185 general practices will deliver the COVID-19 vaccination, while 276 pharmacies and 255 practices will deliver the adult influenza vaccination. Four directly commissioned outreach providers will work with the ICB, local authority public health teams and VCFSE partners to reach communities with lower participation. Delivery, uptake and provider performance will be monitored through established seasonal vaccination governance, with escalation arrangements for emerging operational risks. Coordinated London-wide communications will use a consistent approach where appropriate, alongside locally tailored messaging to improve confidence, convenience and uptake.
- **Reducing hesitancy and improving uptake:** In addition to core planned activities, NHS England have agreed to fund the following initiatives through the NHS Service Development Fund (SDF) to reduce hesitancy and improve uptake:
    - Implementing a Local Incentive scheme (LIS) across all PCNs, requiring GPs to work collaboratively to deliver enhanced vaccination engagement initiatives to both address low uptake in CYP immunisations programmes and boost capacity and access opportunities for the winter vaccination programmes throughout November

- Commissioning a community pharmacy led vaccine hesitancy service, providing an opportunity for people who would like an enhanced conversation about their concerns relating to vaccinations the sufficient time to do so in a local and trusted setting
- Enabling the VCFSE sector to become active participants in the engagement of communities who routinely do not come forward for vaccination through a grant process, supported by Local Authority Public Health leads and overseen by the ICB engagement lead.

Improving staff vaccination rates

Trusts across NEL have plans in place to increase frontline staff vaccination, noting the nationally set ambition of 60% uptake for this cohort. Actions to improve uptake include early Flu Peer Vaccinator (FPVs) recruitment with an aim to ensure vaccinators on each ward and department, pop-up clinics on all sites and clear visible Executive leadership support. The primary care contracting team are encouraging all primary care services to promote vaccinations with staff.

NEL ICB senior leaders are also encouraging ICB staff to contribute to population-level protection by encouraging them to book their flu vaccinations, helping friends and family to make informed decisions to protect themselves and others, promoting ICB vaccination campaigns and being part of the commissioning force to enable flu vaccines through working with local partners such as primary care, schools, care homes and VCFSE sector colleagues.

Public communications and engagement campaign

Delivery of a coordinated winter communications and engagement programme focused on prevention, neighbourhood health and supporting residents to stay well throughout the winter period. Trusted, co-designed campaigns will provide winter wellbeing advice, promote seasonal vaccination uptake and signpost residents to the right support within their local communities to reduce avoidable demand on UEC services. Campaigns will target priority populations including older people, those with long-term conditions, pregnant women, children and families, and will be delivered through a range of digital, community, primary care and partner channels.

5.2 Alternatives to conveyance, attendance and admission

NEL continues to focus on reducing avoidable admissions and attendances through targeted commissioning that shifts care from hospital to community. This aligns with national ambitions to improve access to same-day and out-of-hospital care and support people closer to home through the neighbourhood health model.

Primary Care, Same Day Access, NHS 111 and Attendance Avoidance

NEL has assessed anticipated winter demand and will ensure there is sufficient commissioned primary care capacity to support residents to access timely urgent care within their local communities and reduce avoidable demand on urgent and emergency care services. The Same Day Access (SDA) service provides approximately 230,000 appointments annually, with capacity aligned to seasonal demand, including evenings, weekends, bank holidays and other surge periods. The service provides seven-day urgent primary care access and supports direct booking and clinically appropriate redirection from NHS 111, UTCs and EDs, with ring-fenced access also available to ambulance services through locally agreed pathways. Robust escalation arrangements are in place through established primary care, integrated urgent care (IUC) and system operational processes, with additional capacity available to respond to winter pressures. During winter, we plan to increase appointment capacity to meet the anticipated rise in respiratory presentations and support primary care. LAS are aiming to increase appropriate first selection on the Directory of Services (DoS) for key pathways such as Community Pharmacies and GP Practices to help identify and address referral barriers into these services.

The ICB continues to monitor contractual compliance across general practice, including opening hours, online consultation availability, clinically urgent same-day appointments and NHS 111 direct booking requirements. All practices have declared compliance with requirements relating to online consultations and provision of appointments for direct booking by NHS 111, including delivery of the national requirement for appointment availability. All PCNs participate in the Enhanced Access service and make unused same-day appointment capacity available to NHS 111 in line with DES requirements. In addition, the ICB monitors unscheduled dental care delivery through established contract management arrangements, supported by commissioned Urgent Dental Care services and 24/7 NHS 111 dental triage, ensuring residents can access appropriate urgent dental support throughout the winter period.

Examples of local schemes that are in place to provide alternatives to conveyance, attendance and admission include:

- LAS are testing a revised navigation approach that directs eligible residents in participating care homes in Havering to timely GP-led assessment and treatment, while retaining established UEC pathways for higher-acuity needs. By improving continuity of care and supporting earlier, clinically appropriate intervention, the pilot aims to reduce avoidable NHS 111 and 999 use, ambulance conveyances and hospital attendances, helping relieve pressure on UEC services over winter.
- NELFT are launching a Crisis Car service from October 2026 for six months to manage patients in crisis within the community, focusing on falls, social issues, equipment needs, and lower acuity nursing care such as wound management. In addition to this, the Community Treatment Team (CTT) are commencing intravenous (IV) therapy delivery within the community for patients referred to CTT, expanding CTT Catheter Virtual Ward capacity over winter, and improving referral pathways from GPs, Acute services, and LAS to support admission avoidance.
- Homerton's Intermediate Integrated Team (IIT), ParaDoc, and Frailty Virtual Ward are actively engaged in neighbourhood-level MDTs to support complex patients and reduce ED attendances and their Adult Cardiorespiratory Enhanced and Responsive Service (ACERS) team continues to lead Virtual Ward delivery across respiratory, frailty, and heart failure pathways, with plans to expand capacity through internal workforce realignment.

Community Pharmacy

Community pharmacy provides accessible care, treatment and preventative services close to home whilst reducing pressure on general practice and UEC services. The ICB has assessed community pharmacy demand, activity and capacity and is assured that sufficient provision, including out-of-hours and bank holiday cover, is available across the footprint.

Pharmacy First is delivered by 356 of 366 community pharmacies across North East London (97%), providing advice, assessment and treatment for a wide range of minor illnesses and clinical pathways for seven common conditions, with referral routes from general practice and NHS 111. Community pharmacies also support prevention and early intervention through vaccination services, blood pressure checks and medicines optimisation.

All NEL NHS community pharmacies provide the Discharge Medicines Service (DMS), supporting safe transfers of care following hospital discharge and helping to reduce medicines-related harm and avoidable readmissions. Additional services, including the Community Pharmacy Self-care Advice Service (CPSAS) which helps eligible patients who cannot afford to buy required over the counter medicines recommended by the community pharmacist by providing advice and free of charge medicines, vaccination services, blood pressure checks and urgent access to palliative and end of life medicines - further strengthen local support for residents and help people remain safely cared for within their communities during periods of increased winter pressure.

Care Closer to Home Programme

The NEL-wide Care Closer to Home (CCTH) programme aims to support the shift from hospital to community, reducing avoidable UEC demand and improve outcomes for patients. CCTH is expected to strengthen pathways that support the wider system response to winter pressures, with an initial focus on four priority interventions: Proactive Neighbourhood Care, Same Day Emergency Provision (SDEP), Urgent Community Response (UCR) and Discharge. The CCTH programme will work with Providers and Places to ensure delivery is aligned with wider operational plans and that relevant dependencies and opportunities are reflected in winter planning.

The CCTH diagnostic identified significant longer-term opportunity to reduce avoidable acute utilisation and improve patient flow across NEL. The programme is currently refreshing the diagnostic using 2025/26 activity and validating baselines, benefits and attribution with Providers. Any impact realised during winter 2026/27 will therefore form part of the wider CCTH delivery trajectory rather than representing a separately modelled winter benefit. More specific activity and impact trajectories will be incorporated as individual intervention delivery plans and benefits baselines are agreed.

Single Point of Access (SPoA)/Integrated Care Coordination (ICC) Hub

North East London is progressing Phase 1 of the SPoA/ICC Hub, initially focused on 999 pathways and developing Integrated Urgent Care Clinical Assessment Service (IUC CAS) integration. The ambition for this winter is to strengthen the SPoA/ICC as a clinically-led coordination function, supporting patients to access the right care first time and making better use of alternatives to ED. A key priority is stabilisation of the clinical workforce by moving towards a more sustainable, job-planned consultant model by November, with clinical input from Providers to improve resilience, consistent decision-making and stronger clinical relationships across NEL. REACH and the ICC Hub will operate in parallel while workforce, clinical, digital and governance arrangements are aligned towards a single NEL-wide model.

Priority developments ahead of winter are articulated in the table below, including strengthening visibility and access to UCR pathways, clinically safe case holding, prescribing capability and development of a care home worklist to support earlier clinical review and navigation into frailty, UCR and other community alternatives, informed by learning from other London ICC models. Work continues to strengthen integration with frailty pathways, improve connectivity and interface with NEL Mental Health SPoA and increase engagement with LAS crews. For patients, particularly people with frailty, care home residents and those in mental health crisis, it will provide earlier senior clinical review, fewer referrals and transfers of care, and faster access to the most appropriate support, including treatment at home where clinically safe.

Priority area	What we are reviewing	By when
UCR	Simplify pathways for SPoA/ICC to access UCR, including ‘pull’ and ‘push’ models, and support more non-ED, non-admission community management.	Nov-26
Case Holding	Enable ICC to safely retain oversight of appropriate patients while an alternative pathway is arranged.	Nov-26
Prescribing	Enable appropriate ICC clinicians to prescribe where this supports complete clinical management and care at home.	Nov-26
Care Home Worklist	Explore earlier clinical review of care-home 999 calls and navigation to alternatives.	Nov-26
Frailty & Pathway Integration	Strengthen access to Frailty pathways – Frailty SDEC, Virtual Frailty wards and other admission alternatives through SPoA/ICC	Ongoing
Mental Health	Improve the interface between the NEL Mental Health SPoA and UEC SPoA/ICC.	Nov-26
LAS Engagement	Increase crew awareness of ICC and available alternatives and understand barriers to use.	Ongoing

Priority area	What we are reviewing	By when
Winter Learning & business case development	Use winter delivery and data to inform the recurrent Phase 1 model and business case from April 2027.	Ongoing
Primary care/ SDA/OOHGP	Ensure improvements in SDA availability to CHUB and ICC Hub are equivalent to 111 service and consider availability of OOHGP service for evening referrals.	Nov-26

Additional winter capacity

To further support the system during the winter period, building on existing capacity the intention is to:

- Increase capacity across Same Day Primary Care Hubs, with a focus on respiratory hubs in every NEL borough.
- Review and increase the capacity in Urgent Community Response (UCR) Teams across NEL including testing new models of care to support care homes and increase the acuity in Waltham Forest Frailty Virtual Ward by ensuring IV therapy can be initiated and delivered within the virtual ward.
- Expand the St George's model across Barking and Dagenham, Redbridge and Havering to improve access to comprehensive geriatric assessments for residents at high risk of frailty, aligned with community multidisciplinary teams.

5.3 Ambulance and In-Hospital Pathways and Processes

NEL's ambulance and acute care pathways are aligned to support delivery of constitutional standards and national UEC ambitions, including ambulance response times, ambulance handovers, 4-hour performance, and 12-hour waits in ED, reduction in 24-hour waits for mental health admissions and progress towards elimination of corridor care.

Whole-system demand and capacity modelling has informed winter planning, with providers undertaking pre-winter bed, workforce and occupancy modelling and adjustments to the bed base to improve efficiency, supported by tested surge and extreme surge arrangements for periods of increased demand, including seasonal respiratory illness and severe weather events. The LAS Winter Plan articulates the development of year-round escalation arrangements (including learning from the recent summer escalation), an enhanced Clinical Safety Plan, improved forecasting and planning capabilities and stronger partnerships with System Coordination Centres (SCCs) and ICC Hubs.

Acute providers are expanding Same Day Emergency Care (SDEC) capacity, strengthening front-door triage and streaming, increasing senior decision-maker presence and embedding direct-to-SDEC and direct-to-UTC pathways to reduce avoidable admissions and improve flow. Daily executive oversight of demand, capacity, discharges, patients with no criteria to reside and length of stay supports delivery of 4-hour and 12-hour trajectories, alongside review of the Model Emergency Department and Model Acute Pathway data and implications on service delivery.

Providers have agreed measures to reduce and ultimately eliminate corridor care, including strengthened patient flow processes, expanded assessment capacity, defined safety thresholds and early escalation triggers.

Paediatric escalation protocols, consultant rota changes, and reconfiguration of children's urgent care areas are enabling sustained improvements. Paediatric rapid assessment and treatment models are being expanded, supported by consultant paediatrician-led streaming. System partners are expanding access to paediatric urgent care, including NHS 111 online and telephony, UTC streaming, paediatric same day access in primary care, and timely consultant advice for community clinicians. Providers are implementing robust escalation plans for paediatric surges, including bronchiolitis and winter respiratory illness, supported by rapid triage and

paediatric assessment capacity. Homerton's Children's Emergency Assessment (CEA) unit is supported by targeted training for the Children's Community Nursing Team to manage respiratory illness in the community and facilitate early discharge. To further strengthen paediatric emergency performance, additional ED consultant cover for CEA will be in place throughout winter to improve performance against the internal target. Initiatives to support young children and their families and carer during hospital stays are in place such as the Family Liaison Nurse at Whipps Cross who provides holistic support to families with under-5s frequently attending ED to provide tailored support, community referrals and reduce avoidable visits.

LAS continue to expand "hear and treat" and "see and treat" services and promote use of ICC and alternative care pathways (including MiDoS integration) to reduce unnecessary conveyance and improve Category 2 response times. LAS are focusing on Category 2 segmentation to respond rapidly, maximising use of *5 via iPad for senior clinical decision supporting non-conveyance and enhancing Category 3 & 4 validation to reduce ambulance referral. LAS and acute sites are working in partnership to strengthen ambulance handover processes to support achievement of the 45-minute handover standard and progress towards the longer-term 15-minute ambition. LAS have noted opportunities in relation to ensuring adequate Fit to Sit (with consistent criteria), wheelchair availability and dedicated LAS handover nurse or senior clinical decision maker to facilitate quicker handovers.

Mental Health

System partners report continue to increase the utilisation of mental health crisis services including Integrated Crisis Assessment Hubs (ICAHs), crisis cafés, 111*2 and community crisis alternatives to reduce ED attendances and improve access to timely mental health support. Actions include scaling up seven-day working in community mental health teams, extended operating hours, communications regarding ICAH 24 hour offer and continued monitoring of uptake, opening additional crisis beds, and closer coordination with community mental health teams to prevent unnecessary emergency attendances. Mental health support for children and young people is being strengthened through crisis provision and closer alignment with community and school-based services. There is improved alignment between mental health services and police, including an agreed pathway with NELFT, LAS and Police to utilise the ICAH directly to reduce s136, street triage to support joint assessment at point of police contact, and management of flow in and out of Health Based Place of Safety (HBPoS) to maximise use by police. Homerton continues to work with ELFT to strengthen direct-to-ICAH pathways so that patients in mental health crisis are assessed and supported outside the ED environment where clinically appropriate. Within ED, the Homerton Psychological Medicine (HPM) team provides rapid assessment and onward referral, with escalation processes in place for patients awaiting a mental health bed.

Daily management of mental health bed flow and system-wide escalation processes support reductions in extended waits for mental health admission. NELFT service in place at Queen's and King George to see low risk mental health patients in the UTC and strengthened leadership to support pathway enhancements from acute to Meadow Court. However, mental health bed capacity remains a constraint requiring joint planning with ELFT and NELFT. Where constraints exist, partners are coordinating capacity and pathways across sites. Oversight through system mental health governance forums provides assurance that these initiatives are reducing ED demand and improving timely access to appropriate care. There is ongoing work between ELFT and NELFT to make the best use of the NEL mental health bed base and to ensure all ideas to enhance flow are taken forward with the Mental Health, Learning Disabilities and Autism (MHLDA) Collaborative Improvement Network.

A rapid assessment of mental health investments across NEL has been commissioned, aligned to the Care Closer To Home Programme, which aims to provide clear recommendations on our strategic commissioning plans for mental health, including crisis interventions and capacity across age ranges and pathways. The review

will consider the pace and scale of delivery given the current pressures and will include a one-off piece of bed analysis that bridges current demand and capacity.

Workforce wellbeing, retention, and development

Aligned with national workforce priorities, NEL is committed to ensuring the wellbeing, retention, and development of its urgent and emergency care workforce. Following changes to the ICB blueprint this year, responsibilities for workforce primarily sit with Providers and NHSE, including workforce performance on WTE bank and agency now being directly managed against the operational plan by NHSE. The Provider CPOs are well connected and support each other to review and address any staffing shortages through their CPO Network and the ICB has escalation points where Providers require it.

To support staff welfare through periods of high demand, Trusts have individualised wellbeing offers in place for their staff commissioned at a local level. Trusts report use of early sickness triggers and fast-track occupational health referrals to actively monitor and mitigate against staffing gaps, post-incident support and psychological support in place for ED and critical care teams during winter and protected staff breaks. Year-round staff welfare initiatives remain accessible to all staff, such as Employee Assistance Programmes and staff recognition awards, to ensure staff are cared for, recognised, and supported throughout winter and beyond.

Infection prevention and control (IPC)

All providers have winter IPC plans in place, which focus on early identification and diagnosis of contagious winter pathogens, with clear cohorting policies and step-down processes, and live surveillance of outbreak triggers. IPC leads are embedded in operational planning and escalation frameworks. Patient and staff safety is ensured by face fit-testing, proactive PPE stock management, enhanced cleaning rotas, staff training, and regular IPC audits. Assurance is provided through established provider governance, system IPC oversight arrangements, partnership working with UKHSA and local authority colleagues, and defined outbreak management and escalation processes that support the timely management of infectious disease incidents and outbreaks.

Arrangements are in place to support and commission safe infection prevention and control (IPC) practice across care homes and the wider care sector in North East London. The ICB IPC Team provides system-wide oversight, assurance, specialist advice, outbreak support and quality improvement programmes for adult social care providers.

5.4 Timely discharge and follow-up support

Reducing length of stay, eliminating unnecessary discharge delays and supporting safe transfer of care remain key priorities for winter 2026/27, in line with the ambitions outlined in the Medium-Term Planning Framework. Discharge planning is delivered as a whole-system responsibility. Use of the NHS SAFER patient flow bundle, availability of senior clinical decision-makers, and seven-day discharge capacity are called out as priorities by providers and Places, with Multi-Agency Discharge Events (MADEs) and RESET Weeks planned throughout winter. Providers highlight a renewed focus on tracking Discharge Ready Dates (DRD) for all patients and early identification of any medication, equipment, transport and social requirements to reduce delays at discharge. Barts Health plans to launch a Home Before Lunch programme to increase pre-11am discharges and improve flow.

There are Integrated Discharge Hubs / Transfer of Care Hubs in every borough bringing together health, social care and VCFSE sector partners to coordinate discharge activity, manage demand and capacity, and escalate delays. Partners are working to maximise efficiency of the Hubs and discharge pathways ahead of winter including standardising processes to manage flow and safe discharge of patients, including homeless, collaborative working between Acute and Community providers and implementing additional senior nursing

and care coordination provision for winter. B&B funding is being used to provide safe accommodation for homeless patients to expedite hospital discharge.

Daily review of patients with no criteria to reside for action, escalation and oversight at bed meetings, long-stay case management, discharge-to-assess pathways and real-time oversight of community capacity support timely discharge and rapid resolution of barriers. Home and settle services and care navigators reduce waiting times for patients to return to the place they call home and navigate services across health and social care post-discharge.

Local authority partners are embedded within discharge processes (e.g. Social Workers from Havering and Redbridge Local Authority are on site at BHRUT to help reduce delays), with strengthened arrangements for step-down capacity, intermediate care, home-first pathways and care home coordination. Daily monitoring of equipment following guidance/SOP on equipment prioritisation to ensure a best practice approach to managing patient discharge for those in need of on-going community health and social care services.

Additional support is in place for mental health discharges and pathways for people experiencing homelessness including additional step-down or crisis beds, Mental Health Front Door Discharge Coordinators at Newham starting in October, integrated mental health social work input such as embedded housing officer and close working between NELFT and ELFT to best use the NEL mental health bed base and support flow.

Providers and partners have reviewed current arrangements against Model Discharge Pathway requirements and agreed improvement actions to reduce discharge delays over 48 hours and improve outcomes across acute, community and social care services. Pre-Christmas and holiday-period occupancy reduction plans have been agreed across system partners and will be implemented to maximise flow, maintain resilience and ensure sufficient capacity throughout the winter period.

The System Coordination Centre maintains oversight of discharge performance through daily operational huddles, escalation frameworks and system risk management processes, ensuring delays are identified and addressed at the earliest opportunity. Particular focus is given to people living with frailty, those with complex social needs and people experiencing homelessness. Initiatives such as the Praxis Service (operating at RLH and Homerton) supports vulnerable patients facing immigration or housing crises by providing early legal, welfare, and housing advice to hospital discharge teams, ensuring individuals have safe accommodation and ongoing healthcare access before leaving the hospital to prevent prolonged discharge delays.

Additional winter capacity

To further support the system during the winter period, building on existing capacity the intention is to:

- Increase current intermediate care bedded capacity to support acute flow through utilising current unused beds through the winter period.
- Launch a digital discharge tool at Homerton to improve real-time visibility of discharge readiness and delays ahead of winter.

6. Integrated Strategic System Coordination and EPRR

NEL's Integrated Strategic System Coordination Centre (SCC) and Emergency Preparedness, Resilience and Response (EPRR) team provides strategic system leadership, oversight, risk assessment and incident management, including system and wider coordination related to system and patient safety, flow and resilience. The team operates 365 days a year, with out of hours oversight and leadership from a strategic and tactical on-call cohort which along with the EPRR team also deliver upon the ICB's legal and statutory duties as a Category One responder.

This is replicated across system and regional on-call arrangements with the Trusts and ICB also delivering upon their Category One responder legal and statutory duty arrangements. Effective arrangements are in place in line with the NHSE EPRR Core domains requirements and wider operational frameworks (such as OPEL, MOPEL, clinical escalation pathways, policies, escalation and mutual aid frameworks) that provide consistent approaches to system risk reduction and shared situational awareness.

System oversight is supported through digital innovation via SHREWD and other platforms and is triangulated for real-time, historic and forecasting insights, and inform safe, ethical decision-making with health inequality, inequity and quality as important factors.

A review of winter governance, escalation frameworks, on-call arrangements and business continuity plans is a continuous process, particularly as interdependencies across London and wider due to health restructure, seasonal effects and geopolitical risks weigh into system life.

There is opportunity for neighbourhood and other areas of the ICB and ICS to undertake joint business continuity and resilience exercises across acute, community and primary care services and further inform any update to scenarios such as respiratory infections, severe or adverse weather, workforce disruption and infrastructure failure.

The integrated SCC/EPRR team maintain proactive engagement with NHS England London Region and wider through well-established touchpoints and in emergencies as required through ICB/ICS governance and system convening arrangements.

7. Safeguarding

Safeguarding remains a system-wide priority across NEL, with partners taking a more proactive and risk-focused approach to protecting children, young people and adults who may face heightened vulnerability during winter. This includes (but not limited to) looked after children and care leavers, sex workers, people with drug or alcohol dependence, victims of exploitation, and ex-offenders—groups whose circumstances can increase exposure to harm, reduce access to support, or create barriers to safe discharge.

In recognition of rising complexity and demand, providers have strengthened daily safeguarding oversight, enhanced escalation routes, and increased multi-agency coordination to ensure concerns are identified and acted upon earlier. Safeguarding considerations are now explicitly built into winter escalation triggers, discharge planning, virtual ward pathways and admission-avoidance decisions, ensuring that operational pressures do not compromise safety, dignity or wellbeing.

Multi-agency safeguarding hubs, local authorities, police and VCFSE sector partners work alongside NHS services to support safe discharge, reduce exploitation risk, and ensure timely information sharing where vulnerabilities are identified. System-level assurance is maintained through provider safeguarding committees and the ICB Quality, Safety and Improvement Committee, with enhanced visibility of themes, risks and incidents to enable rapid intervention and targeted support where required.

8. Next steps

The NEL Winter Plan provides assurance that partners have developed a coordinated, whole-system response focused on prevention, neighbourhood health, timely access to appropriate alternatives to hospital, improved ambulance and acute flow, and safe discharge.

Immediate priorities

Following the development of Provider, Place and NEL-wide winter plans and the associated Board level assurance processes, the immediate priorities are:

- continued engagement with system partners on winter plans
- establish winter governance and reporting that complements existing system coordination and oversight arrangements
- finalise design of funded winter schemes and commission additional capacity and local schemes
- undertake local and regional scenario testing, as required
- continued roll out of COVID-19 and influenza vaccination programmes, including frontline staff vaccination
- launch winter campaigns ahead of peak demand

Delivery will be monitored through established system governance and the SCC using established quality, activity and performance measures, with emerging risks—including workforce capacity, mental health provision, hospital flow and community alternatives—escalated promptly and mitigations adjusted collaboratively throughout winter.

Appendix 1: NHS England Winter Planning 2026/27 – Expectations and Assurance (17 July 2026)

To:

- All NHS trust: chairs; chief executives; chief operating officers; medical directors; chief nurses
- Integrated care board: chairs; chief executives; chief operating officers; medical directors; chief nurses
- Primary care networks
- Local government chief executives
- Directors of adult social services
- Directors of public health

cc:

- Regional: directors; chief operating officers; medical directors; chief nurses; directors of commissioning; chairs

Dear colleagues

Winter planning 2026/27 – expectations and assurance

Last winter was the best in 3 years for 4-hour performance and ambulance response times, despite an early flu season, a January norovirus peak and industrial action. That was the result of early planning and strong collaboration across health and care, for which we thank you. This letter sets out the national approach to winter 2026/27 and the actions we need every part of the NHS and local government to take, starting now.

Every local area must have a robust, clinically led and deliverable winter plan. The core expectations are:

- draft winter plans completed by the end of August, developed jointly by all local partners: acute, ambulance, community, mental health, primary care, local authorities, social care and the third sector
- a named executive winter lead in every ICB and provider, accountable for preparedness and response
- plans tested at the NHS England regional stress test exercises in September
- plans signed off by boards using the board assurance statements (BAS), submitted to england.eecpmo@nhs.net by 30 September 2026

Responsibilities for winter reflect the new operating model set out in the Model ICB and Model Region Blueprints: ICBs commission, providers deliver, and regions oversee. The BAS templates set out in full what boards will be asked to assure; use them now to shape your plans rather than as a retrospective checklist. In summary:

Integrated care boards

ICBs are responsible for the winter plan for their area, agreed with all partners and signed off by their board. Plans must be built on a whole-area demand and capacity analysis with robust planning assumptions, tested against surge and extreme surge scenarios. As strategic commissioners, ICBs must lead prevention now – vaccination uptake (particularly maximising uptake in children to help reduce spread), and proactive support for people most at risk from winter viruses and cold weather – and commission the right level of activity for winter so that people are treated closer to home wherever possible. That means sufficient commissioned capacity across:

- primary care: same day urgent appointments, enhanced access, NHS 111 direct booking, community pharmacy and out-of-hours provision including during holidays and over Christmas, alongside delivery of the seasonal vaccination campaigns
- community services: urgent community response, virtual wards and Hospital at Home, with senior clinical decision-making accessible through a single point of access at least 12 hours a day, 7 days a week
- mental health: resilient all-age crisis services, including urgent mental health helplines via NHS 111, crisis alternatives and liaison psychiatry, with crisis plans in place ahead of winter for high-risk patients and frequent attenders

- social care and intermediate care: commissioned jointly with local authorities, with discharge planned together, sufficient capacity through winter and agreed plans to reduce hospital occupancy ahead of Christmas
- infection prevention and control (IPC): commissioning safe IPC practice in care homes and the wider care sector

ICBs must also co-ordinate the winter response day-to-day through their co-ordination centre, operating 7 days a week and using the OPEL framework, with tested escalation, surge and mutual aid arrangements agreed by all partners.

NHS trusts and foundation trusts

Providers are accountable for operational delivery through winter. Trusts must:

- complete an organisational winter plan by the end of August, with staff flu vaccination programmes ready to launch and preparatory actions under way now
- implement the Model emergency department (ED), Model acute pathway and Model discharge pathway requirements ahead of winter, with the elimination of corridor care treated as a patient safety priority, while maintaining elective capacity throughout the winter period
- assure tested arrangements for escalation, infection prevention and control, on-call leadership and staff welfare, and protect delivery of agreed 2026/27 trajectories

Ambulance services

- plan to deliver the 2026/27 category 2 response time trajectory, maximising hear and treat, see and treat and referral into alternatives to ED, to reduce conveyance rates
- work with acute partners to deliver 15-minute handovers, with none exceeding the 45-minute maximum

Regional teams

In line with the Model Region Blueprint, regional teams will oversee winter preparedness and performance across providers and ICBs, arrange the September stress test exercises, assure plans across their geographies and co-ordinate mutual aid across ICB boundaries where needed, including specialised services and pathways commissioned outside of ICB arrangements.

Support available

We will share performance data to support planning by the end of July via NHS Futures, run online sessions in July on improving vaccination uptake, and publish further guidance on urgent community response and community beds shortly. We will continue to work with ICBs and providers to support communication on 111, Pharmacy First, vaccination, and alternatives to ED.

Thank you for everything you and your teams are doing to prepare. Early, collaborative planning delivered a better winter for patients last year; together we can do so again.

Yours sincerely
Sarah-Jane Marsh CBE, NHS England Chief Operating Officer

NHS England » Winter planning 2026/27 – expectations and assurance
Publication reference: PRN02603

Appendix 2: Winter Board Assurance Statements from NEL ICB and Providers

NHS England requested that Winter Plans are signed off by Trust and ICB Boards using the Board Assurance Statements. It is noted that these signed Board Assurance Statements will not be available before this paper is submitted to the NEL ICB Board on 17 September 2026 as they will be going through Trust-level governance in late September. Once received, signed Board Assurance Statements will be retrospectively embedded into this appendix for reference.

Board Assurance Statements	Embedded File
Barts Health	
Homerton University Hospital	
BHRUT	
NELFT	
ELFT	
London Ambulance Service	
NEL ICB	 NHSE%20Winter%20 26%2027%20Board%

Appendix 3: Winter Assurance Matrix Summary

Winter Assurance Matrix submissions have been included from Providers and Places. The RAG rating is a self-assessment from Providers and Places relating to confidence in plans to deliver key ambitions and actions. The narrative provided to support the RAG ratings has been used to inform the development of the NEL Winter Plan 2026/27 and is available on request.

It is noted that Providers will be signing off their final Winter Plans through their Trust-level Boards in September, so RAG ratings remain subject to change. The NEL winter plan will be updated as final, approved information becomes available.

Provider returns received to date (as at 23 September)

WINTER AMBITIONS AND KEY ACTIONS	HEADLINE AMBITIONS			KEY ACTIONS FOR PROVIDERS						
	82% of patients admitted, transferred or discharged within 4 hours in A&E (in March 2027)	Fewer than 10% of patients waiting over 12 hours in A&E (across 26/27)	Average 30 min Cat 2 ambulance response times (across 26/27)	Organisational leadership including surge and on-call arrangements	Frontline staff flu vaccination	Capacity modelling (beds, staff, etc)	Discharge processes and implementation of Model Discharge Pathway	Infection prevention and control, PPE provision	Staff welfare and support	NEW: Summary of approach to implementation of Model ED and Model Acute Pathway
PROVIDERS										
Barts Health	AMBER	GREEN	GREEN	GREEN	GREEN	GREEN	GREEN	GREEN	RAG not provided	RAG not provided
Homerton	GREEN	GREEN	GREEN	GREEN	GREEN	GREEN	GREEN	GREEN	GREEN	GREEN
BHRUT	GREEN	AMBER	AMBER	GREEN	GREEN	GREEN	GREEN	GREEN	GREEN	GREEN
NELFT	AMBER	AMBER	AMBER	GREEN	GREEN	AMBER	GREEN	GREEN	GREEN	N/A
ELFT	GREEN	GREEN	GREEN	GREEN	GREEN	GREEN	GREEN	GREEN	GREEN	N/A
PELC	GREEN	N/A	N/A	GREEN	GREEN	GREEN	N/A	GREEN	GREEN	N/A
GP Care Group	AMBER	N/A	N/A	GREEN	GREEN	GREEN	N/A	GREEN	GREEN	N/A

WINTER AMBITIONS AND KEY ACTIONS	SUPPORTING AMBITIONS											
	Maximum 45-minute ambulance handovers with progress towards delivering 15-minute handovers	Fewer 24 hour waits for MH admission	Fewer discharge delays (starting with 21-day length of stay)	More children seen within 4 hours in A&E	Improve staff vaccination rates	Increase numbers receiving urgent care in primary care (SDA, enhanced access, 111 Direct booking, community pharmacy, urgent dental), community and MH settings (including UCR and VW utilisation) including during holidays and over Christmas	Increase numbers receiving virtual wards and Hospital at Home, with senior clinical decision-making accessible through a single point of access at least 12 hours a day, 7 days a week	Make progress on eliminating corridor care	Increase numbers receiving urgent mental health care: maximise ICAH, resilient all-age crisis services, including urgent mental health helplines via NHS 111, crisis alternatives and liaison psychiatry, with crisis plans in place ahead of winter for high-risk patients and frequent attenders	Eliminate discharge delays longer than 48 hours	Reduce length of stay	Maximise SDEC and UTC to support delivery of 4 hour standard
PROVIDERS												
Barts Health	AMBER	RAG not provided	GREEN	GREEN	GREEN	AMBER	RAG not provided	AMBER	RAG not provided	GREEN	AMBER	GREEN
Homerton	GREEN	RAG not provided	GREEN	GREEN	GREEN	GREEN	AMBER	GREEN	AMBER	GREEN	AMBER	GREEN
BHRUT	AMBER	AMBER	AMBER	GREEN	GREEN	AMBER	GREEN	AMBER	AMBER	AMBER	AMBER	GREEN
NELFT	AMBER	AMBER	AMBER	GREEN	GREEN	GREEN	RAG not provided	AMBER	GREEN	GREEN	GREEN	GREEN
ELFT	GREEN	GREEN	GREEN	GREEN	GREEN	GREEN	RAG not provided	AMBER	RAG not provided	GREEN	GREEN	RAG not provided
PELC	N/A	N/A	N/A	GREEN	GREEN	AMBER	N/A	N/A	GREEN	N/A	N/A	GREEN
GP Care Group	N/A	N/A	N/A	GREEN	GREEN	GREEN	GREEN	N/A	GREEN	N/A	N/A	GREEN

Place returns received to date (as at 23 September)

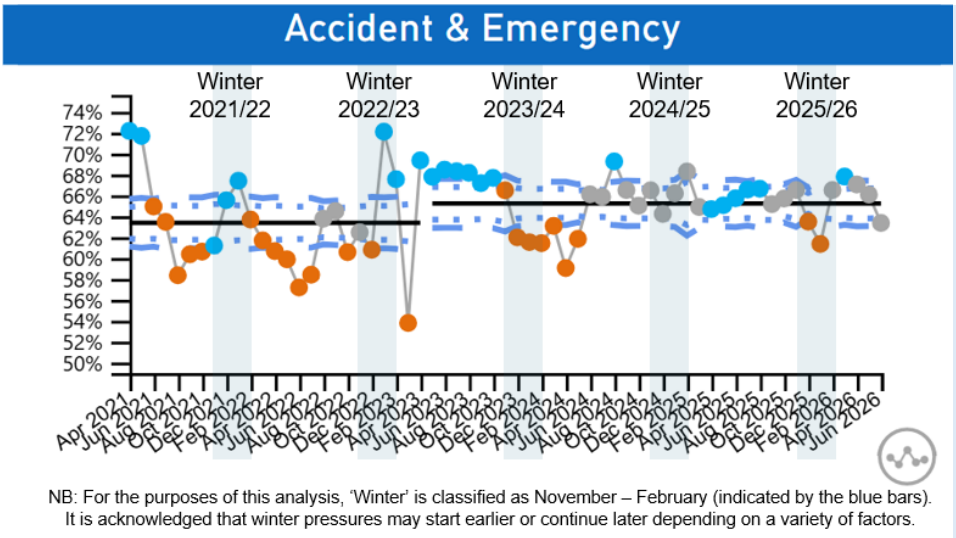
WINTER AMBITIONS AND KEY ACTIONS	KEY ACTIONS FOR ICB/ PLACE/ PROGRAMMES					
	System coordination and leadership	Vaccine programmes (children, RSV for pregnant women and older adults, annual winter flu and covid)	Population stratification, prevention, proactive care and vulnerable cohorts	Alternatives to conveyance, attendance and admission	Timely discharge and follow-up support	Capacity plans across providers
PLACE INCLUDING LOCAL AUTHORITY						
Newham	*GREEN*	*GREEN*	*GREEN*	*GREEN*	*GREEN*	*GREEN*
Waltham Forest	*AMBER*	*GREEN*	*AMBER*	*AMBER*	*AMBER*	*AMBER*
Tower Hamlets	*AMBER*	*GREEN*	*AMBER*	*GREEN*	*GREEN*	*GREEN*
City & Hackney	*AMBER*	*GREEN*	*GREEN*	*GREEN*	*AMBER*	*AMBER*
Redbridge	*GREEN*	*GREEN*	*GREEN*	*RAG not provided*	*GREEN*	*GREEN*
Barking & Dagenham	*GREEN*	*GREEN*	*GREEN*	*GREEN*	*GREEN*	*GREEN*
Havering	*GREEN*	*GREEN*	*GREEN*	*GREEN*	*GREEN*	*GREEN*

Appendix 4: NEL ICB Winter 2026/27 Equality, Quality and Inequality Impact Assessment (EQIA)

The EQIA has been reviewed and endorsed by the relevant ICB teams. The name and date of each endorsement is included within the Impact Assessments for assurance purposes.

Quality Impact Assessment ***Endorsed by Polly Pascoe, Head of Quality & Safety and Patient Safety Specialist (4 September 2026)***	
Patient Safety	Increasing focus on prevention and increasing capacity in community-based urgent care services will provide alternatives to conveyance, attendance and admission and expansion of capacity including Same Day Emergency Care (SDEC), Urgent Community Response (UCR) and virtual wards. If this is not implemented, the system is likely to experience increased risk of harm associated with longer ambulance response times, long waits and overcrowding in Emergency Departments and insufficient acute workforce capacity. This may lead to delayed assessment, treatment or admission, increased deterioration while waiting and reduced ability to isolate infectious patients, leading to higher risk of harm and patient safety concerns. Work continues to improve ambulance handovers (45-min cap with progress towards 15-min handovers) and rapid release protocols. Prolonged ambulance handovers introduce higher risk of patient safety concerns through delayed assessment, treatment and admission, and the inability to release ambulance crews to their next call-out impacting ambulance response times and risking patients deteriorating in the community whilst waiting for an ambulance. Additional winter initiatives to support timely, safe discharge and increase intermediate bed capacity reduce hospital flow issues associated with high bed occupancy and delayed discharges / transfers of care. If this is not implemented, the system is likely to experience increased risk of patients deconditioning while waiting for discharge. Strengthened infection prevention and control (IPC) measures, including testing capacity and enhanced cleaning, support safe environments reduce the potential for harm from infectious disease outbreaks. Staff vaccination programmes and campaigns to increase vaccine uptake reduce the risk of influenza and COVID for patients by reducing the likelihood of patients receiving care from unvaccinated staff. Public winter vaccination campaigns, outreach programmes and interventions to reduce vaccine hesitancy have a positive impact on patient safety by reducing the risk of serious illness, hospital admission and winter-related complications from vaccine-preventable diseases. This helps to protect vulnerable populations and reduce avoidable pressure on UEC services.
Patient Experience	The NEL Winter Plan articulates a variety of interventions (system-wide and localised) that aim to improve access to and capacity of services during the winter period to mitigate against the increased demand and pressure that health and care services experience in winter. The impact of not delivering these interventions would negatively affect patient and carer experience through longer waits for

	appointments, delays to assessment and treatment and poorer privacy and dignity in overcrowded EDs / corridor care. This is likely to lead to reduced confidence in services, increased anxiety for patients and carers, and increased complaints. These are likely to have a long-term lasting impact on how the public interact with health and care services which will influence behaviours beyond the winter months. Acute Trusts continue to optimise and enhance clinical triage and streaming functions at the hospital front door using senior decision-makers to reduce ED long waits which is often cited by patients as one of the main reasons for their negative experience. Delivery of additional services that focus on prevention, proactive care and care closer to home throughout the winter months all seek to reduce the likelihood of negative experiences of patients and carers by reducing avoidable acute activity and delivering a more personalised and proactive service. This is particularly importance for frail and elderly patients for whom prolonged hospital stays are likely to lead to deconditioning and poorer health outcomes. Community-based urgent care alternatives and crisis cafés improve patient dignity, choice and experience, particularly for mental health patients. Additional capacity in crisis services, embedded mental health and local authority support and improved connectivity between acute, mental health and community services outlined in the NEL Winter Plan aim to mitigate against the increased demand seen in winter, and reduce the likelihood of poorer patient experiences compared with less busy periods. The NEL Winter Plan emphasises the importance of community pharmacy services (such as Pharmacy First) that will support patients with common winter minor illnesses and reduce the need for them to try to access advice, treatment or medication from busy GP, UTC or ED services. Winter communications support these messages to increase public awareness of services available close to home. Self-management and digital enablement, through health coaching, peer support and tools such as NHS App, Patients Know Best, ‘What matters to me’ and the Universal Care Plan, ensure continuity and personalisation of care across settings.

	 Patient experience data (source: NHS Friends and Family Test) shows consistent drops in patient experience in ED across NEL over the last three winters, although the overall trend is positive with higher patient experience scores in recent years. This demonstrates the importance of early winter planning and delivery of a co-ordinated system response to increase winter resilience and preparedness and reduce the likelihood of patients having negative experiences when using ED services.
Staff Experience	The NEL Winter Plan articulates interventions that aim to mitigate against the higher demand on ambulance and acute UEC services during winter by increasing capacity and access to services in primary care and the community, and optimising in-hospital processes including discharge pathways. A major reason for implementing the winter is protecting our health and care workforce from the increased pressure that they will experience during periods of peak demand. Joint surge modelling, System Coordination Centre and EPRR functions, and increased clinical presence within workforce planning (where possible) aim to reduce stress from escalation pressures. Local and regional winter testing exercises are being implemented in September/October to test operational resilience and escalation protocols. In addition to implementing schemes that aim to reduce demand and increase capacity, the system has a number of year-round and winter-specific workforce initiatives including (but not limited to) targeted vaccination campaigns and wellbeing support for staff (flu champions, peer vaccinators, incentives), The impact of not implementing these interventions is increased workload, higher operational pressure, poor staff wellbeing (burnout and fatigue, sickness absence, reduced morale) and increased risk of human error. This may also lead to a consequential risk to patient safety, quality of care and staff retention.
Patient Outcomes	The NEL Winter Plan sets out additional prevention, proactive care, community, mental health, urgent care and discharge interventions intended to match increased winter demand with sufficient capacity, support timely assessment and treatment, and help people receive care in the most appropriate setting. These

	interventions aim to reduce avoidable deterioration, emergency attendance and admission, prolonged hospital stays and readmission, while supporting recovery, independence and better outcomes for babies, children, young people and adults. If the additional services are not implemented, patients are more likely to experience delays in accessing advice, assessment and treatment, with increased risk of their condition deteriorating while waiting. Reduced access to these community alternatives, same-day care, urgent mental health support and planned discharge capacity that is being implemented through the Winter Plan is likely to increase avoidable ambulance conveyance, ED attendance and hospital admission, contributing to longer waits, high bed occupancy and delayed transfers of care. The resulting impact on patient outcomes may include poorer recovery, preventable complications, increased deconditioning and loss of independence, longer length of stay and higher risk of readmission. This is particularly relevant for people living with frailty or multiple long-term conditions, care home residents, people experiencing a mental health crisis, and babies, children and young people affected by seasonal respiratory illness. Failure to deliver proactive vaccination and outreach activity may also increase serious illness and hospitalisation from vaccine-preventable disease and widen existing health inequalities. Conversely, implementation of the planned interventions—including vaccination, risk stratification, Same Day Emergency Care, Urgent Community Response, virtual wards, frailty and paediatric pathways, crisis alternatives, community pharmacy, discharge-to-assess and intermediate care—should improve timely access, reduce avoidable acute activity and support safer recovery closer to home. Outcome measures should be monitored through established quality and performance governance, including deterioration and admission rates, long waits, length of stay, delayed discharge, readmission and outcomes for priority and underserved groups. Specific interventions designed to support people who are homeless or in vulnerable housing such as rough sleeping outreach, Warm Hubs, Severe Weather Emergency Protocol (SWEP) and the NEL Community Pharmacy Selfcare Advice Service (CPSAS) provide targeted support to protect and provide for vulnerable people sleeping rough during severe winter weather.
Integrated Care	The NEL Winter Plan sets out a coordinated whole-system approach across primary care, community services, mental health, ambulance and acute providers, local authorities, social care and the VCFSE sector. Additional winter services and schemes are intended to continue to provide strong and coordinated prevention and proactive care, maintain access to alternatives to hospital, support timely transfer between services and increase discharge and intermediate care capacity to meet the increased winter demand. Integrated planning, shared escalation arrangements and multidisciplinary working aim to ensure that patients receive joined-up care in the most appropriate setting, with risks managed collectively across the health and care system in spite of increased operational pressure and demand. If the additional services and winter schemes are not implemented, there is a greater risk that services will operate in isolation, with reduced capacity to

	coordinate care across organisational boundaries. This could result in fragmented pathways, duplicated assessment, inconsistent referrals, poorer information sharing and delays in accessing community, mental health, social care and discharge support. Patients may experience unnecessary ambulance conveyance, emergency attendance or admission, repeated transfers between services and gaps in follow-up care, particularly where needs are complex or require input from several partners. Failure to provide sufficient integrated community and discharge capacity would also be likely to increase delayed transfers of care and length of stay, while shifting operational risk between organisations rather than managing it across the system. This would reduce whole-system resilience during peak demand and limit the ability of partners to respond collectively to emerging pressures, outbreaks or severe weather. Conversely, delivery of neighbourhood multidisciplinary teams, SPoA/ICC coordination, Integrated Discharge Hubs/Transfer of Care Hubs, shared care plans, joint case management and System Coordination Centre oversight should deliver continuity, timely decision-making, use of available capacity and coordinated escalation throughout winter.

Equality Impact Assessment ***Endorsed by Hina Mir, EDI People Business Partner (7 September 2026)***	
Age	**BCYP:** Expansion of paediatric rapid assessment, paediatric escalation protocols and dedicated children's urgent care pathways support the younger population requiring urgent care. Trusts continue to make improvements to see more children in ED within 4 hours. Services such as the Family Liaison Nurse in Waltham Forest provide holistic support to families with under-5s frequently attending ED to provide tailored support, community referrals and reduce avoidable visits. **Frail and elderly patients:** Increased frailty virtual ward capacity, optimisation of frailty pathways and single point of access (e.g. frailty line), falls prevention services, Universal Care Plans, and the expanded comprehensive geriatric assessment model all provide additional support for older people or people living with frailty over winter. Examples of schemes to support frail and elderly residents over winter include: • Expansion of Universal Care Plans (UCPs) across frailty cohorts, residents in proactive care schemes, end of life residents and care home populations. UCP visibility for NHS 111 and LAS will support alternatives to conveyance and improved decision making. • The Falls Proactive Care Model initiated by London Borough of Barking and Dagenham in 2025 is supporting identification of residents at risk of falling which will subsequently link to appropriate services in the community to improve health and wellbeing and reduce future demand on health and care services.

	• The Health Navigator programme uses a clinically validated AI algorithm to identify patients at rising risk of experiencing an extended stay in hospital and offer person-centred clinical coaching to help people take control of their health and stay out of hospital. • Tower Hamlets Link Age+ provides social activities for 50+ to combat loneliness and social isolation over winter. **Care Home residents:** Through the Supplementary Care Home Service, practices are required to deliver proactive care to care home residents, including personalised care planning, regular MDT reviews, Comprehensive Geriatric Assessments (CGAs), medication reviews, falls assessments and immunisation support. The scheme also includes a two-hour response standard during core hours for residents requiring urgent primary care intervention, enabling timely assessment and management in the community. **End of life:** End of life care transformation plans are in place to enable more people to remain in the place they call home and be cared for within their community at the end of life. Through ensuring effective support is in place to increase use of the Universal Care Plan and to embed the delivery of end life care into neighbourhood working and increasing the reach and capacity of established successful Hospice at Home services - inappropriate ambulance conveyances, ED attendances and emergency admissions will be reduced.
Disability	Enhanced urgent community response and mental health crisis alternatives reduce reliance on ED for people with physical and/or mental disabilities. Digital platforms (Connected Care Records) support proactive identification of vulnerable patients. UCR, virtual wards and Hospital at Home may benefit people with mobility, sensory, learning or mental health needs by reducing avoidable travel and hospital attendance. Also note the need for reasonable adjustments, accessible information and non-digital routes. Conversely, digitally-enabled pathways (virtual wards, apps, Connected Care Records, remote monitoring) risk excluding people with learning disabilities, sensory impairments or limited digital access unless easy-read materials, British Sign Language provision, and assured non-digital/telephone routes are in place and actively offered, not just available on request. **Support to housebound residents:** North East London operates a dedicated Housebound Checks and Winter Vaccinations Scheme, which provides proactive outreach to housebound patients who are unable to attend their GP practice. The scheme aims to identify risks prior to the winter period, ensure patients are up to date with flu, COVID-19 and pneumococcal vaccinations, review wider health and wellbeing needs, and implement early interventions where required. This targeted approach supports some of the most vulnerable residents and is designed to reduce unnecessary and inappropriate use of emergency and secondary care services during periods of increased winter pressure.
Ethnicity	Multidisciplinary neighbourhood proactive care MDTs and community outreach vaccination campaigns target high-risk and underserved ethnic groups, including

	vaccine hesitancy programme to increase uptake in communities with lower vaccination participation. Borough-level stratification enables resources to be deployed equitably. Interpreting and translation support is available across NEL in primary and secondary care, community pharmacies, and mobile outreach clinics. This support helps reduce language and communication barriers and promotes equitable access to services.
Gender reassignment	No direct service redesigns identified, but inclusive access to urgent care and crisis services applies to all protected groups. No differential impact or need has been identified for this group in relation to winter urgent and emergency care pathways; this will be kept under review as engagement with trans and non-binary patient groups develops.
Marriage and civil partnership	No specific impact identified, but access to urgent care and crisis services is inclusive. No differential impact by marital or civil partnership status has been identified for winter services; this characteristic will be revisited if evidence emerges (e.g. from carer or bereavement-related pathways).
Pregnancy and maternity	Enhanced IPC and discharge planning reduces risk of infection exposure during maternity care. Targeted RSV vaccination offer for pregnant women from 28 weeks, including call-and-recall and accessible community provision. Improved urgent care capacity supports safer access for pregnant people during winter pressures. Pregnancy payments programme is in place in Tower Hamlets for women from families on low incomes who become pregnant over winter.
Religion or belief	Engagement with voluntary, faith and community sectors in planning supports inclusivity. Multidisciplinary neighbourhood proactive care MDTs and community outreach vaccination campaigns target high-risk and underserved ethnic groups, including vaccine hesitancy programme to increase uptake in communities with lower vaccination participation often associated with religion or belief systems.
Sex	Frailty and maternity pathways ensure tailored care for women, while urgent care access and streaming improvements apply equitably. Consideration should also be given to differential health-seeking behaviour by sex (e.g. lower rates of help-seeking among men for mental health crises), and to whether winter communications and outreach are reaching men and women equitably.
Sexual orientation	No specific impact identified, but access to urgent care and crisis services is inclusive. No differential impact has been identified for this group; services are designed to be inclusive, and this will be kept under review, including in relation to LGBTQ+ mental health crisis provision.

Inequality Impact Assessment *Endorsed by Melody Dhinda-Rees, Senior Officer for Health Equity and Inclusion (7 September 2026)*	
Refugees and recent migrant communities	Targeted vaccination campaigns in communities (pharmacies, schools, mobile clinics) improve access for migrants. Interpreting and translation support is available across NEL in primary and secondary care, community pharmacies, and mobile outreach clinics for people experiencing homelessness and vulnerable migrants. This support helps reduce language and communication barriers and promotes equitable access to services. The Praxis service operates at two NEL hospitals - the Royal London and Homerton Hospital - to support the discharge of people with insecure or restricted immigration status, including those with no recourse to public funds. The service helps enable safe and timely discharge once clinical treatment has been completed, reducing delayed and potentially unsafe discharges. This is particularly important during winter, when pressure on hospital beds is greater. NEL Community Pharmacy Selfcare Advice Service (CPSAS) helps eligible patients who cannot afford to buy required over the counter medicines recommended by the community pharmacist by providing advice and free of charge medicines. Eligible patients include Homeless, Asylum Seekers, Refugees or NEL GP-registered patients receiving financial-related benefits, patients under 18 years old in full time education and have at least one parent eligible for this service, or young people who are under the care of the Local Authority.
People who are homeless or in vulnerable housing	UCR and discharge-to-assess pathways provide support without requiring stable housing. The national autumn/winter COVID-19 and flu vaccination programme, in partnership with local authorities, primary care, community pharmacies and the ICB includes eligible people experiencing homelessness. Outreach support is provided across NEL for people experiencing homelessness and Warm Hubs are operational over winter. Targeted outreach in hostels, day centres and Gypsy, Roma and Traveller sites will provide care and support, opportunistic vaccinations, continuity of care and support into shelter. These services also provide extended hours. Severe Weather Emergency Protocol (SWEP) helps to protect and provide for vulnerable people sleeping rough during severe weather. SWEP is activated when there is a real risk of loss of life because of weather conditions, it must be applied when the night-time temperature is predicted to be zero degrees Celsius or below for three nights in a row. See information from Waltham Forest as an example. NEL Community Pharmacy Selfcare Advice Service (CPSAS) – see above.

Looked after children and care leavers	Paediatric escalation pathways ensure timely assessment and treatment in urgent care settings. NEL Community Pharmacy Selfcare Advice Service (CPSAS) – see above.
Sex workers	No direct interventions, but there is a strong safeguarding focus and crisis alternatives and access to community urgent care reduce barriers.
People with drug or alcohol dependence	Crisis assessment centres and liaison psychiatry support improved access to mental health and urgent care. Community pharmacies provide smoking cessation support and dispense drug and alcohol addiction medication, providing support in the community and reducing demand on hospital-based services. Neighbourhood working brings together partners to ensure co-ordinated care.
Victims of exploitation	Community engagement and safe urgent alternatives reduce reliance on pressured ED environments, supporting safer access.
People living in more deprived areas and/or those on low incomes	Neighbourhood-level risk stratification targets deprived areas. Community-based urgent care and vaccination outreach improve equity of access. Cold, damp, poor-quality or overcrowded housing can contribute to respiratory illness, cardiovascular disease and poor mental health, as well as increasing winter demand on health services. Health, social care and local authority partners will work together to identify and support residents experiencing fuel poverty, damp, mould or cold homes. Particular attention is given to Core20PLUS populations. Support coordinated through social prescribing, welfare services, neighbourhood teams, VCFSE partnerships and local authority-led schemes. Targeted outreach should be used to connect affected communities directly with warm homes and fuel-poverty support. Tower Hamlets has a Crisis and Resilience Fund with cost-of-living support and 'Community Cupboard' programmes to support people on lower incomes. NEL Community Pharmacy Selfcare Advice Service (CPSAS) – see above.
Ex-offenders	Integrated crisis hubs and urgent care alternatives provide timely and non-stigmatised access to care.
Those with caring responsibilities	Integrated crisis hubs and urgent care alternatives provide timely and non-stigmatised access to care. Carers play a key role in preventing admissions, supporting discharge and maintaining people safely at home during winter. As part of winter planning, there will be consideration of carer identification, access to information and advice, respite support where available, and ensuring carers are connected to local support through primary care, neighbourhood teams and social prescribing services.

Those who are digitally excluded	SPoA and telephone-based frailty lines ensure non-digital routes into urgent care remain available. As services increasingly promote NHS 111 Online, the NHS App, Patients Know Best and other digital tools, it is important that digitally excluded residents are not disadvantaged. Digital routes should complement rather than replace existing access channels. Residents who are unable to use digital services (or who prefer not to) should continue to be supported to utilise / directed to primary care, NHS 111 telephony, Integrated Neighbourhood Teams, social prescribing, care coordinators, community pharmacies and VCSFE partners. Where possible, winter communications will also be available through a range of accessible and non-digital formats. Winter communications will use community, primary-care, pharmacy and partner channels, not solely digital routes.

Appendix 5: Winter risks and mitigations

System risks have been identified as risks that occur consistently across Place and Provider Winter Assurance Matrix returns or have been identified by the ICB. Summary mitigations have been developed based on the actions identified by system partners in their Winter Assurance Matrix returns.

ID #	Risk description and impacts	Current risk score			Risk mitigations	Post-mitigation risk score		
		Likelihood	Impact	Score		Likelihood	Impact	Score
1	**Intermediate care capacity and Pathway 2 discharge delays:** Insufficient intermediate care and rehabilitation capacity leading to Pathway 2 discharge delays and increased length of stay. Risk impacts: • Increased patients with no criteria to reside • Reduced acute bed availability • Higher rates of deconditioning and patient safety concerns • Slower recovery due to delayed rehabilitation	4	4	**16**	• Additional intermediate care capacity is being considered for NEL Physical Capacity Funding (funding decisions expected in September). • Expansion of rehabilitation and reablement services e.g. additional capacity within NELFT Integrated Rehabilitation Services (IRS). • Use of Discharge Funding to increase workforce capacity e.g. social workers, therapy staff (Physio and OT) and brokerage officers in Redbridge. • Review configuration of intermediate care capacity e.g. ESD capacity and stroke bed usage. • Joint NELFT and BHRUT intermediate care on-call rotas are being established. • Daily escalation calls including capacity updates on Pathway 2 intermediate beds and IRS capacity. • Maximising efficiency of Transfer of Care Hubs and strengthening coordination with Local Authority and community partners. • Homerton are launching a digital discharge tool ahead of winter to improve real-time visibility of discharge readiness and delays.	3	4	**12**
2	**Mental health length of stay and community capacity:** Long mental health hospital stays, delays in mental health discharge and limited step-down/community capacity. Risk impacts: • Extended lengths of stay • Poorer health outcomes caused by longer stays in hospital • Reduced bed availability and flow • Increased pressure across UEC pathways	4	4	**16**	• Expansion of community crisis alternatives e.g. 24/7 Mental Health Crisis Assessment Hub and crisis cafes, helping to reduce avoidable hospital attendance and admission. • Enhanced discharge coordination • Additional step-down capacity, housing and community support pathways. • Daily operational oversight and escalation processes across mental health, acute, social care and local authority partners.	3	4	**12**

ID #	Risk description and impacts	Current risk score			Risk mitigations	Post-mitigation risk score		
		Likelihood	Impact	Score		Likelihood	Impact	Score
3	**Corridor care:** High demand and limited internal flow can lead to patients being cared for in corridors or non-clinical spaces. Risk impacts: • Increased clinical and safeguarding risk due to reduced visibility from clinical staff or poorer access to equipment • Reduced dignity and experience • Constrained ambulance offloading	4	4	**16**	• Providers have introduced rapid assessment zones, surge protocols, and additional assessment and decision-making spaces. • Patient safety incidents are reviewed through Trust clinical governance routes to ensure patient safety, visibility and clear escalation. • Initiatives to increase hospital flow through improved discharge processes (NHS SAFER patient flow bundle) such as same-day and pre-11am discharges, 7-day discharge capacity, use of discharge lounges, and early action to address medication, equipment, transport or social requirements.	2	4	**8**
4	**Workforce pressures:** Workforce shortages, sickness absence, limited temporary staffing options, and low vaccination uptake may reduce resilience across acute, community, primary and MH services during periods of peak demand. Risk likelihood increased due to prolonged pressure during summer heatwaves. Risk impacts: • Compromised patient flow • Increased clinical risk • Poor staff welfare • Reduced system ability to deliver safe and timely care	4	4	**16**	• Providers are using coordinated surge staffing models, flexible rotas, redeployment protocols, and real-time staffing dashboards to maintain safe staffing levels. • Staff wellbeing support, occupational health resources and vaccination campaigns are in place, supported by daily operational oversight and system-wide escalation to prioritise high-risk services when pressures arise.	2	4	**8**
5	**Frontline staff flu vaccination uptake:** Unvaccinated frontline staff may spread flu or COVID to vulnerable patients and other frontline staff. Risk impacts: • Increased frontline staff sickness • Increased risk to vulnerable patients • Increased pressure on UEC services	4	3	**12**	• Trusts across NEL have plans in place to increase frontline staff vaccination • Early Flu Peer Vaccinator (FPVs) recruitment with an aim to ensure vaccinators on each ward and department • Pop-up clinics on all sites and clear visible Executive leadership support • Primary care contracting team encouraging primary care services to promote vaccinations with staff.	3	2	**6**

ID #	Risk description and impacts	Current risk score			Risk mitigations	Post-mitigation risk score		
		Likelihood	Impact	Score		Likelihood	Impact	Score
6	**Babies, Children and Young People (BCYP):** Seasonal surges in paediatric respiratory illness and mental health presentations may exceed available capacity across paediatric assessment units, wards, and crisis services. Risk impacts: Delays in assessment or admission may increase clinical risk, lead to longer ED waits, and heighten safeguarding concerns, particularly for vulnerable children and young people.	3	4	**12**	• Additional ARI hub capacity being considered for Physical Capacity Funding (decisions expected in September), with ringfenced slots for 48-hour asthma reviews. • System partners have implemented strengthened BCYP escalation pathways, paediatric streaming via UTCs and NHS 111, and surge plans for bronchiolitis and winter respiratory illness. • Rapid triage arrangements ensure timely clinical decision-making, while CYP mental health crisis support is coordinated through CAMHS and community teams. • Referral into specialist paediatric services and retrieval pathway in place for specialised medical transport from local hospitals to paediatric ICU for critically ill patients. • Safeguarding oversight is maintained through multi-agency escalation and follow-up processes. • Family Liaison Nurse-led service in place over winter at Whipps Cross to support with reducing BCYP re-attenders and support signposting back into appropriate community services. • Homerton has activated ID NOW point-of-care testing for COVID-19, influenza and RSV for children admitted to Children's Emergency Admissions (CEA) with respiratory or febrile presentations, enabling rapid diagnosis, cohorting and isolation.	2	4	**8**
7	**Use of alternative care pathways:** Insufficient capacity, inconsistent utilisation or poor visibility of community alternatives results in avoidable ED attendance and admission. Risk impacts: • Increased pressure on ED • Extended waits and crowding • Poorer patient experience • Reduced flow across the system	3	3	**9**	• Strengthen visibility, referral pathways and utilisation of community alternatives through SPoA/ICC, accurate DoS management and enhanced access to UCR, virtual wards, SDA, SDEC, community pharmacy and MH crisis services. • Additional winter capacity will be commissioned where required, with utilisation and performance monitored through system governance. • Type 3 improvement plans in place for Barts Health sites to address recent challenging performance.	2	3	**6**

ID #	Risk description and impacts	Current risk score			Risk mitigations	Post-mitigation risk score		
		Likelihood	Impact	Score		Likelihood	Impact	Score
8	**Maximum 45-minute ambulance handovers:** Delays in handover at acute front doors due to ED crowding and bed flow constraints. Risk impacts: • Prolonged ambulance waits delay response times in the community • Increase clinical risk for patients • Overcrowding in receiving areas	3	3	**9**	• Providers have implemented improved ambulance receiving areas, real-time handover dashboards, and senior clinical decision-maker presence at peak times. • System-wide escalation processes to maintain flow and prioritise timely transfers. • LAS have asked acute trusts to ensure adequate Fit to Sit (with consistent criteria), wheelchair availability and, where possible, dedicated LAS handover nurse or senior clinical decision maker in every ED.	2	3	**6**
9	**Frailty:** Increasing demand from older, frail patients with multiple long-term conditions presents ongoing pressures on urgent and emergency care, particularly during surges in respiratory illness and seasonal exacerbations. Risk impacts: • Longer hospital stays • Higher rates of deconditioning and patient safety concerns • Increased risk of readmission if proactive management and timely discharge are not achieved • Reduced flow and delayed discharges	3	3	**9**	• System partners are expanding frailty pathways, including rapid assessment via Same Day Emergency Care, Urgent Community Response, virtual wards, and the use of a Single Point of Access (SPoA) in NEL to support consistent navigation and faster access to community-based support. • Use of criteria-to-reside oversight and discharge-to-assess approaches to maintain flow and improve outcomes.	2	3	**6**
10	**ICB restructure and reduced capacity:** Reduced ICB capacity and loss of institutional knowledge following implementation of the Model ICB Blueprint impacts the ability to coordinate winter planning, assurance, escalation and commissioning decisions. Risk impacts: • Reduced resilience for escalation periods • Loss of historical knowledge & relationships • Delays in decision-making and winter scheme mobilisation	3	3	**9**	• Clear governance through the NEL UEC Board and SCC arrangements. • Defined escalation routes and executive sponsorship. • Use of established provider, Place and neighbourhood forums. • Cross-cover arrangements and strengthened system leadership oversight.	2	3	**6**

ID #	Risk description and impacts	Current risk score			Risk mitigations	Post-mitigation risk score		
		Likelihood	Impact	Score		Likelihood	Impact	Score
11	**ICB staff flu vaccination uptake:** Unvaccinated ICB staff may spread flu or COVID to frontline workers and/or vulnerable residents. Risk impacts: • Increased ICB staff sickness • Increased spread of flu or COVID pathogens • Increased pressure on UEC services	3	3	**9**	• All-staff communications from ICB CEO about the importance of our contribution to population-level protection. • Encouraging staff to book their own flu vaccinations. • Encouraging staff to support friends and family to make informed decisions to protect themselves and others. • Promotion of ICB vaccination campaigns. • Work with local partners such as primary care, schools, care homes and VCFSE colleagues.	2	3	**6**

Winter Planning 26/27
Board Assurance Statement (BAS)
Integrated Care Board (ICB)

1. Purpose
The purpose of the Board Assurance Statement is to ensure the ICB's Board has oversight that all key considerations have been met. It should be signed off by both the ICB Accountable Officer and Chair.

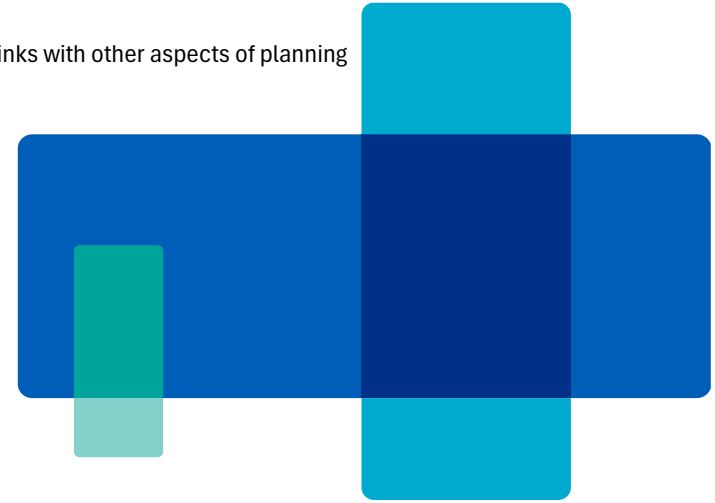
2. Guidance on completing the Board Assurance Statement (BAS)

Section A: Board Assurance Statement
Please include the Integrated Care Board's (ICB) name at the top of the tab.
This section gives ICBs the opportunity to describe the approach to creating the winter plan, and demonstrate how links with other aspects of planning have been considered.

Section B: 26/27 Winter Plan checklist
This section provides a checklist of what Boards should ensure is covered in their 2026/27 Winter Plans.

3. Submission process and contacts

Completed Board Assurance Statements should be submitted to the national UEC team via england.eecpmo@nhs.net by **30 September 2026.**



Winter Planning 2026/27: Section A Board Assurance Statement			
Assurance statement	**Is this in place? (Y/N)**	**Additional comments or qualifications (optional)**	**Blockers/Support requirements (optional)**
The Board has assured the ICB Winter Plan for 2026/27.	To be completed following ICB Board approval on 30 September	The Winter Plan is presented to the North East London ICB Board on 30 September for assurance and approval. Trust Boards are undergoing their own Winter Plan assurance and approval processes in parallel to meet the 30 September NHS England deadline.	None identified.
The Board is assured that a robust quality and equality impact assessment (QEIA) informed development of the ICB's plan and this has been reviewed by the Board.	Yes	An Equality, Quality and Inequality Impact Assessment (EQIA) has been developed and has been used to inform the development of the NEL Winter Plan. This has been endorsed by the NEL ICB EDI, Quality, and Health Equity and Inequalities Teams.	None identified.
The Board is assured that the ICB's plan was developed with active engagement and input of all system partners, including primary care, 111 providers, Community Health Service Providers (including NHS and non-NHS providers), acute and specialist trusts, mental health, ambulance services, local authorities and social care provider colleagues.	Yes	All system partners have been involved in the development of Provider, Place/Neighbourhood and the overarching Integrated Care System winter plans. Winter planning events have been held with system partners to develop cohesive plans for winter preparedness and resilience that have been used to inform the NEL-wide plan.	None identified.
The Board has identified a named Executive Winter Sponsor with clear accountability for winter preparedness, delivery, escalation and system oversight.	Yes	The North East London Executive Winter Sponsor is Charlotte Pomery (Chief Strategic Commissioning Officer).	None identified.
The Board has tested the plan during a regionally-led winter exercise, reviewed the outcome, and incorporated lessons learned.	Yes	No additional comments	None identified.
The Board is assured that clear winter governance arrangements, escalation processes and operational leadership arrangements are in place across the system.	Yes	Detail contained within the NEL Winter Plan addresses all key actions in Section B, guided by previously agreed national and regional ambitions and actions.	None identified.
The Board is assured that fit testing arrangements, PPE provision and other key infection prevention and control mitigations, as set out in the Health and Social Care Act 2008: code of practice on the prevention and control of infections are in place across the system and are supported by plans to scale rapidly in response to infectious disease surges, ensuring continuity of safe care and workforce protection.	Yes	System-wide IPC arrangements (including fit testing, proactive PPE stock management, staff training, enhanced cleaning, surveillance, audit and outbreak escalation) are in place in line with the Health and Social Care Act 2008 Code of Practice, with established governance and surge plans to protect patients, staff and continuity of safe care during infectious disease outbreaks.	None identified.
The Board has received assurance that health protection governance, escalation processes, and provider response arrangements are effective and support timely management of infectious disease incidents and outbreaks, as set out in the ICB Commissioning Guidance - https://www.england.nhs.uk/long-read/clinical-response-outbreaks-of-infectious-disease-commissioning-guidance-icbs/	Yes	Established system health protection governance, UKHSA and local authority partnership arrangements, defined escalation routes and provider outbreak response plans align with the ICB Commissioning Guidance and enable timely identification, coordination and management of infectious disease incidents and outbreaks.	None identified.

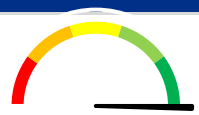
Winter Planning 2026/27: Section B Winter Plan Checklist			Confirm (Y/N)	Additional comments or qualifications (optional)	Blockers/Support requirements (optional)
Prevention and vulnerable cohorts					
1	1.1	The Board is assured that those eligible have access to priority vaccination programmes, including childhood vaccinations. This includes 1. RSV vaccination for pregnant women, older adults aged 75 years and over, and older adult care home residents; 2. pneumococcal vaccination for all adults aged 65 years and over and children and adults in a clinical risk group; 3. the annual winter flu and COVID-19 vaccination campaigns.	Yes	Community, frontline staff and ICB staff vaccination programmes are in place across NEL which have been designed to increase uptake, reduce hesitancy, build confidence, and maximise convenience. Priority programmes include childhood vaccinations, RSV vaccination for pregnant women and older adults and the annual winter flu and covid vaccination campaigns. NHS North East London ICB is assured that appropriate arrangements are in place to deliver the Autumn/Winter 2026/27 COVID-19 and influenza vaccination programmes in accordance with NHS England requirements, as documented in NEL's assurance submission to NHSE on 28 July 2026.	None identified.
	1.2	The Board is assured that a risk-stratified approach is in place to identify individuals at increased risk of winter-related deterioration or admission, including those with co-morbidities that leave them susceptible to hospital admission as a result of winter viruses, younger people with significant co-morbidities, and other vulnerable cohorts. Targeted interventions are in place, including vaccination, pre-winter health checks, personalised care planning, access to antivirals where appropriate, and community-based support to reduce avoidable admissions and support continuing care in the community.	Yes	North East London operates a risk-stratified approach to identify and support residents at increased risk of winter-related deterioration, admission and poor health outcomes, including care home residents, housebound patients, people with multiple long-term conditions and other clinically vulnerable cohorts. Targeted interventions include vaccination programmes, proactive care, personalised care planning, multidisciplinary reviews and community-based support.	None identified.
Flow, occupancy and discharge					
2	2.1	The Board is assured there are clear operational plans and daily processes to reduce avoidable bed occupancy and improve patient flow seven days a week, including pre-winter review of bed demand and capacity, delivery of model discharge pathway requirements, and executive oversight of required and actual daily discharges, patients with no criteria to reside by pathway, discharge performance and long discharge delays.	Yes	Robust system-wide arrangements are in place to reduce avoidable bed occupancy and improve patient flow seven days a week. Acute providers have undertaken pre-winter demand and capacity modelling and reviewed discharge pathways against Model Discharge requirements, supported by executive oversight of daily discharges, patients with no criteria to reside, long length of stay patients and discharge delays. Transfer of Care Hubs / Integrated Discharge Hubs provide coordinated oversight of discharge across health, social care and community partners, supported by daily operational reviews, escalation processes and real-time monitoring of community capacity. Place and provider plans include targeted actions to expand reablement, intermediate care, step-down capacity, virtual wards and discharge-to-assess pathways, alongside agreed pre-Christmas occupancy reduction plans. System performance, discharge metrics and flow risks are monitored through established governance, discharge escalation calls and System Coordination Centre oversight to ensure timely intervention throughout the winter period.	None identified.
	2.2	The Board is assured that system partners have agreed and implemented measures to eliminate corridor care, with defined patient safety thresholds, supported by coordinated patient flow, discharge and escalation arrangements, with system level oversight and mitigation of risk during periods of increased demand.	Yes	System partners have agreed and implemented measures to make progress towards the elimination of corridor care as a patient safety priority, supported by defined escalation thresholds, coordinated patient flow processes and whole-system oversight. Providers have reviewed and strengthened front-door pathways, SDEC utilisation, discharge arrangements and ambulance handover processes to manage and optimise capacity, occupancy and flow. Acute, community, mental health, primary care and local authority partners operate established escalation frameworks, including OPEL processes, discharge escalation arrangements and System Coordination Centre oversight, enabling rapid identification and mitigation of emerging risks.	None identified.
	2.3	The Board is assured that discharge planning is being undertaken jointly with local authorities and social care partners, with effective system discharge coordination arrangements, clear processes to identify, escalate and resolve discharge delays, and real-time visibility of demand and community capacity.	Yes	Discharge planning is undertaken jointly with local authorities, social care partners, community providers and acute trusts through established Transfer of Care Hub / Integrated Discharge Hub arrangements across North East London. Daily multidisciplinary review processes provide oversight of discharge readiness, patients with no criteria to reside, long length of stay patients and community capacity, supported by clear escalation processes at Place, provider and system level. Real-time visibility of discharge demand and capacity is maintained through operational huddles, discharge escalation calls, system coordination arrangements and integrated discharge teams, enabling barriers to discharge to be identified and resolved promptly.	None identified.
	2.4	The Board is assured that providers have baselined current discharge arrangements against the model discharge pathway, identified gaps, and agreed improvement actions across acute, community and social care partners.	Yes	Acute providers have reviewed current discharge arrangements against the Model Discharge Pathway and agreed improvement actions that will support safe, timely discharge before and during winter. Plans are in place with acute, community, local authority and social care partners to continue to reduce discharge delays and improve outcomes.	None identified.
	2.5	The Board is assured that pre-Christmas and holiday-period occupancy reduction plans have been agreed and will be implemented.	Yes	System partners have agreed pre-Christmas and holiday-period plans to reduce occupancy and maintain patient flow. These include enhanced discharge coordination, Multi-Agency Discharge Events (MADEs) and RESET weeks, seven-day discharge capacity, senior clinical decision-making and escalation arrangements with community and social care partners.	None identified.
Demand, capacity, and surge management					
3	3.1	The Board is assured that whole system demand and capacity modelling has informed winter planning and that tested surge and extreme surge response arrangements are in place to support safe operational delivery during periods of increased demand.	Yes	Winter planning has been informed by whole-system demand and capacity modelling undertaken across acute, community, primary care, mental health and social care services. Providers have completed pre-winter demand, bed and workforce modelling, reviewed anticipated activity levels and identified additional capacity requirements, while Places have assessed local pressures and continue to deliver targeted interventions to support admission avoidance, discharge and patient flow. Tested surge, escalation and extreme surge arrangements are in place across system partners, supported by established OPEL processes, business continuity arrangements, mutual aid protocols and System Coordination Centre oversight. Scenario testing exercises, operational escalation frameworks and daily system coordination processes provide assurance that the system can respond safely to periods of increased demand, severe weather, seasonal illness outbreaks and other operational pressures while maintaining patient safety and service continuity.	None identified.
	3.2	The Board is assured that workforce, NHS 111 and mutual aid arrangements are sufficient to respond safely to sustained periods of increased demand.	Yes	Workforce, NHS 111 and mutual aid arrangements have been reviewed and are sufficient to support safe operational delivery during periods of sustained winter pressure. Providers have undertaken workforce and capacity planning, including review of staffing levels, escalation rotas, on-call arrangements, vaccination programmes, wellbeing support and business continuity processes. NHS 111 pathways continue to support direct booking into primary care, community services and urgent care alternatives, underpinned by ongoing review of Directory of Services information and referral pathways. System partners operate established escalation and mutual aid arrangements through OPEL processes, provider collaboration, Place-based operational oversight and System Coordination Centre leadership, enabling pressures, capacity constraints and emerging risks to be identified and managed proactively. These arrangements are further supported by tested surge and escalation processes, cross-system operational coordination and established relationships across providers, community services, mental health, primary care, local authorities and ambulance services.	None identified.
	3.3	The Board is assured that bed escalation capacity arrangements have been agreed and tested across system partners	Yes	Bed escalation capacity arrangements have been agreed and tested across system partners to support safe operational delivery during periods of increased demand. Acute, community and mental health providers have undertaken pre-winter bed and capacity modelling, reviewed escalation capacity and established surge and extreme surge plans aligned to local and NEL-wide escalation frameworks. Providers maintain agreed bed escalation processes, flex capacity where clinically appropriate, and operate tested OPEL, business continuity and mutual aid arrangements supported by executive oversight and on-call leadership. System partners coordinate capacity and escalation through established operational forums, discharge escalation arrangements and the System Coordination Centre, enabling early identification of pressures and deployment of mitigations. Regional and local scenario testing exercises, together with established provider collaboration arrangements, provide assurance that escalation capacity can be mobilised safely and effectively to maintain patient flow, service continuity and patient safety throughout the winter period.	None identified.
Primary Care, NHS 111 and admission avoidance					
4	4.1	The Board is assured that anticipated general practice demand has been assessed, and that: • sufficient capacity has been commissioned, including for surge periods (e.g. ARI hubs) • robust escalation process are in place • through the IUC Clinical Assessment Service (or equivalent), sufficient capacity will be commissioned for out of hours general practice, including weekends and bank holidays.	Yes	The Same Day Access (SDA) service delivers approximately 230,000 appointments annually, with capacity weighted towards the winter months to accommodate the anticipated increase in demand associated with seasonal respiratory illnesses and winter pressures. The SDA hubs also provide additional coverage during winter bank holiday periods to support system resilience. In addition, all PCNs across NEL provide Enhanced Access services, offering further primary care capacity outside core hours, alongside access to Community Pharmacy services through the Pharmacy First programme. Together, these services provide a range of alternative access routes for patients and help mitigate pressure on both general practice and urgent care services. Where requires, additional capacity will be commissioned through ARI Hubs using Physical Capacity Funding.	None identified.
	4.2	The Board is assured that practices are compliant with contractual requirements, including opening hours, use of online consultation, clinically urgent same day appointments (working towards 90% target) and practices making 1 appointment per 3,000 registered population available to NHS 111 for direct booking.	Yes	The ICB continues to monitor and support compliance with key contractual requirements across general practice, including core opening hours, online consultation access, clinically urgent same-day appointments, and the provision of appointments for direct booking by NHS 111. All practices have declared compliance with contractual requirements relating to online consultations and the provision of appointments for direct booking by NHS 111 through the practice e-declaration process. Delivery of clinically urgent same-day appointments remains a key priority within the Primary Care Medium Term Plan, with the ambition of achieving 90% same-day provision for clinically urgent requests. GP Appointments Data (GPAD) is reviewed on a monthly basis to monitor progress and identify practices requiring additional support.	None identified.
	4.3	The Board is assured that PCN enhanced access appointments are available and that PCNs make available to NHS 111 any unused on the day slots as per DES requirements.	Yes	All PCNs across North East London participate in the Enhanced Access service. Utilisation of Enhanced Access appointments remains high; however, where appointments remain unfilled, practices offer these for direct booking via NHS 111. Compliance with this requirement is monitored through the Enhanced Access claims process and followed up where necessary.	None identified.
	4.4	The Board is assured that unscheduled dental care delivery is monitored as part of the mandated number of urgent dental care appointments.	Yes	The ICB has adequate commissioned Urgent Dental Care delivery togther with 24/7 dental triage accessed via NHS111. Additionally, general dental practices are mandated to deliver 8.2% of their contracted activity as 'unscheduled care'. Both aspects are monitored as part of normal contract management processes and reviewed on a regular basis throughout the year.	None identified.
	4.5	The Board is assured that for community pharmacy, demand, activity and capacity has been assessed, including sufficient out of hours provision across the ICB footprint; and delivery of Pharmacy First and the Discharge Medicines service.	Yes	There is sufficient out of hours and bank holiday cover across community pharmacy aligned to urgent care centres and GP OOH services where possible. Pharmacy First: 97% (n=356) pharmacies in NEL have signed up to provide Pharmacy First. The Medicines and Pharmacy team will work with the NEL LPC to support and address any interface issues between community pharmacies and primary/secondary care (re Pharmacy First, DMS respectively). Discharge Medicines Service (DMS): DMS is an essential service for every community pharmacy with an NHS contract. NEL ICS will continue with alternate monthly ICS DMS group to review data and share learning, alongside the Regional DMS group.	None identified.
	4.6	The Board is assured that the NHS 111 Directory of Services profiles are accurate, regularly reviewed, and aligned to available capacity across primary care and community services.	Yes	London Ambulance Service are increasing appropriate first selection on the NHS 111 Directory of Services (DoS) for key pathways such as Pharmacy and GP Practices / SDEC and UCR for CAS and identify and address referral barriers into services.	None identified.
	4.7	The Board is assured that for all provision, both in and out of hours, there will be communications activity to ensure the public is aware of what services are available.	Yes	The ICB will implement a coordinated communications approach throughout the winter period to ensure patients are aware of the range of services available both in and out of hours and can access the most appropriate service for their needs. This will include targeted communications through GP practices, PCNs, NHS websites, social media channels, community networks and system partners.	None identified.
Community Health Capacity					
5	5.1	The Board is assured that sufficient urgent community capacity, including UCR, Virtual Wards, Hospital at Home and community diagnostic interventions where applicable, has been commissioned, is monitored and utilised effectively to support admission avoidance and discharge, with appropriate escalation arrangements in place and access to senior clinical decision-making through SPOAs at least 12 hours a day, 7 days a week.	Yes	Sufficient urgent community capacity has been commissioned across North East London to support admission avoidance, early discharge and care closer to home. This includes Urgent Community Response, virtual wards, frailty and respiratory pathways, Hospital at Home models, intermediate care services and community rehabilitation schemes, with additional winter capacity identified where required. System partners monitor service utilisation, referral activity and capacity through established operational oversight and escalation arrangements. Access to senior clinical decision-making is supported through Single Points of Access, Integrated Care Coordination arrangements and community clinical advice services operating seven days a week, enabling timely navigation of patients to the most appropriate community pathway.	None identified.
Whole System Response					
6	6.1	The Board is assured that a system wide approach is in place to optimise the use of out-of-hospital capacity and admission avoidance pathways to reduce avoidable admissions and support timely discharge.	Yes	A whole-system approach is in place to optimise out-of-hospital capacity and admission avoidance pathways across North East London. The Same Day Access service provides urgent same-day appointments, with NHS 111 and Urgent Treatment Centres able to direct book patients, while all PCNs deliver Enhanced Access services outside core hours. Proactive care schemes, including the LTC LIS, housebound support, care home services and borough-specific risk stratification initiatives, identify and support vulnerable patients before deterioration occurs. Community pharmacies further support admission avoidance through Pharmacy First, vaccination services, medicines optimisation, discharge medicines support and the CPSAS scheme. These arrangements are complemented by UCR, virtual wards, community rehabilitation and mental health crisis alternatives, supporting timely discharge and reducing avoidable admissions.	None identified.
Leadership, operational grip and resilience					

7	7.1	The Board is assured that on-call arrangements are in place and have been tested. Staff on-call have access to medical and nursing leaders for urgent advice if needed.	Yes	24/7 executive, senior operational and clinical on-call rotas are in place, with clear escalation routes to senior medical and nursing leaders for urgent advice. On-call staff receive structured winter briefings covering roles, responsibilities and escalation procedures. Arrangements have been tested through pre-winter exercises with system partners, with learning incorporated into plans to support safe, timely decision-making during periods of increased pressure.	None identified.
	7.2	The Board is assured there will be an appropriately skilled and resourced operating model for system co-ordination over the winter period to enable the sharing of intelligence and risk balance to ensure this is appropriately managed across all partners.	Yes	The integrated SCC/EPRR team operates a seven-day roster, 365 days a year, supported out-of-hours by a strategic and tactical on-call team which is replicated across system on-call arrangements with the Trusts and ICB also delivering upon their Category One responder legal and statutory duty arrangements. Escalation frameworks, mutual aid arrangements and system partner approaches are well established (ICB and London-wide), however the changing regional SCC arrangements (NWL and SWL/SCL) may introduce risk and instability for NEL.	None identified.
	7.3	The Board is assured that tested command, control and escalation arrangements are in place, including OPEL reporting, EPRR arrangements and clearly defined leadership responsibilities.	Yes	See Section 7.2. In addition, governance arrangements are in place in terms of EPRR framework and core domains, including but not limited BRF, LHRP Local and Regional EPRR fora. ICB has EPRR steering group which leads the programme around business continuity around EPRR arrangements. There is an Accountable Emergency Officer (AEO) for the ICB (Dr Paul Gilluley) and Deputy Director (Jo Scott-Jones) who lead on these arrangements.	None identified.
	7.4	The Board is assured that mutual aid arrangements, business continuity plans, severe weather preparedness arrangements and industrial action contingencies have been reviewed and tested where appropriate.	Yes	See Section 7.2 and 7.3. The mutual aid arrangements, business continuity plans, severe weather preparedness arrangements and industrial action contingenices are in place and have been reviewed and go through a continual cycle of testing. Governance and oversight arrangements are in place through the NEL ICB constituted fora. It is important to note that these arrangements have significant interdependencies and inform (and are informed by) population health data, integrated needs assessments, commissioning arrangements and assessment of provider service sustainability (annual self-assurance arrangements and regular interaction with system partners).	None identified.
	7.5	The Board is assured that plans are in place to support staff welfare through periods of high demand.	Yes	To support staff welfare through periods of high demand, Trusts have individualised wellbeing offers in place for their staff commissioned at a local level. Psychological support is in place for ED and critical care teams during winter. Staff flu vaccination plans have been developed to ensure all staff have the opportunity to obtain a flu vaccine when working on shift e.g. Flu Peer Vaccinator, Flu Nurse or Occupational Health colleagues. Communications plans are in place for weekly reminders of the importance of the vaccine and winter pressures. At the ICB level, SCC/EPRR policies refer to the importance of staff welfare, not just within an incident or operational pressures, but also through 1:1 appraisals and as part of briefing and debriefing approaches.	None identified.
System Co-ordination Centres					
8	8.1	The Board is assured that SCCs will be proactive in resolving operational pressures in hours, and that ICBs will maintain on-call functions for the oversight and resolution of operational pressures and patient flow and safety out of hours.	Yes	See Section 7. SCC is not limited to only resolving operational pressures (proactively, preventatively or reactively) but also has a strategic approach to system and patient safety as the predominant lens and works with pathways (clinical and non-clinical), with commissioners, with neighbourhoods, with partners in health and social care and with regional and national bodies (including out of London stakeholders) in this regard. This approach is reflected in the on-call arrangements.	None identified.
	8.2	The Board is assured that SCCs will be fully operational 08:00 to 18:00 7 days per week and have the ability to extend hours of operation during periods of extreme pressure.	Yes	The integrated SCC/EPRR team operates a seven-day roster, 365 days a year, supported out-of-hours (24/7) by a strategic and tactical on-call team which is replicated across system on-call arrangements with the Trusts and ICB also delivering upon their category 1 responder legal and statutory duty arrangements. On-call arrangements provide extended hours of operations during periods of extreme pressure.	None identified.
	8.3	The Board is assured that ICB SCCs will maintain proactive engagement and communication with NHSE Regional teams.	Yes	NEL SCC/EPRR team will maintain proactive engagement and communication with NHSE Regional teams through existing cadence and touchpoints in place for day-to-day as well as short-, medium- and long-term plans including coordination and management of incidents and issues.	None identified.

BAS Self-Assessment Dashboard

ICB

Overall Assessment Rating: 5 - Full assurance



		Rating
Section A	Governance	5 - Full assurance
Section B	Prevention and vulnerable cohorts	5 - Full assurance
	Flow, occupancy and discharge	5 - Full assurance
	Demand, capacity, and surge management	5 - Full assurance
	Primary Care, NHS 111 and admission avoidance	5 - Full assurance
	Community Health Capacity	5 - Full assurance
	Whole System Response	5 - Full assurance
	Leadership, operational grip and resilience	5 - Full assurance

Executive Approval

The purpose of the Board Assurance Statement is to ensure the Trust's Board has oversight that all key considerations have been met. It should be signed off by both the ICB Accountable Officer and Chair.

Integrated Care Board (ICB) name:	North East London ICB
ICB CEO/AO name:	Dr Nnenna Osuji
Date reviewed:	30/09/2026
ICB Chair name:	Dame Marie Gabriel CBE
Date reviewed:	30/09/2026

Please update following completion of the BAS template

Please provide name and contact details of designated Senior Responsible Officer (Executive Level)

Charlotte Pomery, Chief Strategic Commissioning Officer, charlotte.pomery@nhs.net

Please provide confirmation that the SRO has reviewed and approved the responses within this document, confirming they are correct and complete.

To be updated following ICB Board - The SRO has reviewed and approved the responses within this document, confirming they are correct and complete.

Please confirm date reviewed

30/09/2026

North East London
Integrated Care Board

NHS North East London ICB board

30 September 2026

Title of report	North east London maternity and neonatal service demand and capacity stock take
Author	Diane Jones, Director Clinical and Quality commissioning
Presented by	Dr Paul Gilluley, Chief Clinical and Quality commissioning officer
Contact for further information	Diane Jones Director, clinical and quality commissioning diane.jones11@nhs.net
Executive summary	Demand and capacity review outlines our north east London model of care for integrated maternity and neonatal services spanning primary, community and acute settings.
Action / recommendation	The Board is asked to approve next steps to progress with plans for engagement with residents and providers to deliver the models of care.
Previous reporting	Previously discussed and supported at the Quality Safety and Improvement Committee (September 2026)
Next steps/ onward reporting	Maternity and neonatal collaborative provide oversight on the delivery of the programme.
Conflicts of interest	None known.
Strategic fit	The ICS aims this report aligns with are: • To improve outcomes in population health and healthcare • To tackle inequalities in outcomes, experience and access
Impact on local people, health inequalities and sustainability	The model will streamline services, so women and babies receive the best care regardless of place of residence in north east London at vital points in the pathway e.g. booking, planned induction, labour and post birth.
Has an Equalities Impact Assessment been carried out?	Yes, with previous board paper
Impact on finance, performance and quality	Improvement in performance and resident experience
Risks	Please state any risks to the delivery and if possible, relate to the BAF risks

1.0 Introduction

1.1 The proposed North East London Maternity and Neonatal Model of Care (appendix 1) provides a clear framework for delivering this change through six core enablers:

- **A centralised digital booking and induction of labour (IOL) system** – improving access, transparency, and equity by ensuring capacity is allocated according to need and choice.

- **Care closer to home** – expanding continuity into multidisciplinary teams and community-based pathways to reduce unnecessary hospital visits and improve family experience.
- **Community maternity hubs** – if possible, repurposing existing birthing centre sites to provide enhanced antenatal, postnatal, and women's health services.
- **Redesignation of delivery suite and postnatal capacity** – optimising beds, flow, and theatre efficiency to relieve system pressures and meet rising demand.
- **Optimising neonatal flow** –ensuring optimal utilisation of Level 3 neonatal cot capacity through the planned and timely receiving and repatriation of babies between Level 2 and Level 3 neonatal units
- **Strengthen strategic commissioning.** Reframe the commissioning group to monitor outcomes and improvement, with emphasis on women's voices and organisational cultures.

1.2 Board is asked to approve the next steps in the programme to deliver the model of care.

2.0 Key messages

2.1 Maternity and neonatal services in north east London are under pressure due to increasing demand. The model of care is about optimising and modernising capacity to ensure services are safe, sustainable, and equitable across north east London through the proposed models of care which is a five-year programme (started 2024).

3.0 Body of report

3.1 The north east London demand and capacity review has provided valuable insight into where women are choosing to give birth and how demand is distributed across our maternity system. The output of the engagement has been the developed models of care which we are seeking approval to implement. This includes maintaining the closed stand-alone birthing centres at Barking and Barkingside and focussing on strengthening provision of ante-natal, post-natal and broader women's services at these sites.

Insight and feedback from women, families and communities helped to shape the proposed models of care. We will continue to listen and learn from residents' experience to ensure our future configuration of maternity and neonatal services addresses their priorities and provide a better experience for our communities.

We are also planning a proactive engagement process to allow women, families and communities a meaningful opportunity to influence our plans for community maternity hubs. This is vital to maintaining trust, transparency, and shared ownership of the changes, ensuring that the model reflects both clinical evidence, living experiences and priorities of our population.

4.0 Risks and mitigations

4.1 As the programme develops there may be a risk of reduced engagement from residents and stakeholders, hence a co design approach will increase involvement across participants.

Not delivering the overall changes carries a risk of delays to improvement within the maternity and neonatal pathways.

5.0 Conclusion

5.1 The paper has been supported at the quality safety and improvement committee and is shared with board to approve the nest steps to delivering the model of care.

6.0 Appendices

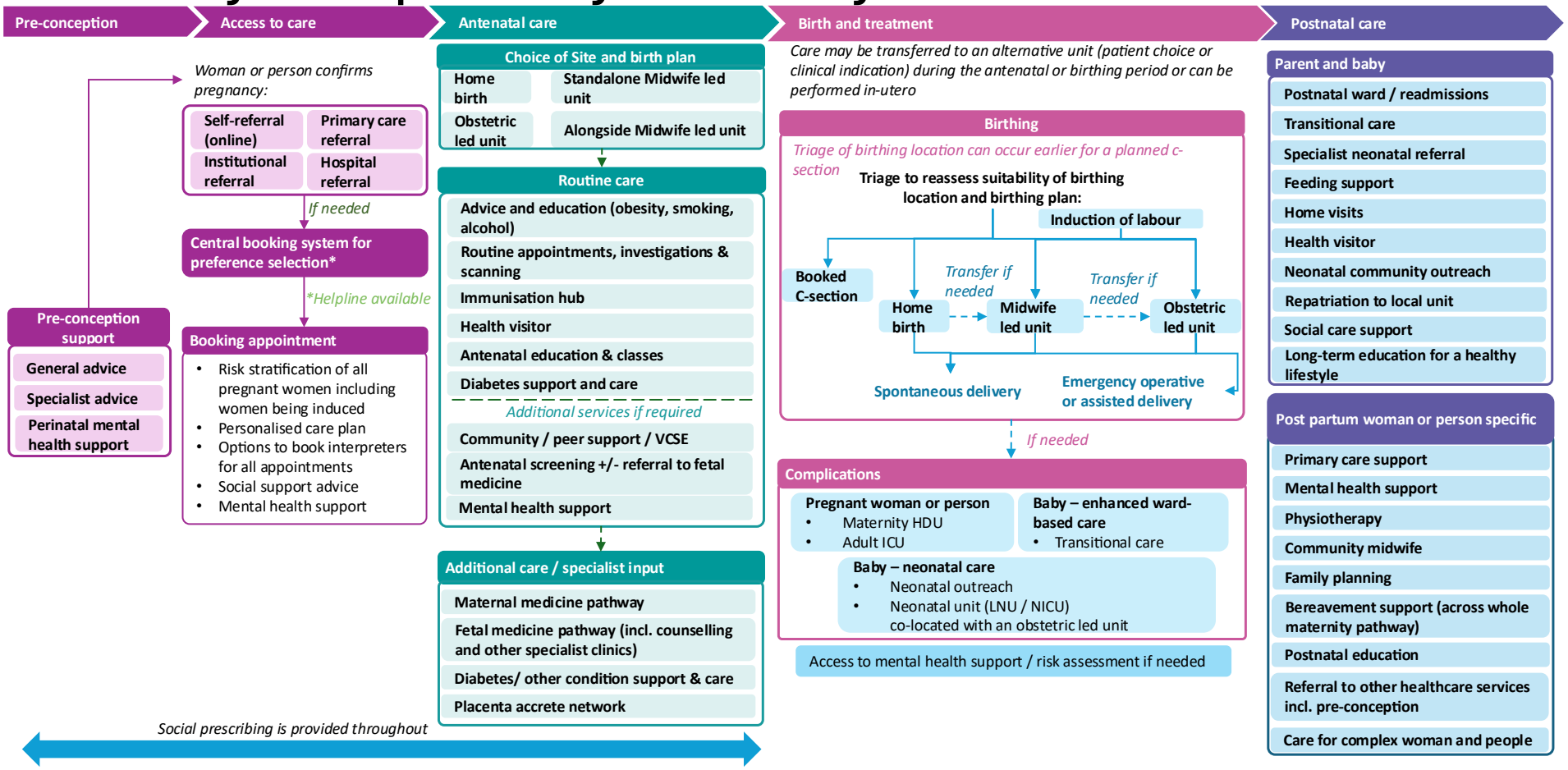
6.1 Appendix 1 Models of care

7.0 End

7.1 Diane Jones
23 September 2026

Maternity care pathway summary

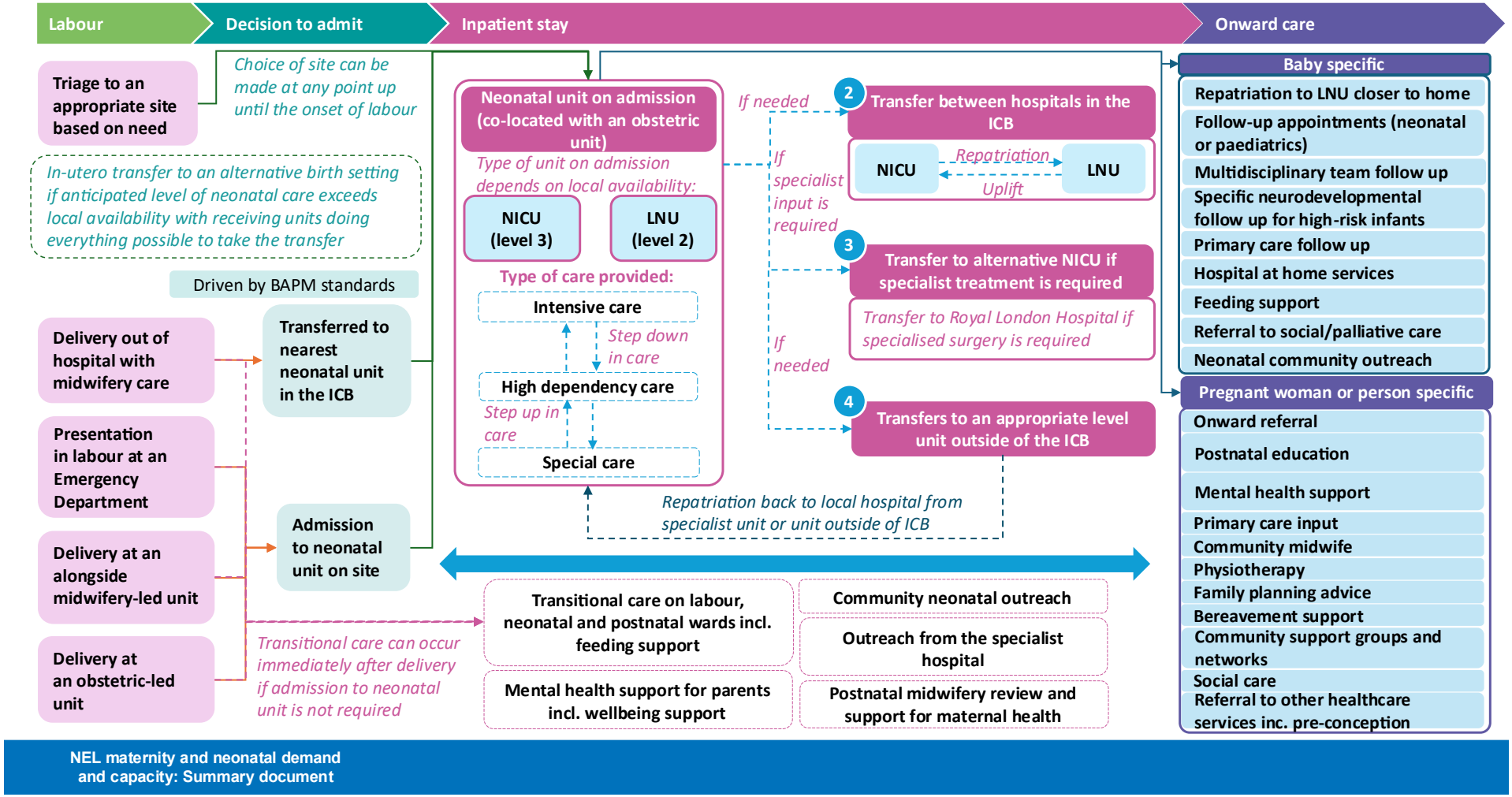
This is a best practice model of care and represents how care could be delivered in the future and does not reflect the current care pathway



NEL maternity and neonatal demand and capacity: Summary document

Neonatal care pathway summary

This is a best practice model of care and represents how care could be delivered in the future and does not reflect the current care pathway



North East London
Integrated Care Board

NHS North East London ICB board

30 September 2026

Title of report	Once for London ICB collaborative commissioning
Author	Henry Black, Chief Finance Officer
Presented by	Henry Black, Chief Finance Officer
Contact for further information	Henry Black, Chief Finance Officer
Executive summary	This paper sets out the proposal for the four London ICBs to work collaboratively on a number of jointly funded programmes of work. There are currently three programmes identified: • Million hearts and minds • Digital access • Neighbourhood health The proposal is for each programme to be allocated £1m of funding per year, with the costs shared on a capitation basis by the four ICBs. This amounts to a funding commitment from 2027/28 as follows: ICB / Pop'n Share / Contribution £'000 WNL 42% 1,270.20 NEL 25% 740.10 SWL 17% 499.80 SEL 16% 489.90 100% 3,000.00 In 2026/27 there is a pro-rata funding commitment of £750k for Million Hearts and Minds only, allocated as follows: ICB / Contribution £'000 WNL 318 NEL 185 SWL 125 SEL 122 750 The paper sets out the purpose, scope and intended benefits of each programme, as well as the principles for collaboration and proposed governance.

Action / recommendation	The Board is asked to approve the proposal to contribute towards the shared London programmes.
Previous reporting	Commissioning board
Next steps/ onward reporting	n/a
Conflicts of interest	There are no known conflicts in relation to this report.
Strategic fit	The ICS aims this report aligns with are: • To tackle inequalities in outcomes, experience and access • To enhance productivity and value for money • To support broader social and economic development
Impact on local people, health inequalities and sustainability	The anticipated benefits of joint working across pan-London priorities include harnessing greater scale and sharing of best practice.
Impact on finance, performance and quality	Better working at scale should develop opportunities to improve finance, performance and quality.
Risks	n/a

NHS
North East London

A joint approach to funding once for London deliverables in the LHP

ICB Board Meeting, 30 September

Context and purpose

This paper provides an overview of current Londonwide priorities, programmes and governance and invites a discussion about how the four London ICBs can best take advantage of opportunities to work at scale across London. This document is a single paper presented to all four boards in line with the uniform commitment to a single approach.



A number of agreed London NHS priorities for delivery with three main programmes as part of the LHP aligned with the 10YP

Million hearts and minds – Prevention – KF - WNL
Neighbourhood health – Left shift – AB – SEL/SWL
Digital Access – Analogue to digital – NCO - NEL



Whilst there may be elements of delivery to the three shifts in individual ICBs, there is merit in programmatically delivering elements at scale.



To avoid ad-hoc asks for funding (recurrent or non-recurrent) – a single collective funding approach from ICBs is proposed.

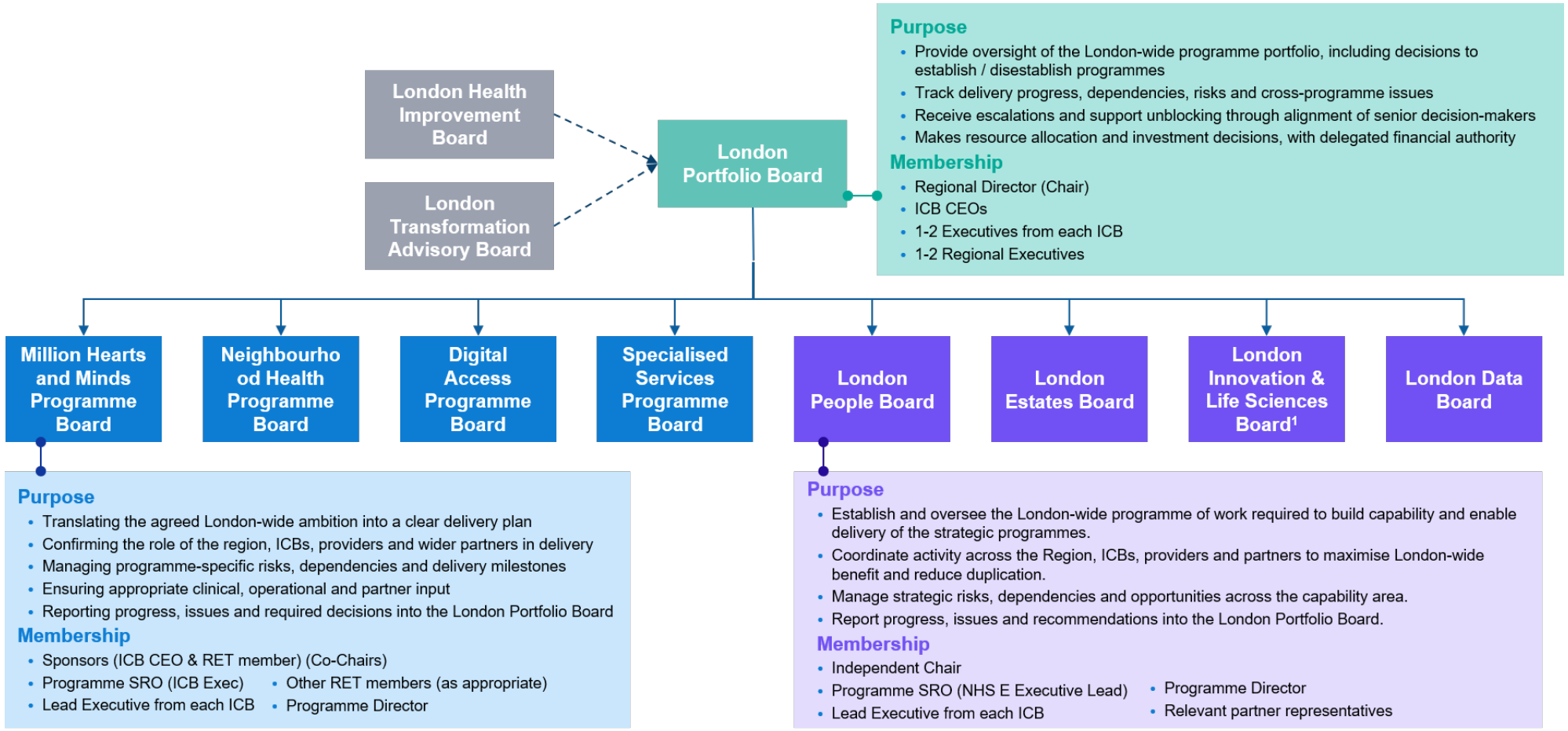


Elements of consideration:

Source of funding (ring fenced, national, other)
Fund holding and management – where does all/proportion of the money sit
Fund spending and monitoring

Emerging collective London governance and key priority programmes

This diagram shows the emerging governance for London transformation and enabling priority programmes, including the new Transformation Advisory Board. This replaces a more disparate and fragmented existing structure.



1. **Note**: There is a wider piece of work determining how the London Life Sciences strategy is delivered – the detail of this forum will be shaped by that work

Funding Agreement

The programmes are allocated £1m per annum each from 2027/28. In 2026/27 the pro-rata share for MH&M is £750k.

Programme	2026/27 £'000	2027/28 £'000
Million Hearts & Minds	750	1,000
Neighbourhood Health	-	1,000
Digital & Data	-	1,000
Total	750	3,000

ICB contributions are proposed on a capitation basis

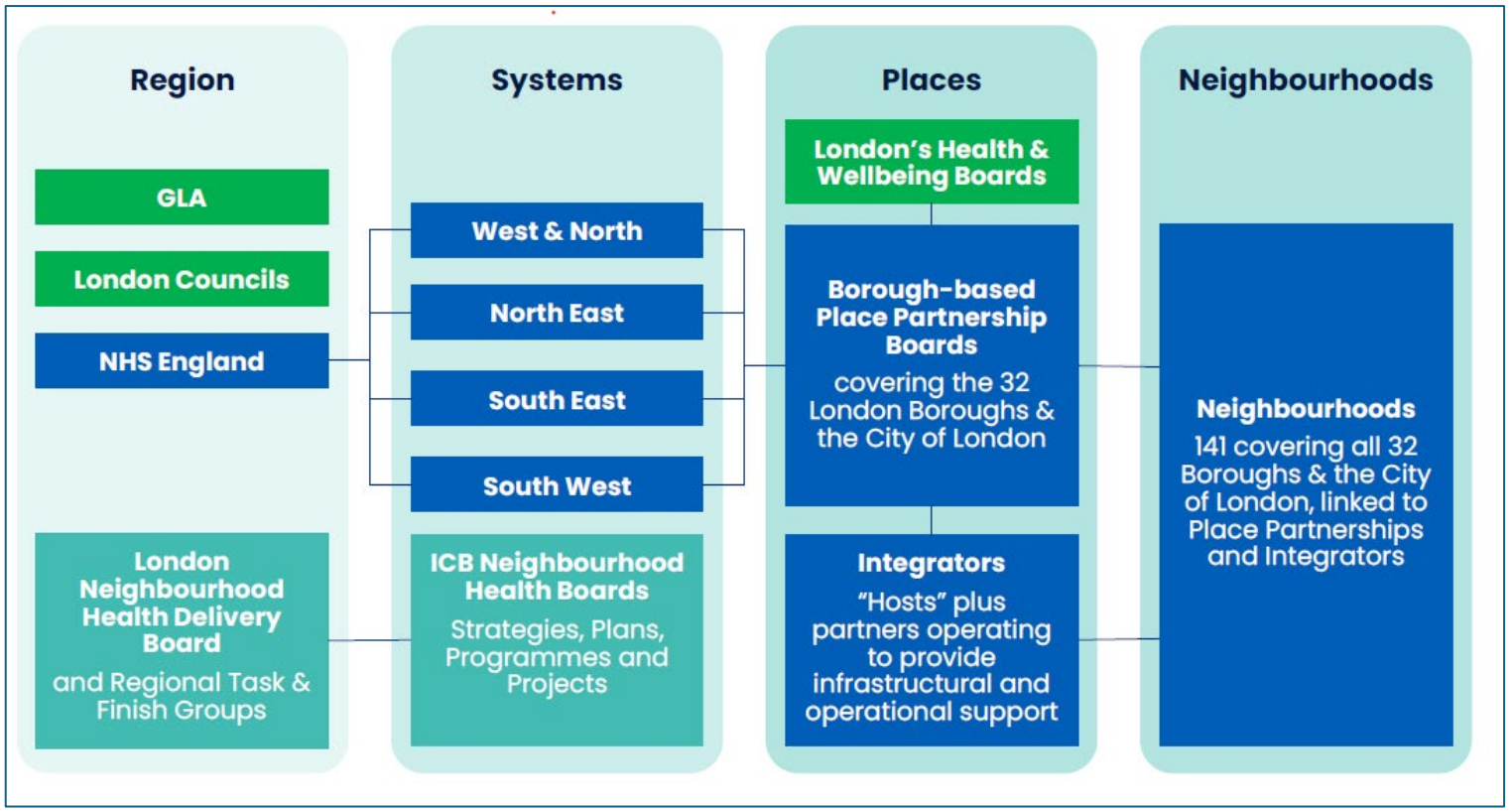
ICB	Pop'n Share	Contribution £'000	Contribution £'000
WNL	42%	318	1,270
NEL	25%	185	740
SWL	17%	125	500
SEL	16%	122	490
	100%	750	3,000

Other non-health partners have also confirmed their contributions as follows

- ADPH London - £275k
- GLA - £50k
- Novo Nordisk - £25k

Key priorities: neighbourhood health

Neighbourhood health development remains a core priority for London-wide working with established governance, programme and support arrangements



£160m+ committed investment

SIF and transformation funding identified to support neighbourhood health delivery

Integrator arrangements in all 32 boroughs

Infrastructure to enable delivery across organisational boundaries.

100% London coverage

141 neighbourhood footprints jointly agreed, based on natural communities, with data and analysis aligned with each neighbourhood

Population Health Management

across commissioners and providers, with shared modelling across primary and secondary care, and with LA and VCFSE partners

Governance in Place

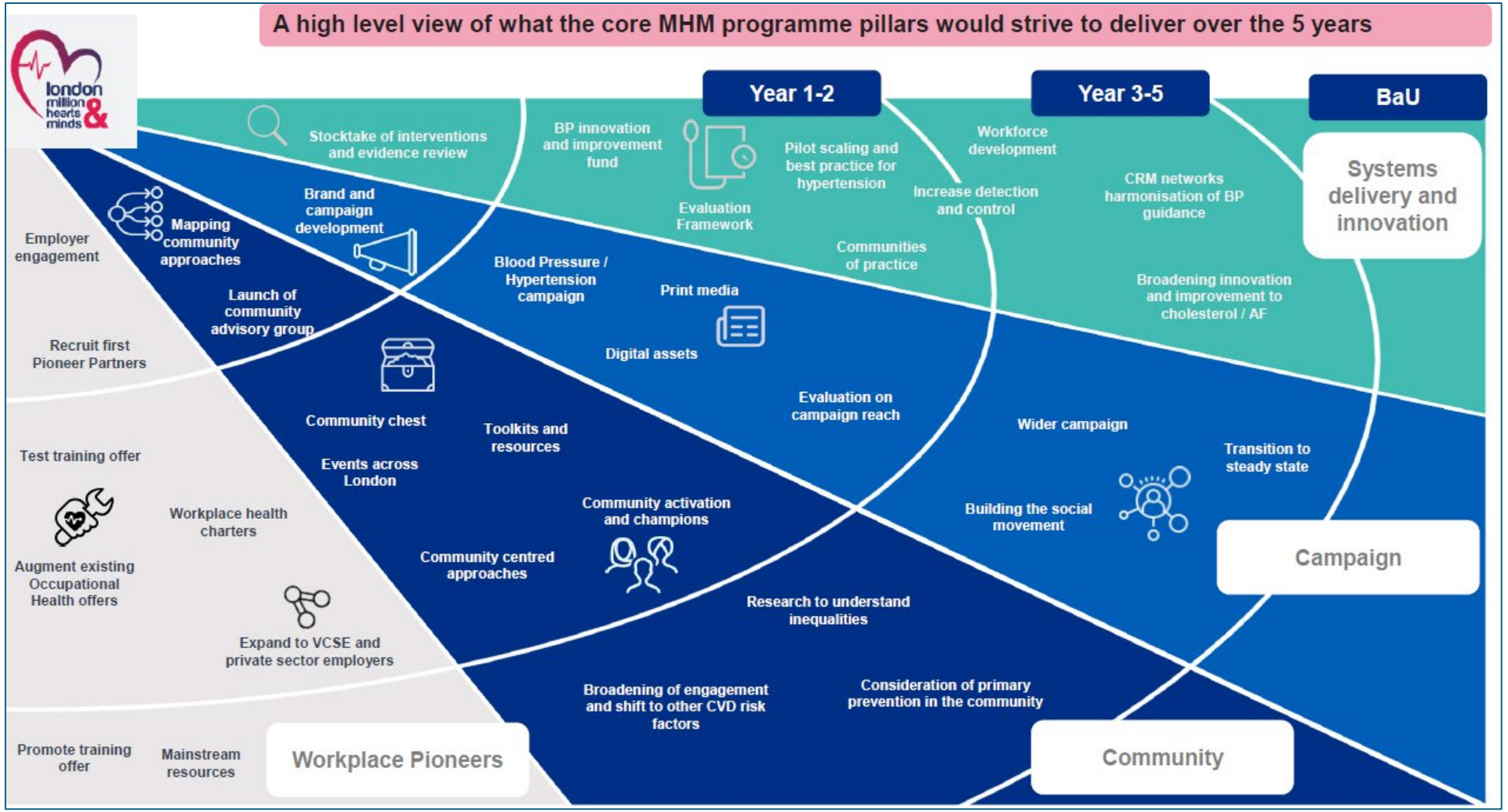
Place based boards covering all 32 London Boroughs and the City of London empowered to provide decision-making and accountable delivery

Pan London Enablers

Being jointly developed across organisations and sectors, including our neighbourhood workforce, digital and estates

Key priorities: Million Hearts and Minds

Million Hearts and Minds is the signature public health / health improvement programme for London, focusing on preventing cardiometabolic disease through a range of collective interventions



Key priorities: Digital access and the London Health Mission

The London digital landscape remains complex. Digital Access is a subset of the wider digital enabling programme, with close links to data improvement.

The regional transformation programme focuses on a range of related changes to improve digital access to services, including embedding the NHS App as the core digital access route...

... while the London Health Mission, led by the Mayor, is focused on using improved and fairer digital access as a means to reduce health inequalities...

Three domains of Digital Access

1 Access and self-management

NHS APP

- One trusted route into routine and planned NHS services
- Appointments, prescriptions, health records, messages and online consultations
- Access to provider portals and pathways through Wayfinder
- Greater ability for residents to manage and follow up their care

2 Assessment and navigation

INTELLIGENT NAVIGATION + NHS 111

- Digital assessment of a resident's needs
- Improved NHS 111 classification and routing
- Direction to the right service or pathway
- Greater use of digital routes where clinically appropriate

3 Digitally enabled urgent care

DIGITAL URGENT + EMERGENCY CARE

- Digital front doors and streamlined assessment
- AI-enabled triage and clinical decision support
- Digital transfer of information between services
- Technology supporting monitoring, patient flow and earlier intervention

Long-term goal:

Make London the healthiest global city, through tackling health inequalities and using London's unique assets.

Aim: LLG supportive of the rephrased mission

`By 2035 we will have significantly *closed the gap in health inequalities in our city, utilising London's innovation and digital transformation opportunities.*`

How do we get there:

- Focus on how the analogue to digital shift can reduce inequalities
- Improve digital inclusion and health literacy.
- Work with the emerging Neighbourhood Health Service to promote healthier communities.
- Identify areas where biggest opportunity to reduce inequality – recognising 'digital' can be interpreted in different ways.
- Ensuring we embed national priorities and take distinctly London action.

North East London
Integrated Care Board

NHS North East London ICB board

30 September 2026

Title of report	Neighbourhood health: progress, self-assessment learning and next steps
Author	Jo Frazer-Wise, Director of Neighbourhood Development
Presented by	Charlotte Pomery, Chief Strategic Commissioning Officer
Contact for further information	Jo Frazer-Wise, Director of Neighbourhood Development, jo.frazer-wise@nhs.net
Executive summary	This paper provides the ICB Board with a further update on neighbourhood health implementation across north east London. It builds on the report received in July including a focus on what we have learned from the first self-assessment against the national neighbourhood health framework. The assessment confirms that many of the building blocks for neighbourhood health are in place, while also showing where further work is needed across north east London and within individual Places. The findings highlight strong and consistent arrangements for general practice access. They also identify priorities around community waiting times, confirmation of integrator arrangements, clearer leadership accountability and continued development of neighbourhood delivery models. Significant progress has been made in reducing 52-week waits for adults, with further focus needed on children and young people.
Action / recommendation	The Board is asked to: • Note progress in the development of neighbourhood health across north east London; • Note the findings and learning from the first neighbourhood health self-assessment; • Support the proposed next phase of jointly owned Place and system development; and • Comment on the priorities identified for the period ahead.
Previous reporting	The paper 'Neighbourhood health: next steps' was reported to the ICB Board on 29 July 2026. Neighbourhood working is also reported through the Strategic Commissioning Committee and Commissioning Board.
Next steps/ onward reporting	Progress will continue to be reported to the Strategic Commissioning Committee, the ICB Board and through relevant system and Place governance. Neighbourhood Health Plans will also be developed with Place partners and approved through Health and Wellbeing Boards. We propose that detailed updates are prepared for Strategic Commissioning Committee, with the Board taking a higher level summary of developments.

Conflicts of interest	None identified.
Strategic fit	This paper aligns to all four Integrated Care System aims: • improving outcomes in population health and healthcare; • tackling inequalities in outcomes, experience and access; • enhancing productivity and value for money; and • supporting broader social and economic development.
Impact on local people, health inequalities and sustainability	Neighbourhood health is intended to improve access to joined-up, proactive and preventative support close to home. Using population health management and local insight should help target support to communities with the greatest unmet need. The next phase will strengthen community and partner involvement in locally owned plans.
Has an Equalities Impact Assessment been carried out?	An Equalities Impact Assessment has not been undertaken for this update and assurance paper. Assessments will be completed where required for individual commissioning or service change decisions arising from the programme.
Impact on finance, performance and quality	There are no additional resource implications arising directly from this report. The previous Board report noted that £4.8m per annum for three years had been identified from the Transformation Fund to support integrated neighbourhood working. Delivery, benefits and any related quality or performance implications will be tracked through programme governance.
Risks	Key risks are uneven pace between Places; delay in confirming integrator arrangements; gaps in shared evidence and accountability; community waits, particularly for children and young people; and emerging national guidance on Neighbourhood Health Plans and Better Care Fund reform.

1.0 Introduction and purpose

1.1 This paper provides the Board with a further update on neighbourhood health implementation across north east London. It builds on the reports received to date, which set out our shared vision, delivery arrangements, investment and governance for neighbourhood health.

2.0 Key messages

North east London has established the foundations for neighbourhood health, including agreed neighbourhood footprints, integrated neighbourhood delivery models, system working across all partnerships and supporting governance arrangements, including all partners (local government, Voluntary, Community, Faith and Social Enterprise Sector, the NHS across all aspects of health care and the independent sector).

The self-assessment confirms a strong and consistent position on general practice access across all seven Places. The next phase will focus on maintaining this position and demonstrating impact on take-up and delivery.

Community waiting times remain a shared challenge. Significant progress has been made in reducing 52-week waits for adults, with further focus required on children and young people.

Variation between Places reflects different stages of development and local context rather than a lack of commitment to neighbourhood working.

Currently Places are confirming integrator arrangements, and part of this work will be to align leadership accountability through the development of memoranda of understanding within Place partnerships

3.0 Progress since July

3.1 Each Place Partnership has engaged across the system to develop proposals for spend of their Neighbourhood Place Transformation Fund, to support the development of neighbourhood proactive care models focused on populations with the greatest complexity and need, typically the top 3 – 5% of our population across our geography. The proposals will come to the Proactive Care Commissioning Group at the end of September for approval and to ensure a focus on consistency and shared learning.

3.2 Each Place Partnership has confirmed how the ICB funding to strengthen GP clinical leadership into neighbourhood health in each Place is to be deployed. Clinical leads in some Places are already confirmed; other roles are being recruited to – the overall process should be completed in the next 4 – 6 weeks.

3.3 The ICB has invited each Place to name how their integrator function will be satisfied and to identify a provisional list of integrator organisations. Responses have now been received reflecting a strong desire at Place to continue to work together as system partners, bound by a Memorandum of Understanding. Further updates on identification of integrators will be reported to the next Board meeting.

3.4 North east London took part in a visit from Claire Fuller to London ICBs in July as part of a national programme of visits to every ICB in the country carried out by Claire Fuller and team. We have now received a letter and summary document from Claire Fuller and this is attached as Appendix 1.

3.4.1 As a follow up from the visit, all ICBs were asked to complete a self-assessment against the 10 Stage One requirements set out in the national Neighbourhood Health Framework, providing a developmental baseline by Place, helping to identify where there is a strong shared position, where local approaches differ and where further support is needed. While it provides an early view, it also gives us a useful starting point for understanding strengths, variation and where we need to focus next. It further enables some points of comparison with other systems both in London and more widely.

3.4.2 The self-assessment is already informing the next phase of neighbourhood health implementation:

- The assessment confirms that north east London has made progress in establishing the core foundations required for neighbourhood health.
- Place variation reflects different starting points, local contexts and operating models rather than differing levels of commitment.
- Several domains require a clearer north east London response, particularly organisational leadership, planned care, the primary and secondary care interface and data sharing.
- The exercise highlighted the need to confirm integrator arrangements and align leadership accountability more explicitly against the ten Stage 1 requirements.
- The greatest value of the exercise is not the score itself, but the shared understanding and improvement actions it creates across Places and partners.
- Future reviews should be undertaken jointly with partners to create a mature and collectively owned assessment of progress and priorities.

3.4.3 Taken together, the findings suggest that north east London has established many of the foundations required for neighbourhood health, but now needs to focus on demonstrating impact, strengthening accountability arrangements and using neighbourhoods as the delivery mechanism for broader system transformation.

3.4.4 The self-assessment also underlines the importance of sharing learning, adapting and testing, an approach embedded within neighbourhood health development in north east London.

3.5 Finally, it is proposed that the Strategic Commissioning Committee will in the future take detailed updates, with relevant escalation to the Board as required.

4.0 Next steps

4.1 The self-assessment has led to agreement on a series of next steps, as described in the table below:

Priority	Next step
Demonstrate impact	Move beyond evidence of arrangements to demonstrate impact and outcomes from general practice access and wider neighbourhood delivery.
Community waits	Sustain progress for adults and focus improvement on children and young people, in line with commissioning intentions.
Integrator arrangements and accountability	Confirm accountability arrangements and ensure existing leadership responsibilities are clearly aligned to the ten Stage 1 requirements. Supported through the development of a memorandum of understanding in each Place.
Place and neighbourhood plans	Use the assessment with partners to shape local and north east London plans, recognising that Neighbourhood Health Plan and revised Better Care Fund guidance are not yet available.
Care Closer to Home	Use the developing urgent response in the community model to build capacity and consistency, fully aligned to neighbourhoods.

Priority	Next step
Footprint alignment and equity	Complete Primary Care Network and neighbourhood alignment plans in Newham and Barking and Dagenham, and ensure an equitable approach across north east London.
System-wide domains	Agree clearer north east London positions on planned care, primary-secondary interface and data sharing, aligned with wider system and London work.

4.2 In addition, we are continuing to implement our existing plans through the Neighbourhood Transformation Fund for consistent proactive care delivery, additional clinical leadership and organisational development.

4.3 There are a number of risks and mitigations in delivering integrated neighbourhoods in north east London. A high-level summary, with mitigations, is set out below:

- Places are at different stages of development, which may lead to uneven progress. Mitigation: proportionate support, shared learning and clearer Place actions.
- Delay in confirming integrator arrangements could affect accountability and the pace of local development. Mitigation: prioritise confirmation in each Place and identify leads for all ten Stage 1 requirements through a shared memorandum of understanding.
- Community waits remain a shared challenge, particularly for children and young people. Mitigation: align improvement work with commissioning intentions, the community collaborative and neighbourhood delivery of proactive care.
- National guidance for Neighbourhood Health Plans and Better Care Fund reform is not yet available. Mitigation: keep plans under review, remain engaged with national and regional developments and manage any effect on existing arrangements carefully.
- The self-assessment may be interpreted as a ranking. Mitigation: continue to present it as a developmental baseline and focus discussion on learning (including from each other), outcomes and improvement.

5.0 Conclusion and recommendations

5.1 This paper reflects further progress in delivering integrated neighbourhoods. The self-assessment offers a useful developmental baseline for the next phase of implementation. It identifies common system issues and shows where local maturity and evidence vary.

5.2 The Board is asked to note the update and support the next phase of jointly owned Place and system development.

Jo Frazer-Wise
September 2026

ICB Chief Executive
ICB Chair

NHS England
Wellington House
133-155 Waterloo Road
London
SE1 8UG

15 September 2026

Dear Marie and Nnenna

Neighbourhood Health – North East London

Thank you to you and your team for meeting with us to discuss progress following the recent Neighbourhood Health Readiness Visit.

We appreciated the continued openness and constructive engagement from you and your colleagues. The follow-up discussion provided an opportunity to further review progress and consider the system's trajectory as neighbourhood health continues to develop.

There are a number of strong examples of good practice across North East London.
These include:

- Havering Liberty PCN launched its PCN Aligned Community Team (PACT) in July 2024 as a proactive neighbourhood-based frailty service designed to help older people age well at home. PACT brings together a multidisciplinary team of clinical and non-clinical practitioners with expertise in frailty, who provide comprehensive geriatric assessment, anticipatory care planning, optimisation of long-term conditions, and holistic identification of unmet health and wellbeing needs through home visits. In its first year, PACT demonstrated a 50% reduction in unplanned hospital admissions, a 33% reduction in emergency department attendances, and more than 50% fewer falls and delirium episodes among patients supported.

- St George's Health and Wellbeing Hub is a neighbourhood health centre that provides a true "one-stop shop" for integrated care through collaboration between system partners. Volunteers play a pivotal role in welcoming residents, providing practical support, connecting people with community services and helping to tackle loneliness and social isolation. The hub also features a community café and is embedding a creative health approach, recognising the value of arts, culture and community activities in improving health and wellbeing. The hub enables multidisciplinary teams to work seamlessly together, supporting people with complex needs, addressing the wider determinants of health, promoting prevention, reducing pressure on urgent and acute services, and improving the overall experience of care through an integrated neighbourhood model.

- The FIND (Focused Identification and Navigation of Disease) programme: More than 680,000 residents in NEL live with at least one long-term condition, with residents in NEL developing long-term conditions significantly earlier than national averages. The FIND

programme uses an innovative AI clinical support tool developed by Barts Health, which has the potential to disrupt the growth trend in LTCs by identifying people with CKD, Diabetes, Liver Disease and CVD earlier, and preventing or slowing disease progression and addressing health inequalities and embedding into neighbourhoods. A pilot across one Place has demonstrated strong outcomes including earlier intervention, more equitable identification of high risk groups, reduction in avoidable deterioration and release of specialist capacity by supporting people better in the community. Using non-recurrent transformation funding over three years, FIND will methodically roll out across all long term conditions and all Places in NEL, engaging with neighbourhood teams and complementing neighbourhood development.

- Point of Care lipid testing in Community Pharmacies: This is a community pharmacy-led cardiovascular disease prevention programme designed to reach people who may not engage with conventional NHS health checks, particularly residents of deprived communities and people from global majority ethnicities. This community based model working into neighbourhoods has secured two years of ICB funding and is expanding across London and nationally. The work has been shortlisted for an HSJ award - Bridging the Prevention Gap: Cardiovascular Risk Assessments in Community Pharmacies.

- Digitally Enabled Cardiovascular Care at Barts Heart Centre: Barts Heart Centre has developed a comprehensive digitally enabled cardiovascular programme spanning heart failure, acute coronary syndromes, inter-hospital transfer and patients awaiting major cardiac surgery at home in their neighbourhood. The outcomes have been published in leading journals including the European Heart Journal and BMJ's Open Heart, START-HF has been recognised in the national Cardiovascular Disease Modern Service Framework, and the models are now being taken forward for wider replication across the NHS.

Taking together the visit and follow-up process, alongside the subsequent system and Place level self – assessments, the accompanying single page summary provides an agreed reflection of the system's current position.

We would encourage you to share the agreed single page summary with your Board and system partners to support local oversight of neighbourhood health priorities, agreed next steps and future delivery.

As discussed, the key areas of focus over the coming months are to:

1. Use the development of our urgent response in the community model through Care Closer to Home to build greater capacity and consistency across northeast London, fully aligned to neighbourhoods.

2. Complete PCN alignment plans in Newham and Barking & Dagenham, and share Havering's approach to footprint alignment.

3. Strengthen evidence of integrated delivery and devolved decision-making in Tower Hamlets and Barking & Dagenham, and share Havering's approach.

4. Work across the Planned care team to confirm a NEL-wide position for elective pathways, recognising place variation.

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5. Use the shared NEL improvement approach to reduce variation and strengthen therapy, rehabilitation and specialist community pathways. Note that 52 week waits have largely been eliminated for adults but not for children and young people.

6. Maintain primary and secondary care interfaces as a key area of action, building on work with BHRUT, Barts and the Homerton. The Clinical Cabinet is taking a leading role in driving this work forward.

7. Make progress on confirming integrator arrangements in each Place. Ensure leads are identified for the 10 Stage 1 requirements. Mesh in new strategic arrangements.

8. Confirm the NEL-wide data-sharing position, aligned to work across London in this space.

Thank you again for the time and commitment shown by you and your teams throughout both the visit and follow-up process. We look forward to continuing to work with you as neighbourhood health develops across North East London.

Yours sincerely



Professor Claire Fuller MRCGP DRCOG MBBS
National Priority Programme Director – Neighbourhood Health

Annex:

Single slide summary

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NHS North East London ICB

14 September 2026

ICB at a glance

ICB CEO	Dr Nnenna Osuji (substantive March 2026)	**Local authorities**	4 of London's 6 most deprived boroughs; over 50% global majority; 26% of children in poverty.
ICB Chair	Dame Marie Gabriel CBE (unchanged through the 1 April 2026 reset, one of four nationally)	**Providers**	5 NHS trusts; BHRUT viability work with NHSE. 37 neighbourhoods; 48 PCNs, the highest density in London.
Registered population	2.4m+	**Self-reported**	ICB and most providers breaking even 2026/27; allocation 1.2% below target, a comparative reduction of £169m since 2019/20; £42m Transformation Fund 2026/27.
Places	7 Places co-terminus with HWBs (City and Hackney one Place with 2 HWBs).	**Pilots and designations**	NNHIP Wave 1: Barking and Dagenham only, from 7 of 7 Place submissions. NHC: pipeline submission 28 May 2026. Mackey J1-J4 SUBMITTED 14 May 2026.

Place level maturity score

Place (fill = maturity)	Tower Hamlets SCORE: 65	City and Hackney SCORE: 66	Barking and Dagenham SCORE: 60	Newham SCORE: 59	Havering SCORE: 66	Redbridge SCORE: 59	Waltham Forest SCORE: 59
Population	~340,000	~290,000	~233,000	~360,000	~265,000	~310,000	~280,000
HWB sign-off	YES (long-standing)	YES	YES (Cabinet 2025)	PARTIAL	YES	YES	YES
NbH readiness	4 NbHs, 7 PCNs Barnsley St NMHC, first 24/7 nationally (ELFT + Look Ahead + TH MH Alliance); THT Board is a HWB sub-committee with delegated authority; named-leader matrix per neighbourhood. 1 CDC Mile End; 14 FH. Acute CEO Is Place Lead.	8 NbHs, 8 PCNs Homerton dual acute and community mandate; Homerton CEO is Place Lead. Band 8C Head of INTs from Oct 2025; Proactive Care Service 73 QALYs, £1.6m benefit, 854 residents. 1 SH Homerton; 4 FH	4 NbHs, 6 PCNs NNHIP Wave 1 (£10m, 9 Sep 2025); Committees in Common with HWB since Jun 2023; AI dermatology triage 40-60% referral reduction. 1 CDC Barking;1 UTC Barking Community Hospital: 3 community hubs; PiP Impact £1.5m LA CEO is Place Lead.	8 NbHs, 9 PCNs Year of Care CVD early adopter; neighbourhood footprint aligned to local communities, and ASC and Public Health have been reshaped to neighbourhood footprint; 3PCNs not aligned to neighbourhood footprint . 2 SH; 4 FH; PiP Impact £1.5m. Acute CEO Place Lead.	5 NbHs, 5 PCNs LA CEO is also Place Executive Lead, ; St George's Hub HSJ shortlisted. 1 CDC St George's Hornchurch; PiP Phase 2 Harold Hill East £20m	4/5 NbHs, 6 PCNs HASS model since 2016, predating the framework by 9 years; Independence Model; Well House crisis alternative 182 residents/yr. 1 SH King George. LA CEO is also Place Lead	3 NbHs, 7 PCNs Coronation Square Leyton NHC opens early 2027; Care Home Proactive Pathway 141 admissions and 200 A&E attendances avoided, £331.9k net; HWB integrated with HCP. 1 SH Whipps Cross; 4 FH LA CEO and NELFT CEO joint Place Leads.
HLE (M/ F)	M 59.5 / F 59.2	M 59.3 / F 58.9	M 59.6 / F 59.0	M 62.8 / F 63.1	M 63.8 / F 63.3	M 63.5 / F 63.3	M 64.4 / F 64.5
GP federation /integrator	TH GP Care Group (32 practices)	C&H Integrated Primary Care	Together First CIC	Newham Health Collaborative	Havering Health Ltd	Healthbridge Direct	WF GP FedNet (36 practices)

Examples of practice surfaced across Places

Five Goals (RAG vs England; see Annex B for scores)

G1 Health outcomes (+0.9 above average)	G2 GP access and experience (-2.1 bottom)	G3 Planned care (-2.44 bottom)	G4 Urgent and emergency care (+0.81 above average)	G5 Patient and staff satisfaction (+0.28 above average)

Goals commentary: G3 (-2.44) and G2 (-2.10) both sit in the BOTTOM band and are jointly the dominant signal. G3 has moved from at-England on the May quartile basis, driven by long-wait volume: 52+ weeks 2,714, 65+ weeks 188, 78+ weeks 27. G2 is interaction-quality driven, not access driven: preferred HCP -3.12 is the single weakest metric while phone ease is -0.24. G1 carries the structural paradox: rank 1 of 42 on Type 2 diabetes care processes against rank 42 of 42 on Type 2 HbA1c control. Structured care is being delivered at scale without proportionate disease control.

Enablers (Section 4)

Enabler (fill = rating)	Key point
Data	Optum Pathfinder PHM across all 7 Places with Section 251 approval; NEL Discovery; Eclipse, Snowflake, Johns Hopkins frailty tool. Half of neighbourhoods have fully completed a PHM cycle ; all 37 targeted by early 2026/27. REGIONAL LAYER (London pre-read, June 2026): London Neighbourhood Impact Model built on the OneLondon SDE across 9m population and 5+ years of real-world data, offering one predictive model, one resource model, one evidence base and one platform. London Neighbourhood Explorer maps neighbourhood boundaries onto LSOA, enabling Census, ONS, IMD, QOF and Police data at neighbourhood level; note neighbourhoods are not a recognised geography in central government publications, which is why sub-Place data has been unavailable to date. Owned by the Delivery Board digital T&F group.
Workforce	£10.5m of the £42m fund to provider workforce; primary care growth trajectory 2%, 2.5%, 3% across three years. Around a third of GPs and practice nurses are 55+. No neighbourhood-model workforce plan. REGIONAL LAYER: five-tier London model of care agreed (Resident, Hyper-Local, Consistent, Tailored and Aligned Functions), co-developed in 2024/25 with clinical, managerial, local government, community and wider partners. VCSE sector c.44,000 organisations, c.388,000 employees, 2m+ volunteers, with the majority under £100k income. Owned by the people T&F group. No costed regional workforce plan. LOCAL ADOPTION is what the rating reflects.
Finance	£42m Transformation Fund 2026/27 allocated by area, plus £10m LTC, 19.8m elective and £20m mental health growth funding. Population up 266,000 since 2019/20 against an allocation moving from 2.5% above to 1.2% below target. REGIONAL LAYER: c.£160m+ committed across the four ICB funds per the June pre-read. London asks the centre for a 60% cash-release rule on avoided activity, gateway-based stranded-cost cover and year-of-care tariffs piloted in 2026/27. Owned by the commissioning and contracting T&F group.
Digital	London Care Record and London SDE; 111 recommissioned with AI navigation, go-live 1 Aug 2026, projected 43,260 AI-diverted calls in year one; ambient voice technology rolling out; Barts digital outpatient letters c.50k/week. REGIONAL LAYER: London SDE data sharing agreement signed by 1,097 of 1,131 GP practices with 1,013 flowing data at end Feb 2026. London Care Record 29m views in 2025, up 22%, valued at £56m in time saved. 85,000+ Universal Care Plans, including 11,000+ frailty and dementia and 1,000+ LD and autism since January. eMHA via Thalamos used 225,000 times in 2025 at c.1,700 patients a month.
Estates	More than 40 centres with co-located services; capital plan targets NHC coverage across all 37 neighbourhoods. NHC pipeline submitted 28 May 2026; NEL absent from NHC Wave 1 first-27. REGIONAL LAYER: estates assets collated per ICB covering primary, acute, community, void space and selected LA assets, overlaid on weighted Core20, deprivation, age and population mapping, in an interactive tool alongside Strata to which ICB estates leads hold access. A functional requirements framework for neighbourhood hubs is mapped to NHSE archetypes with investment principles attached. Owned by the estates T&F group. NB Centre Manor Park in Newham has been named as an NHC Wave 1 site and is in active development from a physical and care model perspective.

Framework readiness: Stage 1 (foundations) and Stage 2 (NbH Plan)

Ref	Stage 1 minimum requirement (2026/27): Ref fill shows rating	Self-assessed score
S1	Reduce non-elective admissions via UCR and reablement	6.7
S2	Reduce variation; improve access to general practice	10
S3	Neighbourhood footprints agreed	6.4
S4	INTs for high-priority cohorts; devolved budgets	7.5
S5	Neighbourhood approach to elective pathways	5.8
S6	18-week community waits; eliminate 52-week waits	4
S7	BCF pooled funding plans confirmed with LAs	7
S8	Primary / secondary care interface (RTC)	5.5
S9	Organisational ownership of planned deliverables	2.8
S10	Data-sharing arrangements for patient identification	6
Stage 1 overall:		**62**

Ref	Stage 2 NbH Plan step (2027/28): Ref fill shows rating
1	National objectives via three reform agendas
2	Support wider local goals and public service reform
3	Local objectives informed by JSNA
4	Confirm final geographies
5	Confirm organisational delivery (Integrator: KLOE 3)
6	Confirm governance and operations (Accountability: KLOE 1)
7	Align other initiatives (Family Hubs; Pride in Place; MH hubs; employment)
Stage 2 overall	

Forward look - Key actions over next 6 months:

1. Use the development of our urgent response in the community model through Care Closer to Home to build greater capacity and consistency across northeast London, fully aligned to neighbourhoods.
2. Complete PCN alignment plans in Newham and Barking & Dagenham, and share Havering's approach to footprint alignment.
3. Strengthen evidence of integrated delivery and devolved decision-making in Tower Hamlets and Barking & Dagenham, and share Havering's approach.
4. Work across the Planned care team to confirm a NEL-wide position for elective pathways, recognising place variation.
5. Use the shared NEL improvement approach to reduce variation and strengthen therapy, rehabilitation and specialist community pathways. Note that 52 week waits have largely been eliminated for adults but not for children and young people.
6. Maintain primary and secondary care interfaces as a key area of action, building on work with BHRUT, Barts and the Homerton. The Clinical Cabinet is taking a leading role in driving this work forward.
7. Make progress on confirming integrator arrangements in each Place. Ensure leads are identified for the 10 Stage 1 requirements. Mesh in new strategic arrangements.
8. Confirm the NEL-wide data-sharing position, aligned to work across London in this space.

Forward look – Key actions over next 18 months

1. Deliver our urgent response in the community model through integrated neighbourhood models.
2. Continue to build the NEL-wide position for elective pathways in neighbourhoods, recognising place variation.
3. Continue to maintain primary and secondary care interfaces as a key area of action. The Clinical Cabinet is taking a leading role in driving this work forward.
4. Focus on developing our approach for rising need, working through our segmentation model to focus on those in lower need segments
5. Ensure connectivity of the various public sector reforms in play currently, including for children and young people (SEND) and mental health.
6. Commission core offer for community services, to reduce variation and strengthen therapy, rehabilitation and specialist community pathways.

ICB anticipate working through the priorities identified for the next six months into the future period, given the complexity of some of the issues raised.

North East London
Integrated Care Board

NHS North East London ICB board

30 September 2026

Title of report	Financial Overview - Month 5 2026-27
Author	Robert Adcock, Director of Finance
Presented by	Henry Black, Chief Finance Officer
Contact for further information	henryblack@nhs.net
Executive summary	**Key Items:** • The finance report provides a dashboard overview of the ICB's core financial performance at Month 5. • At Month 5 the ICB is reporting a broadly breakeven position, with a small year to date surplus of £49k and expenditure £1.0m favourable to plan. The year end forecast remains at breakeven. • Emerging pressures and risks include weight management services, Autism Spectrum Disorder (ASD) and Attention-Deficit/Hyperactivity Disorder (ADHD), Right to Choose activity, prescribing costs, Section 117 expenditure, continuing healthcare (CHC) savings delivery and wider contractual exposure. • Efficiency delivery at Month 5 is £26.7m against a £27.5m phased plan, with a £0.8m adverse variance. The full year forecast remains £80.0m, in line with plan. • The capital programme is forecast to fully utilise the £14.6m programme allocation, although scheme approvals and mobilisation continue to require close management. • The wider ICS position remains challenged and has reported a year to date £29.1m adverse variance.
Action required	The Board is asked to note the Month 5 financial position and associated risks.
Previous reporting	Finance reports are provided to each Finance and Investment Committee and Board meetings.
Next steps/ onward reporting	Future reports will continue to be shared with the Finance and Investment Committee and the Board.
Conflicts of interest	No conflicts of interest have been identified.
Strategic fit	The report provides an update on delivery against the 2026/27 financial plan and supports oversight of financial sustainability across north east London.
Impact on local people, health inequalities and sustainability	• The financial position reflects investment in key services, core financial performance and the delivery of efficiencies supporting delivery of improved population health outcomes and reduction in health inequalities.

Impact on finance, performance and quality	• Delivery of the financial plan, control total and efficiency programme remains critical, alongside maintaining performance against key constitutional standards.
Risks	• The ICB faces a range of financial risks, including prescribing volatility, ADHD/ASD pathways, weight management services, CHC, Section 117 expenditure and wider contractual pressures. • The base case includes £14.0m of identified risks matched by £14.0m of mitigating actions, although the risk assessment indicates a potential downside of £18.0m and upside of up to £4.0m. • The forecast position remains sensitive and requires continued focus on demand management, contract management, prescribing trends, cost improvement plan (CIP) delivery and use of agreed mitigations where required.

1.0 Purpose of the report

The purpose of the report is to update the Board on the Month 5 2026/27 financial position, provide an assessment of the underlying position, risks and mitigations facing the ICB.

2.0 Month 5 finance overview

The Month 5 financial overview is included within the attached report.

NHS
North East London

Month 5 2026-27 Finance Overview

Meeting name: North East London ICB Board Meeting

Presenter: Henry Black

Date: 30 September 2026

NEL ICB - Financial Summary at Month 5 (August 2026)

ICB Financial Position Summary

Financial Position

The ICB is reporting a broadly breakeven position at Month 5, with a small year to date surplus of £49k which is £1m favourable variance to plan. The year end forecast remains breakeven, with run rate pressures in acute and mental health services being mitigated through a range of actions.

Key Risks and Mitigations

While the forecast remains in line with plan, a range of risks have been identified. Emerging pressures include weight management services, ADHD/ASD Right to Choose activity, prescribing costs, Section 117 expenditure, and delivery of CHC savings. The current assessment assumes these risks can be effectively managed within the overall financial position through a combination of planned investment, contractual mechanisms, delivery of CIP programmes, and the use of non-recurrent mitigation measures where appropriate.

Underlying Position and Savings

The ICB has strengthened its underlying financial position over recent years, moving from a material deficit to a balanced position. CIP delivery is currently forecast in line with plan, although some risks remain, particularly in relation to CHC savings, which will require continued control over key cost drivers.

Capital and Cash Position

The capital programme remains within available funding and is forecast to fully utilise the £14.6m programme allocation, although scheme approvals and mobilisation continue to require close management. Cash drawdown remains in line with plan with no immediate liquidity concerns.

Mental Health Investment Standard (MHIS)

The ICB is forecasting to invest £10m additional resources compared to the MHIS target. This relates to the additional MH investment agreed through the operating plan process. Appendix 1 includes a summary of the MHIS position.

ICB Financial Duties

Duty	RAG	Position
Maintain expenditure within the revenue resource limit	A	Although the ICB is reporting in line with plan, the plan contains some risk and as such an Amber risk rating is considered appropriate.
Ensure running costs are within the running cost resource limit.	G	Running costs are currently forecast to be within budget, we are delivering savings from the nationally mandated reductions and transition costs are within the current funded envelope.
Maintain capital expenditure within the delegated limit	G	The capital programme is £14.6m, we currently do not anticipate any major risks around spending to this level.
Maintain expenditure within the allocated cash limit	G	The ICB continues to maintain expenditure within its allocated cash limit, demonstrating effective financial control and delivery against
Ensure compliance with the better payment practice code	G	In month performance has detreated due to the backlog of ADHD invoices, however the ytd performance is still meeting target.

ICS Financial Position Summary

- The NEL system is reporting a £29.1m adverse variance year to date, with a £60.4m deficit against a planned deficit of £31.3m. NEL accounts for 65% of London's £45.4m adverse variance.
- The variance is driven by providers (£30m adverse), partly offset by the ICB's £1.0m favourable position.
- Barts Health (£16.1m adverse) and BHRUT (£14.2m adverse) account for the majority of the system variance, reflecting ongoing cost pressures and delivery risks.
- ELFT (£0.7m favourable) and NELFT (£0.0m favourable) remain broadly in line with plan, while Homerton (£0.4m adverse) reports a small adverse variance.
- All organisations continue to forecast delivery of their year-end plans, subject to delivery of savings, expenditure controls and mitigation of operational and demand-led pressures.

NEL ICB - Financial Summary at Month 5 (August 2026)

YTD surplus
£49k
small surplus at M5

YTD variance
£1m fav.
£2,645.5m vs £2,647.5m

Forecast
Breakeven
in line with plan

YTD Efficiency
£26.7m against £27.5m
£0.8m adverse variance

Forecast Efficiency
£80m
In line with Plan

	YTD Budget £m	YTD Actual £m	Variance £m	FY Budget £m	Forecast £m	Variance £m	Forecast as % of overall spend %	RAG
Acute	1,176.6	1,175.9	(0.7)	2,810.2	2,816.5	6.3	44%	
Mental Health & LD	247.4	249.7	2.3	593.9	599.3	5.5	9%	
Community Health Services	239.9	240.3	0.4	574.4	575.2	0.8	9%	
Continuing Care	89.5	89.6	0.2	213.0	213.1	0.0	3%	
Other Programme	27.6	20.6	(7.0)	73.2	50.2	(23.0)	1%	
Prescribing	135.9	140.0	4.1	326.2	336.9	10.7	5%	
Primary Care Services	26.8	26.6	(0.3)	64.5	64.1	(0.4)	1%	
Primary Care Co-Commissioning	211.6	211.7	0.0	507.9	508.0	0.1	8%	
Dental, Opthalmic and Pharmacy	115.0	115.0	0.0	276.0	276.0	0.0	4%	
Specialised Commissioning	341.3	341.3	(0.0)	819.0	819.0	0.0	13%	
Programme Corporate	21.3	21.3	0.0	51.0	51.0	0.0	1%	
Running Costs	14.6	14.6	(0.0)	34.9	34.9	(0.0)	1%	
Total Expenditure	2,647.5	2,646.5	(0.9)	6,344.3	6,344.3	(0.0)		
Revenue Resource Limit Total	(2,646.6)	(2,646.6)	0.0	(6,344.3)	(6,344.3)	0.0		
In Year (Surplus) / Deficit	0.9	(0.0)	(0.9)	0.0	0.0	0.0		

Programme Performance and forecast

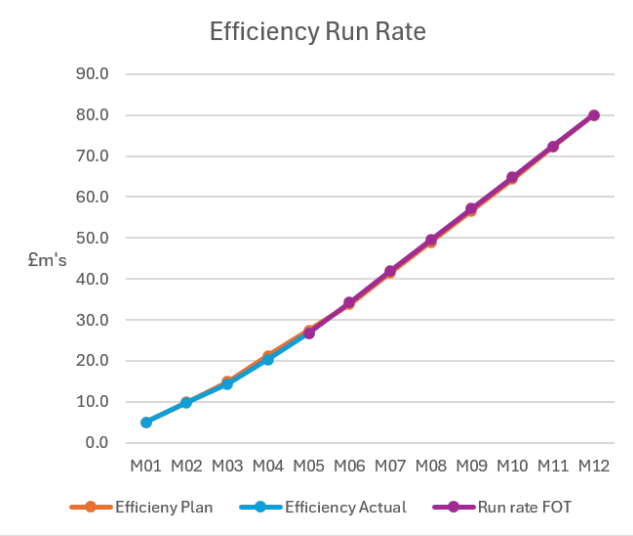
- The table to the left summarises the financial position by functional area.
- Month 5 spend is £2,646.5m against plan of £2,647.5m, giving a £1m favorable variance and a breakeven forecast.
- Acute services are reporting a £0.7m favourable variance year to date; however, a £6.3m forecast pressure remains, driven mainly by weight management and contractual cost and volume risks.
- Mental health services are reporting a £2.3m adverse variance year to date, driven by ADHD/ASD and Section 117 pressures; the full-year forecast pressure is £5.5m.
- Primary care services are broadly in line with plan; however, prescribing is reporting a £4.1m adverse variance year to date and a forecast pressure of £10.7m.
- Other programme areas are reporting a £7m favourable variance year to date and a £23m favourable forecast variance, reflecting planned mitigations and investment slippage available to offset underlying pressures.

Key cost drivers and sensitivities

- Demand-led pressures remain the principal area of financial risk, particularly across weight management, ADHD/ASD pathways, prescribing, CHC and Section 117 expenditure.
- Prescribing is reporting a £10.7m forecast pressure and remains highly sensitive to national price concessions, Category M changes and demand growth.
- Cost and volume contractual exposure remains across acute and community arrangements, including associates.
- CHC is currently forecasting breakeven but remains exposed to package price and volume changes, with delivery dependent on continued package reviews, cost management actions and CIP delivery.
- Corporate and running costs are forecast to break even; however, risks remain around restructure delivery, CHC agency costs, legal fees and void costs, which are currently being mitigated within existing resources.

NEL ICB - Financial Summary CIP Saving at Month 5 (August 2026)

Commissioning Group	Month 5 YTD			Forecast			High Risk	Medium Risk	Low Risk
	Plan £'000	Actual £'000	Variance £'000	Plan £'000	Actual £'000	Variance £'000			
Continuing Health Care (CHC)	625	459	-166	3,225	3,225	0	396	642	2,187
Prescribing	2,792	2,792	0	6,700	6,701	1	0	1,825	4,876
Acute Planned Care	2,325	1,852	-473	10,178	10,178	-0	1,061	4,976	4,141
Proactive Care	986	980	-7	6,069	6,069	0	4,367	634	1,068
UEC Care closer to home	0	0	0	2,584	2,584	-1	2,584	0	0
Primary Care	62	62	0	205	205	0	0	60	145
Corporate	20,697	20,534	-164	51,039	51,039	-0	3,500	539	47,000
Total	**27,488**	**26,679**	**-809**	**80,000**	**80,000**	**-0**	**11,908**	**8,675**	**59,417**



Savings delivery

- £26.7m of efficiency savings have been delivered year to date against a £27.5m phased plan, resulting in £0.8m slippage. This is expected to be recovered during the latter part of the year.
- 58% of the £80.0m full-year efficiency target is phased into Q3 and Q4, reflecting the planned delivery profile for the programme.
- The current efficiency run rate forecasts delivery of the full-year target, with performance expected to improve through the second half of the year.
- The CIP risk profile remains strong, with 74% of schemes rated low risk, broadly consistent with the previous month.
- Ongoing oversight, recovery plans and additional pipeline schemes remain in place through the Commissioning Groups to support delivery of the full-year target.

Savings detail and residual risk

- CHC is £166k behind the year to date operating plan target, mainly driven by the complex case programme. A review of the programme has been completed and the forecast remains break-even.
- Prescribing savings remain on track, with available data showing delivery of the target to date and early indications that the full-year CIP target will be achieved.
- Primary Care savings have been achieved through internal service reviews and the successful procurement of the information facilitation service at a lower cost.
- Acute Planned Care is reporting £473k of slippage against the year to date target due to procurement delays. The associated savings have been deferred to 2027/28 and are expected to be offset through other mitigations.
- Proactive Care remains the most significant delivery risk, with savings linked to Care Closer to Home and End of Life programmes. Planned savings from weight management prescribing are no longer expected to materialise and End of Life savings have slipped into Q4.

NEL ICB - Financial Summary at Month 5 (August 2026)

Underlying Forecast	Base risks	Base Mitigations
£200k	£14m	£14m
in line with plan	In line with plan	In line with plan

Underlying position

- The ICB has significantly strengthened its underlying financial position over the past two years, moving from a material underlying deficit to a broadly balanced position.
- The underlying position reflects the full-year impact of recurrent and non-recurrent investments, together with the ongoing cost of organisational change and service transformation programmes.
- Key financial pressures include £23.0m of investment commitments which support delivery of strategic and operational priorities.
- These pressures are substantially offset by £14.2m of recurring efficiency savings and other underlying improvements, resulting in a forecast underlying position of £0.2m.
- Delivery of a balanced underlying position remains dependent on continued control of demand-led expenditure, particularly prescribing, CHC and acute activity.
- The three-year financial plan assumes a balanced position throughout the planning period and will require sustained delivery of efficiencies, robust financial management and ongoing scrutiny of emerging cost pressures.

Risk and mitigations

- The ICB continues to forecast a breakeven position. The base case includes £14.0m of identified risks matched by £14.0m of mitigating actions, although the risk assessment indicates a potential downside of £18.0m and upside of up to £4.0m.
- The principal risk areas are prescribing, ADHD/ASD pathways, weight management, CHC and wider contractual pressures.
- Investment plans within the 2026/27 budget are expected to help offset run-rate pressures and support transformational change.
- The ICB is reviewing available non-recurrent mitigations and contractual levers to manage cost and volume activity. Mitigations include investment slippage, balance sheet opportunities and efficiency stretch where required.
- Targeted review is underway of weight management and ADHD/ASD investment to confirm the impact on underlying pressures and inform future financial plans.
- Continued focus on demand management, contract management, prescribing trends and CIP delivery is required to maintain financial balance. Further updates will be provided as data quality improves, particularly in relation to prescribing and demand-led activity.

Appendix 1 ICS Financial Summary and Mental Health Investment Standard

Organisation	YTD Plan £m	YTD Actual £m	Variance £m	FY Plan £m	Forecast £m	Variance £m
BHRUT	22.0	36.2	(14.2)	0.0	0.0	0.0
Barts Health	5.9	22.1	(16.1)	0.0	0.0	0.0
East London NHSFT	0.8	0.2	0.7	0.0	0.0	0.0
Homerton	1.6	2.0	(0.4)	0.0	0.0	0.0
NELFT	(0.0)	(0.0)	0.0	0.0	0.0	0.0
Total NEL Providers	30.4	60.4	(30.0)	0.0	0.0	0.0
NEL ICB	0.9	(0.0)	0.9	0.0	0.0	0.0
NEL System Total	31.3	60.4	(29.1)	0.0	0.0	0.0

Mental Health Investment Standard	FY Plan £m	Forecast £m	Variance £m	Target spend £m	Excess / Shortfall in MHIS Delivery £m	Variance %
Mental Health Investment Standard	509.1	522.7	(13.6)	515.1	7.6	1.5%
Specialised Commissioning Mental Health Investment Standard	92.8	94.9	(2.1)	92.8	2.1	2.3%

Appendix 2 – Left Shift

Area	2025/26 £m	2026/27 £m	Change £'m	Change %
Acute	2,661.3	2,655.2	-6.1	-0.2%
Acute Independent Sector	109.9	104.9	-5.1	-4.6%
Mental Health & LD	537.4	577.6	40.2	7.5%
Community Health Services	525.8	573.4	47.6	9.1%
Continuing Care	210.0	213.0	3.0	1.4%
Other Programme	57.9	58.3	0.4	0.7%
Prescribing	321.3	315.2	-6.1	-1.9%
Primary Care (Delegated & Mainstream)	540.2	560.8	20.6	3.8%
Dental, Opthalmic and Pharmacy	242.6	261.1	18.4	7.6%
Specialised Commissioning	747.0	819.0	72.0	9.6%
Corporate (Programme & Admin)	117.2	82.4	-34.8	-29.7%
Total	**6,070.8**	**6,221.0**	**150.2**	**2.5%**

- The table above is intended to provide an illustrative comparison of the ICB's planned expenditure between 2025/26 and 2026/27, highlighting the shift in investment across commissioning areas in support of the strategic objective of delivering more care closer to home and reducing reliance on acute services
- To enable a like-for-like comparison, the 2025/26 figures reflect opening budgets only and exclude in-year non-recurrent allocations, funding adjustments and other budget movements received during the financial year. The comparison should therefore be viewed as directional rather than a direct reconciliation between years.
- The largest areas of growth are within Specialised Commissioning (+£72.0m), Community Health Services (+£47.6m) and Mental Health & Learning Disabilities (+£40.2m), reflecting increased investment in community, mental health and specialist services.
- This investment has been partially offset by reductions in Acute Services (-£6.1m), Acute Independent Sector (-£5.1m), Prescribing (-£6.1m) and Corporate budgets (-£34.8m), demonstrating the planned redistribution of resources towards out-of-hospital and preventative models of care.
- The values are presented for illustrative purposes only and it should not be attempted to reconcile the movements directly back to current 2026/27 budgets. Both years have been subject to numerous in-year adjustments, allocations, transfers and non-recurrent funding changes, meaning any direct reconciliation would not provide a meaningful measure of the underlying strategic investment shifts being illustrated.

North East London
Integrated Care Board

NHS North East London ICB board

30 September 2026

Title of report	Provider quality and performance report
Author	ICB Performance Improvement Team ICB Quality and Safety Team
Presented by	Ralph Coulbeck, Chief Strategy Officer Dr Paul Gilluley. Chief Clinical and Quality Commissioning Officer
Contact for further information	Helen Pace; Head of Performance; helen.pace@nhs.net Paul Isaacs; Senior Performance Manager; p.isaacs@nhs.net Polly Pascoe; Head of Quality and Safety; polly.pascoe1@nhs.net
Executive summary	**Integration of performance and quality reporting** In line with the evolution of reporting to support the strategic commissioning focus of the ICB, the Board is presented for the first time with a single integrated performance and quality report. The revised performance report (received positively by the executive management team (EMT) and the Board previously) has been combined with elements of the system quality report to provide an Integrated Performance and Quality (IPAQ) report. The IPAQ report is aligned to National Oversight Framework (NOF) domains – access, experience of care, effectiveness of care and population health, and patient safety – and will serve as a single 'golden thread' presented to EMT, Quality, Safety and Improvement Committee, and the Board. The ICB Performance Improvement team and Quality and Safety team welcome feedback. **Performance and quality overview** • Performance data presented is July 26 (M4). For Urgent and Emergency Care (UEC) August 26 (M5) data is available. **Access** • Continued delivery of referral to treatment (RTT) waiting list size, positioning the system positively going into Quarter 2. • RTT 18-week and more than (>)52-week performance has improved overall; however, recent month-on-month trends and widening gap from plan indicate an increasing level of risk. • North East London ICB diagnostics (DM01) performance is improving, equating to an 8%-point improvement from April 26. Despite this progress, further improvement is required to close the gap to

	plan and waiting times for some tests remain comparatively high. • Performance against all three cancer measures (faster diagnosis standard (FDS), 62-day and 31-day) is improving overall, 62-day performance marginally below plan for the month, driven by Barking, Havering and Redbridge University Hospitals Trust (BHRUT). BHRUT is expected to return to compliance for September 26 reporting. • The published position against the 4-hour emergency department (ED) standard in August was 79%, Barts Health being the most challenged Trust based on distance from plan. • Urgent and emergency Care (UEC) attendances were down in August in line with seasonality but remain above planned levels (Barts Health and Homerton Healthcare) and compared to the same period last year (all three north east London acute Trusts). • The number of type 1 and 2 patients admitted, transferred, or discharged within 12-hours of arrival remains stable (89%). • General practice appointment activity continues to perform strongly. In July, practices delivered 1.17 million appointments, 3.2% above plan and higher compared to the same period in 2025. Activity was above the March 27 target benchmark, indicating sustained growth in access to primary care services • The longest community health services waiting times are within autism spectrum disorder (ASD) services for children and young people. An ASD Improvement Programme to support a coordinated approach to reducing waits, improving access, and enhancing the overall quality of ASD services across the system is being developed. • Individual placement support remains below plan across all places but is showing an improving trend. Talking therapies completed treatments and average length of stay for adults and Psychiatric Intensive Care Unit (PICU) is above plan. Average length of stay for older adults remains below plan. **Experience of care** • Friends and Family Test (FFT) scores for A&E have shown a downward trend throughout this financial year, with negative special cause variation in June and July of 2026, where we would normally see positive scores returning following winter pressures. • Acute inpatient FFT scores have returned to the mean following special cause variation in May. The Care Quality Commission (CQC) Inpatient Survey was published in August 2026, with worse than expected experience reported within BHRUT.

	• North East London ICB fell below plan in July for talking therapies reliable improvement but exceeded plan for reliable recovery. **Effectiveness of care and population health** • BHRUT's Summary Hospital-level Mortality Indicator (SHMI) has shown a steep decrease since November 2025, this is likely due to a quality data issue. • While Barts Health SHMI is within expected range, negative special cause variation continues to be seen. **Patient safety** • While numbers of methicillin-resistant Staphylococcus aureus (MRSA) infections remain within expected limits, and below the mean for the past five months, numbers are regularly above the threshold of 0. Model Health System benchmarking places North East London ICB as the ICB with the highest MRSA infection rates in London and second highest rates nationally. • While numbers of healthcare-associated E.coli infections remain within expected limits, Model Health System shows north east London has the highest rates in London and third highest nationally. • Numbers of clostridioides difficile (C.diff) infections have sat below the mean between March and June 2026, following reduced control throughout 2025, however this has returned to the mean in July 2026. As of June 2026, Model Health System shows north east London has the lowest C.diff infection rate nationally. • The rate of grade 3 and 4 tears has sat above the mean for the past six months, with special cause variation seen in March 2026; if this is sustained, there we will see a negative step change in performance. • The rate of postpartum haemorrhage (PPH) of 1500ml or more continues to sit above the mean with special cause variation seen in February and March 2026. **Other key updates** • On 10 September, NHS England published the Q1 2026/27 oversight segment scores for providers in line with the national NHS Oversight Framework for 2026/27. The 2026/27 framework uses updated metrics and methodology. Any comparison with segmentation results from 2025/26 must be considered in this context. • Barts Health, BHRUT, and Homerton Healthcare are all in segment 3 (below average and / or financial deficit) for Q1, financial deficit applies. East London NHS Foundation Trust (ELFT) and North East London

	NHS Foundation Trust (NELFT) are in segment 2 (above average), financial deficit does not apply. London Ambulance Service is also placed in segment 2, financial deficit does not apply. • Formal publication of ICB segmentation for Q1 is awaited.
Key areas of improvement / concern	1. Diagnostics (DM01) performance is improving following electronic patient record implementation at BHRUT and reporting changes at Barts Health; however, further improvement is required to close the gap to plan and some diagnostic tests continue to experience significantly longer waits than planned. 2. Community health service 18-week performance and >52-week waits are improving; the longest waits continue to be within children and young people's ASD services. 3. General practice appointment activity remains strong, supporting improved access to primary care services. 4. As of June 2026, Model Health System shows north east London has the lowest C.diff infection rate nationally. 5. FFT scores for A&E have shown a downward trend throughout this financial year, with negative special cause variation in June and July of 2026, where we would normally see positive scores returning following winter pressures.
Action / recommendation	The Board is asked to note the significant programme of work underway to evolve reporting. The ICB performance improvement team and quality and safety team welcomes feedback. All performance related queries may be raised with the ICB performance improvement team. All quality and safety queries may be raised with the quality and safety team.
Previous reporting	Each of the performance domains has associated system wide improvement activity managed through ICB programmes, Commissioning Boards, and governance.
Next steps/ onward reporting	The report is presented at EMT, Quality, Safety and Improvement Committee and ICB Board.
Conflicts of interest	No known conflicts of interest
Strategic fit	This report aligns with the following ICS aims: • To improve outcomes in population health and healthcare

	• To tackle inequalities in outcomes, experience, and access • To enhance productivity and value for money • To support broader social and economic development
Impact on local people, health inequalities and sustainability	Improving access to healthcare and the speed of treatment is likely to benefit disadvantaged groups among residents, as well improve performance, quality, equity of access and reduction of health inequalities for the NEL population.
Has an Equalities Impact Assessment been carried out?	
Impact on finance, performance and quality	Balance between delivery of the required financial position versus delivery of performance, quality, and activity requirements.
Risks	The risks and issues are described against the relevant performance and quality domains.

NHS
North East London

Integrated Performance and Quality Report

September 2026

Introduction

This report presents the first Integrated Performance and Quality Report presented to the NHS North East London Board. It is an important step towards a more joined-up approach, providing a foundation on which future iterations can be refined and enhanced.

Aligned to NHS Oversight Framework (NOF) domains, Access, Experience of Care, Effectiveness of Care and Population Health, and Patient Safety the report is intended to identify areas of performance by exception, highlight emerging risks and variation, and provide assurance regarding the actions being taken to improve outcomes, reduce health inequalities and support sustainable service delivery.

Through regular review of this information, the Integrated Care Board can fulfil its role as strategic commissioner, alongside statutory responsibilities for quality improvement, system performance and stewardship of NHS resources, while maintaining a clear focus on delivering better outcomes and experiences for local people.

Contents

Access

- Elective RTT (18-week performance)
- Elective Waiting Lists
- Elective RTT (52-week performance)
- Diagnostic Waiting Times
- Cancer (Faster Diagnosis Standard)
- Cancer (31-day performance)
- Cancer (62-day performance)
- UEC (4 Hour Waits)
- UEC (Attendances)
- Community Services (18-week performance)
- Community Services (52-week performance)
- Appointments in General Practice
- Talking Therapies Completed Treatments
- Mental Health Individual Placement Support
- Average LOS Adults, PICU and Older Adults

Experience of Care

- Friends & Family Test

Effectiveness of Care & Population Health

- Mental Health (reliable improvement rate)
- Mental Health (reliable recovery rate)
- Mortality

Patient Safety

- Maternity
- Infection Prevention & Control
- Speaking Up

Executive Summary by Exception

Access

- Continued delivery of RTT waiting list size, positioning the system positively going into Q2.
- RTT 18-week and >52-week performance has improved overall; however, recent month-on-month trends and widening gap from plan indicate an increasing level of risk.
- Diagnostics (DM01) performance is improving following EPR implementation at BHRUT and reporting changes at Barts Health; however, further improvement required to close the gap to plan; some diagnostic tests continue to experience significantly longer waits than planned.
- Performance against all three cancer measures (FDS, 62-day, and 31-day) is improving overall.
- CHS 18-week performance and >52-week waits are improving; the longest waits continue to be within Children and Young People's ASD services.
- General practice appointment activity remains strong, supporting improved access to primary care services.
- Mental Health Individual Placement Support while below plan is improving.
- Talking therapies completed treatments and average length of stay for adults & PICU are above plan. Average length of stay for older adults remains below plan.

Experience of Care

- FFT scores for A&E have shown a downward trend throughout this financial year, with negative special cause variation in June & July of 2026, where we would normally see positive scores returning following winter pressures.
- Acute inpatient FFT scores have returned to the mean following special cause variation in May. The CQC Inpatient Survey was published in August 2026, with worse than expected experience reported within BHRUT.
- NEL ICB fell below plan in Jul-26 for talking therapies reliable improvement but exceeded plan for reliable recovery.

Effectiveness of Care and Population Health

- BHRUT's SHMI has shown a steep decrease since November 2025, this is likely due to a quality data issue.
- While Barts SHMI is within expected range, negative special cause variation continues to be seen.

Patient Safety

- While numbers of MRSA infections remain within expected limits, and below the mean for the past five months, numbers are regularly above the threshold of 0. Model Health System benchmarking places NEL ICB as the ICB with the highest MRSA infection rates in London and second highest rates nationally.
- While numbers of healthcare-associated E.Coli infections remain within expected limits, Model Health System shows NEL ICB has the highest rates in London and third highest nationally.
- Numbers of C.Diff infections have sat below the mean between March and June 2026, following reduced control throughout 2025, however this has returned to the mean in July 2026. As of June 2026, Model Health System shows NEL has the lowest C.diff infection rate nationally.
- The rate of grade 3 and 4 tears has sat above the mean for the past six months, with special cause variation seen in March 2026; if this is sustained, there we will see a negative step change in performance.
- The rate of PPH of 1500ml or more continues to sit above the mean with special cause variation seen in February and March 2026.

NHS
North East London

Access to Services

NHS Oversight Framework Access to Services Domain Scores				
Barts	BHRUT	ELFT	Homerton	NELFT
3 – Below Average	3 – Below Average	1 – High Performing	1 – High Performing	3 – Below Average

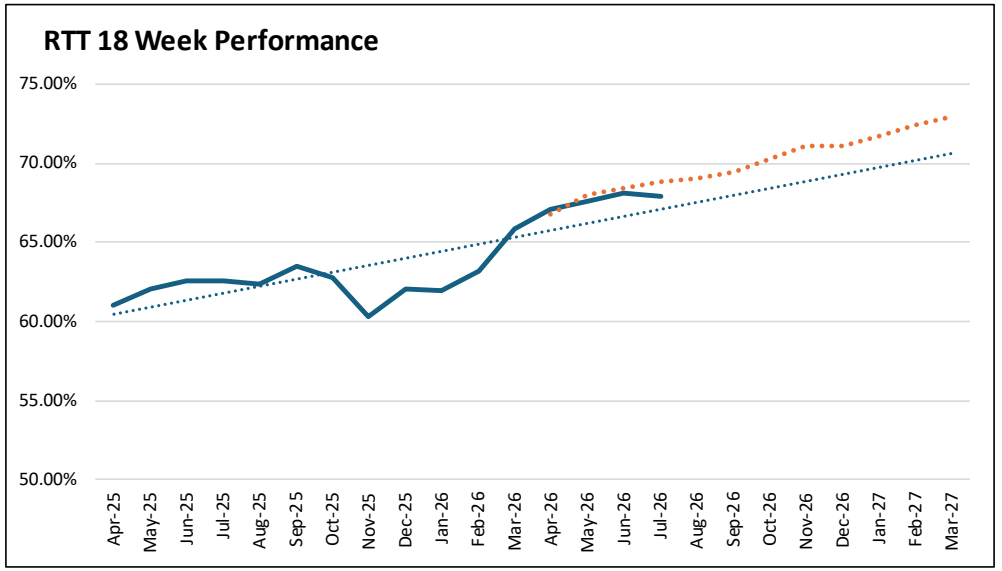
Elective | 18-week performance

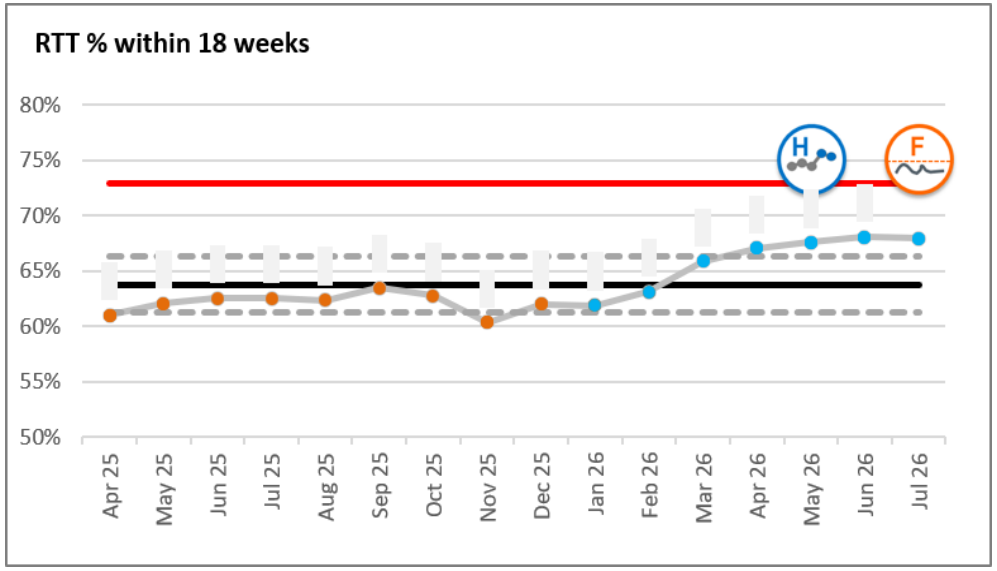
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	**62.5%**	**68.1%**	**67.9%**	**68.8%**	**73.0%**
Barts	57.1%	63.9%	63.8%	63.7%	67.6%
BHRUT	71.1%	68.7%	68.6%	68.9%	77.5%
Homerton	65.1%	75.4%	73.6%	75.5%	80.0%

Change / Variance		
Prior Month	Prior Year	Plan
-0.2%	5.4%	-0.9%
-0.1%	6.7%	0.1%
-0.1%	-2.6%	-0.4%
-1.8%	8.5%	-1.9%

- NEL ICB 18-week RTT performance overall is improving (SPC).
- Performance in July however, fell marginally below plan at NEL level. Homerton (for the first month) and BHRUT behind plan.
- In line with ICB recovery funding, specialty level recovery plans and revised trajectories (waiting list and RTT performance) have been agreed with BHRUT at Trust level. BHRUT marginally behind revised plan (-0.4%) for the month.

Commissioner action: Overarching support to BHRUT recovery incl. ICB funding to support specialty level interventions to meet revised trajectories. Additional financial support also provided to Barts Health for the first half of the year (H1). Robust tracking, monitoring, evidence, and assurance against spend and delivery.
The commissioner led NEL 92% T&F owns the overarching quantified delivery plan and supporting architecture to deliver 92% RTT and reduce the size of the NEL ICB PTL by half by Mar-29.





London Rank: NEL ICB 2; SEL ICB 4; SWL ICB 1; NWNL ICB 3
National Rank: London 3; East of England 7; Midlands 5; North East & Yorkshire 1; North West 4; South East 6; South West 2
Data source: Published RTT data - NHSE

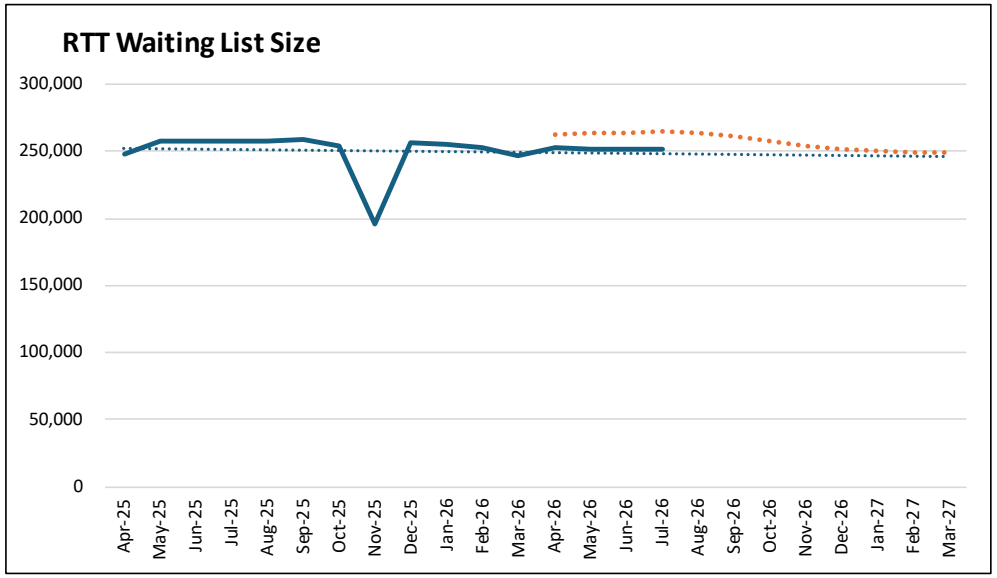
Elective | waiting list

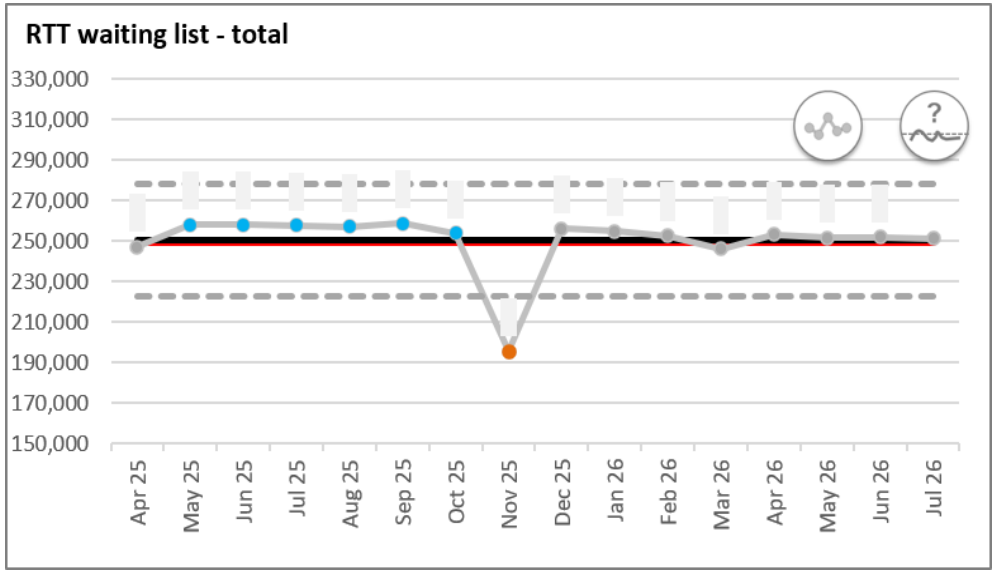
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	**257,804**	**252,020**	**251,544**	**264,845**	**249,069**
Barts	134,381	121,464	121,541	125,976	123,382
BHRUT	57,908	68,893	67,651	70,525	51,974
Homerton	35,456	26,863	26,141	26,396	25,148

Change		
Prior Month	Prior Year	Plan
-0.2%	-2.4%	-5.0%
0.1%	-9.6%	-3.5%
-1.8%	16.8%	-4.1%
-2.7%	-26.3%	-1.0%

- NEL ICB waiting list is stable (SPC).
- In Jul-26 the ICB delivered against the planned waiting list size, continuing to position the system positively going into Q2.
- The ICB and all three Trusts met waiting list size trajectories for the month.
- In line with ICB recovery funding, specialty level recovery plans and revised trajectories (waiting list and RTT performance) have been agreed with BHRUT at Trust level (BHRUT position is against the revised plan).

Commissioner action: Overarching support to BHRUT recovery incl. ICB funding to support specialty level interventions to meet revised trajectories. Additional financial support also provided to Barts Health for the first half of the year (H1). Robust tracking, monitoring, evidence, and assurance against spend and delivery.
The commissioner led NEL 92% T&F owns the overarching quantified delivery plan and supporting architecture to deliver 92% RTT and reduce the size of the NEL ICB PTL by half by Mar-29.





London Rank: NEL ICB 3; SEL ICB 2; SWL ICB 1; NWNL ICB 4
National Rank: London 6; East of England 2; Midlands 7; North East & Yorkshire 3; North West 4; South East 5; South West 1
Data source: Published RTT data - NHSE

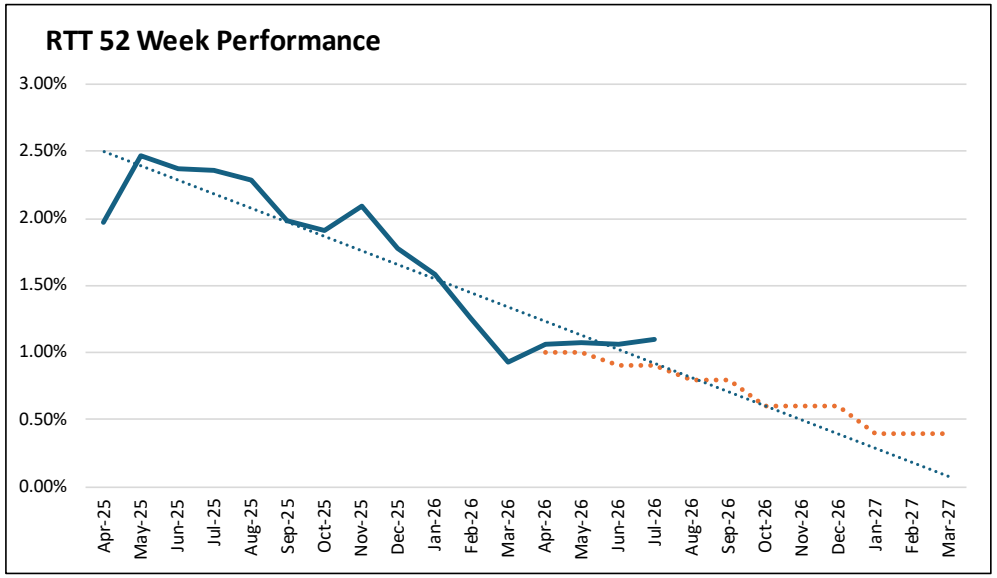
Elective | 52-week performance

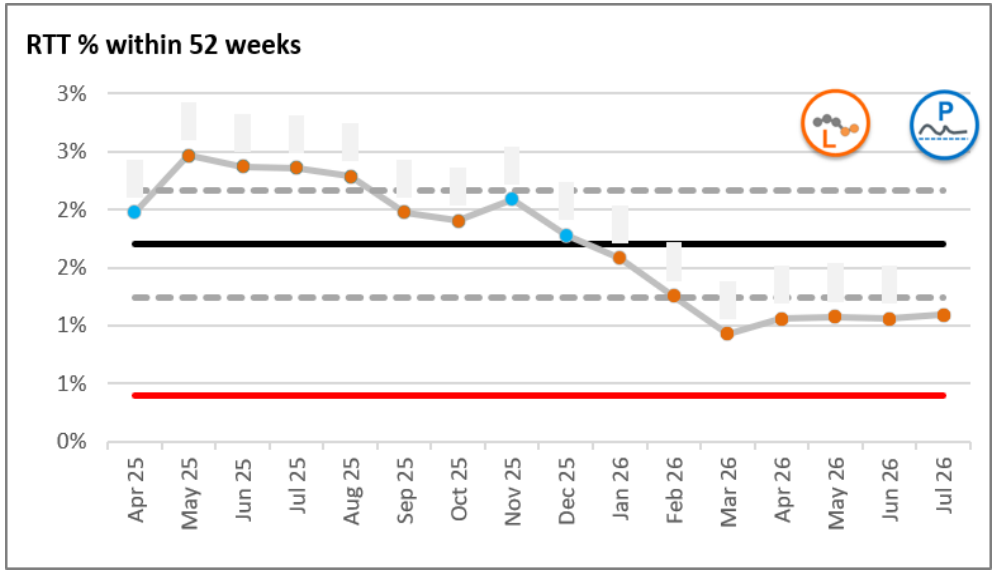
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	**2.4%**	**1.1%**	**1.1%**	**0.9%**	**0.4%**
Barts	3.5%	0.9%	0.8%	0.0%	0.0%
BHRUT	1.0%	1.5%	1.8%	1.1%	0.4%
Homerton	1.1%	0.6%	0.5%	0.0%	0.0%

Change		
Prior Month	Prior Year	Plan
0.0%	-1.3%	0.2%
-0.1%	-2.6%	0.8%
0.3%	0.8%	0.7%
-0.1%	-0.6%	0.5%

- NEL ICB % >52ww is improving overall (SPC).
- In Jul-26 the ICB position however remains above planned volumes (+368 pathways adverse to plan) but is holding at 1.1% of PTL.
- All three NEL Acute Trusts above plan for the month, noting >52ww volumes at both Barts Health and Homerton are below 1% of the waiting list. In line with ICB recovery funding, specialty level recovery plans and revised trajectories (waiting list and RTT performance) have been agreed with BHRUT at Trust level (BHRUT position is against the revised plan).

Commissioner action: Overarching support to BHRUT recovery incl. ICB funding to support specialty level interventions to meet revised trajectories. Additional financial support also provided to Barts Health for the first half of the year (H1). Robust tracking, monitoring, evidence, and assurance against spend and delivery.
The commissioner led NEL 92% T&F owns the overarching quantified delivery plan and supporting architecture to deliver 92% RTT and reduce the size of the NEL ICB PTL by half by Mar-29.





London Rank: NEL ICB 3; SEL ICB 1; SWL ICB 4; NWNL ICB 2
National Rank: London 5; East of England 1; Midlands 3; North East & Yorkshire 7; North West 4; South East 2; South West 6
Data source: Published RTT data - NHSE

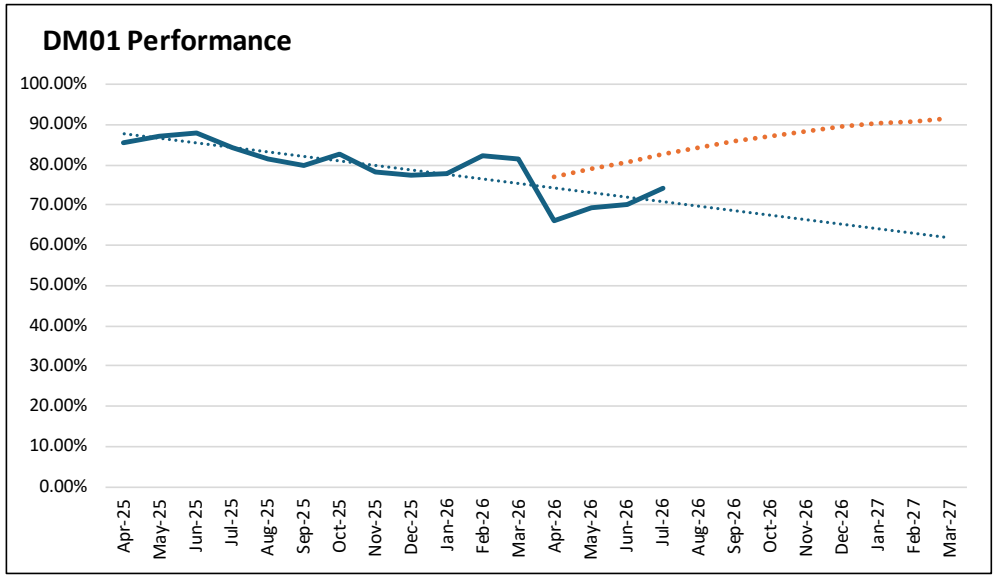
Diagnostics | DM01 performance

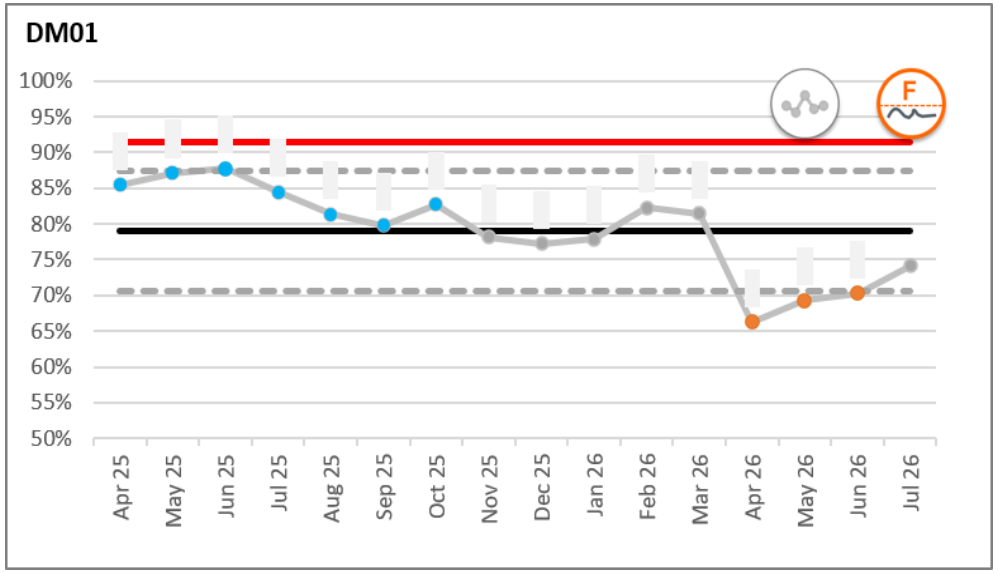
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	**84.4%**	**70.3%**	**74.1%**	**82.4%**	**91.4%**
Barts	77.2%	67.7%	70.0%	76.3%	88.6%
BHRUT	95.0%	51.9%	60.1%	91.8%	95.6%
Homerton	91.4%	98.2%	97.0%	96.9%	98.0%

Change		
Prior Month	Prior Year	Plan
3.8%	**-10.3%**	**-8.4%**
2.3%	-7.2%	-6.3%
8.1%	-34.9%	-31.7%
-1.1%	5.6%	0.2%

- NEL ICB diagnostics (DM01) performance is improving (SPC) and is above the lower control limit for the first month since Apr-26, post impact of the return to national reporting by BHRUT following EPR implementation, and changes to diagnostic reporting logic at Barts Health.
- There has been a +6.3% performance improvement at Barts Health and +12.7% performance improvement at BHRUT from April.

Commissioner action: Overarching support to BHRUT recovery actions including MSK diagnostics SPoA to reduce the backlog and improve compliance, and support to the national bid for CDC funding (pending national approval). If approved, funding would expedite compliance, especially in MRI and ultrasound.
Work underway to ensure full utilisation of available capacity and assets including unused IS capacity.
Updated ultrasound triage/ referral guidelines being discussed in all Trusts and IS Providers to manage demand. Action also underway to support development of new CDCs, capital and digital investments including iRefer to reduce low value imaging requests and additional physical assets to enhance capacity and speed of testing.



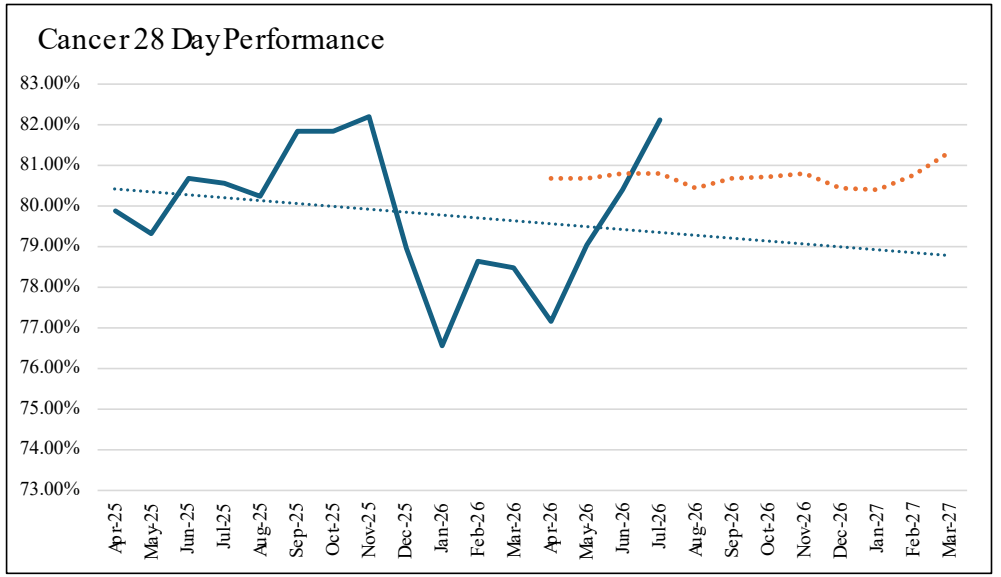


London Rank: NEL ICB 4; SEL ICB 3; SWL ICB 1; NWNL ICB 2
National Rank: London 4; East of England 7; Midlands 6; North East & Yorkshire 3; North West 1; South East 5; South West 2
Data source: Published Diagnostics data - NHSE

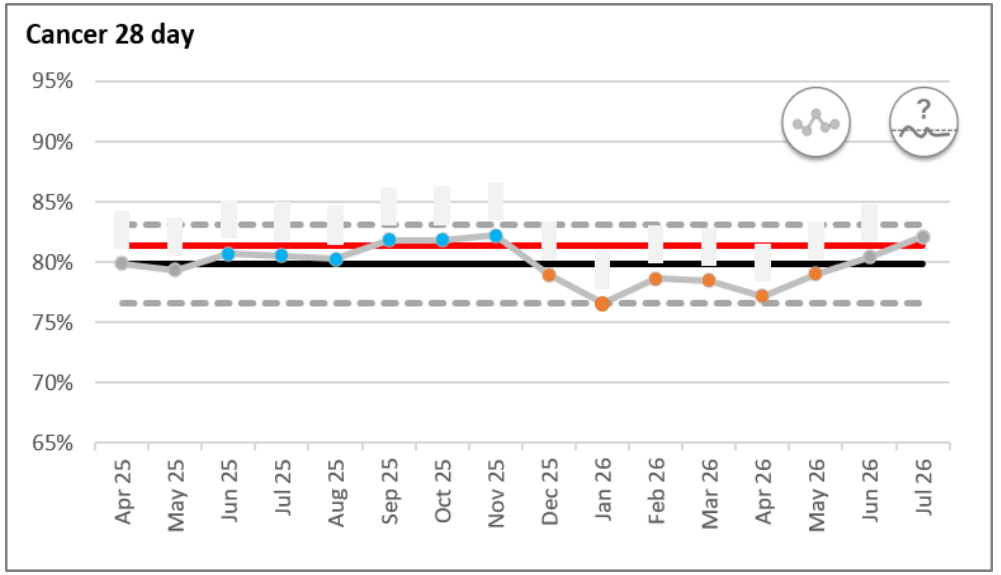
Cancer | FDS performance

	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	**80.6%**	**80.4%**	**82.1%**	**80.80%**	**81.3%**
Barts	81.1%	81.0%	81.4%	80.0%	80.0%
BHRUT	78.6%	77.0%	80.2%	80.4%	80.2%
Homerton	83.2%	86.2%	87.2%	83.3%	85.5%

Change		
Prior Month	Prior Year	Plan
1.7%	1.6%	1.3%
0.4%	0.3%	1.4%
3.2%	1.6%	-0.2%
1.0%	4.0%	3.8%



- At ICB level cancer FDS performance has improved from Apr-26, above plan for the first month and back in line with performance pre EPR implementation at BHRUT (moving towards the upper control limit)
- There has been a +9.5% improvement at BHRUT vs. the Dec-25 reported position post EPR implementation (Nov-25). BHRUT return to compliance against plan is anticipated for Aug-26.



Commissioner action: Cancer Alliance support to BHRUT, including weekly oversight meetings, expert reference group leads, and a cash injection of £600,000, plus an additional £200,000 for sustainability plans. Operational support includes working with clinicians and focusing on tumour site challenges.

London Rank: NEL ICB 4; SEL ICB 2; SWL ICB 1; NWNL ICB 3
National Rank: London 1; East of England 6; Midlands 2; North East & Yorkshire 3; North West 5; South East 4; South West 7
Data source: Published Cancer data - NHSE

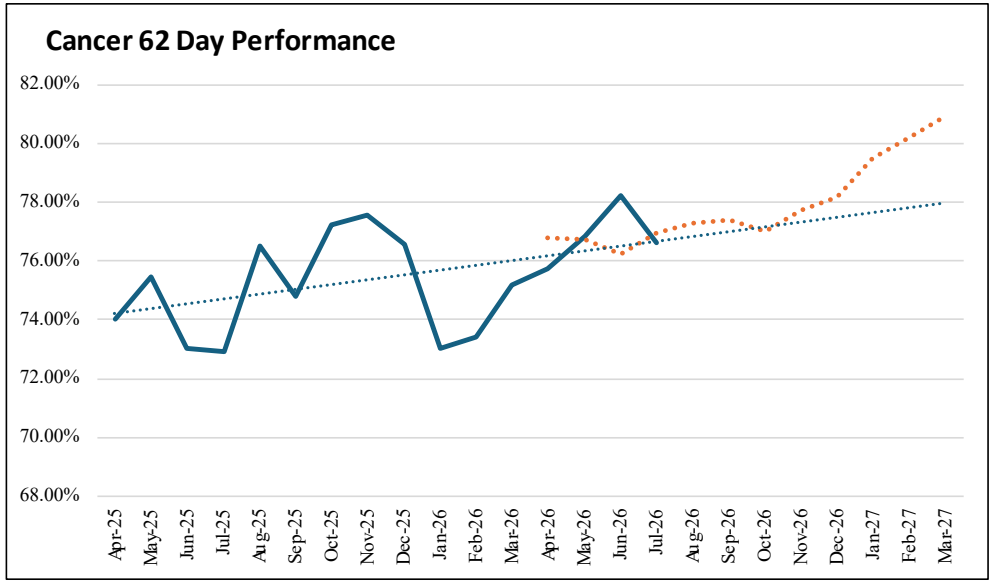
Cancer | 62-day performance

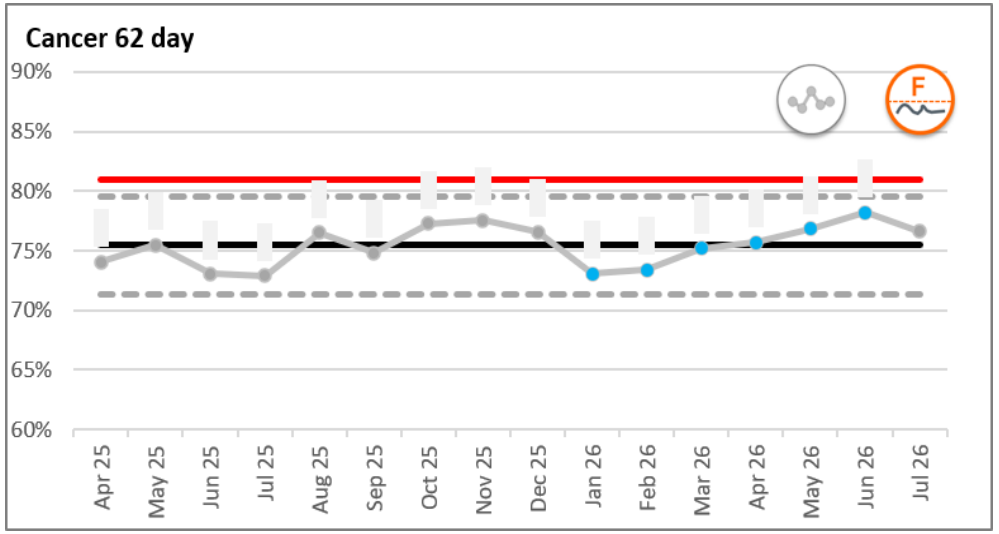
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	**72.9%**	**78.2%**	**76.6%**	**76.96%**	**80.9%**
Barts	75.3%	80.3%	81.1%	76.2%	80.1%
BHRUT	72.2%	72.2%	74.7%	75.5%	80.0%
Homerton	82.8%	87.8%	88.3%	83.3%	86.3%

Change		
Prior Month	Prior Year	Plan
-2.0%	5.1%	-0.3%
1.0%	7.7%	4.9%
3.4%	3.5%	-0.8%
0.6%	6.7%	5.0%

- At ICB level cancer 62-day performance is improving overall (SPC).
- Performance for the month (Jul-26) marginally below plan at NEL level. Barts Health and Homerton above trajectory for the month.
- There has been a +6% improvement at BHRUT vs. the Dec-25 reported position post EPR implementation (Nov-25) and a +7.7% improvement from Apr-26. BHRUT return to compliance against plan is anticipated for Sep-26.

Commissioner action: BHRUT collaboration with the Cancer Alliance, including weekly oversight meetings, expert reference group leads, and a cash injection of £600,000, plus an additional £200,000 for sustainability plans. Operational support includes working with clinicians and focusing on tumour site challenges.





London Rank: NEL ICB 3; SEL ICB 4; SWL ICB 1; NWNL ICB 2
National Rank: London 2; East of England 7; Midlands 6; North East & Yorkshire 5; North West 1; South East 3; South West 4
Data source: Published Cancer data - NHSE

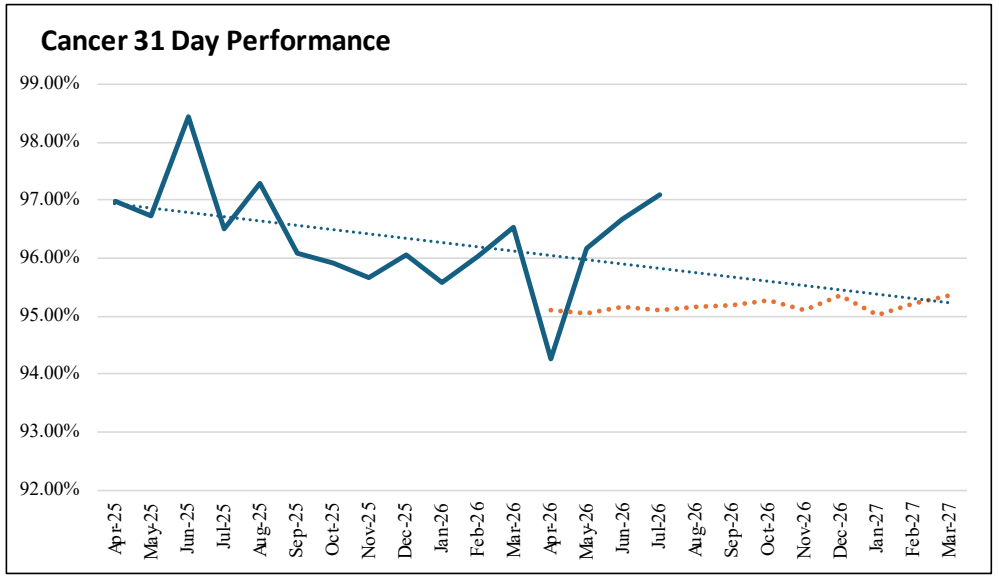
Cancer | 31-day performance

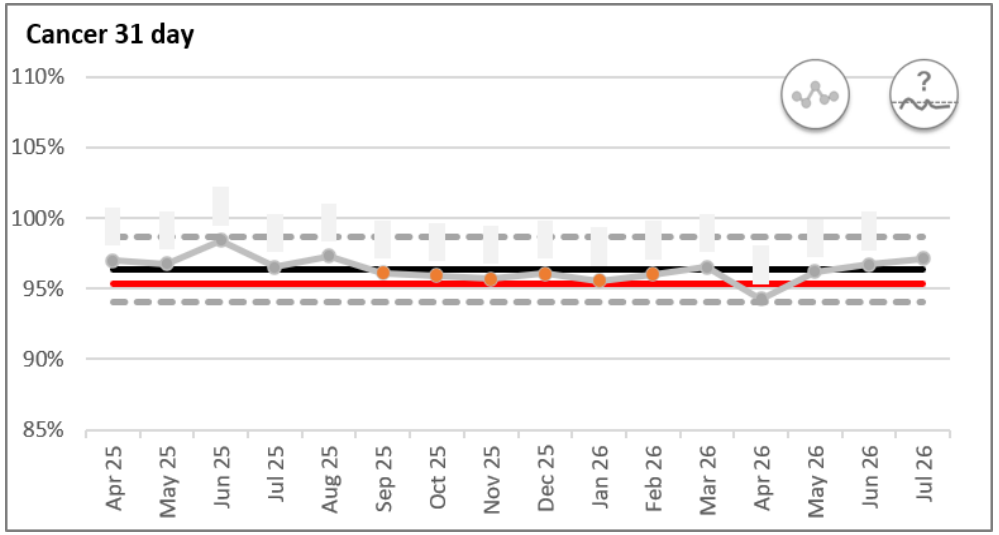
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	**96.5%**	**96.7%**	**97.1%**	**95.12%**	**95.4%**
Barts	98.2%	96.4%	98.5%	94.1%	94.1%
BHRUT	97.3%	98.9%	98.8%	96.3%	96.8%
Homerton	100.0%	100.0%	100.0%	98.4%	98.3%

Change		
Prior Month	Prior Year	Plan
0.4%	**0.6%**	**2.0%**
2.2%	0.3%	4.4%
-0.1%	1.5%	2.4%
0.0%	0.0%	1.6%

- At ICB level cancer 31-day performance has improved from Apr-26 but within expected variation (common cause).
- Performance above plan at NEL level for the month, all three NEL Trusts positive against plan.

Commissioner action: N/A





London Rank: NEL ICB 2; SEL ICB 4; SWL ICB 1; NWNL ICB 3
National Rank: London 1; East of England 6; Midlands 3; North East & Yorkshire 7; North West 2; South East 5; South West 4
Data source: Published Cancer data - NHSE

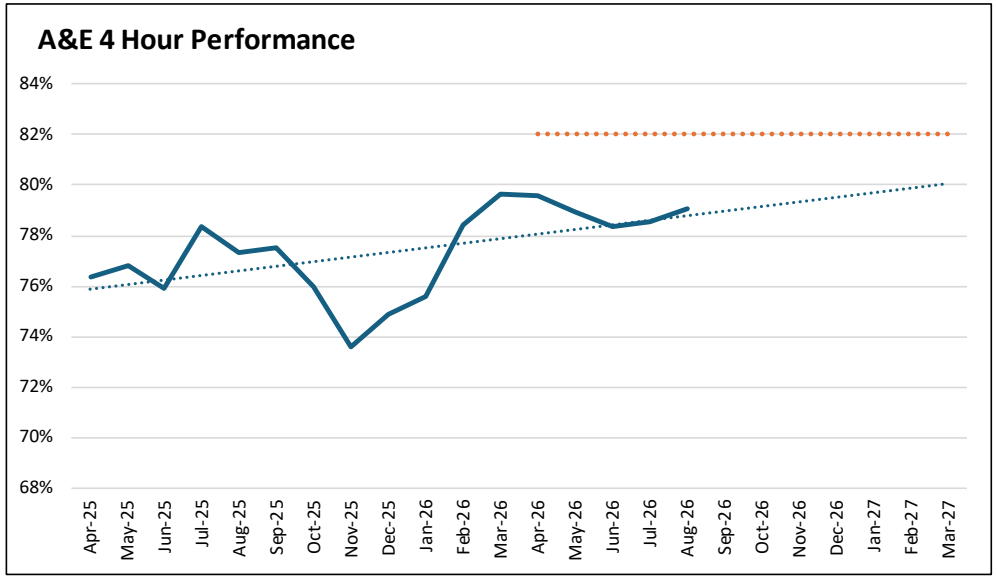
UEC | 4-hour performance

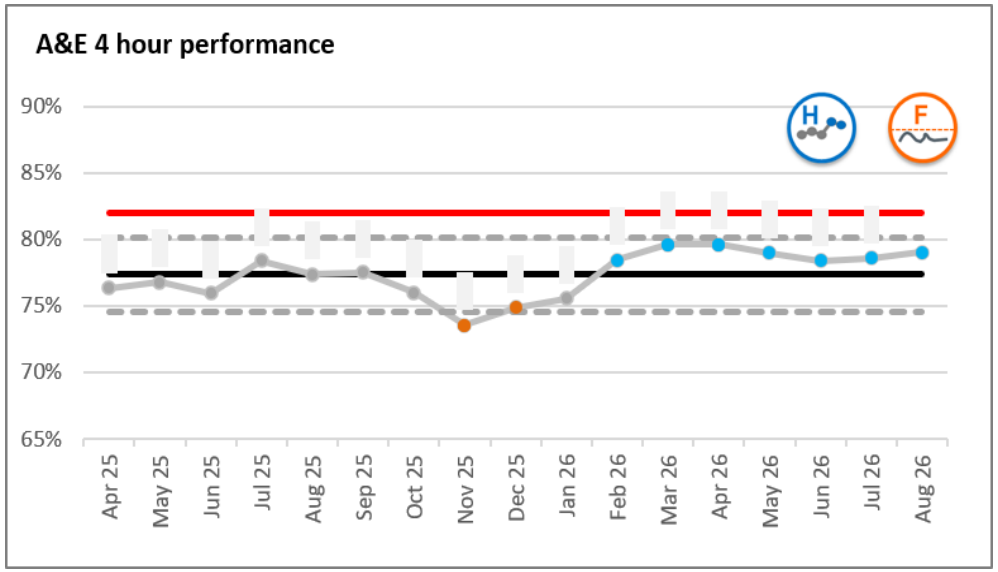
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Aug-25	Jul-26	Aug-26	Aug-26	Mar-27
NEL ICB	**77.3%**	**78.6%**	**79.0%**	**82.0%**	**82.0%**
Barts	72.0%	73.6%	73.1%	79.3%	82.1%
BHRUT	78.7%	81.3%	81.9%	82.0%	82.0%
Homerton	81.1%	79.1%	83.4%	84.0%	84.0%

Change		
Prior Month	Prior Year	Plan
0.5%	1.7%	-3.0%
-0.5%	1.1%	-6.2%
0.6%	3.2%	-0.2%
4.3%	2.3%	-0.6%

- The published position against the 4-hour ED standard at NEL system level in Aug-26 is below trajectory for the month but remains stable within control limits, 79% achievement.
- At Provider level, all three Providers are below planned performance for the month. Barts Health the most challenged Trust based on distance from plan.

Commissioner action: The ICB has agreed an improvement plan with Tower Hamlets GP Care Group. The plan includes creating a demand and capacity tool and a robust breach analysis.
A Type 3 Improvement Plan is in place for Newham UTC, which will go live in January.
Type 1 improvement plans are being sought through contract management meetings in coming months, aligned to trajectories agreed as part of operating plan performance.



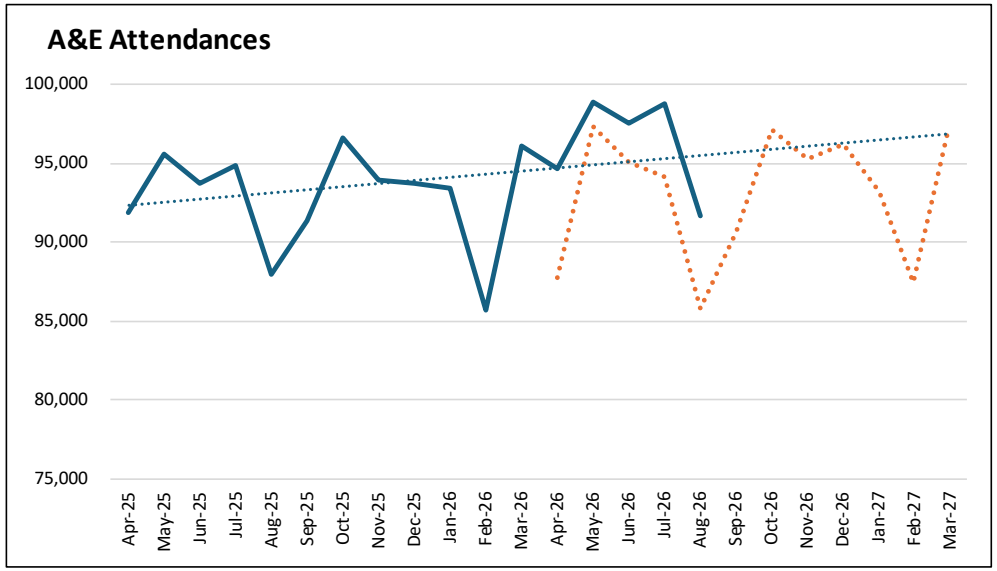


London Rank: NEL ICB 2; SEL ICB 3; SWL ICB 4; NWNL ICB 1
National Rank: London 1; East of England 4; Midlands 5; North East & Yorkshire 2; North West 6; South East 3; South West 7
Data source: Published A&E data - NHSE

UEC | attendances

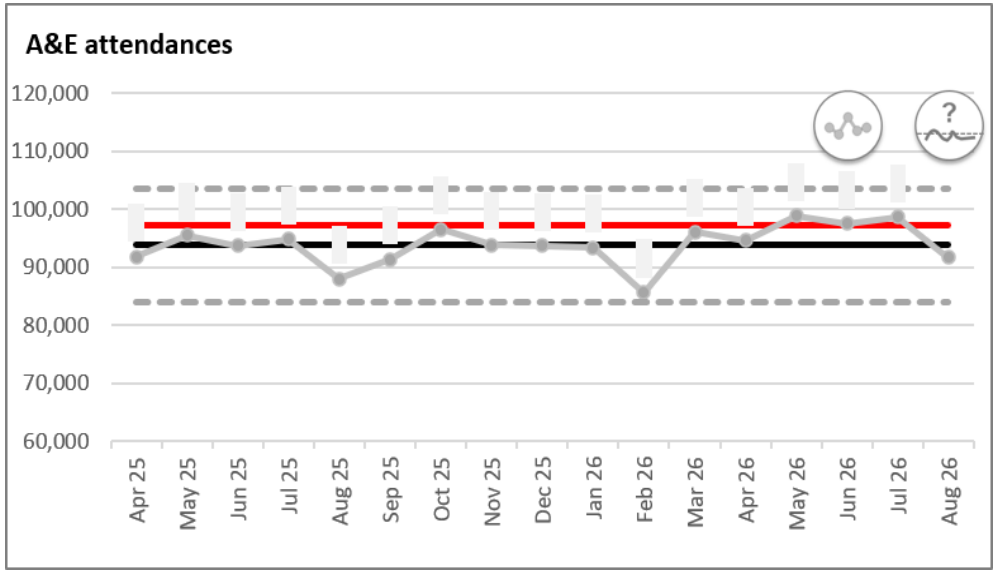
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Aug-25	Jul-26	Aug-26	Aug-26	Mar-27
NEL ICB	**87,974**	**98,725**	**91,681**	**85,761**	**97,349**
Barts	42,424	47,580	43,986	42,476	47,430
BHRUT	28,167	32,315	29,984	32,749	38,479
Homerton	10,343	11,507	11,151	10,536	11,440

Change		
Prior Month	Prior Year	Plan
-7.1%	4.2%	6.9%
-7.6%	3.7%	3.6%
-7.2%	6.5%	-8.4%
-3.1%	7.8%	5.8%



- UEC attendances were down in Aug-26 in line with seasonality but remain above planned levels (Barts Health and Homerton) and compared to the same period last year (all three NEL Trusts).

Commissioner action: Continue delivery of system-wide commissioning intentions on demand management initiatives, including Single Point of Access/ Integrated Clinical Coordination, Same Day Access to Primary Care, Urgent Treatment Centres, Same Day Emergency Care, Urgent Community Response, Virtual Wards, Acute Frailty pathways, alongside optimisation of admission criteria and alternatives to hospital admission.
The Care Closer to Home programme will build on these foundations by redesigning and scaling community-based models of care to help moderate future growth in avoidable attendances, admissions and bed days and aligns to wider context focus on drivers of deficit in Outer NEL.



London Rank: NEL ICB 2; SEL ICB 3; SWL ICB 4; NWNL ICB 1
National Rank: London 2; East of England 6; Midlands 1; North East & Yorkshire 3; North West 5; South East 4; South West 7
Data source: Published A&E data - NHSE

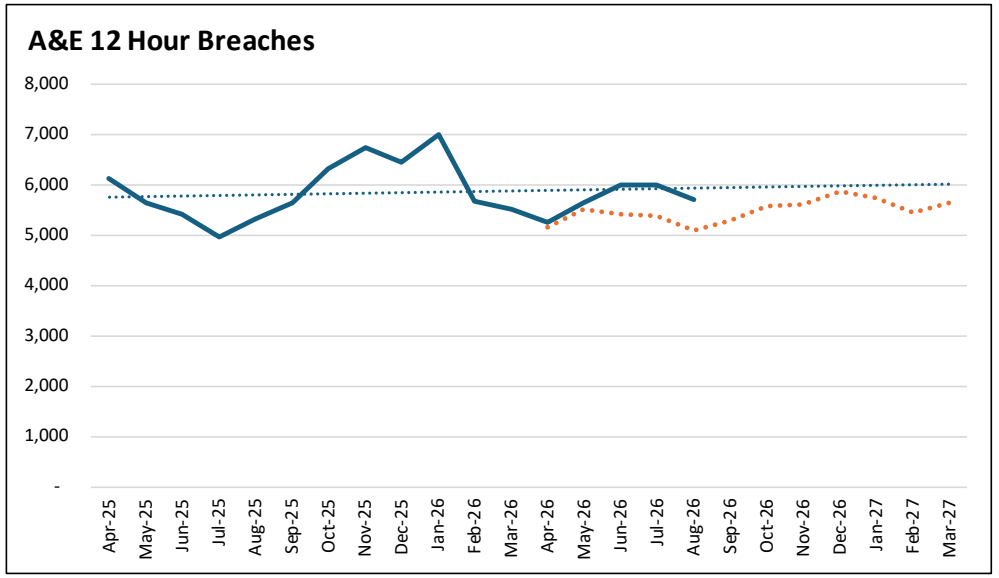
UEC | 12-hour performance

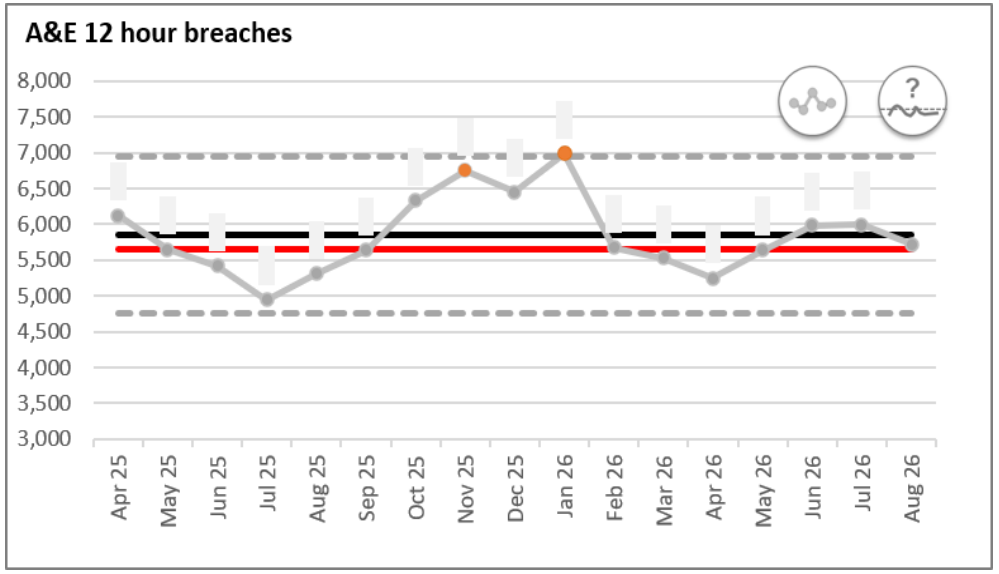
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Aug-25	Jul-26	Aug-26	Aug-26	Mar-27
NEL ICB	**5,308**	**5,998**	**5,719**	**5,080**	**5,646**
Barts	3,488	4,024	3,987	3,235	3,505
BHRUT	2,293	2,443	2,229	1,774	2,084
Homerton	93	70	59	71	57

Change		
Prior Month	Prior Year	Plan
-4.7%	7.7%	12.6%
-0.9%	14.3%	23.2%
-8.8%	-2.8%	25.7%
-15.7%	-36.6%	-16.9%

- The number of type 1 and 2 patients admitted, transferred, or discharged within 12-hours of arrival is higher than plan but remains stable within control limits (89%).
- At Provider level, Barts and BHRUT are below planned performance for the month.

Commissioner action: Continue delivery of system-wide commissioning intentions on demand management initiatives, including Single Point of Access/ Integrated Clinical Coordination, Same Day Access to Primary Care, Urgent Treatment Centres, Same Day Emergency Care, Urgent Community Response, Virtual Wards, Acute Frailty pathways, alongside optimisation of admission criteria and alternatives to hospital admission.
The Care Closer to Home programme will build on these foundations by redesigning and scaling community-based models of care to help moderate future growth in avoidable attendances, admissions and bed days and aligns to wider context focus on drivers of deficit in Outer NEL.





Data source: Published A&E data - NHSE

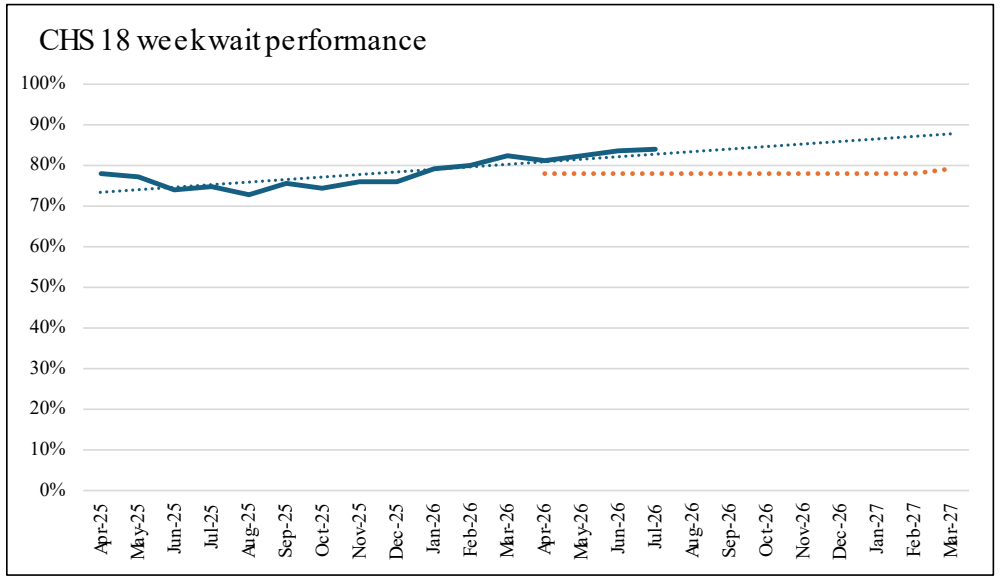
Community Health Services | 18-week performance

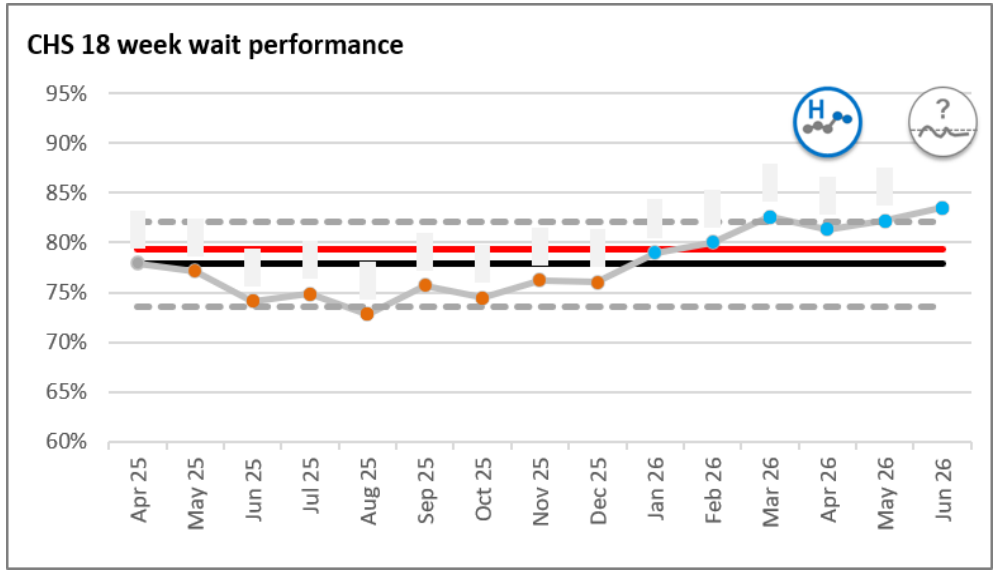
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	74.8%	83.5%	84.1%	78.0%	79.3%

Change		
Prior Month	Prior Year	Plan
0.7%	12.4%	7.8%

- NEL ICB 18-week CHS performance in Jul-26 was 84.1%, above plan.
- NEL ICB CHS 18-week performance is improving (SPC).

Commissioner action: The longest waiting times are within Autism Spectrum Disorder (ASD) services for Children and Young People. To address this funding has been identified and a business case has been agreed to reduce waiting times in Newham to 12 weeks.
We are have also started with other borough services to agree next steps to reduce waiting lists. In addition, we are developing a North East London ASD Improvement Programme to support a coordinated approach to reducing waits, improving access, and enhancing the overall quality of ASD services across the system





Data source: Published CHS data - NHSE

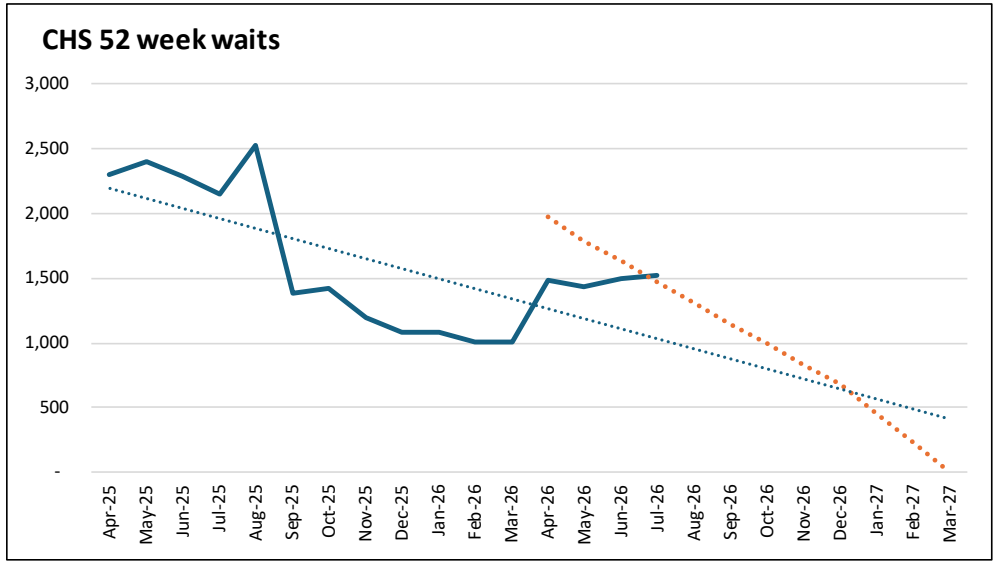
Community Health Services | 52-week performance

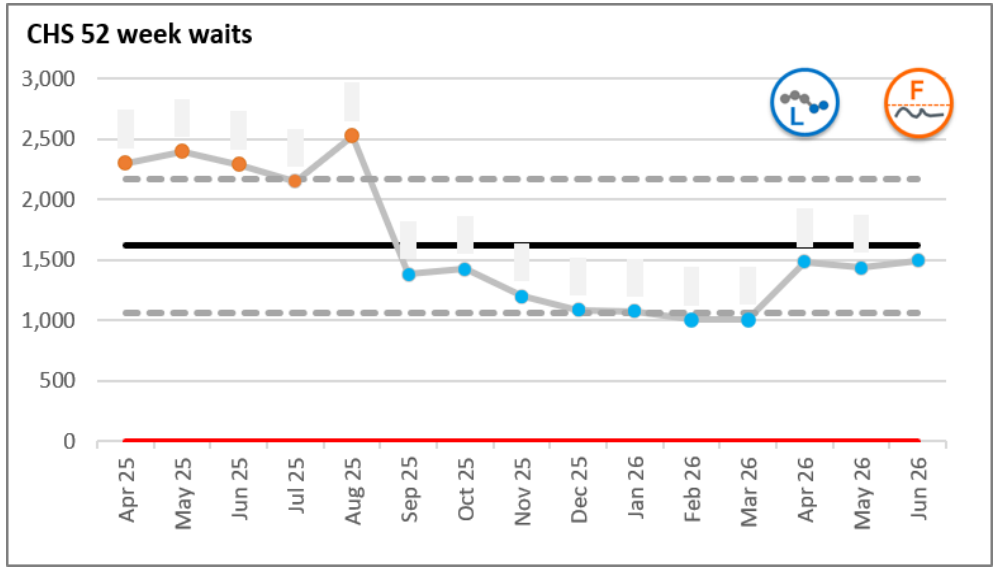
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	2,153	1,495	1,524	1,469	0

Change		
Prior Month	Prior Year	Plan
1.9%	-29.2%	3.7%

- There were 1,469 CHS waits >52-weeks in Jul-26, above trajectory. NEL ICB CHS 18-week performance is improving (SPC).
- All providers are committed to eliminating 52 week waits outside of ASD.

Commissioner action: The longest waiting times are within Autism Spectrum Disorder (ASD) services for Children and Young People. To address this funding has been identified and a business case is currently progressing through the ICB approval process to reduce waiting times in Newham. We are have also started with other borough services to agree next steps to reduce waiting lists. In addition, we are developing a North East London ASD Improvement Programme to support a coordinated approach to reducing waits, improving access, and enhancing the overall quality of ASD services across the system.





Data source: Published CHS data - NHSE

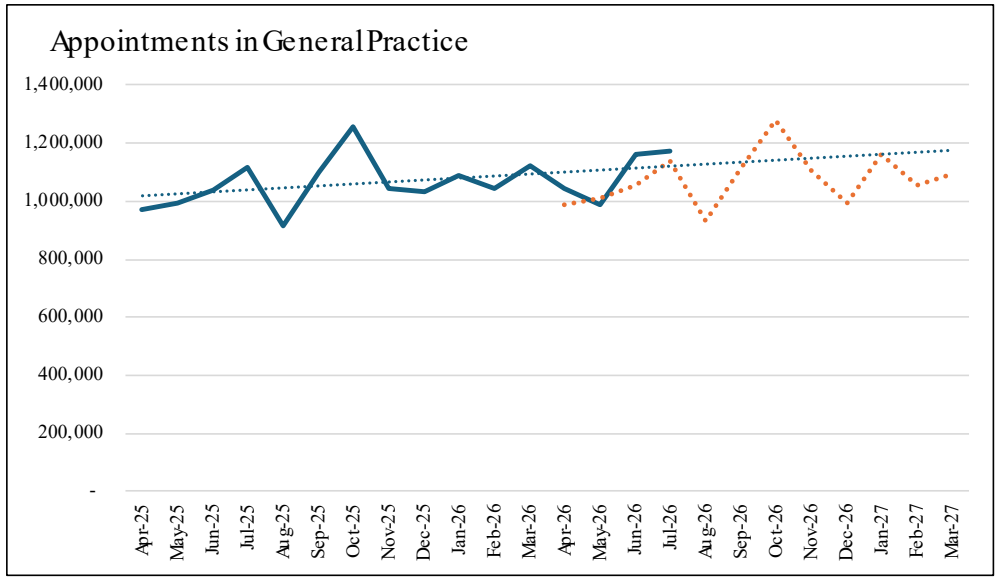
Primary Care | appointments in general practice

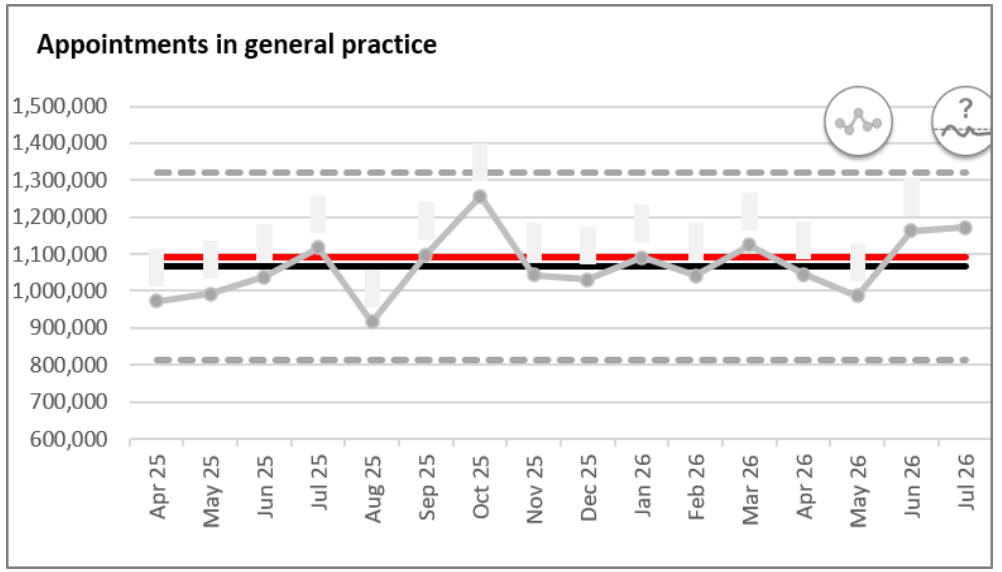
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	1,116,727	1,163,448	1,172,251	1,136,014	1,092,782

Change		
Prior Month	Prior Year	Plan
0.8%	5.0%	3.2%

- July activity reached 1,172,251 appointments, 36,237 (3.2%) above plan, 0.8% higher than June and 5.0% above July 2025. Activity remained within expected limits of variation and was 79,469 (7.3%) above the March 2027 target benchmark; however, further monthly data are required to confirm sustained performance.
- As the LTC LIS rollout progresses, increased proactive long-term condition management is expected to support further growth in appointment activity during 2026/27.

Commissioner action: Continue monthly monitoring, including practice-level variation, to confirm whether above-plan performance is sustained and promptly investigate any material reduction with providers.





London Rank: NEL ICB 2; SEL ICB 3; SWL ICB 4; NWNL ICB 1
National Rank: London 3; East of England 5; Midlands 1; North East & Yorkshire 2; North West 6; South East 4; South West 7
Data source: Published Primary Care data - NHSE

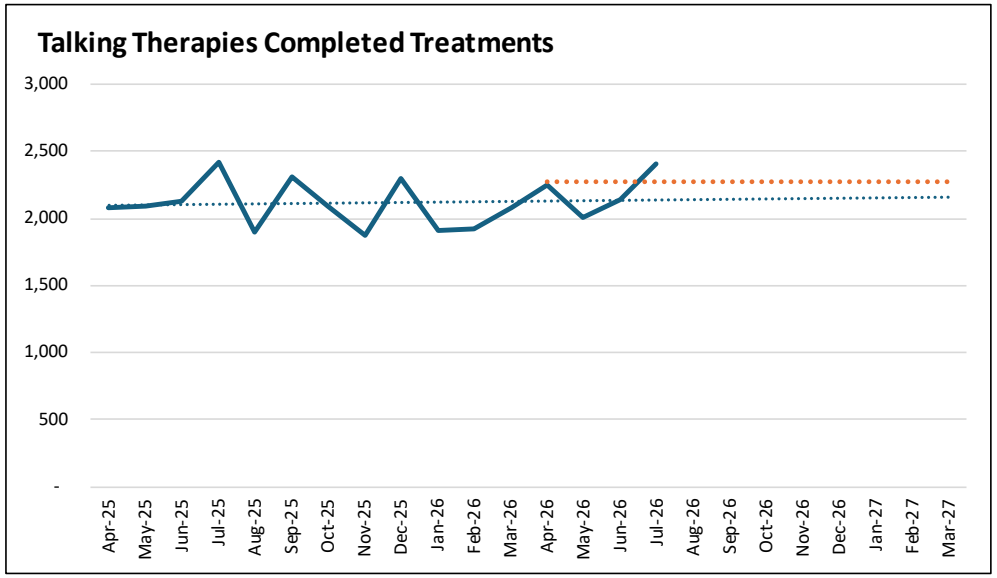
Mental Health | talking therapies completed treatments

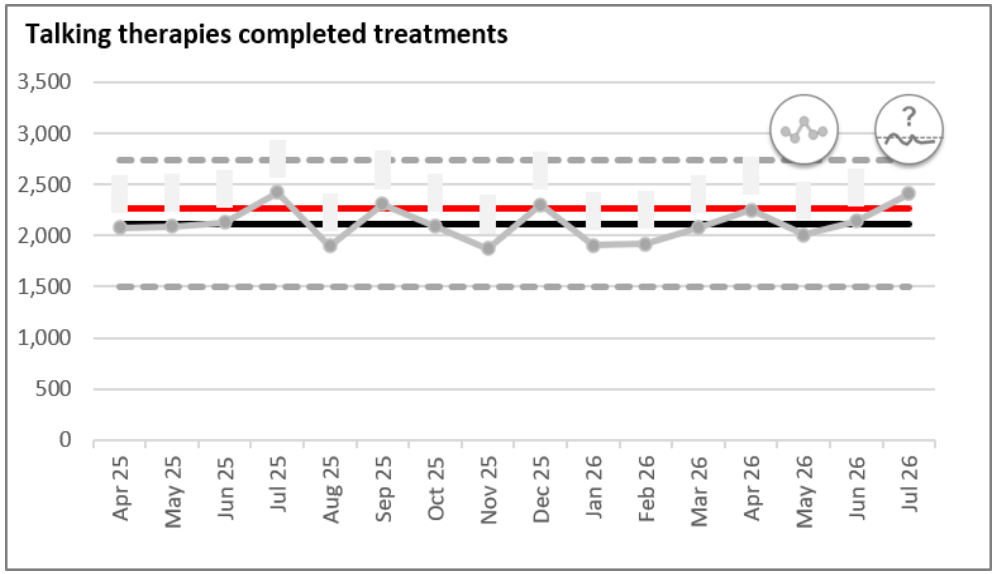
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	2,420	2,140	2,410	2,271	2,271

Change		
Prior Month	Prior Year	Plan
12.6%	-0.4%	6.1%

- NEL ICB completed treatments performance stable (SPC).
- In Jul-26 the ICB met the planned completed treatments performance trajectory.
- The NHSE Talking Therapies Deep Dive is providing an opportunity to understand the drivers of performance variation and support reliable, sustainable improvement across NEL.

Commissioner action: Work collaboratively with NHSE and providers to implement the agreed actions arising from the Talking Therapies Deep Dive, provide strategic oversight of provider improvement plans, and strengthen performance assurance to support sustainable improvements in reliable improvement performance. Review and understand current service models, workforce approaches and barriers across boroughs to identify variation and opportunities for improvement, ensuring equitable access to Talking Therapies services across NEL. Use this intelligence to inform future commissioning intentions, including consideration of both place-based and NEL-wide delivery models.





London Rank: NEL ICB 2; SEL ICB 3; SWL ICB 4; NWNL ICB 1
National Rank: London 2; East of England 6; Midlands 1; North East & Yorkshire 4; North West 5; South East 3; South West 7
Data source: Published Mental Health data - NHSE

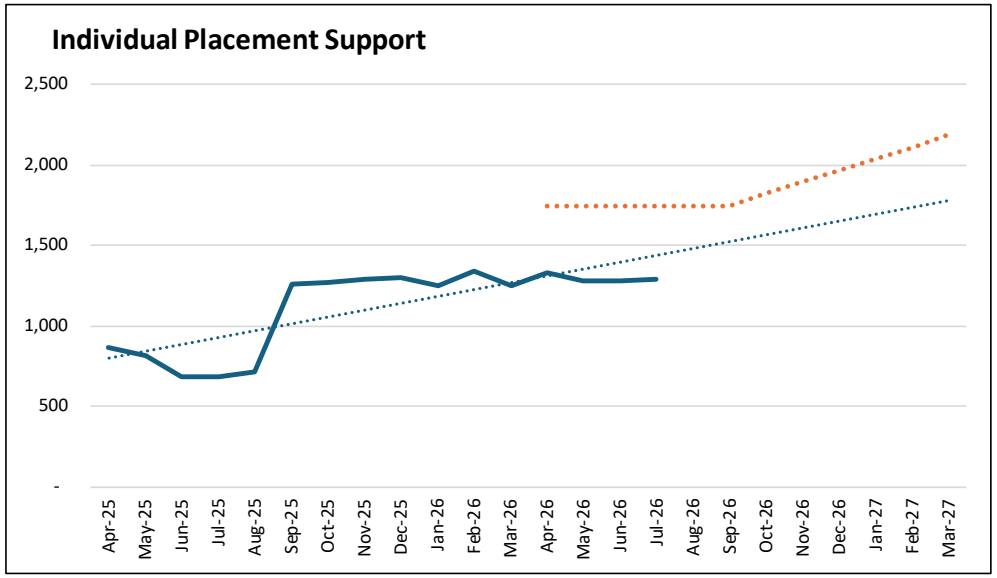
Mental Health | individual placement support

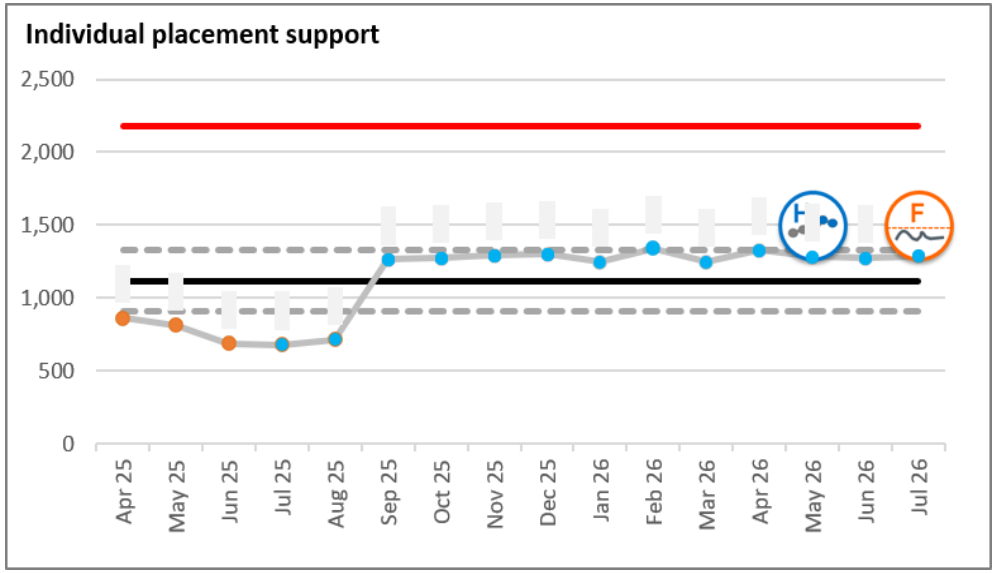
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	680	1,275	1,285	1,746	2,183

Change		
Prior Month	Prior Year	Plan
0.8%	89.0%	-26.4%

- NEL ICB individual placement support performance improving (SPC).
- In Jul-26 the ICB failed to deliver against planned individual placement support performance.
- Place level performance (not shown here) poor, with all places not achieving planned performance.
- Provider improvement plans are in place, with commissioner oversight to support recovery. The NHSE Deep Dive is providing an opportunity to understand the drivers of performance variation and support reliable, sustainable improvement across NEL.

Commissioner action: Provide overarching strategic oversight of the IPS programme, including monitoring provider improvement plans to support performance improvement. Map IPS delivery across NEL to understand variation in service models, integration with clinical teams, referral pathways and operational challenges, identifying best practice and common themes across providers. Work collaboratively with NHS England through the planned Deep Dive to review performance, recovery plans and opportunities for improvement. Use the findings to inform future commissioning intentions, strengthen integration, improve equitable access and consider opportunities for greater consistency through place-based and NEL-wide commissioning approaches.





London Rank: NEL ICB 3; SEL ICB 2; SWL ICB 4; NWNL ICB 1

Data source: Published Mental Health data - NHSE

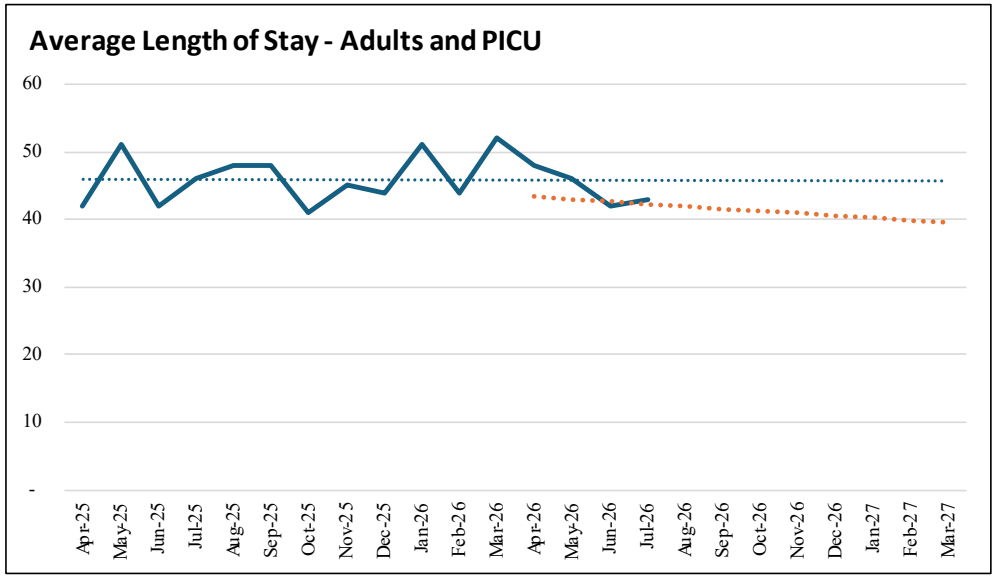
Mental Health | average length of stay for adults and PICU

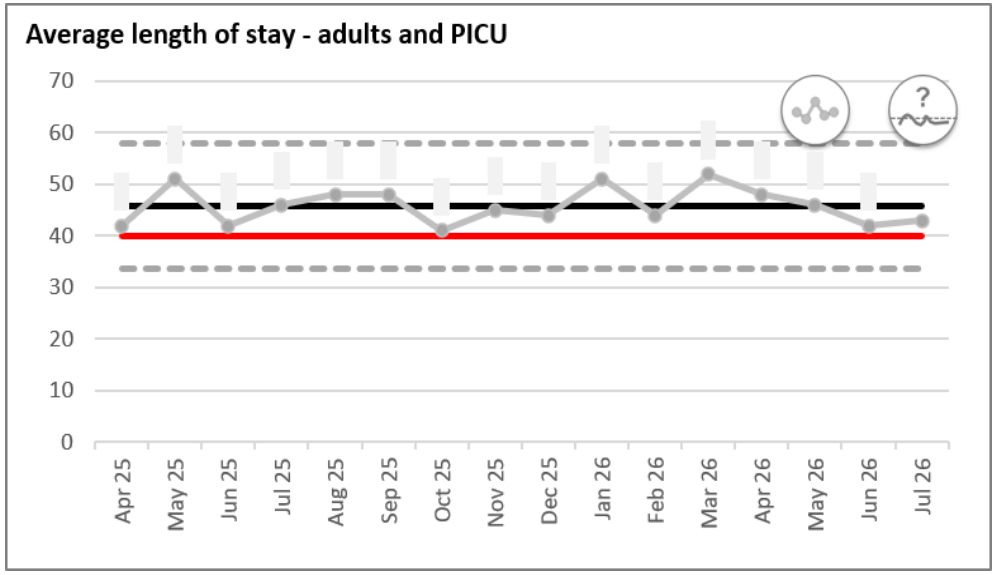
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	46	42	43	42	40

Change		
Prior Month	Prior Year	Plan
2.4%	-6.5%	1.8%

- NEL ICB average length of stay for adults and PICU performance stable (SPC).
- In Jun-26 the ICB delivered against planned performance.

Commissioner action: N/A





London Rank: NEL ICB 3; SEL ICB 4; SWL ICB 2; NWNL ICB 1

Data source: Published Mental Health data - NHSE

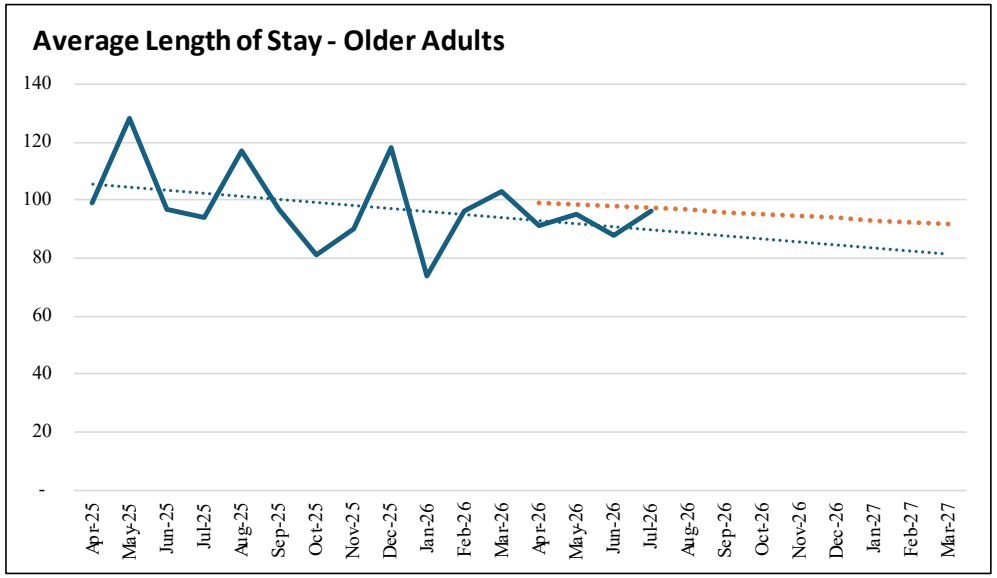
Mental Health | average length of stay for older adults

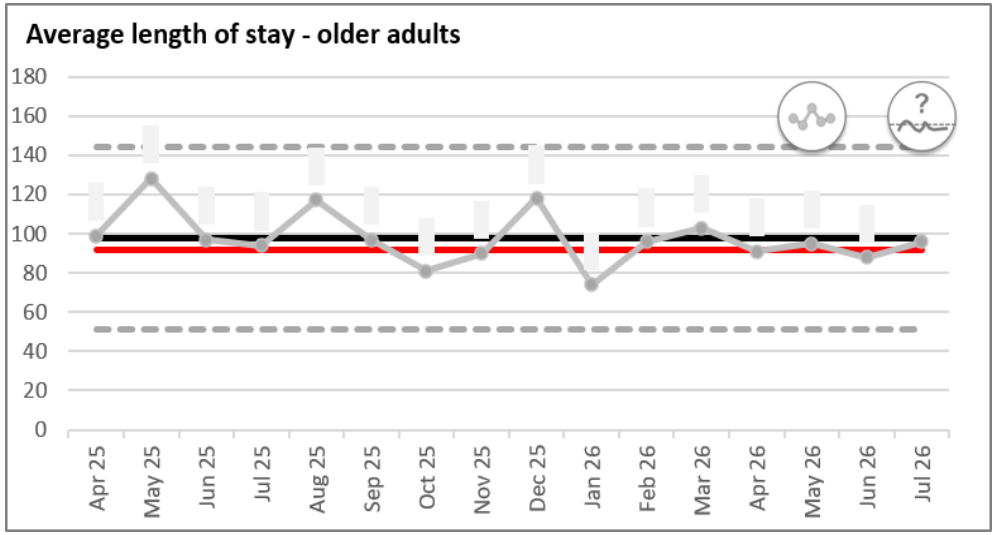
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	94	88	96	97	92

Change		
Prior Month	Prior Year	Plan
9.1%	2.1%	-1.3%

- NEL ICB average length of stay for older adults performance stable (SPC).
- In Jul-26 the ICB failed to deliver against planned performance.

Commissioner action: N/A





London Rank: NEL ICB 4; SEL ICB 3; SWL ICB 2; NWNL ICB 1

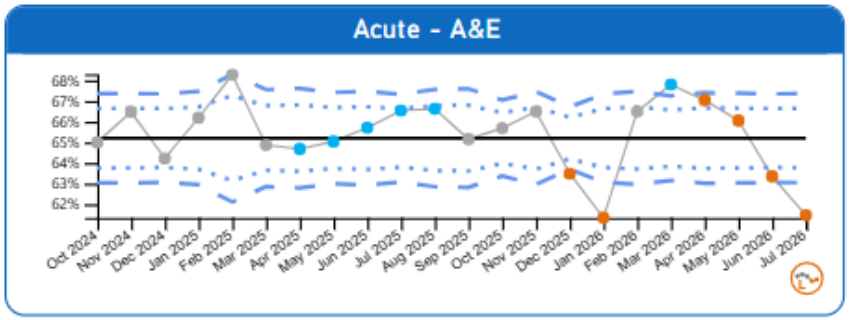
Data source: Published Mental Health data - NHSE

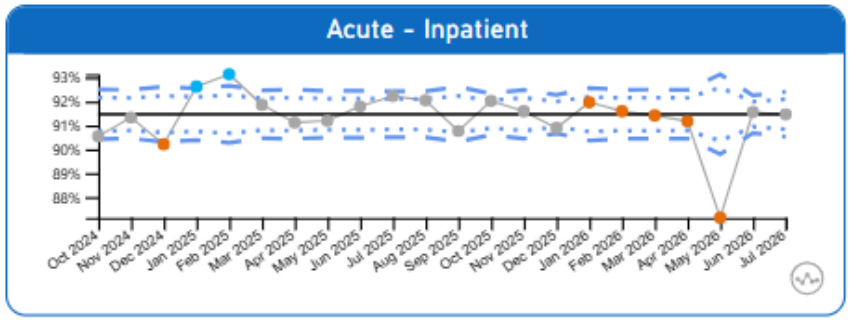
NHS
North East London

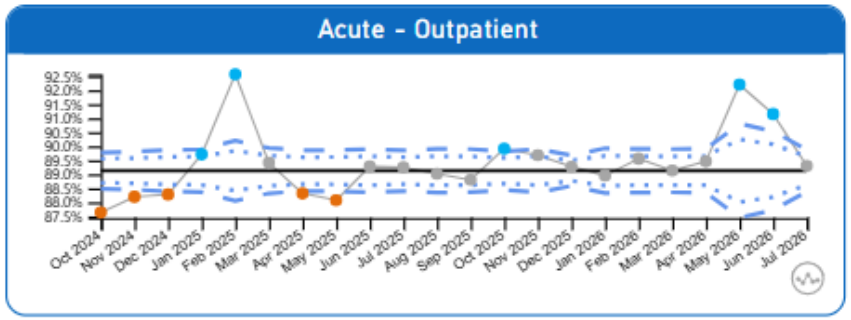
Experience of Care

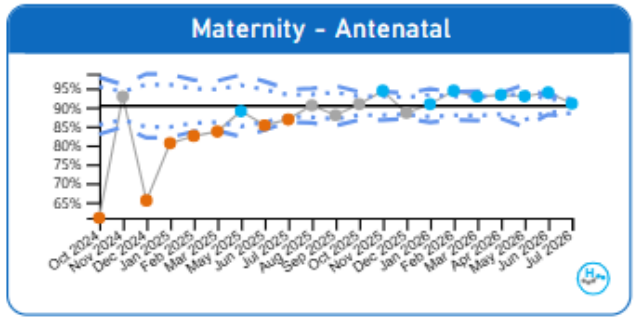
NHS Oversight Framework Experience of Care Domain Scores				
Barts	BHRUT	ELFT	Homerton	NELFT
2 – Above Average	4 – Low Performing	3 – Below Average	1 – High Performing	1 – High Performing

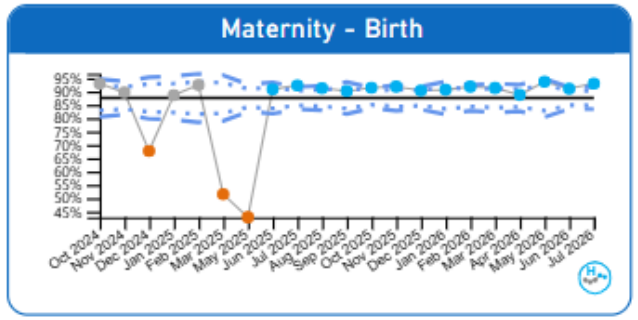
Effectiveness & Experience: Friends & Family Test (FFT)

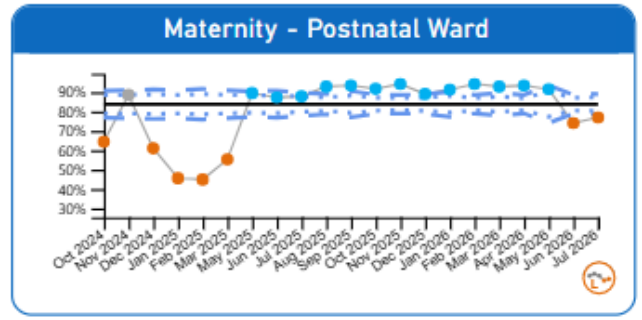


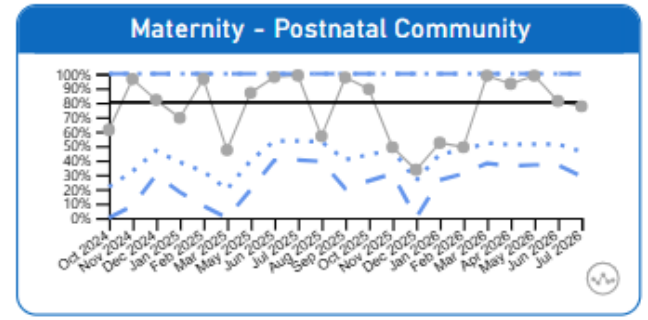












Narrative

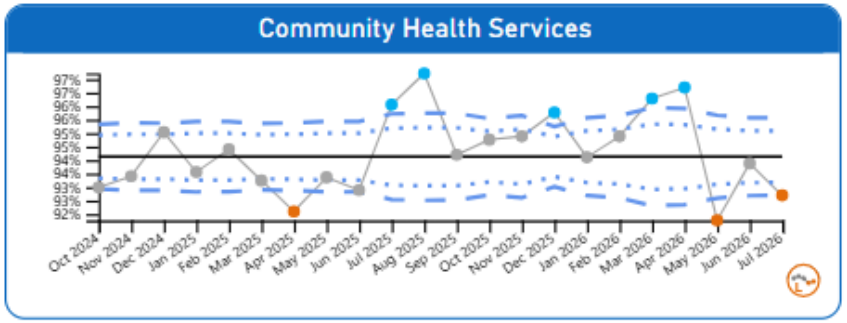
FFT scores for A&E have shown a downward trend throughout this financial year, with negative special cause variation in June & July of 2026, where we would normally see positive scores returning following winter pressures. Work will be undertaken via the System Quality Group to explore the impact of the recent heatwaves on the quality & safety of services across NEL. Following a May 2026 inspection, the CQC downgraded Newham Hospital's A&E to 'Requires Improvement' due to operational pressures affecting timely care, despite a positive patient-centred culture. Barts has introduced a dedicated waiting-room nurse, enhanced deterioration checks and escalation, standardised assessment documentation, and a hospital-wide flow and triage improvement programme.

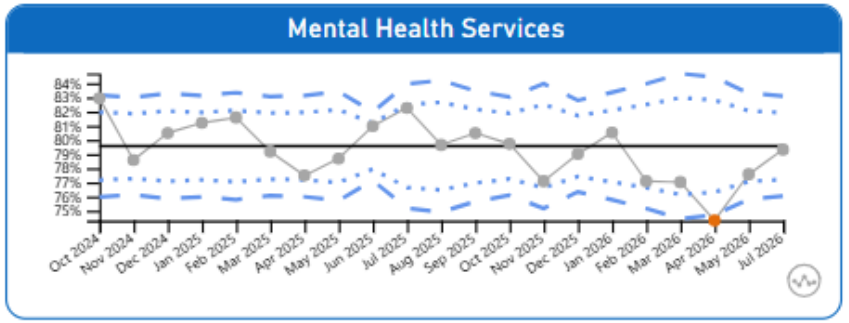
Acute inpatient FFT scores have returned to the mean following special cause variation in May. The CQC Inpatient Survey was published in August 2026, with worse than expected experience reported within BHRUT; we will work to gain assurance regarding planned improvements in this area. Outpatient services have shown positive special cause variation through May and June 2026; this cannot be attributed to one part of the system but may relate to a lower response rate across a number of our providers during this time.

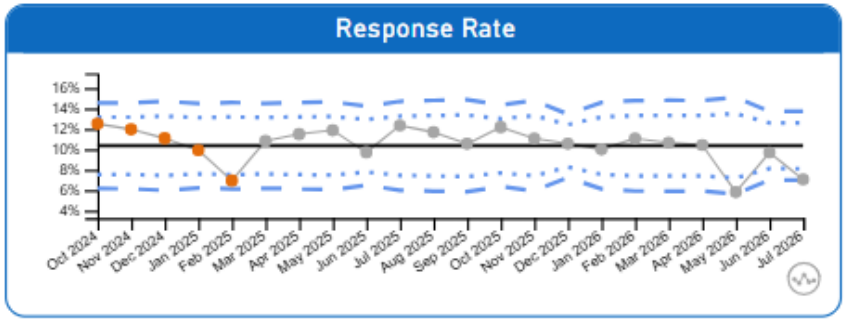
FTT for Maternity services, particularly antenatal and postnatal (ward) are showing higher scores across 2025/26 in comparison to 2024/25. Negative special cause variation in June & July 2026 for postnatal ward FFT scores is likely due to data suppression for one provider, meaning their scores are not counted as part of this data. Further benchmarking will be undertaken to ensure ongoing improvements or deterioration can be accurately detected now that FFT scores across these services are more controlled. Responses related to postnatal community services are the least controlled across maternity and likely reflect larger variation in response rates month to month.

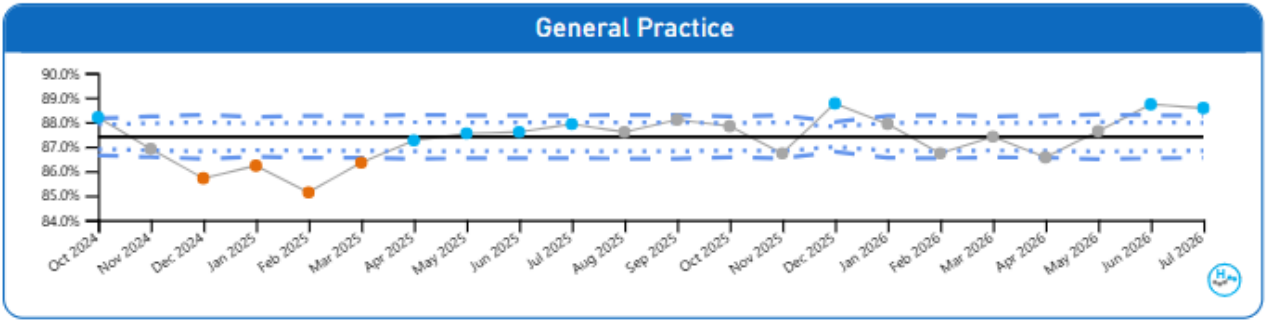
Data source: NHS England » Friends and Family Test data

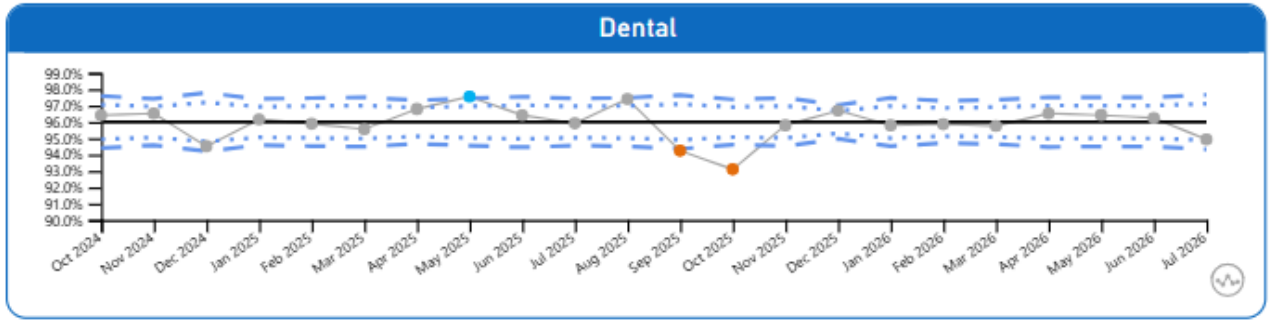
Effectiveness & Experience: Friends & Family Test (FFT)











Narrative

FFT scores for General Practice have shown positive special cause variation across June & July 2026. The latest GP Patient Survey findings show a five-year positive trend in satisfaction rates related to getting through to the GP practice via the phone, experience with receptionists, confidence in professionals and the numbers having conversations about their conditions. Despite this, results of the GP Patient Survey shows North East London continues to perform lower than other London ICBs across several core metrics, including overall satisfaction with GP, Pharmacy and Dental services, confidence in professionals and patients feeling they were treated with care and concern and that their needs were met.

Both community and mental health services have seen negative special cause variation in Q1 of 2026/27; response rates for these areas have been particularly low and therefore this data should be interpreted with caution. Response rates are often lower across our mental health and community health services due to patients sometimes being harder to reach, and are less likely to have a single, clearly defined episode of care than patients in acute hospital settings.

Data source: NHS England » Friends and Family Test data

NHS
North East London

Effectiveness of Care & Population Health

NHS Oversight Framework Effectiveness of Care & Population Health Domain Scores				
Barts	BHRUT	ELFT	Homerton	NELFT
2 – Above Average	2 – Above Average	1 – High Performing	2 – Above Average	1 – High Performing

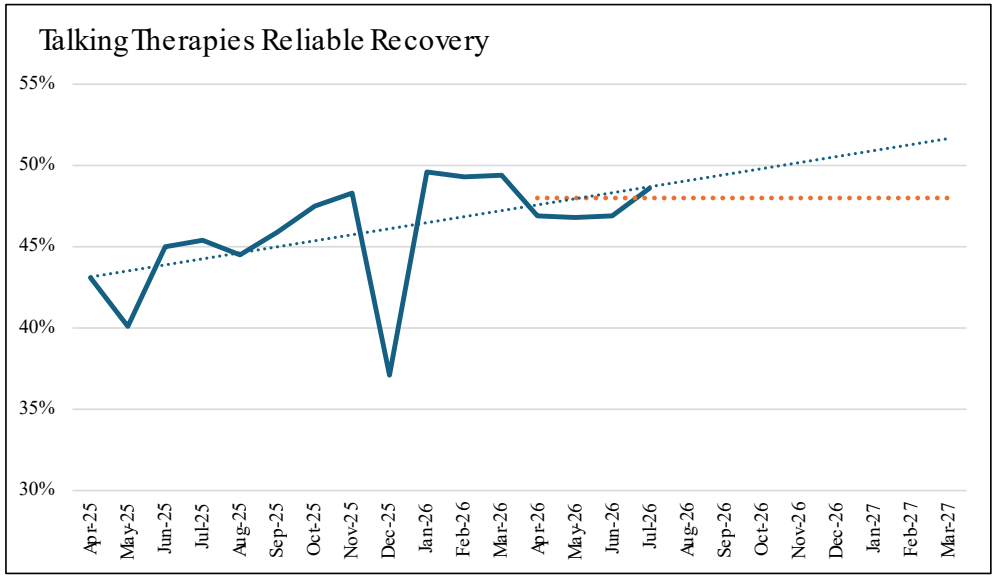
Mental Health | talking therapies reliable recovery

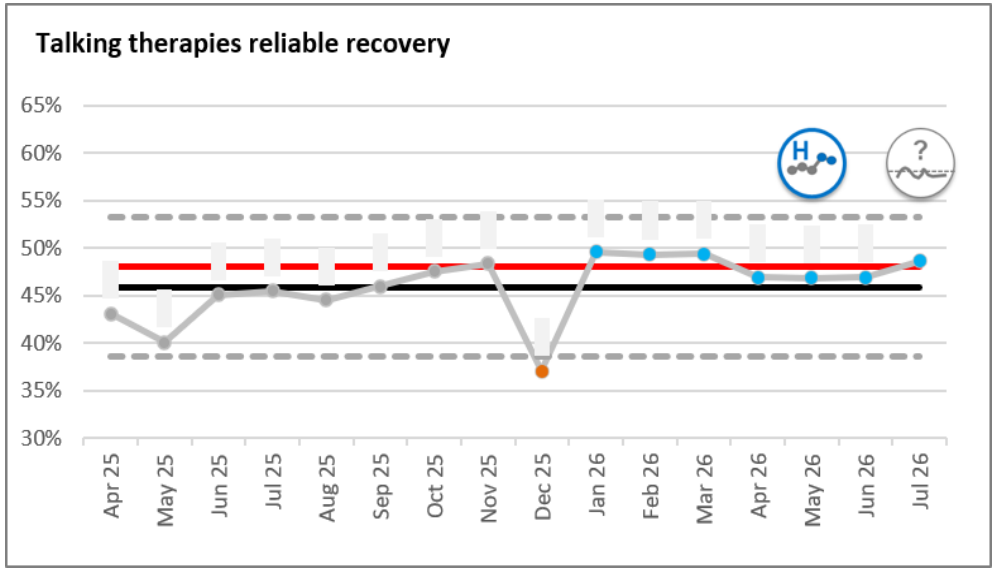
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	45.4%	46.9%	48.6%	48.0%	48.0%

Change / Variance		
Prior Month	Prior Year	Plan
1.7%	3.1%	0.5%

- NEL ICB reliable recovery performance stable (SPC).
- In Jun-26 the ICB delivered against planned reliable recovery performance.
- The NHS England Talking Therapies Deep Dive provides an opportunity to understand the underlying drivers of performance, validate recovery plans and support sustainable improvement across NEL.

Commissioner action: Work collaboratively with NHSE and providers to implement the agreed actions arising from the Talking Therapies Deep Dive, provide strategic commissioning oversight of provider improvement plans, and strengthen assurance to support sustainable improvements in reliable recovery performance. Review and understand current service models, workforce approaches and barriers across boroughs to identify variation and opportunities for improvement, ensuring equitable access to Talking Therapies services across NEL. Use this intelligence to inform future commissioning intentions, including consideration of both place-based and NEL-wide delivery models.





London Rank: NEL ICB 2; SEL ICB 3; SWL ICB 2; NWNL ICB 4
National Rank: London 2; East of England 4; Midlands 7; North East & Yorkshire 2; North West 6; South East 6; South West 2
Data source: Published Mental Health data - NHSE

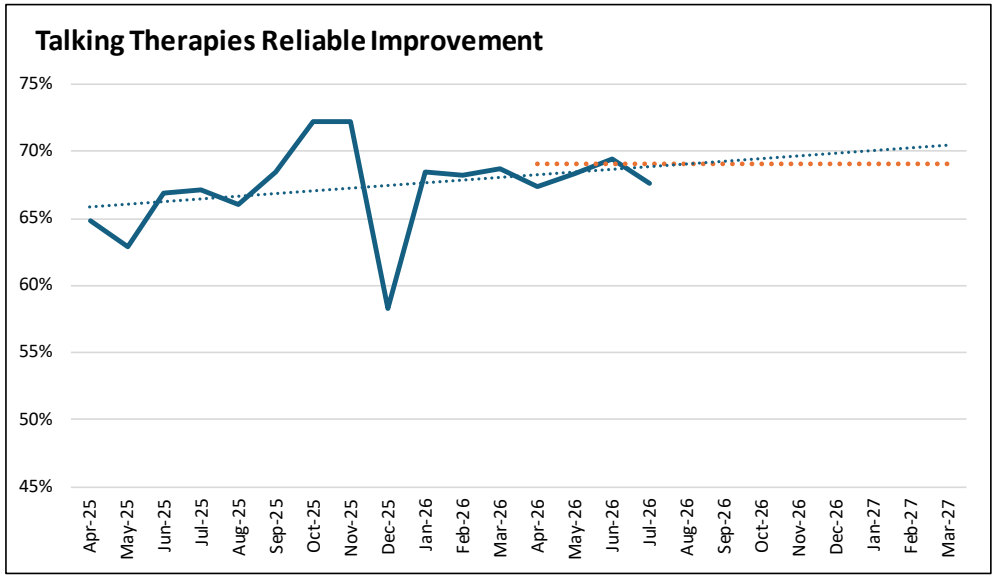
Mental Health | talking therapies reliable improvement

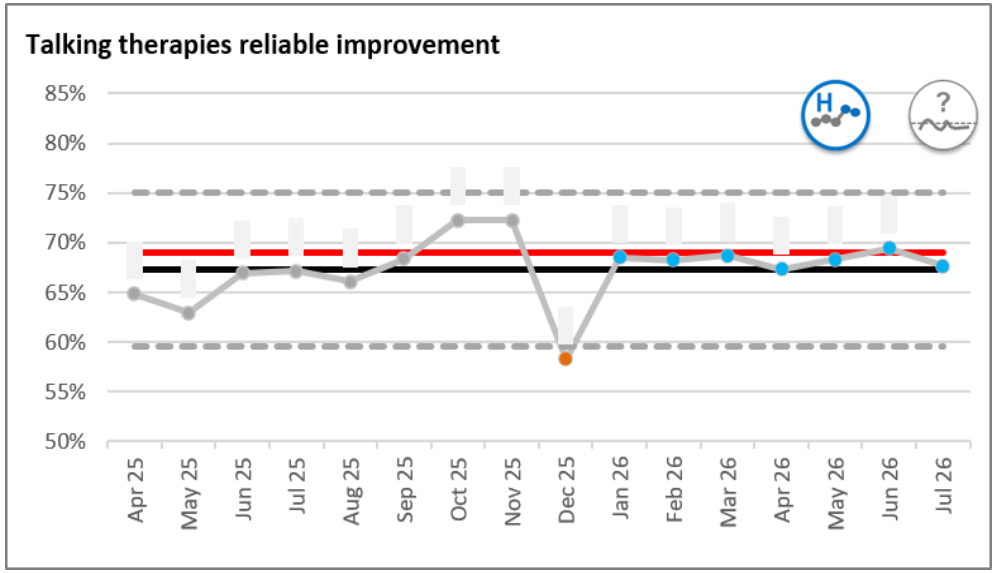
	Prior Year	Prior Month	Current Month	Month Plan	Year End Target
	Jul-25	Jun-26	Jul-26	Jul-26	Mar-27
NEL ICB	67.1%	69.4%	67.6%	69.0%	69.0%

Change / Variance		
Prior Month	Prior Year	Plan
-1.8%	0.5%	-1.4%

- NEL ICB reliable improvement performance stable (SPC).
- In Jun-26 the ICB did not meet the planned reliable improvement performance trajectory.
- The NHSE Talking Therapies Deep Dive is providing an opportunity to understand the drivers of performance variation and support reliable, sustainable improvement across NEL.

Commissioner action: Work collaboratively with NHSE and providers to implement the agreed actions arising from the Talking Therapies Deep Dive, provide strategic oversight of provider improvement plans, and strengthen performance assurance to support sustainable improvements in reliable improvement performance. Review and understand current service models, workforce approaches and barriers across boroughs to identify variation and opportunities for improvement, ensuring equitable access to Talking Therapies services across NEL. Use this intelligence to inform future commissioning intentions, including consideration of both place-based and NEL-wide delivery models.

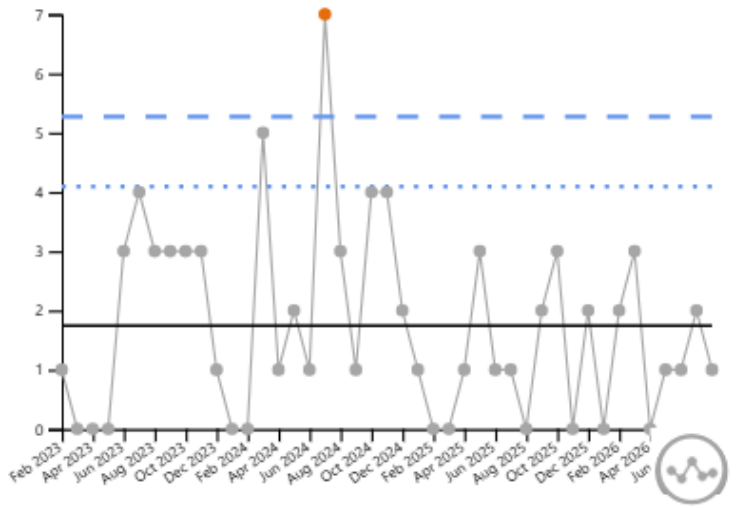




London Rank: NEL ICB 3; SEL ICB 2; SWL ICB 1; NWNL ICB 4
National Rank: London 4; East of England 6; Midlands 2; North East & Yorkshire 2; North West 6; South East 7; South West 4
Data source: Published Mental Health data - NHSE

Effectiveness of care & population health: Mortality

Number of PFDs



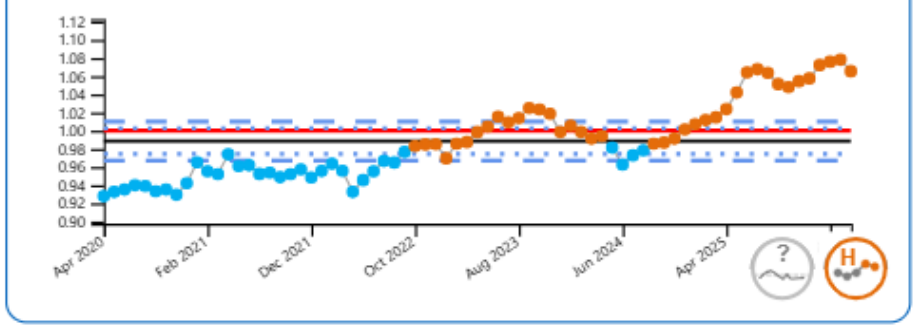
Narrative

A total of four Prevention of Future Death reports were received by five organisations across North East London Integrated Care Board in Q1 of 2026/27 including our acute providers, mental health providers and local authorities. One PFD related to a suicide, one to a homicide, one to a fall at home and one to drowning. Three of the deceased were adults, and one was a child. All were female.

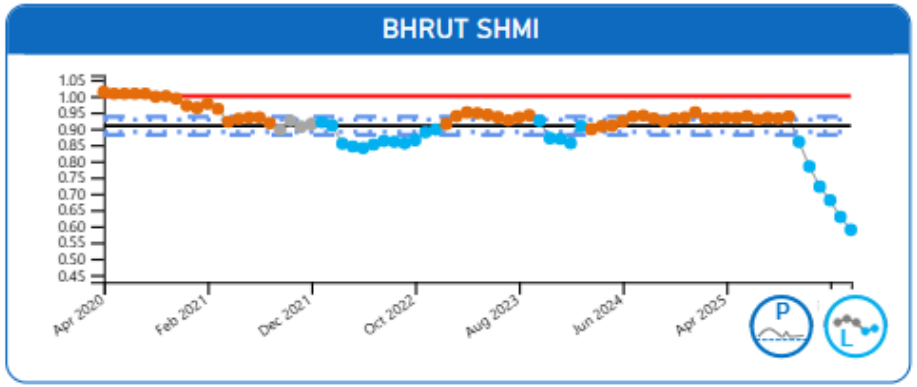
Two PFDs have been received by North East London providers so far in Q2 of 2026/27.

Providers are required to respond to Prevention of Future deaths detailing their immediate and ongoing improvement actions.

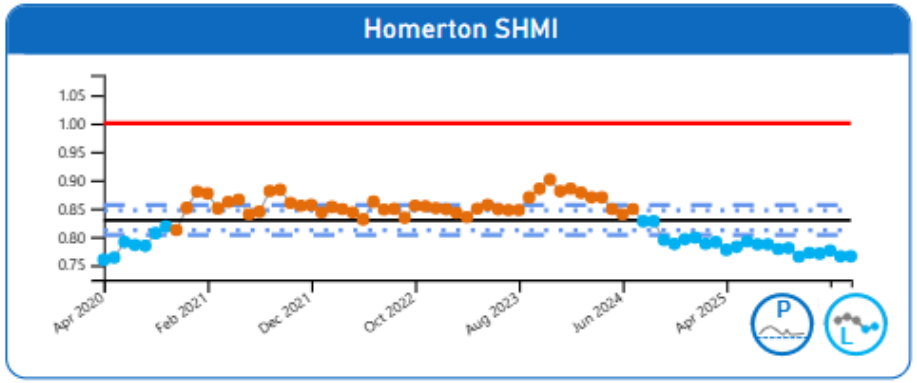
Barts SHMI



BHRUT SHMI



Homerton SHMI



Narrative

Both BHRUT and Homerton's SHMI are 'lower than expected'. BHRUT's SHMI has shown a steep decrease since November 2025, which is likely due to data quality issues related to EPR implementation. Work is being undertaken by the organisation to ensure this issue can be rectified.

While Barts SHMI is within expected range, colleagues from Barts presented at the ICB Quality Safety & Improvement Committee regarding the continued upward trend and negative special cause variation that was seen. In response, Barts presented on their SHMI position to the Quality, Safety & Improvement Committee. SHMI has increased over the past 18 months but remains within the expected range. Barts Health says that the increase is not due to more deaths but is linked to changes in recording Same Day Emergency Care (SDEC) activity, which is recognised by NHS England (NHSE) as having the potential to increase the SHMI. Committee members acknowledged the work taking place and processes implemented and requested that an update is presented at the September committee including updated guidance from NHSE if received.

Mortality will become a regular focus at our System Quality Group, where partners from across health, social care, public health and the wider ICS will be able to share insight and intelligence from Learning from Deaths processes, Prevention of Future Deaths and Patient Safety Incident Investigations to identify concerns or risks to quality and opportunities for improvement, coming together to develop system responses and improve the safety and quality of care we provide.

Data source: (PFDs) – Internal Log (SHMI) Summary Hospital-level Mortality Indicator (SHMI) - Deaths associated with hospitalisation - NHS England Digital

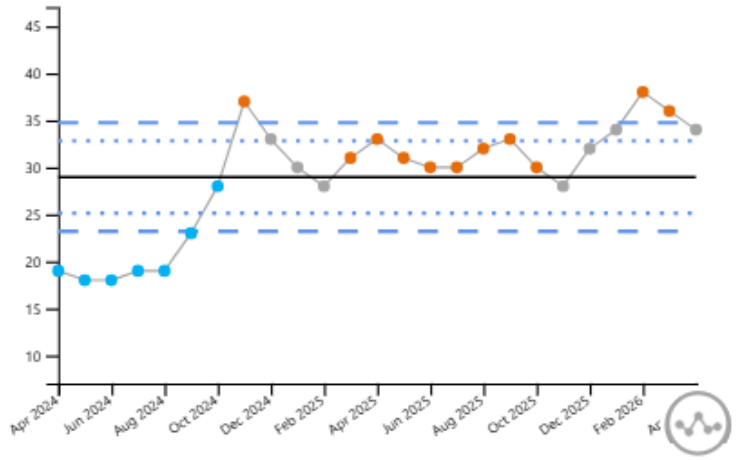
NHS
North East London

Patient Safety

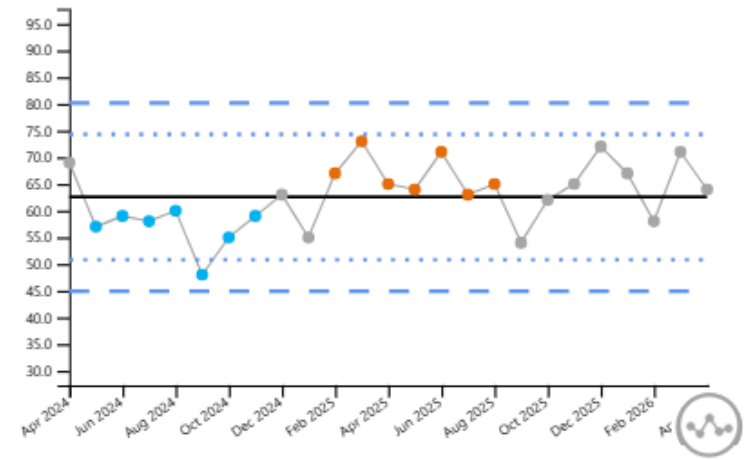
NHS Oversight Framework Patient Safety Domain Scores				
Barts	BHRUT	ELFT	Homerton	NELFT
4 – Low Performing	3 – Below Average	2 – Above Average	1 – High Performing	1 – High Performing

Safety: Maternity

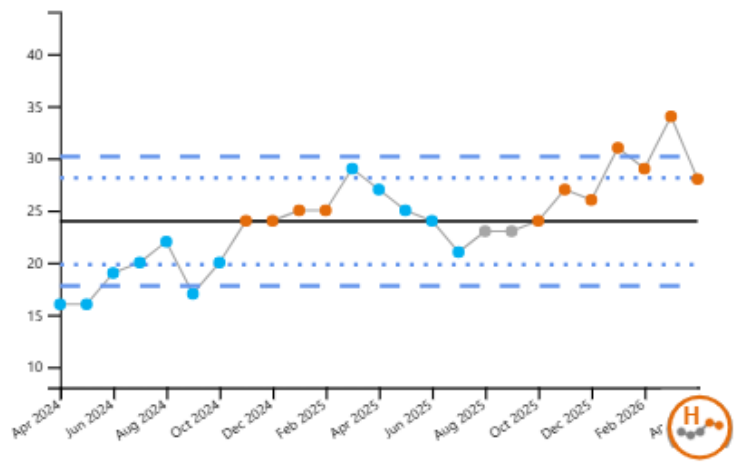
Rate of PPH of 1500ml or more



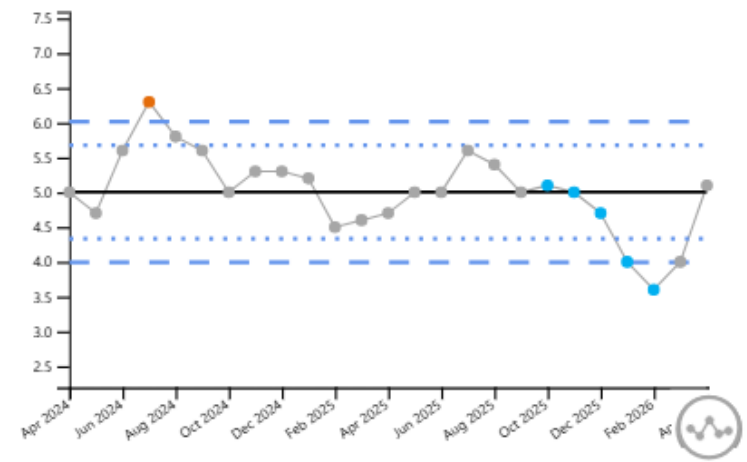
Rate of Pre Term Births



Rate of Grade 3/4 Tears



Rate of Readmissions



Narrative

The rate of PPH of 1500ml or more continues to sit above the mean, reflecting ongoing improvements in measurement of blood loss during birth due to involvement of some of our Trusts in the Obs UK trial. Whipps Cross at Barts have undertaken a harm review from before the trial, focusing on transfusion and post-delivery haemoglobin rates and are assured that there hasn't been an increase in harm. All providers have QI work to reduce PPH and review cases of blood loss >1,500ml. The LMNS are currently reviewing cases for emerging themes. Obstetric haemorrhage is now included in the Maternal Care Bundle, launched in March 2026, and forms part of Year 8 of the Maternity Incentive Scheme and the ten-point action plan following the Ockenden Report and Amos Review.

The rate of grade 3 and 4 tears has sat above the mean for the past six months, with special cause variation seen in March 2026; if this is sustained, there we will see a negative step change in performance. As part of implementing perinatal pelvic health services, all NEL maternity providers are implementing the OASI Care Bundle, a set of interventions to improve outcomes and reduce obstetric anal sphincter injuries (OASI), including 3rd- and 4th-degree tears.

The rate of pre-term births remains within expected variation and while there has been a downward trend in the rate of readmissions across winter 2025/26, this has now returned to the mean. Providers continue to monitor pre-term birth rates through their local maternity governance and quality improvement processes.

The rate of readmissions has seen a downward trend across winter 2025/26, however recent data has reverted to the mean. There is currently no clear explanation for the lower winter rates. Providers continue to monitor the data and review readmissions to identify any emerging themes or changes in practice that may help explain the variation.

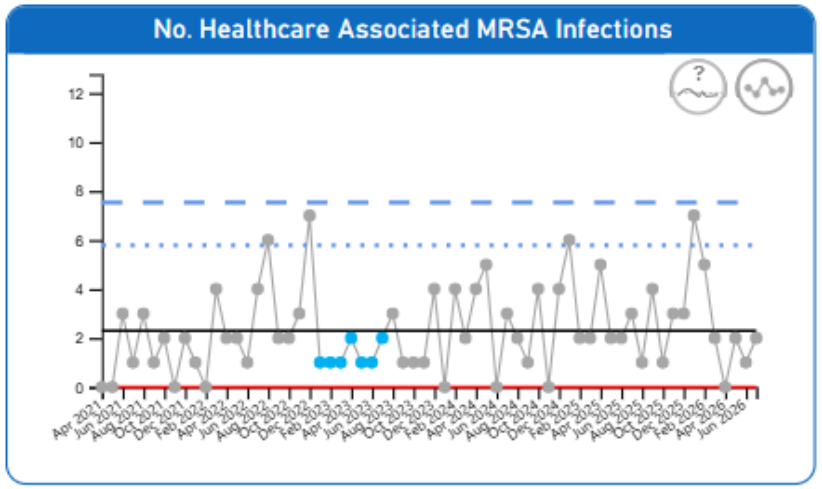
Data source: Maternity Services Monthly Statistics - NHS England Digital

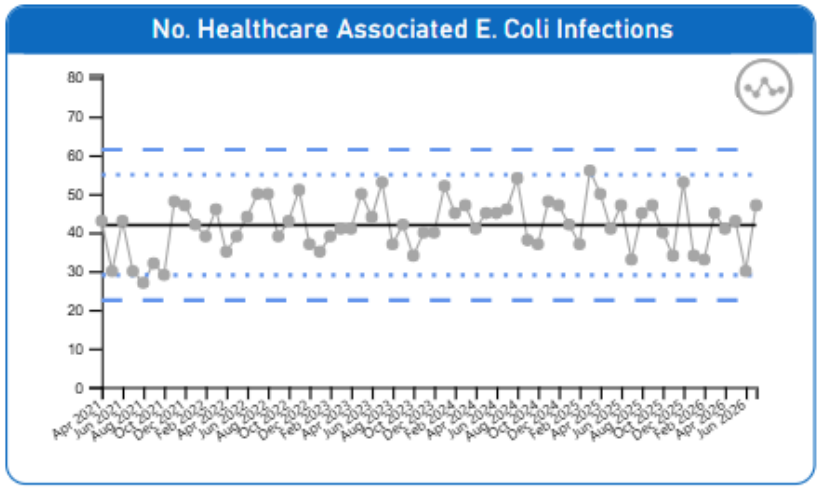
Safety: Infection Prevention & Control

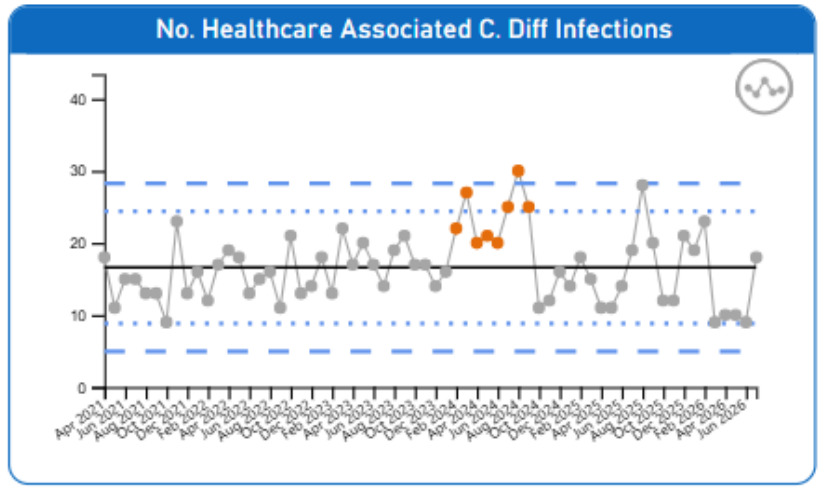
NHS Oversight Framework: No. MRSA Infection Score		
Barts	BHRUT	Homerton
4.00	3.60	2.24

NHS Oversight Framework: E.Coli Infection Score		
Barts	BHRUT	Homerton
2.00	3.00	4.00

NHS Oversight Framework: C.Diff Infection Score		
Barts	BHRUT	Homerton
2.00	2.00	2.00







Narrative

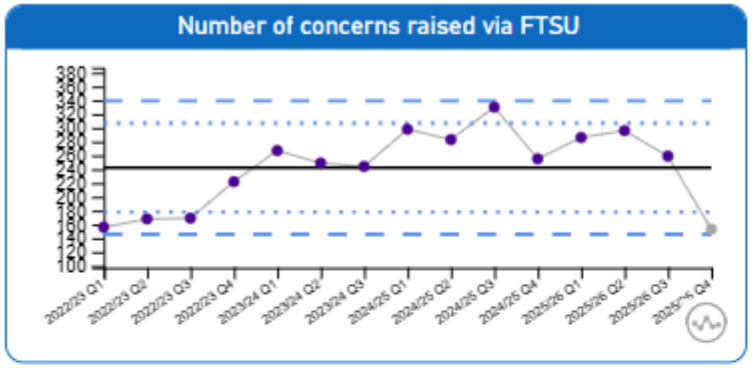
While numbers of healthcare associated MRSA infections remain within expected limits, and below the mean for the past five months, numbers are regularly above the threshold of 0. Model Health System benchmarking places NEL ICB as the ICB with the highest MRSA infection rates in London and second highest rates nationally. Acute providers have implemented MRSA improvement plans focused on enhanced screening and decolonisation, strengthened post-infection review processes, improved device management, and renewed emphasis on hand hygiene, environmental cleanliness and ANTT. Barts Health has established a formal trust-wide MRSA reduction programme, while Homerton has enhanced surveillance and investigation of neonatal MRSA acquisitions. Learning from all cases is reviewed through NEL IPC governance structures to support system-wide improvement.

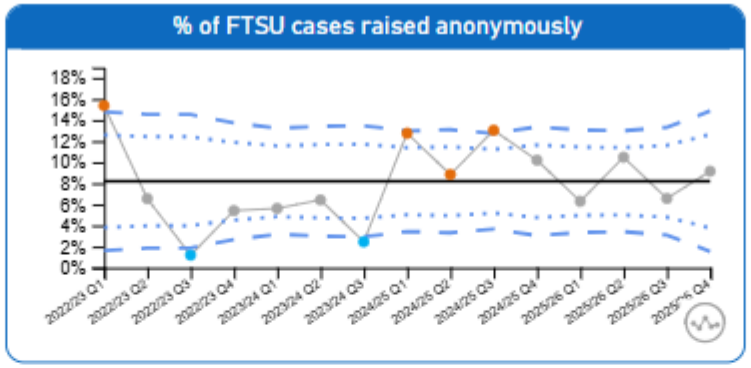
While numbers of healthcare-associated E.Coli infections remain within expected limits, Model Health System shows NEL ICB has the highest rates in London and third highest nationally. Barts numbers have sat below the mean for five out of the first six months of 2026; this has been supported by targeted GNBSI reduction work including enhanced post-infection review processes, improved catheter and device management, surveillance of infection sources, and quality improvement initiatives focused on preventing urinary tract-related bloodstream infections.

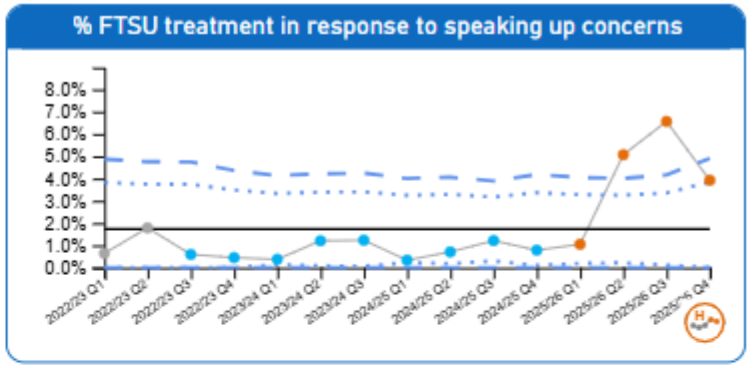
Numbers of healthcare associated C.Diff infections have sat below the mean between March and June 2026, following reduced control throughout 2025, however this has returned to the mean in July 2026. Healthcare-associated C. Diff infections have reduced following implementation of strengthened antimicrobial stewardship, enhanced post-infection review processes, improved environmental and infection prevention controls, and the establishment of the NEL C. difficile Collaborative. Providers have worked together to standardise practice, align policies with national guidance, review every case for learning, and improve antibiotic prescribing, contributing to a sustained reduction in infections across the system. As of June 2026, Model Health System shows NEL has the lowest C.diff infection rate nationally.

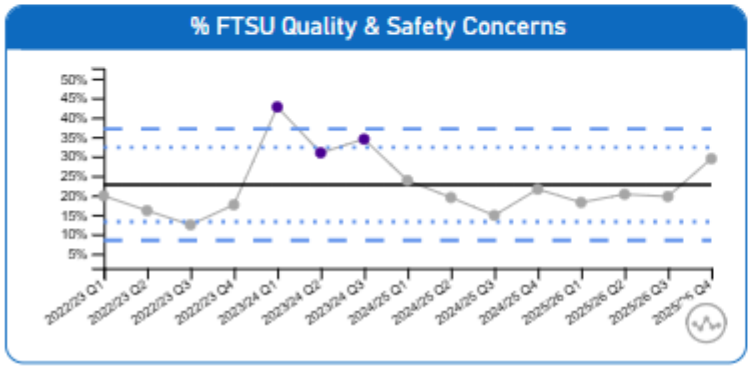
Data source: Healthcare-associated infections | UKHSA data dashboard

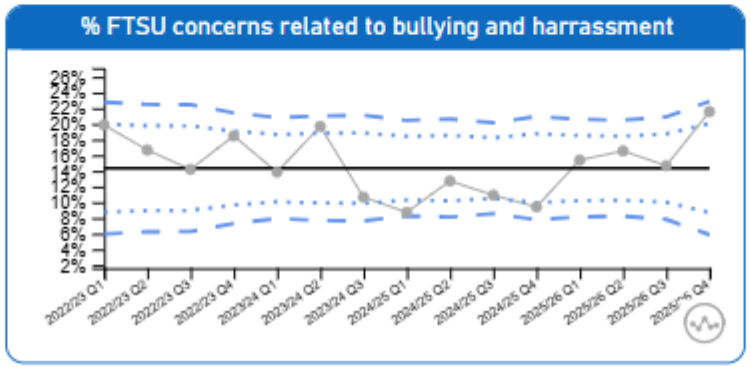
Safety: Speaking Up

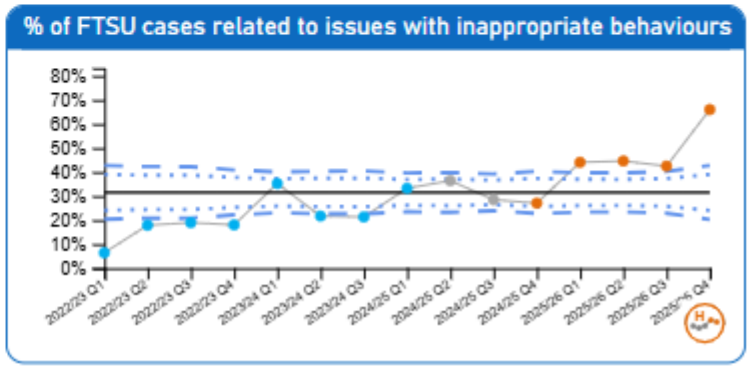












Narrative

As this data is published quarterly, there are no new insights to share since the previous report.

It is important to note that the National Guardian's Office (NGO) who collect and publish this data closed on the 30 June 2026. Responsibility for the national collection and publication of speaking up data has now shifted to NHSE who will continue to collect data from Trusts and ICBs but have decided to pause collections for primary care and independent healthcare providers while the approach is reviewed, although these organisations should continue to collect and analyse data locally. Data will likely be shared on Model Health System, however at this point we're not sure if that information will be shared in as detailed as a way as it is by the NGO.

Data source: NHS England » Friends and Family Test data

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Additional Materials

Reading the Report

Statistical Process Control (SPC) Charts

The Integrated Performance & Quality Report is developed in line with the Making Data Count guidance, which makes the case for and demonstrates the use of Statistical Process Control. Statistical Process Control (SPC) is an analytical technique that plots data over time. It helps us understand variation and guides us to take the most appropriate action. It is widely used to understand whether change results in improvement and whether quality is controlled. Statistical Process Control charts can also be used to provide assurance, demonstrating where performance does not meet intended targets.

The use of Statistical Process Control charts can:

- Alert us to a situation that may be deteriorating;
- Show us where a situation/system is improving;
- Demonstrate how capable a system is of delivering standards or targets; and
- Detail whether our processes and ways of working are reliable and lead to expected results.

While SPC aids with interpretation, examination, understanding and analysis of the data by a subject matter expert is still required.

Key

Most charts contained within this report have a corresponding icon which highlights what is happening in the system. Most importantly:

- Blue icons indicate that we are either assuring or improving quality within the system
- Orange icons indicate that we are not assuring quality within the system, or that performance is declining



NHS Oversight Framework Scores

The Integrated Performance & Quality Report also includes scores pulled from the NHS National Oversight Framework. The NHS Oversight Framework (NOF) provides NHS England's national assessment of provider performance. Organisations are scored across five domains: Access to Services, Effectiveness and Experience, Patient Safety, People and Workforce, and Finance and Productivity. Domain scores range from 1 (high performing) to 4 (low performing) and provide a comparative view of performance against national peers. These scores contribute to the overall segmentation of providers and help inform the level of support, improvement activity and oversight required.

NHS
North East London
Integrated Care Board

NHS North East London ICB board

30 September 2026

Title of report	Place-based performance reporting
Author	Graham Crawford – Intelligence and Insight
Presented by	Ralph Coulbeck – Chief Strategy Officer
Contact for further information	Graham Crawford – Intelligence and Insight, graham.crawford@nhs.net
Executive summary	The latest iteration of the North East London (NEL) Place Based Reporting Dashboard provides a more comprehensive view of performance, utilisation and variation across the seven NEL places. The dashboard brings together population characteristics and key indicators across primary care, community and mental health services, urgent and emergency care, and admitted patient care to support a more place-based understanding of demand, service use and outcomes.
Action / recommendation	The Board is asked to: • Review the latest iteration of the Place Based Reporting Dashboard. • Consider whether the information presented provides sufficient insight into place-level performance and variation. • Identify any specific domains, pathways or population groups that would benefit from deeper analysis in future iterations of the reporting framework. • Provide feedback on the continued development of the dashboard to ensure it remains focused on the issues most relevant to strategic oversight and system improvement.
Previous reporting	Wireframe of a place-based report was presented to a previous ICB Board and Quality, Safety and Improvement Committee (QSIC).
Next steps/ onward reporting	The future iterations of the Place Based Performance report will be presented to QSIC and future ICB boards.
Conflicts of interest	No conflicts of interest have been identified.
Strategic fit	The ICS aims this report aligns with are: • To improve outcomes in population health and healthcare • To tackle inequalities in outcomes, experience and access • To enhance productivity and value for money
Impact on local people, health inequalities and sustainability	Reporting helps to understand variation of service provision across the diverse and growing population of NEL. The aim is for the place-based reporting alongside other analytics to

	ensure commissioning decisions are driven by the most relevant and useful insight.
Has an Equalities Impact Assessment been carried out?	Not applicable
Impact on finance, performance and quality	Not applicable
Risks	Organisational change has delayed delivery and engagement of the place-based performance report. Future iterations will be driven by the relative priority of this development alongside other reporting requests. There are limitations to the analysis conducted. The NEL patient level linked dataset comprises health and demographic data sources – where scope extends to wider determinants – we are limited to what is held in core health datasets. A good example of limited data is our special educational needs and disabilities (SEND) population – ICB analysts are seeking to secure data flows to support our future special educational needs and disabilities SEND strategic commissioning role. Data quality, electronic patient record implementations and recording patterns in providers are potential sources of variation across or places. Surfacing reports in this way highlight areas of discussions at provider data assurance meetings. The Clinical Effectiveness Group (CEG), a source of several primary care reports for example - health checks of severe mental illness registered populations - were unsuccessful in the GP facilitation procurement and so several primary care reports will require developing from scratch to be reflected in this report.

1.0 Background and purpose of the report

1.1 This report provides the Board with an overview of the latest iteration of the North East London (NEL) Place Based Reporting Dashboard. The intention is to provide members with a concise summary of performance and utilisation across the seven places within NEL, drawing together key contextual, operational and system flow indicators into a single reporting framework.

1.2 The report seeks to move beyond the presentation of traditional performance indicators, spotlight trends and areas requiring further consideration. It also provides an opportunity for Board members to comment on the usefulness of the current reporting approach and identify areas where additional analysis or reporting would add value.

1.3 The Board is asked to:

- Review the latest iteration of the Place Based Reporting Dashboard.
- Consider whether the information presented provides sufficient insight into place-level performance and variation.

- Identify any specific domains, pathways or population groups that would benefit from deeper analysis in future iterations of the reporting framework.
- Provide feedback on the continued development of the dashboard to ensure it remains focused on the issues most relevant to strategic oversight and system improvement.

2.0 Key messages

2.1 The aim of the dashboard is to highlight variation across our places. There are 5 domains – the population can be found at the top of the report. Then metrics and indicators related to 4 areas of delivery – primary care, community/mental health, urgent emergency care and admitted patient care.

2.2 The report continues to evolve but it limited by the linked patient data set. Through the vagaries of data flow and national reporting cadence not all metrics reflect the latest reporting month and delays in provider clinical coding mean readmissions and avoidable admissions are a further reporting month behind. A more detailed definition of the metric definition and reporting cadence is available.

3.0 Place-based performance report

3.1 Population context – at the top of each page are the population characteristics showing population size and trend in GP registrations, deprivation, levels of multimorbidity, prevalence of severe mental illness and the proportion of residents whose first language is not English. These contextual factors are important when interpreting operational performance and utilisation patterns and reinforce the need for place-specific solutions rather than a single system-wide approach.

3.2 The data highlights significant variation in Core20 populations and indicators associated with health inequalities, underscoring the importance of maintaining a focus on equitable access and outcomes across all services.

3.3 Primary care – Primary care indicators show variation in both access and digital engagement across places. NHS App uptake continues to differ considerably between places Havering is 3% higher than the NEL uptake value, with Barking and Dagenham 6% lower. There will opportunities to share learning from areas achieving higher levels of digital adoption and our whether there are cultural, economic factors involved. Digital engagement is increasingly important as services seek to improve self-management, communication and access to care.

3.4 Measures relating to same-day access similarly demonstrate variation across places. City and Hackney are delivering the highest percentage levels of same day access, the % of clinically urgent appointments dealt with on the day indicators is 12% higher than the NEL level. Whilst some degree of variation is expected because of differing population need and service models, the data suggest there may be opportunities to better understand the factors underpinning performance and identify approaches that could be adopted more widely. Havering are achieving better cervical screening rates than the rest of NEL.

3.5 Community and mental health – Community and mental health activity continues to represent a significant component of place-based service delivery. The place pages show variation in community service contact rates, mental health service utilisation and measures associated with serious mental illness. Some of this variation is likely to reflect differences in underlying need and age of the relative place populations – with community particularly supporting the older populations. That said, City and

Hackney has the highest community contact rate, closely followed by Havering (as expected with a young (2nd largest proportion of young people under 18) and the largest older adult population) the places in NEL. Whilst Waltham Forest registered population receives the highest mental health contact rate.

3.6 The graph for physical health checks has not been updated but has been left in the report as this measure is used in the national oversight framework and understanding local variation will be important to address improvement in this indicator.

3.7 **Urgent and Emergency Care** – Urgent and emergency care remains one of the most significant areas of variation across places. Differences in emergency department attendance rates persist, reflecting a combination of population need, health inequalities, access to alternative care pathways and local service utilisation patterns. Outliers in 12-hour ED waits are driven by relative pressure in local ED departments. Barking and Dagenham registered patients continue to utilise at a higher rate per capita than other places in NEL.

3.8 Interestingly there is very little positive or negative correlation between metrics. For example, ED rates bare little relationship to same day access rates/availability, similarly mental health in ED to SMI or mental health contact rates.

3.9 **Admitted patient care** - Elective care indicators provide a useful overview of planned activity and demand across places. The inclusion of inequalities measures within the elective care section is particularly important and supports the system's strategic objective of reducing unwarranted variation in access and outcomes.

3.10 As the reporting framework continues to mature, there may be scope to strengthen the narrative around elective care inequalities, particularly by identifying where specific population groups experience longer waits, lower rates of access or different outcomes.

4.0 Risks and mitigations

4.1 The place-based reporting is looking to focus on the drivers of utilisation/demand/experience and evolve away from the traditional constitutional measures where coding practice has had greater scrutiny and focus and therefore has levels of greater assurance. Some of the variation in the report is driven by data quality and clinical recording practices in our providers. Although the impact of the BHRUT EPR implementation can't be seen in the sparklines/trend, apart from the 12-hour waits in ED where in November 25 you can see a dip in recording levels. To mitigate that risk the ICB will use contracting data assurance meetings with providers to challenge recording practice to provide the ICB with greater levels of assurance.

4.2 There is a national programme to cleanse GP lists. The full impact of the reduction in our registered population on our reporting has not been completed. The cleanse is expected to take up to a year and can be seen in the GP registration sparklines, this influences the denominator in the 'rate' indicators.

4.3 Formatting in the attached report has been distorted by the creation of the dashboard and through pdf conversion. Future iterations of the report will be amended and shared via the NEL Information Portal to minimise distortion in the final report.

5.0 Conclusion / Recommendations

5.1 The latest iteration of the Place Based Reporting Dashboard represents a significant step forward in developing a comprehensive and consistent view of performance across North East London places.

5.2 The dashboards bring together a broad range of indicators into a concise format with the aim of providing a useful basis for understanding variation between places and identifying areas for further investigation.

5.3 The initial thoughts of the reporting team is the reporting framework should evolve to support:

- Additional inequalities reporting.
- Identification of actionable insights rather than data alone.
- Supported by other reporting that provides an enhanced understanding of patient flow and pathway utilisation by place.

5.4 The board is asked to:

- Review the latest iteration of the Place Based Reporting Dashboard.
- Consider whether the information presented provides sufficient insight into place-level performance and variation.
- Identify any specific domains, pathways or population groups that would benefit from deeper analysis in future iterations of the reporting framework.
- Provide feedback on the continued development of the dashboard to ensure it remains focused on the issues most relevant to strategic oversight and system improvement.

6.0 Attachments

6.1 Place Based Performance Report – August 26

7.0 End

Graham Crawford - 9 September 2026.

Place Based Report - August 2026

Barking & Dagenham

NHS North East London

Registered Patients @ Aug-26
254,651
change -212 | 6-month trend | 7th in NEL

Housebound @Aug-26
1449 | -2 diff | 0.57% | 7th in NEL | 7th in NEL

Main language not English
79,743 | 6th in NEL | -107 diff | 31.31% | 5th in NEL

NEL Health & Care partnership priorities

Children	LTC	SMI	Core 20
25.54% (1st)	30.22% (2nd)	0.96% (6th)	27.17% (4th)

KPI KEY:
Metric Name
Place Value (NEL Value)
Trend ▲ 5% change since last period (NEL Rank)

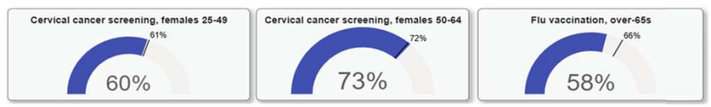
⚠ *data cadence for certain metrics vary*

NHS App Uptake
67.00% | 6-month trend | 73.00%
Trend ▲ | 0.98% Change in mth | (7th) Rank

% clinically urgent appts dealt with on day
67% | 6-month trend | 71%
Trend ▲ | 0.29% Change in mth | (6th) Rank

Same Day Access
46.20% | 6-month trend | 46.12%
Trend ▼ | -0.25% Change in mth | (4th) Rank

Cervical cancer screening and flu vaccination coverage, benchmarked against London average



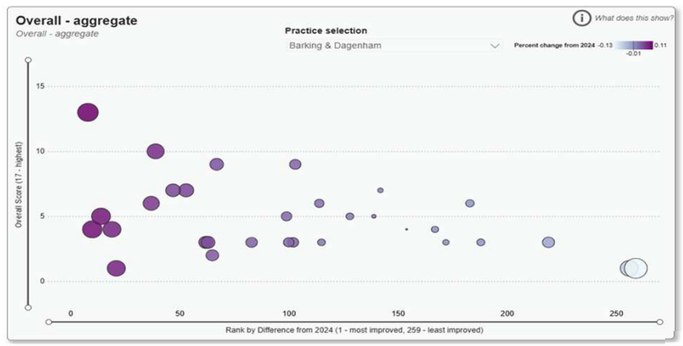
NEL average - 62% , 75% and 58% - Havering best performing place

Top 3 Most Improved

Practice Name	Difference (%) from 2024
Dr Bi Jalaval's Practice	172.2%
Dr M Fateh's Practice	153.1%
Salisbury Avenue Surgery	141.9%

Top 3 Least Improved

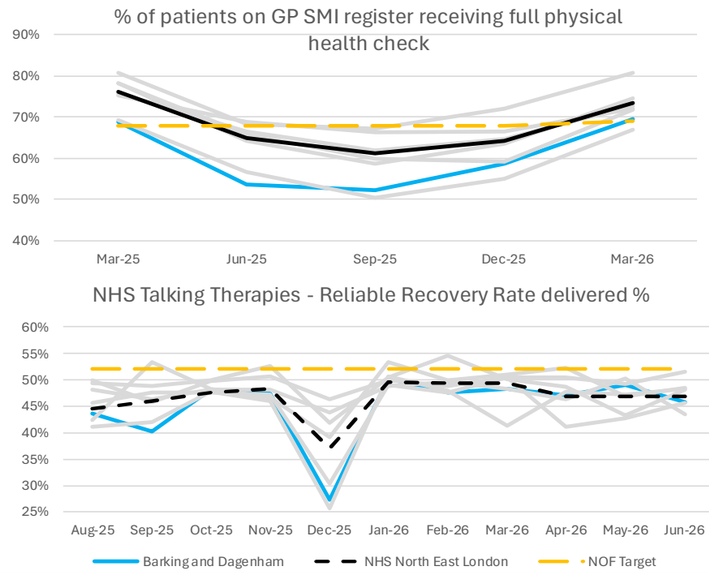
Practice Name	Difference (%) from 2024
John Smith Medical Centre	-212.3%
Green Lane Surgery	-126.6%
Highgrove Surgery	-53.0%



monthly rate of comm. contacts per 1,000
75.17 | 6-month trend in contacts | 76.35
Trend ▼ | -1.59 Change in mth | (4th) Rank

Monthly rate of MH contacts per 1,000
43.64 | 6-month trend in contacts | 33.39
Trend ▲ | 1.66 Change in mth | (2nd) Rank

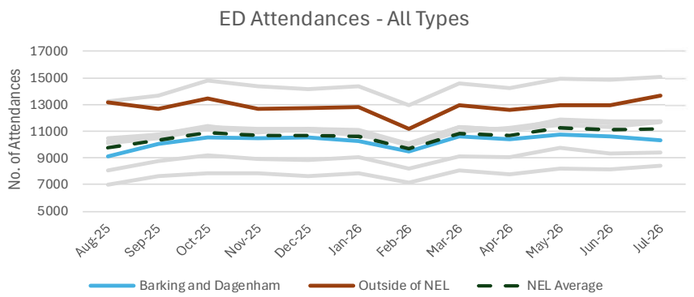
Prevalence of Learning Disability per 1,000
5.92 | 6-month trend of recorded LD | 5.19
Trend ▲ | 0.03 Change in mth | (1st) Rank

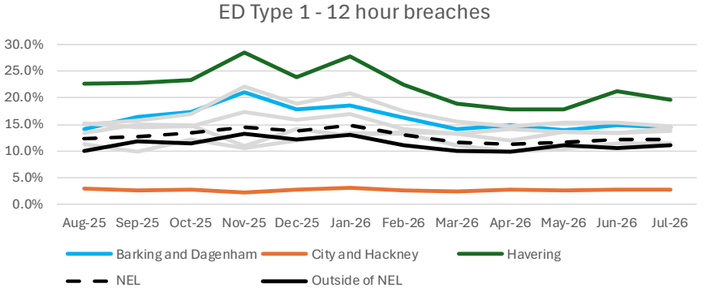


Mental Health Attendance in ED
134 | 6-month trend | 1217
Trend ▲ | 5 Change in mth | (6th) Rank

Self Harm Attendance in ED
38 | 6-month trend | 287
Trend ▼ | -13 Change in mth | (5th) Rank

Monthly ED rates per 1000 patients
53.1 | 6-month attendance trend | 42.9
Trend ▼ | -0.22 change in mth | (1st) Rank





Readmissions
766 | 6-month trend | 4274
Trend ▼ | -43 Change in mth | (2nd) Rank

Avoidable admissions @Jun26
123 | 6-month trend | 992
Trend ▼ | -19 Change in mth | (2nd) Rank

Monthly rate of elective IP adm per 1,000
1.24 | 6-month trend in elective IP admissions | 1.32
Trend ▲ | 0.29 Change in mth | (4th) Rank

Inequality profile of the admitted waiting list for B&D's registered patients



Place Based Report - August 2026

City and Hackney

NHS North East London

Registered Patients @ Aug-26
351,416
change +74 | 6-month trend | 4th in NEL

Housebound @Aug-26
2816 (1st in NEL) | 0 diff | 0.80% (2nd in NEL)

Main language not English
81,963 (5th in NEL) | -199 diff | 23.32% (6th in NEL)

NEL Health & Care partnership priorities

Children	LTC	SMI	Core 20
18.88% (6th)	28.09% (6th)	1.46% (1st)	40.15% (3rd)

KPI KEY:
Metric Name
Place Value (NEL Value)
Trend ▲ 5% change since last period (NEL Rank)

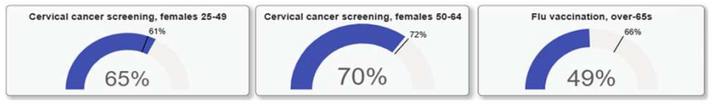
⚠ *data cadence for certain metrics vary*

NHS App Uptake
69.00% | 6-month trend | 73.00%
Trend ▲ | 0.64% Change in mth | (6th) Rank

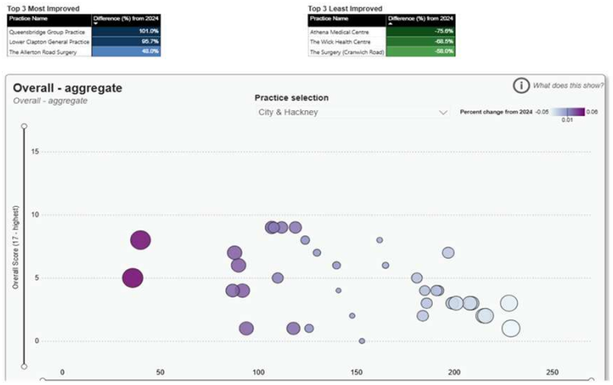
% clinically urgent appts dealt with on day
83% | 6-month trend | 71%
Trend ▼ | -0.73% Change in mth | (1st) Rank

Same Day Access
54.83% | 6-month trend | 46.12%
Trend ▼ | -0.37% Change in mth | (1st) Rank

Cervical cancer screening and flu vaccination coverage, benchmarked against London average



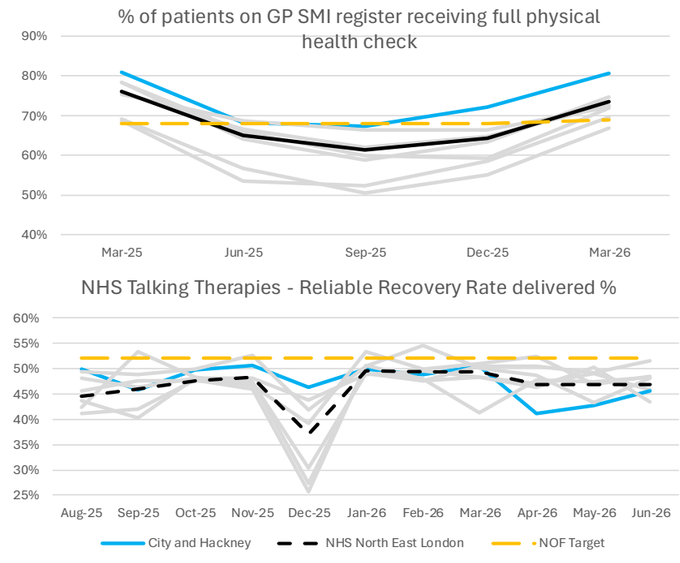
NEL average - 62% , 75% and 58% - Havering best performing place



monthly rate of comm. contacts per 1,000
92.68 | 6-month trend in contacts | 76.35
Trend ▼ | -4.54 Change in mth | (1st) Rank

Monthly rate of MH contacts per 1,000
29.83 | 6-month trend in contacts | 33.39
Trend ▲ | 7.22 Change in mth | (5th) Rank

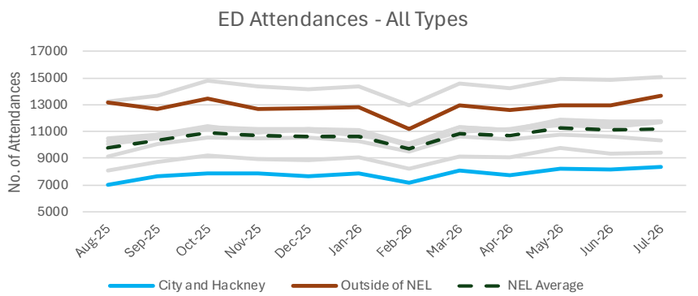
Prevalence of Learning Disability per 1,000
4.91 | 6-month trend of recorded LD | 5.19
Trend ▲ | 0.00 Change in mth | (6th) Rank

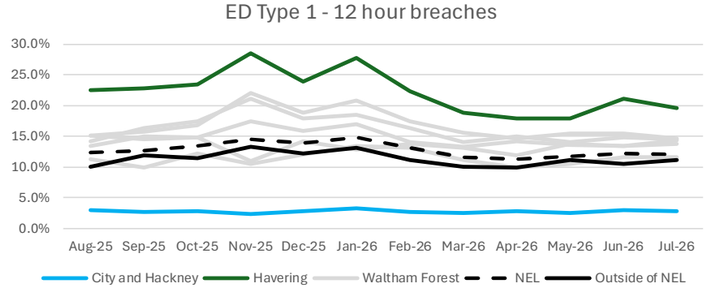


Mental Health Attendance in ED
165 | 6-month trend | 1217
Trend ▲ | 14 Change in mth | (5th) Rank

Self Harm Attendance in ED
6 | 6-month trend | 287
Trend ▼ | -3 Change in mth | (7th) Rank

Monthly ED rates per 1000 patients
33.6 | 6-month attendance trend | 42.9
Trend ▲ | 0.86 change in mth | (7th) Rank





Readmissions
458 | 6-month trend | 4274
Trend ▲ | 16 Change in mth | (5th) Rank

Avoidable admissions @Jun26
151 | 6-month trend | 992
Trend ▼ | -27 Change in mth | (3rd) Rank

Monthly rate of elective IP adm per 1,000
1.19 | 6-month trend in elective IP admissions | 1.32
Trend ▲ | 0.25 Change in mth | (6th) Rank

Inequality profile of the admitted waiting list for C&H's registered patients



Place Based Report - August 2026

Havering

NHS North East London

Registered Patients @ Aug-26
302,629
change -192 | 6-month trend | 6th in NEL

Housebound @Aug-26
2741 | -2 diff | 0.91% 1st in NEL
3rd in NEL

Main language not English
53,694 7th in NEL | 74 diff | 17.74% 7th in NEL

KPI KEY:
Metric Name
Place Value (NEL Value)
Trend ▲ 5% change since last period (NEL Rank)

NEL Health & Care partnership priorities

Children	LTC	SMI	Core 20
22.06% (2nd)	35.31% (1st)	0.80% (7th)	7.55% (7th)

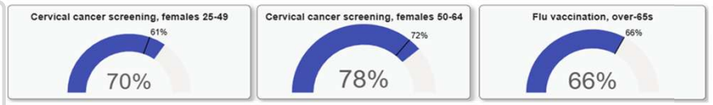
data cadence for certain metrics vary

NHS App Uptake
77.00% | 6-month trend | 73.00%
▲ Trend | 0.69% Change in mth | (1st) Rank

% clinically urgent appts dealt with on day
69% | 6-month trend | 71%
▼ Trend | -0.63% Change in mth | (4th) Rank

Same Day Access
40.10% | 6-month trend | 46.12%
▼ Trend | -0.71% Change in mth | (7th) Rank

Cervical cancer screening and flu vaccination coverage, benchmarked against London average



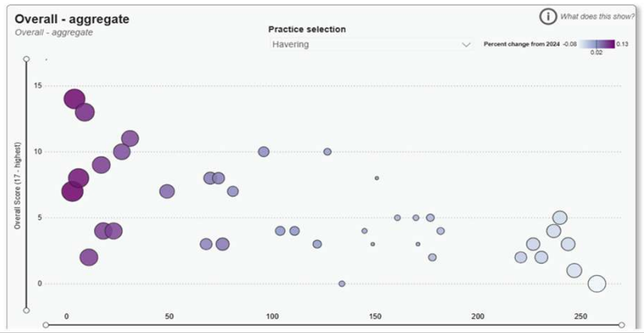
NEL average - 62% , 75% and 58% - Havering best performing place

Top 3 Most Improved

Practice Name	Difference (%) from 2024
Billet Lane Medical Practice	206.5%
South Hornchurch Medical Practice	190.0%
Ashton Gardens Surgery	183.7%

Top 3 Least Improved

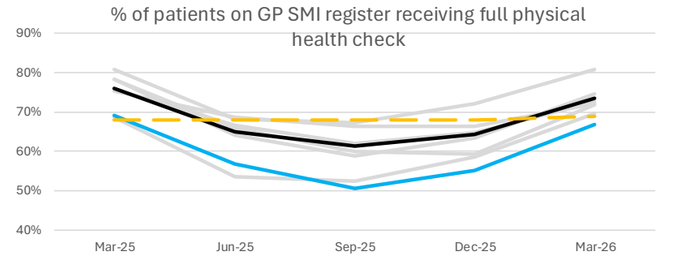
Practice Name	Difference (%) from 2024
Chase Cross Medical Centre	-135.8%
Central Park Surgery (Havering)	-93.1%
The Saheecha's Practices	-84.7%

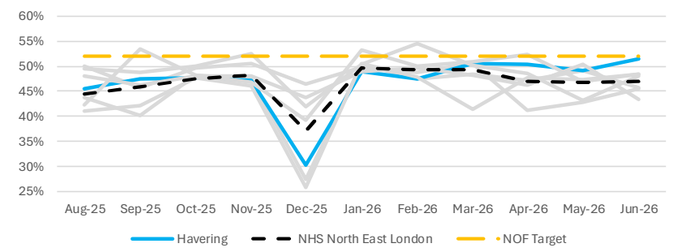


monthly rate of comm. contacts per 1,000
92.48 | 6-month trend in contacts | 76.35
▲ Trend | 2.30 Change in mth | (2nd) Rank

Monthly rate of MH contacts per 1,000
41.10 | 6-month trend in contacts | 33.39
▼ Trend | -0.99 Change in mth | (3rd) Rank

Prevalence of Learning Disability per 1,000
5.24 | 6-month trend of recorded LD | 5.19
▲ Trend | 0.01 Change in mth | (3rd) Rank

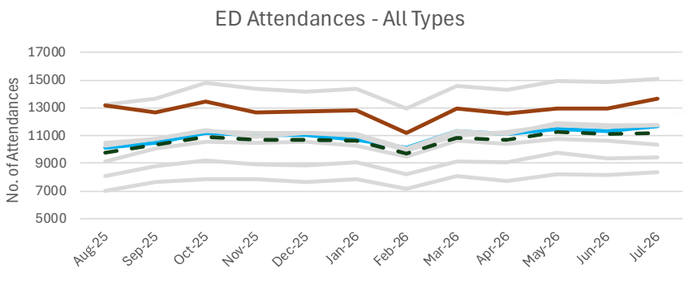


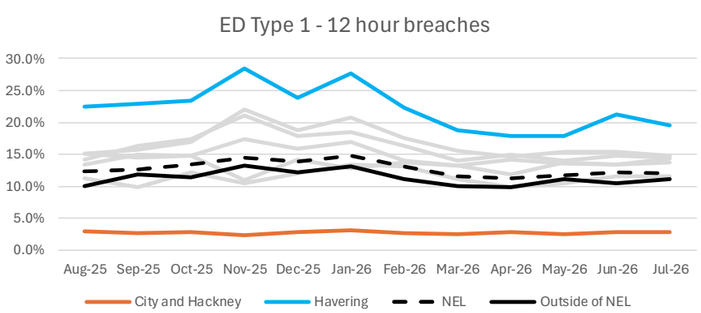


Mental Health Attendance in ED
131 | 6-month trend | 1217
▲ Trend | 3 Change in mth | (7th) Rank

Self Harm Attendance in ED
37 | 6-month trend | 287
▼ Trend | -1 Change in mth | (6th) Rank

Monthly ED rates per 1000 patients
51.1 | 6-month attendance trend | 42.9
▲ Trend | 2.61 change in mth | (2nd) Rank





Readmissions
1315 | 6-month trend | 4274
▲ Trend | 111 Change in mth | (1st) Rank

Avoidable admissions @Jun26
163 | 6-month trend | 992
▼ Trend | -24 Change in mth | (1st) Rank

Monthly rate of elective IP adm per 1,000
1.80 | 6-month trend in elective IP admissions | 1.32
▲ Trend | 0.29 Change in mth | (1st) Rank

Inequality profile of the admitted waiting list for Havering's registered patients



Place Based Report - August 2026

Newham

NHS North East London

Registered Patients @ Aug-26
464,884
change +89 — 6-month trend — 1st in NEL

Housebound @Aug-26
2672 | -2 diff | 0.57% 6th in NEL
4th in NEL

Main language not English
202,833 1st in NEL | -205 diff | 43.63% 1st in NEL

KPI KEY:
Metric Name
Place Value (NEL Value)
Trend ▲ 5% change since last period (NEL Rank)

NEL Health & Care partnership priorities

Children	LTC	SMI	Core 20
19.83% (5th)	28.32% (5th)	1.17% (4th)	43.81% (1st)

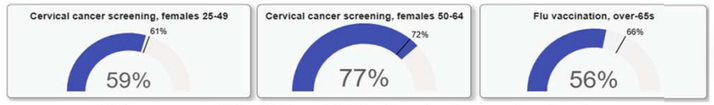
⚠ *data cadence for certain metrics vary*

NHS App Uptake
72.00% — 6-month trend — 73.00%
Trend ▲ | 0.93% Change in mth | (5th) Rank

% clinically urgent appts dealt with on day
59% — 6-month trend — 71%
Trend ▲ | 2.09% Change in mth | (7th) Rank

Same Day Access
40.53% — 6-month trend — 46.12%
Trend ▲ | 1.40% Change in mth | (6th) Rank

Cervical cancer screening and flu vaccination coverage, benchmarked against London average



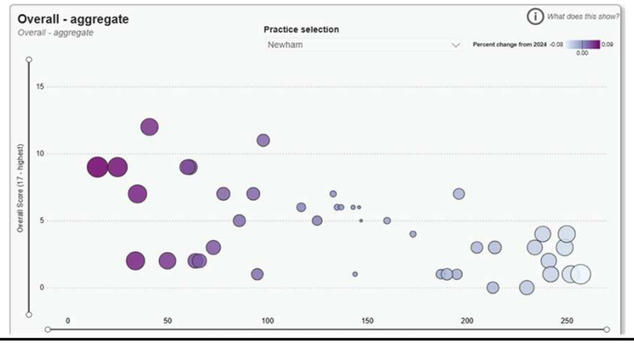
NEL average - 62%, 75% and 58% - Havering best performing place

Top 3 Most Improved

Practice Name	Difference (%) from 2024
Claremont Clinic	130.7%
First 4 Health Group - ET Health	123.7%
The Manor Park Practice	106.5%

Top 3 Least Improved

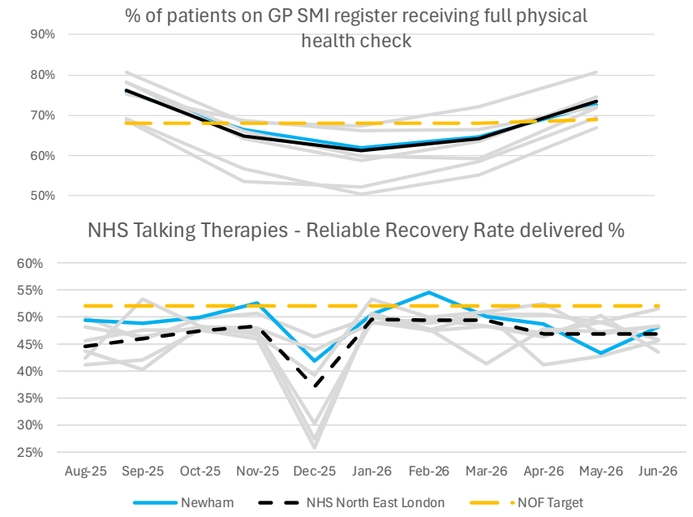
Practice Name	Difference (%) from 2024
Woodgrange Medical Practice	-125.0%
E12 Health	-94.4%
Stratford Health Centre	-91.6%



monthly rate of comm. contacts per 1,000
86.43 — 6-month trend in contacts — 76.35
Trend ▼ | -4.33 Change in mth | (3rd) Rank

Monthly rate of MH contacts per 1,000
23.50 — 6-month trend in contacts — 33.39
Trend ▲ | 0.62 Change in mth | (7th) Rank

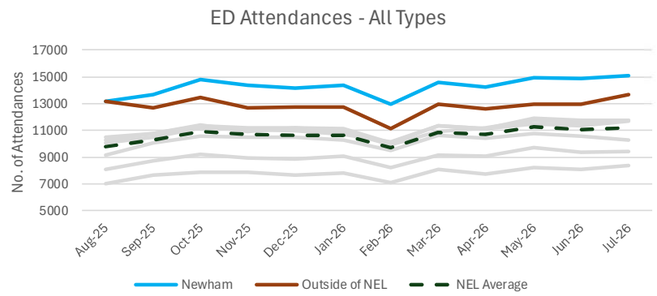
Prevalence of Learning Disability per 1,000
5.21 — 6-month trend of recorded LD — 5.19
Trend ▲ | 0.01 Change in mth | (4th) Rank

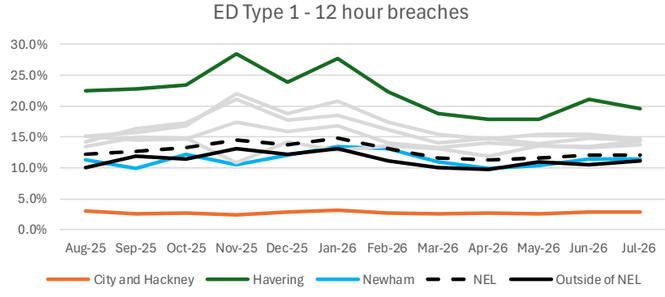


Mental Health Attendance in ED
211 — 6-month trend — 1217
Trend ▼ | -50 Change in mth | (2nd) Rank

Self Harm Attendance in ED
69 — 6-month trend — 287
Trend ▼ | -15 Change in mth | (1st) Rank

Monthly ED rates per 1000 patients
43.0 — 6-month attendance trend — 42.9
Trend ▲ | 1.03 change in mth | (4th) Rank





Readmissions
480 — 6-month trend — 4274
Trend ▼ | -10 Change in mth | (4th) Rank

Avoidable admissions @Jun26
188 — 6-month trend — 992
Trend ▲ | 19 Change in mth | (4th) Rank

Monthly rate of elective IP adm per 1,000
1.20 — 6-month trend in elective IP admissions — 1.32
Trend ▲ | 0.42 Change in mth | (5th) Rank

Inequality profile of the admitted waiting list for Newham's registered patients



Place Based Report - August 2026

Redbridge

NHS North East London

Registered Patients @ Aug-26: 360,076 — change +215 — 6-month trend — 3rd in NEL

Housebound @Aug-26: 2143 — 6th in NEL — -21 diff — 0.60% — 5th in NEL

Main language not English: 143,641 — 2nd in NEL — -126 diff — 39.89% — 2nd in NEL

NEL Health & Care partnership priorities

Children	LTC	SMI	Core 20
22.05% (3rd)	29.04% (4th)	0.99% (5th)	11.45% (6th)

KPI KEY: Metric Name — Place Value (NEL Value) — Trend ▲ 5% change since last period (NEL Rank)

⚠ *data cadence for certain metrics vary*

NHS App Uptake

75.00%	6-month trend	73.00%
▲ Trend	0.81% Change in mth	(3rd) Rank

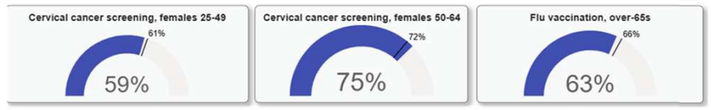
% clinically urgent appts dealt with on day

68%	6-month trend	71%
▼ Trend	-0.64% Change in mth	(5th) Rank

Same Day Access

49.59%	6-month trend	46.12%
▼ Trend	-0.20% Change in mth	(2nd) Rank

Cervical cancer screening and flu vaccination coverage, benchmarked against London average



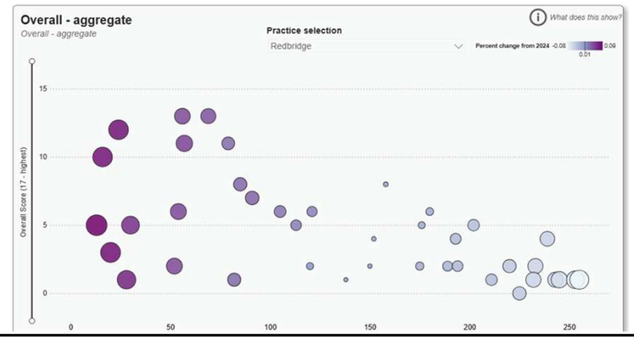
NEL average - 62% , 75% and 58% - Havering best performing place

Top 3 Most Improved

Practice Name	Difference (%) from 2024
St Clement's Surgery	148.3%
Gants Hill Med Ctr & The Redbridge Surg	135.1%
Mathulia's Surgery	131.0%

Top 3 Least Improved

Practice Name	Difference (%) from 2024
The Elmhurst Practice	-121.1%
Chadwell Heath Surgery	-103.0%
Queen Mary Practice	-66.1%



monthly rate of comm. contacts per 1,000

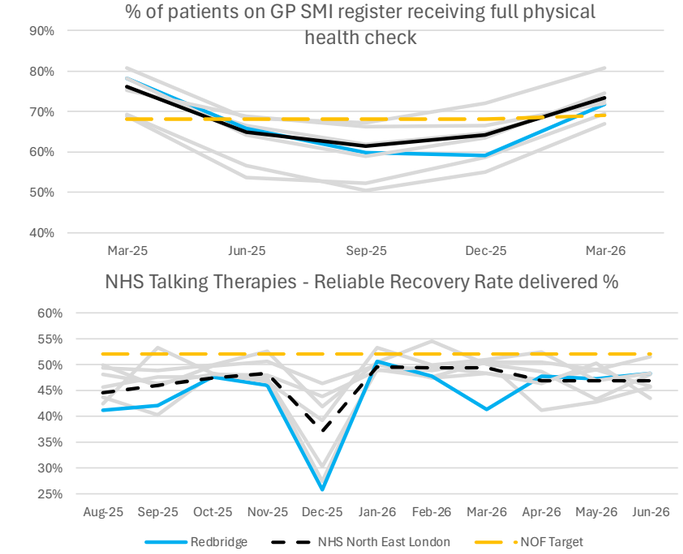
63.43	6-month trend in contacts	76.35
▲ Trend	1.25 Change in mth	(6th) Rank

Monthly rate of MH contacts per 1,000

33.59	6-month trend in contacts	33.39
▲ Trend	1.59 Change in mth	(4th) Rank

Prevalence of Learning Disability per 1,000

5.16	6-month trend of recorded LD	5.19
▲ Trend	0.01 Change in mth	(5th) Rank



Mental Health Attendance in ED

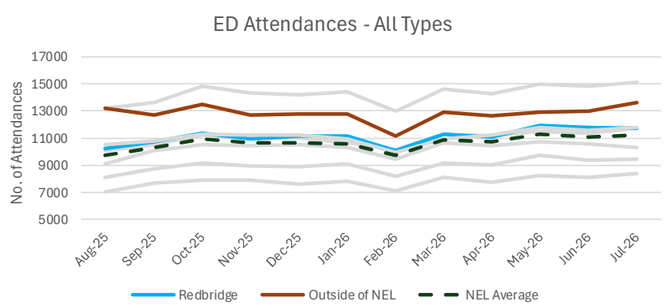
169	6-month trend	1217
▲ Trend	1 Change in mth	(4th) Rank

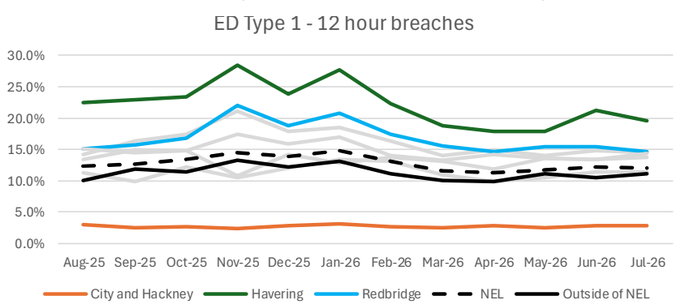
Self Harm Attendance in ED

43	6-month trend	287
– Trend	0 Change in mth	(3rd) Rank

Monthly ED rates per 1000 patients

44.1	6-month attendance trend	42.9
▲ Trend	0.48 change in mth	(3rd) Rank





Readmissions

682	6-month trend	4274
▲ Trend	13 Change in mth	(3rd) Rank

Avoidable admissions @Jun26

134	6-month trend	992
▼ Trend	-24 Change in mth	(6th) Rank

Monthly rate of elective IP adm per 1,000

1.39	6-month trend in elective IP admissions	1.32
▲ Trend	0.45 Change in mth	(3rd) Rank

Inequality profile of the admitted waiting list for Redbridge's registered patients

4,027 Pathways Waiting	23.1 Average Weeks Waiting	221 Waiting Over 52 Weeks	(Blank) Waiting Over 78 Weeks	(Blank) Waiting Over 104 Weeks	3,938 Distinct Patients Waiting

NHS North East London



Place Based Report - August 2026

Tower Hamlets

NHS North East London

Registered Patients @ Aug-26
389,820
change +414 | 6-month trend | 2nd in NEL

Housebound @Aug-26
2742 | -20 diff | 0.70% | 2nd in NEL | 3rd in NEL

Main language not English
140,372 | 3rd in NEL | -25 diff | 36.01% | 3rd in NEL

KPI KEY:
Metric Name
Place Value (NEL Value)
Trend ▲ 5% change since last period (NEL Rank)

NEL Health & Care partnership priorities

Children	LTC	SMI	Core 20
17.17% (7th)	25.53% (7th)	1.36% (2nd)	41.73% (2nd)

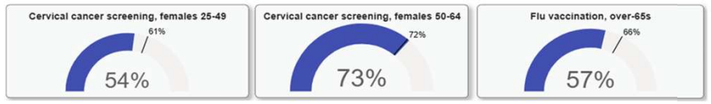
⚠ *data cadence for certain metrics vary*

NHS App Uptake
76.00% | 6-month trend | 73.00%
Trend ▲ | 0.68% Change in mth | (2nd) Rank

% clinically urgent appts dealt with on day
72% | 6-month trend | 71%
Trend ▼ | -1.54% Change in mth | (3rd) Rank

Same Day Access
43.27% | 6-month trend | 46.12%
Trend ▼ | -1.22% Change in mth | (5th) Rank

Cervical cancer screening and flu vaccination coverage, benchmarked against London average



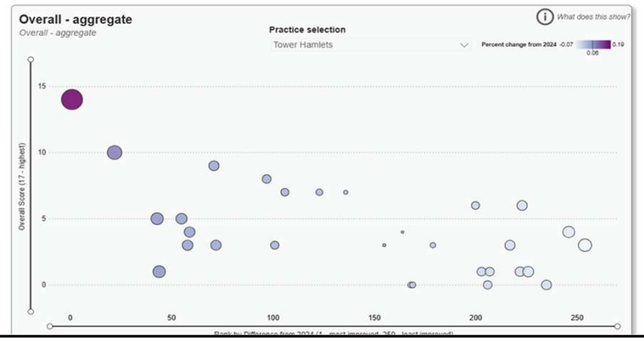
NEL average - 62% , 75% and 58% - Havering best performing place

Top 3 Most Improved

Practice Name	Difference (%) from 2024
The Wapping Group Practice	305.6%
The Jubilee Street Practice Ltd	133.6%
Gough Walk Practice	97.0%

Top 3 Least Improved

Practice Name	Difference (%) from 2024
Xx Place Health Centre	-108.4%
Health E1	-95.2%
The Christ Street Hlth Ctr	-71.2%

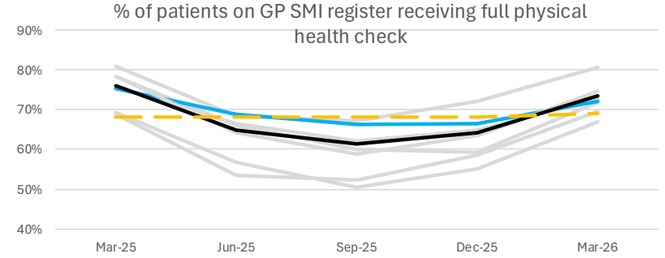


monthly rate of comm. contacts per 1,000
53.30 | 6-month trend in contacts | 76.35
Trend ▼ | -0.71 Change in mth | (7th) Rank

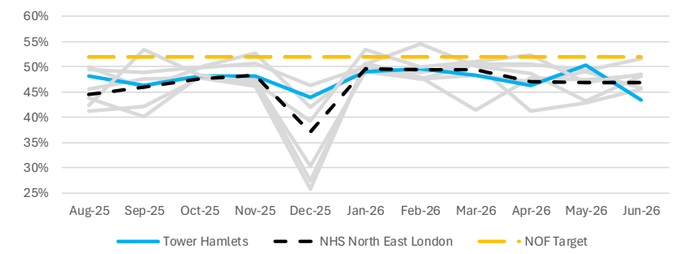
Monthly rate of MH contacts per 1,000
26.80 | 6-month trend in contacts | 33.39
Trend ▲ | 1.71 Change in mth | (6th) Rank

Prevalence of Learning Disability per 1,000
4.89 | 6-month trend of recorded LD | 5.19
Trend ▲ | 0.02 Change in mth | (7th) Rank

% of patients on GP SMI register receiving full physical health check



NHS Talking Therapies - Reliable Recovery Rate delivered %

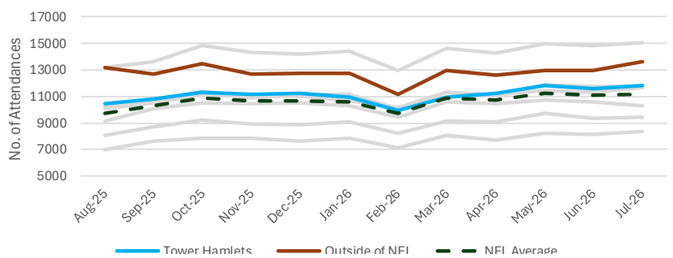


Mental Health Attendance in ED
237 | 6-month trend | 1217
Trend ▼ | -37 Change in mth | (1st) Rank

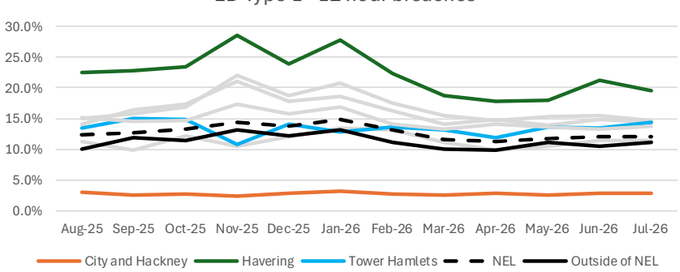
Self Harm Attendance in ED
51 | 6-month trend | 287
Trend ▼ | -4 Change in mth | (2nd) Rank

Monthly ED rates per 1000 patients
38.8 | 6-month attendance trend | 42.9
Trend ▲ | 0.86 change in mth | (6th) Rank

ED Attendances - All Types



ED Type 1 - 12 hour breaches

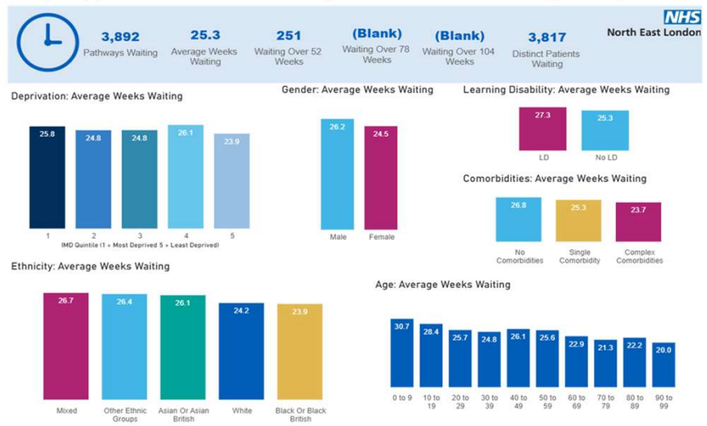


Readmissions
284 | 6-month trend | 4274
Trend ▼ | -44 Change in mth | (7th) Rank

Avoidable admissions @Jun26
111 | 6-month trend | 992
Trend ▼ | -21 Change in mth | (7th) Rank

Monthly rate of elective IP adm per 1,000
1.01 | 6-month trend in elective IP admissions | 1.32
Trend ▲ | 0.31 Change in mth | (7th) Rank

Inequality profile of the admitted waiting list for Tower Hamlets's registered patients



Place Based Report - August 2026

Waltham Forest

NHS North East London

Registered Patients @ Aug-26
325,083
change -313 | 6-month trend | 5th in NEL

Housebound @Aug-26
2175 | -11 diff | 0.67% | 5th in NEL | 4th in NEL

Main language not English
108,172 | 4th in NEL | -276 diff | 33.28% | 4th in NEL

NEL Health & Care partnership priorities

Children	LTC	SMI	Core 20
20.74% (4th)	30.20% (3rd)	1.26% (3rd)	14.12% (5th)

KPI KEY:
Metric Name
Place Value (NEL Value)
Trend ▲ 5% change since last period (NEL Rank)

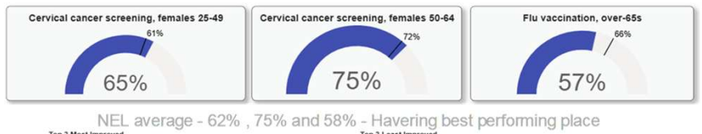
⚠ *data cadence for certain metrics vary*

NHS App Uptake
73.00% | 6-month trend | 73.00%
Trend ▲ | 0.75% Change in mth | (4th) Rank

% clinically urgent appts dealt with on day
79% | 6-month trend | 71%
Trend ▼ | -0.23% Change in mth | (2nd) Rank

Same Day Access
47.16% | 6-month trend | 46.12%
Trend ▼ | -0.20% Change in mth | (3rd) Rank

Cervical cancer screening and flu vaccination coverage, benchmarked against London average



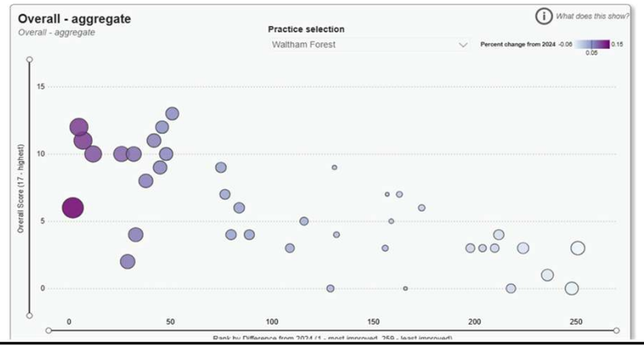
NEL average - 62% , 75% and 58% - Havering best performing place

Top 3 Most Improved

Practice Name	Difference (%) from 2024
The Forest Surgery	247.4%
The Microfaculty	178.4%
Francis Road Medical Centre	177.8%

Top 3 Least Improved

Practice Name	Difference (%) from 2024
Grove Surgery (Waltham Forest)	-87.8%
The Ecclesbourne Practice	-87.8%
The Manor Practice	-71.1%

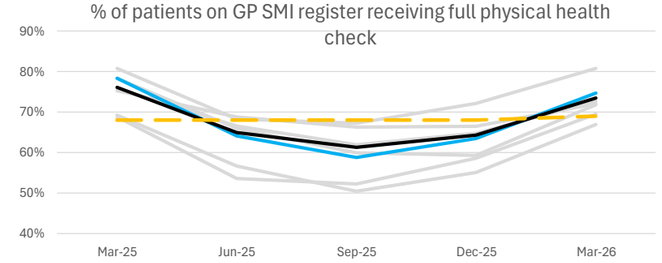


monthly rate of comm. contacts per 1,000
72.12 | 6-month trend in contacts | 76.35
Trend ▲ | 0.46 Change in mth | (5th) Rank

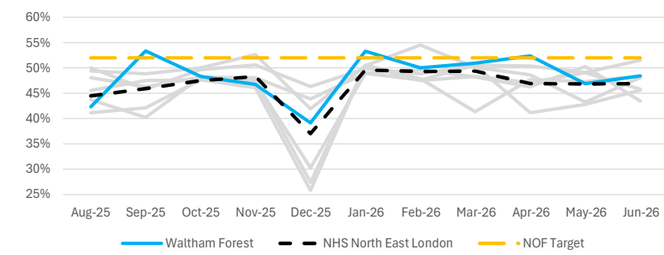
Monthly rate of MH contacts per 1,000
43.80 | 6-month trend in contacts | 33.39
Trend ▼ | -0.49 Change in mth | (1st) Rank

Prevalence of Learning Disability per 1,000
5.26 | 6-month trend of recorded LD | 5.19
Trend ▲ | 0.01 Change in mth | (2nd) Rank

% of patients on GP SMI register receiving full physical health check



NHS Talking Therapies - Reliable Recovery Rate delivered %

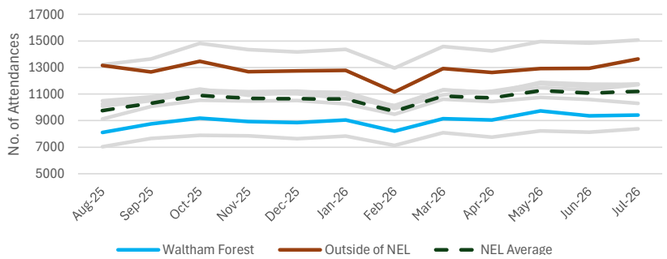


Mental Health Attendance in ED
170 | 6-month trend | 1217
Trend ▼ | -18 Change in mth | (3rd) Rank

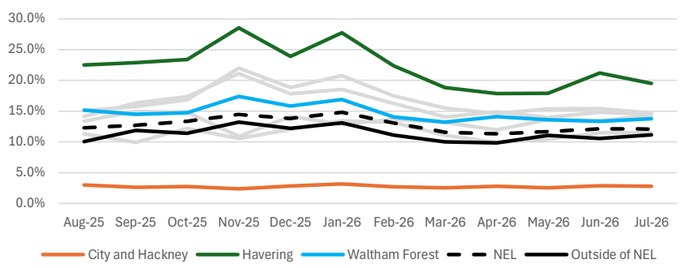
Self Harm Attendance in ED
43 | 6-month trend | 287
Trend ▼ | -6 Change in mth | (3rd) Rank

Monthly ED rates per 1000 patients
40.6 | 6-month attendance trend | 42.9
Trend ▲ | 0.59 change in mth | (5th) Rank

ED Attendances - All Types



ED Type 1 - 12 hour breaches



Readmissions
289 | 6-month trend | 4274
Trend ▲ | 26 Change in mth | (6th) Rank

Avoidable admissions @Jun26
122 | 6-month trend | 992
Trend ▼ | -7 Change in mth | (5th) Rank

Monthly rate of elective IP adm per 1,000
1.57 | 6-month trend in elective IP admissions | 1.32
Trend ▲ | 0.45 Change in mth | (2nd) Rank

Inequality profile of the admitted waiting list for Waltham Forest's registered patients



NHS North East London ICB board

30 September 2026

Title of report	Annual report and accounts – 2025/2026
Author	Anne-Marie Keliris, Head of Governance and Company Secretary
Presented by	Henry Black, Chief Finance Officer
Contact for further information	Annemarie.keliris@nhs.net
Executive summary	Integrated Care Boards (ICBs) are required to publish as a single document, an annual report, accountable officer statement and annual accounts. National annual reporting guidance refers to compliance with the Manual of Accounts issued by the Department of Health and Social Care and this in turn complies with the Government's Financial Reporting Manual. There is a common format for ease of national collation. NHS North East London ICB submitted an annual report and signed final audited year end accounts for the period 1 April 2025 to 31 March 2026 to NHS England by 19 June 2026. The ICB annual report celebrates the achievements of the Integrated Care System (ICS) with significant progress to reflect on. The ICB is required to publish the ICB annual report and accounts alongside the External Auditor's annual report on its public facing website by 30 September 2026.
Action / recommendation	The ICB board is asked to note the annual reports and annual accounts.
Previous reporting	Finance, Performance and Investment Committee Audit and Risk Committee ICB Board
Next steps/ onward reporting	The annual report and accounts were published on the ICB's website by the required deadline of 30 September 2026.
Conflicts of interest	There are no conflicts of interest identified in relation to this report.
Strategic fit	The annual report content reflects on the strategic priorities.
Impact on local people, health inequalities and sustainability	The annual report includes details on the impact of the ICB's activities on local people, health inequalities and sustainability and provides information and assurance on progress. An addendum report, Health Inequalities Data Summary, published September 2026, summarises the findings from analysis of 18 core indicators from the NHS England Inequalities guidance and will be used to inform our 26/27 annual reporting process.
Impact on finance, performance and quality	As above, the annual reports and accounts provide transparent information on the ICB's progress in these areas.

Risks	The key risk to the ICB is that the annual reports and annual accounts are not published, however all deadlines so far have been met.

North East London
Integrated Care Board

NHS North East London ICB Board

30 September 2026

Title of report	Governance update
Author	Anne-Marie Keliris, Head of Governance and Company Secretary
Presented by	Dr Nnenna Osuji, Chief Executive Officer
Contact for further information	annemarie.keliris@nhs.net
Executive summary	The Board approved a revised governance and decision-making framework in July 2026, including an Executive Commissioning Board reporting to the Strategic Commissioning Committee and a Contracts Sub-Committee reporting to the Finance and Investment Committee. This report completes the next stage of implementation by enacting the Executive Commissioning Board and the Contracts Sub-Committee. The Executive Commissioning Board formalises and continues the work of the existing Commissioning Board. The Contracts Sub-Committee replaces the Primary Care Contracts Sub-Committee and will oversee contract awards and contract management for primary care and other health services. The updated Scheme of Reservation and Delegation (SoRD) requires further refinement and will be considered by the Audit and Risk Committee in October before being brought to the November 2026 Board meeting for approval.
Action / recommendation	The ICB Board is asked to: • Enact the Executive Commissioning Board, reporting to the Strategic Commissioning Committee, continuing and formalising the work of the existing Commissioning Board. • Approve the Executive Commissioning Board Terms of Reference. • Enact the Contracts Sub-Committee as a formal sub-committee of the Finance and Investment Committee, replacing the Primary Care Contracts Sub-Committee. • Approve the Contracts Sub-Committee Terms of Reference and note the consequential closure of the Primary Care Contracts Sub-Committee when the new arrangements take effect. • Note that the updated Scheme of Reservation and Delegation is not being presented to this meeting and will be brought to the November 2026 Board meeting following further refinement and consideration by the Audit and Risk Committee. • Note the implementation and transitional arrangements.

Previous reporting	ICB Board: March, June and July 2026 Commissioning Board: September 2026 Strategic Commissioning Committee 2026 Finance and Investment Committee September 2026
Next steps/ onward reporting	Following Board approval, the Governance Handbook, meeting map, forward plans and published Terms of Reference will be updated. The Scheme of Reservation and Delegation will be refined and presented to the Audit and Risk Committee before being brought to the November 2026 Board meeting for approval.
Conflicts of interest	No conflicts of interest have been identified in relation to this report. Conflicts arising in sub-committee business will be managed in accordance with the ICB Constitution, Standing Orders, policies and the relevant Terms of Reference.
Strategic fit	The proposals support the ICB's role as a strategic commissioner and all four Integrated Care System aims: • improving outcomes; • tackling inequalities; • enhancing productivity and value for money; • supporting broader social and economic development.
Impact on local people, health inequalities and sustainability	The governance arrangements support transparent, evidence-based and system-aligned commissioning and contracting decisions, including consideration of health inequalities, engagement, quality, sustainability and value.
Has an Equalities Impact Assessment been carried out?	An Equality Impact Assessment is not required for the enactment of internal governance arrangements. Equality and health inequality impacts will continue to be assessed for individual commissioning and contracting proposals.
Impact on finance, performance and quality	There are no additional resource implications arising from the governance changes themselves. Financial, procurement, performance, clinical, quality and safety implications arising from individual proposals will continue to be considered through the appropriate governance routes.
Risks	The principal risk is that the arrangements are implemented without sufficiently clear delegations, reporting or escalation, resulting in gaps, duplication or reduced oversight. Mitigations include approved Terms of Reference, regular assurance and exception reporting, clear escalation of matters outside delegated authority, transitional arrangements, and further refinement and review of the SoRD through the Audit and Risk Committee before Board approval in November 2026.

1.0 Introduction

1.1 The Board has implemented governance change through a phased process to support the Integrated Care Board's transition to operating as a strategic commissioner. The revised framework is intended to clarify where decisions are made, where assurance is received and how matters are escalated.

1.2 At its meeting on 29 July 2026, the Board approved the revised Board committee architecture and refreshed Terms of Reference for the Committees of the Board. The architecture identified two sub-committees for subsequent enactment: the Executive Commissioning Board, reporting to the Strategic Commissioning

Committee, and the Contracts Sub-Committee, reporting to the Finance and Investment Committee.

1.3 This report brings the related changes together so that the Board can enact the Executive Commissioning Board and the Contracts Sub-Committee. The updated Scheme of Reservation and Delegation is not being presented to this meeting; it requires further refinement and will be considered by the Audit and Risk Committee before being brought to the November 2026 Board meeting for approval.

2.0 Key messages

- The Executive Commissioning Board formalises and continues the work of the existing Commissioning Board. The intention is continuity and formalisation, not the creation of a separate or duplicative forum.
- The Contracts Sub-Committee will replace the Primary Care Contracts Sub-Committee and provide a single formal route for oversight of contract awards and contract management.
- Each body will operate within its approved Terms of Reference and the authority currently delegated through the Scheme of Reservation and Delegation. The Scheme will be refined to reflect the new arrangements and brought forward separately for approval.
- Assurance and exception reporting will flow to the relevant parent committee, with material matters escalated onwards to the Board.
- Transitional arrangements will preserve continuity of live business, actions, risks and forward plans.

3.0 Body of report

3.1 Executive Commissioning Board

3.1.1 The existing Commissioning Board has operated as an executive forum for coordinating and overseeing commissioning activity. Enactment of the Executive Commissioning Board formalises this work within the ICB's governance structure and establishes a clear line of accountability to the Strategic Commissioning Committee.

3.1.2 The Executive Commissioning Board will provide strategic leadership, oversight and accountability for commissioning decisions and implementation of the North East London Commissioning Framework. Its scope includes commissioning plans, decommissioning, recommissioning or contract renewal and new expenditure proposals across primary care, community, acute, mental health, specialist and independent-sector services.

3.1.3 The Executive Commissioning Board will take decisions within its delegated authority. Strategic commissioning decisions outside that authority will be recommended or escalated to the Strategic Commissioning Committee. Matters requiring separate financial, procurement, quality or safety assurance will continue through the appropriate governance route.

3.2 Contracts Sub-Committee

3.2.1 The Contracts Sub-Committee will be enacted as a sub-committee of the Finance and Investment Committee and will replace the Primary Care Contracts Sub-Committee. This creates a single formal route aligned to the revised governance framework.

3.2.2 The sub-committee will oversee functions relating to contract awards and contract management, including associated financial and commercial matters, and provide assurance on proposed contract awards in accordance with its Terms of Reference and the applicable delegated authority.

3.2.3 The sub-committee will maintain an effective relationship with the Executive Commissioning Board so that commissioning intent, procurement and contracting decisions are coherent and routed appropriately. Relevant business, actions and assurance requirements will transfer through a controlled handover, including continued compliance with delegated primary care commissioning requirements.

3.3 Scheme of Reservation and Delegation

3.3.1 The updated Scheme of Reservation and Delegation is not being presented to this meeting. Further work is required to refine the proposed delegations and ensure alignment with the revised committee architecture and associated governance arrangements.

3.3.2 The refined Scheme will be presented to the Audit and Risk Committee for consideration before being brought to the November 2026 Board meeting for approval. The further refinement will:

- reflect both new governance bodies and replace references to the Primary Care Contracts Sub-Committee;
- align delegated decision-making with the approved committee architecture and Chief Officer roles;
- clarify the interface between strategic commissioning, finance, procurement, contracting, quality and safety governance;
- confirm escalation routes for matters outside delegated authority or requiring parent committee or Board consideration; and
- maintain financial controls and alignment with the Board Assurance Framework.

3.3.3 Pending approval of the updated Scheme, decisions must continue to follow the applicable governance and delegation arrangements. Where the correct route or extent of authority is uncertain, governance and finance advice should be obtained before a decision is taken.

4.0 Risks and mitigations

4.1 The principal risk is that enactment is treated as a change of meeting title rather than a formal change to authority, accountability and assurance. This could create gaps, duplication, inappropriate decision routes or reduced strategic oversight.

4.2 Mitigation is provided through approved Terms of Reference, regular assurance and exception reporting, clear escalation of matters that are material, novel, contentious, high risk or outside delegated authority, and further refinement and review of the Scheme of Reservation and Delegation through the Audit and Risk Committee before Board approval in November 2026.

4.3 The Executive Commissioning Board will report to the Strategic Commissioning Committee. The Contracts Sub-Committee will report to the Finance and Investment Committee. Where a matter crosses committee responsibilities, the chairs and executive leads will agree the primary route while ensuring all necessary assurance is obtained.

4.4 Transitional arrangements will include the controlled transfer of forward-plan items, decisions, actions and risks; confirmation of membership and meeting arrangements; and updates to the Governance Handbook and meeting map. A governance stocktake in early 2027 will review effectiveness and identify any gaps or duplication.

5.0 Recommendations

5.1 The proposed enactments complete the next stage of the Board-approved governance framework by formalising existing commissioning arrangements and establishing a clear contracting route. The updated Scheme of Reservation and Delegation will be brought forward separately once it has been further refined and considered by the Audit and Risk Committee.

5.2 The Board is asked to:

- Enact the Executive Commissioning Board, reporting to the Strategic Commissioning Committee, continuing and formalising the work of the existing Commissioning Board.
- Approve the Executive Commissioning Board Terms of Reference at Appendix 1.
- Enact the Contracts Sub-Committee as a formal sub-committee of the Finance and Investment Committee, replacing the Primary Care Contracts Sub-Committee.
- Approve the Contracts Sub-Committee Terms of Reference at Appendix 2 and note that the Primary Care Contracts Sub-Committee will close when the new arrangements take effect.
- Note that the updated Scheme of Reservation and Delegation is not being presented at this meeting and will be brought to the November 2026 Board meeting following further refinement and consideration by the Audit and Risk Committee.
- Note the assurance, escalation, implementation and transitional arrangements set out in this report.

6.0 Attachments

Appendix 1: Executive Commissioning Board Terms of Reference.

Appendix 2: Contracts Sub-Committee Terms of Reference.

North East London
Integrated Care Board

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Executive Commissioning Board

North East London ICB

Terms of Reference

Not for Comment

Status	1. The ICB Board has established the Strategic Commissioning Committee (the "SCC"). The Executive Commissioning Board ("the Board") has been established by the SCC to oversee and coordinate commissioning activity on behalf of the ICB and to exercise delegated authority as set out in these Terms of Reference. 2. These Terms of Reference set out the membership, the remit, responsibilities and reporting arrangements of the Board and may only be changed with the approval of the SCC and the ICB Board. Additionally, the membership of the Board must be approved by the Chair of the ICB. 3. The Board and all of its members are bound by the ICB's Constitution, Standing Orders, Standing Financial Instructions, policies and procedures of the ICB.
Authority	4. The Board is authorised by the SCC, within the authority delegated to the SCC by the ICB Board, to take all necessary actions to fulfil the remit described within these terms of reference, including obtaining professional (including legal) advice, commissioning reports and creating groups. The Board will follow the processes described by the ICB for commissioning any professional advice. 5. The Board does not have the authority to delegate any functions delegated to it. However, the Board may establish groups (e.g. working, advisory or task and finish groups), which do not have any decision-making powers but may inform the work of the Board. Such groups must operate under terms of reference approved by the Board, and these must reflect appropriate arrangements for the management of conflicts of interest. 6. The Board is authorised to make decisions relating to the effective delivery of the north east London Commissioning Framework. The scope of its commissioning decisions will include: (a) Commissioning Plans (b) Decommissioning of services/service provisions (c) Recommissioning or contract renewal of existing services

(d) New expenditure proposals

For the avoidance of doubt, this will include any joint commissioning arrangements to which the ICB is party as commissioner.

7. The Board will take a forward strategic view, taking commissioning decisions within its delegated authority and making recommendations to the SCC where matters fall outside that authority. Agreed decisions will be implemented through the appropriate executive and system arrangements.

8. The Board will take a system-wide view, ensuring coherence of individual commissioning plans; contributing to equity of resources and outcomes across our system (for example by Place, by Condition and by Care Group); improving access, experience and outcomes for our local population; and overseeing implementation of the strategic shifts set out in national policy, including the Darzi Review and the 10 Year Health Plan for England. This will include ensuring that the allocation of resources reflects the required strategic shifts.

9. Where matters fall outside the delegated authority of the Board, recommendations will be made to the SCC for consideration and decision.

10. Where there are financial or procurement implications that fall within the scheme of delegation the business cases or contract awards would need to be taken through the relevant ICB Board governance. Where there are particular implications for quality or safety, or the commissioning process has been in effect initiated through this route, Quality Safety and Improvement Committee (QSIC) will also be engaged as part of the commissioning process.

Purpose and responsibilities

11. The role of the Board shall be to oversee the exercise of functions relating to commissioning decisions, ensuring that commissioning activities align with national strategies and the NEL integrated care strategy and deliver high-quality and cost-effective healthcare services that contribute to financial sustainability across North East London in line with the NEL medium term financial strategy (MTFS) and North East London Commissioning Framework.

12. Where decommissioning is required, the Board will ensure all due process has been followed in line with the NEL commissioning framework, including delivery of any required engagement and or consultation, with both local government and local communities.

13. The specific matters delegated to the Board are set out in the ICB's Financial Scheme of Reservation and Delegation, which is in the Handbook and is available at this link.

14. The Board is established to provide strategic leadership, oversight and accountability for commissioning decisions within the Integrated Care Board (ICB).

15. The Board serves as the strategic commissioner for health services, including but not limited to primary care, community health, acute, mental

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health and specialist services, and across the independent sector and the voluntary, community, faith and social enterprise sector, overseeing both the allocation of resources and the implementation of the Strategic Commissioning Framework. It ensures that commissioning, decommissioning, recommissioning, contract renewals, and new expenditure proposals are conducted in a transparent, evidence-based, and system-aligned manner which maintain a focus on our local population

16. The Board will ensure strategic commissioning plans are aligned across our system architecture. While system partners will be actively engaged in supporting the commissioning cycle, they will not hold membership or decision-making authority within the Board.

Key duties relating the exercise of the Board's functions

17. The Board will:

(a) Ensure an effective and productive working relationship between the SCC and the Board, reducing duplication and improving strategic fit and alignment

(b) Develop and implement commissioning strategies that align with NHS policy and local health needs, ensuring equitable resource distribution and access to services across the system to reduce health inequalities, improve outcomes and delivery quality improvement.

(c) Oversee and govern commissioning, decommissioning, recommissioning, contract renewals, and new expenditure proposals, including any joint commissioning arrangements in which the ICB is a party in its commissioning role.

(d) Ensure effective allocation and management of resources in line with the ICB aims of improving population health, reducing health inequalities, ensuring value for money and supporting economic and social development and to ensure resources reflect the three key strategic shifts set out in national policy: treatment to prevention; acute to community; analogue to digital. This will involve proposing moving resources across our system in new ways, including the Voluntary, Community, Faith and Social Enterprise Sector.

(e) Identify commissioning and decommissioning requirements and agree actions within the Board's delegated authority. Where implementation is required, the Board will refer agreed decisions or recommendations through the appropriate executive, committee and system governance arrangements, ensuring due process is followed.

(f) Ensure compliance with legal, regulatory, and governance requirements including ensuring adequate engagement with the JHOSC and the public as required.

(g) Ensure appropriate and meaningful engagement with stakeholders, including patients, residents, communities, clinicians and partners including but not limited to local

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	authorities, NHS Trusts and the voluntary and community sector, to inform commissioning decisions. (h) In line with the above, ensure the agreed decommissioning process is followed where plans are made to cease, reduce or pause services. (i) Ratify the decisions of and monitor the work of the Commissioning Groups. (j) Ensure strategic alignment across the system architecture. (k) Oversee the implementation of the MTFS to ensure responsible Commissioning Stewardship and accountability. (l) Oversee the implementation of national priorities (the three shifts): analogue to digital, hospital to community, and treatment to prevention. (m) Report to the SCC on the exercise of its delegated functions and refer matters outside its authority to the SCC for decision or onward recommendation to the ICB Board. Matters requiring financial, procurement, quality or safety approval will follow the relevant ICB governance route.
Chairing arrangements	18. The Board will be chaired by the ICB Chief Executive Officer (CEO) who is appointed on account of their specific knowledge, skills and experiences making them suitable to chair the Board and will agree the Board's agenda and ensure that its work and discussions meet the objectives set out in these terms of reference. 19. Board members may appoint a Vice Chair from amongst the members. If the Chair has a conflict of interest then the Vice Chair or, if necessary, another member of the Board will be responsible for deciding the appropriate course of action.
Membership	20. Members of the Executive Commissioning Board shall be appointed by the ICB Board in accordance with the ICB Constitution, and the Chair of the ICB will approve its membership. 21. The Board shall have 5 members as follows: a) Chief Executive Officer – Chair b) Chief Strategy Officer c) Chief Strategic Commissioning Officer d) Chief Clinical and Quality Commissioning Officer e) Chief Finance Officer
Participants	22. The following will be regular participants:

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a) Director of Statutory Clinical Services

b) Director of Acute Commissioning for Planned Care

c) Director of Strategy & Population Health

d) Deputy Director of Workforce Programmes

e) Director of Clinical and Quality Commissioning

f) Director for Proactive Care

g) Director for Primary Care

h) Chief Pharmacist and Strategic Commissioner for Medicines and Prescribing

23. Only members of the Board have voting rights. The Chair may invite relevant officers and partners to attend meetings as appropriate to the business of the Board.

24. The Chair may invite contracting and/or commissioning staff to the meeting as necessary in accordance with the business of the Board.

25. The Chair will invite relevant local authority or other public sector body commissioning leads, including Directors of Children's and or Adult Services, as appropriate including where papers or recommendations include joint commissioning proposals.

Meetings Quoracy and Decisions

26. The Board will operate in accordance with the ICB's governance framework, as set out in its Constitution and Handbook and wider ICB policies and procedures, except as otherwise provided below:

Scheduling meetings

27. The Board shall ordinarily meet once a fortnight for an hour and a half. Where meetings are not required, they will be stood down.

Quoracy

28. For a meeting to be quorate there must be 3 members present, which must include:

(a) The Chief Executive Officer or the Chief Strategic Commissioning Officer

(b) Chief Clinical and Quality Commissioning Officer or Chief Finance Officer (or an appropriate deputy) A formally nominated deputy attending in place of a member will count towards the quorum and may exercise that member's voting rights.

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29. If any member of the Board has been disqualified from participating on an item in the agenda, by reason of a declaration of conflicts of interest, then that individual shall no longer count towards the quorum.

30. If the quorum has not been reached, then the meeting may proceed if those attending agree, but no decisions may be taken.

Voting

31. Decisions will be taken in accordance with the Standing Orders. The Board will ordinarily reach decisions by consensus. When this is not possible, the Chair may call a vote. Only members of the Board may vote. Each member is allowed one vote and a simple majority will be conclusive on any matter. Where there is a split vote, with no clear majority, the Chair of the Board will hold the casting vote. The result of the vote will be recorded in the minutes.

Papers and notice

32. A minimum of two clear working days' notice is required of the date and time of a meeting. Notice of all meetings shall comprise venue, time and date of the meeting, together with an agenda of items to be discussed. Supporting papers must be distributed at least two clear working days ahead of the meeting.

33. On occasion it may be necessary to arrange urgent meetings at shorter notice. In these circumstances the Chair will give as much notice as possible to members. Urgent virtual meeting by email or MS teams shall be permitted in exceptional circumstances (i.e. remedial action) at the discretion of the Chair.

Virtual attendance

34. It is for the Chair to decide whether or not the Board will meet virtually by means of telephone, video or other electronic means. Where a meeting is not held virtually, the Chair may nevertheless agree that individual members may attend virtually. Participation in a meeting in this manner shall be deemed to constitute presence in person at such meeting. How a person has attended a meeting shall be specified in the meeting minutes.

Recordings of meetings

35. Except with the permission of the Chair, no person admitted to a meeting of the Board shall be permitted to record the proceedings in any manner whatsoever, other than in writing.

Confidential information

36. Where confidential information is presented to the Board, all those who are present will ensure that they treat that information appropriately in light of any confidentiality requirements and information governance principles. This includes any information governance requirements

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expected of the ICB under the Delegation Agreement (e.g. clauses 17 and 21) and the MOU (e.g. clause 22).

Meeting minutes

37. The minutes of a meeting will be formally taken in the form of key points of debate, actions and decisions and a draft copy circulated to the members of the Board together with the action log as soon after the meeting as practicable. The minutes shall be submitted for agreement at the next meeting where they shall be signed by the Chair.

Governance support

38. Governance support to the Board will be provided by the ICB's governance team.

Conflicts of interest

39. It is recognised that conflicts of interest may exist in relation to commissioning activity undertaken by the ICB. It is important that all members of the Board as well as those attending, keep up to date their register of interests and proactively identify the nature of any conflicts of interest which may have an impact on the business being discussed.

40. Conflicts of interest will be managed in accordance with the policies and procedures of the ICB and those contained in the Handbook and shall be consistent with the statutory duties contained in the National Health Service Act 2006 and any statutory guidance issued by NHS England. It is an obligation on all members and attendees to declare and act on any conflict of interest.

Behaviours and Conduct

41. Members must act with integrity, transparency, and accountability.

42. Decisions should be patient-centred, evidence-based, and aligned with system priorities.

43. Members will be expected to behave and conduct business in accordance with:

    (a) The ICB's policies and procedures including its Constitution, Standing Orders and Standards of Business Conduct Policy; which sets out the expected behaviours that all members of the Board and its committees will uphold whilst undertaking ICB business;

    (b) The NHS Constitution;

    (c) The Nolan Principles;

44. Members must demonstrably consider equality diversity and inclusion implications of the decisions they make.

45. Collaboration and constructive challenge are encouraged to drive continuous improvement.

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Accountability and Reporting	46. The Board is accountable to the SCC and will provide regular reports on decisions taken, recommendations made, assurance matters and issues requiring escalation. The SCC remains accountable to the ICB Board.
Review	47. The Board will review its effectiveness at least annually. 48. These terms of reference will be reviewed at least annually and more frequently if required. Any proposed amendments to the terms of reference will be agreed by the Chief Executive and Chief Finance Officer and will be submitted to SCC and the ICB Board for approval.

Date of approval:

Date of review

Version:

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Integrated Care Board

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Contracts Sub-Committee

TERMS OF REFERENCE

Status	1. The Board of the ICB has established the Finance and Investment Committee (the "**FIC**") and, in turn, the Contracts Sub-Committee ("the **sub-committee**") is established as an executive sub-committee of the FIC, replacing the Primary Care Contracts Sub-Committee, which is disestablished. 2. These Terms of Reference set out the membership, the remit, responsibilities and reporting arrangements of the sub-committee and may only be changed with the approval of the FIC and the Board. Additionally, the membership of the sub-committee must be approved by the Chair of the Board. 3. The sub-committee and all of its members are bound by the ICB's Constitution, Standing Orders, Standing Financial Instructions, policies and procedures of the ICB.
Relationship with other committees	4. The sub-committee will operate as part of the ICB's wider commissioning and governance framework and will maintain a clear and effective relationship with the Commissioning Sub-Committee. 5. Where commissioning strategies, service models or procurement approaches are developed or overseen by the Commissioning Sub-Committee, the Contracts Sub-Committee will provide assurance on the contractual, commercial and financial aspects of proposed contract awards and contract variations. 6. The Chairs of the respective committees will ensure appropriate alignment, information flow and escalation arrangements are in place to support coherent decision-making and assurance across the commissioning lifecycle.
Authority	7. The sub-committee is authorised by the Board to take all necessary actions to fulfil the remit described within these terms of reference, including obtaining professional (including legal) advice, commissioning reports and creating groups. The sub-committee will follow the processes described by the Board for commissioning any professional advice. 8. The sub-committee does not have the authority to delegate any functions delegated to it. However, the sub-committee may establish groups (e.g. working, advisory or task and finish groups), which do not have any decision-making powers but may inform the work of the sub-committee. Such groups must operate under terms of reference approved by the sub-committee, and these must reflect appropriate arrangements for the management of conflicts of interest.

Purpose and responsibilities

9. The role of the sub-committee shall be to oversee the exercise of functions relating to contract awards and contract management, including associated financial and commercial matters, and to provide assurance to the Finance and Investment Committee and the Board on all proposed contract awards, whether the decision to award is delegated to the sub-committee or reserved to the Finance and Investment Committee, in accordance with the ICB's Scheme of Reservation and Delegation.

10. The specific matters delegated to the sub-committee are set out in the ICB's Scheme of Reservation and Delegation and any associated operational schemes of delegation. The sub-committee is authorised by the ICB to take decisions in relation to those matters described in the scheme(s), subject to applicable procurement and contracting rules and the management of conflicts of interest.

    (i) In the first part, to bring together all delegated and non-delegated primary care contracting decisions within a single executive forum.

    (ii) In the next part, to extend the remit to cover contract awards across all healthcare services including primary care, the independent sector and NHS provider Trusts, up to the value of £xxxm (or such other limit as set out in the Scheme of Reservation and Delegation).

    (iii) To ensure decisions are taken with appropriate commercial, financial, quality and clinical input, and that decisions are properly recorded and reported through the ICB governance framework.

11. Financial management in relation to contract awards will be assured through the sub-committee, which may escalate matters to the FIC where required by the Scheme of Reservation and Delegation, the Standing Financial Instructions, or where matters are novel, contentious or repercussive.

12. The sub-committee shall provide assurance on the robustness, compliance and value for money of all contract award proposals, including those where the decision to award is ultimately taken by the Finance and Investment Committee.

13. This assurance will include consideration of procurement compliance, commercial and financial risk, affordability, value for money, quality and clinical considerations, and the management of conflicts of interest. The sub-committee will ensure that contract award recommendations are supported by appropriate documentation and are escalated with clear assurance commentary where decisions sit outside its delegated authority.

14. The sub-committee's assurance responsibilities will be facilitated by receiving reports and advice from relevant directorates and groups (as appropriate to the scope of items being considered), including finance, contracting, commissioning, quality and clinical leadership.

Key duties relating the exercise of the sub-committee's functions	15. The ICB's responsibilities relating to primary care have been delegated to it by NHS England in reliance on section 65Z5 of the NHS Act 2006 and in accordance with the Primary Care Delegation Agreement. The sub-committee shall ensure that it adheres to the agreement at all times and the requirements of any assurance arrangements made by NHS England. 16. The Delegation Agreement imposes wide-ranging contractual obligations on the ICB and refers to relevant guidance documents, policy and expectations, and Mandated Guidance issued by NHS England from time-to-time.[1] In certain circumstances, decisions may require the approval of the ICB's Chief Executive Office or Chief Finance and Performance Officer and require approval of NHS England in accordance with the financial limits set out in the Delegation Agreement, which includes where a matter in relation to the Delegated Functions is *novel contentious or repercussive*[2]. The sub-committee will ensure that it takes advice on any matter where these requirements may apply and escalate matters as appropriate. 17. In relation to Pharmacy, Optometry and Dentistry services (PODS), the sub-committee shall also take into account any advice, guidance or recommendations made by the London Commissioning Oversight Group (COG).
Chairing arrangements	18. The sub-committee will be chaired by the Chief Finance Officer (or nominated deputy) who will agree the sub-committee's agenda and ensure that its work and discussions meet the objectives set out in these terms of reference. 19. Sub-committee members may appoint a Vice Chair from amongst the members. If the Chair has a conflict of interest then the Vice Chair or, if necessary, another member of the sub-committee will be responsible for deciding the appropriate course of action.
Membership	20. The sub-committee members shall be appointed by the Board in accordance with the ICB Constitution and the Chair of the ICB will approve the membership of the sub-committee. 21. The sub-committee shall have 7 members as follows: (a) Independent Clinical Chair (b) Chief Finance Officer or nominated deputy (c) Chief Clinical & Quality Commissioning Officer or nominated clinical deputy

[1] See Schedule 9 of the Delegation Agreement.
[2] See Schedule 5 of the Delegation Agreement.

(d) Director for Primary Care or nominated deputy

(e) Director for Proactive Care or nominated deputy

(f) Director for Acute Commissioning or nominated deputy

(g) Director of Procurement and Contracting or nominated deputy

Participants

22. Only members of the sub-committee have the right to attend meetings, but the Chair may invite relevant staff to the meeting as necessary in accordance with the business of the sub-committee.

23. The following will have a standing invitation to attend for part of the meeting of the sub-committee when items relating to primary care are discussed

    (a) Representatives from Healthwatch and the Local Medical Committee (London-wide and Barking & Dagenham and Havering), Local Dental Committee, Local Optical Committee and Local Pharmaceutical Committee.

24. The sub-committee may, invite others to attend meetings to support the sub-committee in discharging its responsibilities.

Meetings Quoracy and Decisions

25. The sub-committee will operate in accordance with the ICB's governance framework, as set out in its Constitution and Handbook and wider ICB policies and procedures, except as otherwise provided below:

Scheduling meetings

26. The sub-committee shall ordinarily meet on a bi-monthly basis, with six meetings each financial year. Additional meetings may be convened on an exceptional basis at the discretion of the Chair.

27. The Board, Chair, Chief Executive or FIC may ask the sub-committee to convene further meetings to discuss particular issues on which they want the sub-committee's advice.

Quoracy

28. For a meeting to be quorate there must be four members present.

29. If any member of the sub-committee has been disqualified from participating on an item in the agenda, by reason of a declaration of conflicts of interest, then that individual shall no longer count towards the quorum.

30. If the quorum has not been reached, then the meeting may proceed if those attending agree, but no decisions may be taken.

Voting

31. Decisions will be taken in accordance with the Standing Orders. The sub-committee will ordinarily reach conclusions by consensus. When this is

not possible, the Chair may call a vote. Only members of the sub-committee may vote. Each member is allowed one vote and a simple majority will be conclusive on any matter. Where there is a split vote, with no clear majority, the Chair of the sub-committee will hold the casting vote. The result of the vote will be recorded in the minutes.

Papers and notice

32. A minimum of five clear working days' notice is required of the date and time of a meeting. Notice of all meetings shall comprise venue, time and date of the meeting, together with an agenda of items to be discussed. Supporting papers must also be distributed at least five clear working days ahead of the meeting.

33. On occasion it may be necessary to arrange urgent meetings at shorter notice. In these circumstances the Chair will give as much notice as possible to members. Urgent virtual meeting by email or MS teams shall be permitted in exceptional circumstances (i.e. remedial action) at the discretion of the Chair.

Virtual attendance

34. It is for the Chair to decide whether or not the sub-committee will meet virtually by means of telephone, video or other electronic means. Where a meeting is not held virtually, the Chair may nevertheless agree that individual members may attend virtually. Participation in a meeting in this manner shall be deemed to constitute presence in person at such meeting. How a person has attended a meeting shall be specified in the meeting minutes.

Recordings of meetings

35. Except with the permission of the Chair, no person admitted to a meeting of the sub-committee shall be permitted to record the proceedings in any manner whatsoever, other than in writing.

Confidential information

36. Where confidential information is presented to the sub-committee, all those who are present will ensure that they treat that information appropriately in light of any confidentiality requirements and information governance principles. This includes any information governance requirements expected of the ICB under the Delegation Agreement (e.g. clauses 17 and 21) and the MOU (e.g. clause 22).

Meeting minutes

37. The minutes of a meeting will be formally taken in the form of key points of debate, actions and decisions and a draft copy circulated to the members of the sub-committee together with the action log as soon after the meeting as practicable. The minutes shall be submitted for agreement at the next meeting where they shall be signed by the Chair.

Governance support

38. Governance support to the sub-committee will be provided by the ICB's governance team.

Conflicts of interest

39. Conflicts of interest will be managed in accordance with the policies and procedures of the ICB and those contained in the Handbook and shall be consistent with the statutory duties contained in the National Health Service Act 2006 and any statutory guidance issued by NHS England.

40. The ICB acknowledges that it shall comply with sections 17 and 23 of the MOU which, in summary, requires the ICB to:

    (a) Openly declare conflicts of interest in any decision-making forum convened for the purposes of making a decision under the provisions of the MOU.

    (b) Comply with relevant guidance and maintain a publicly available register of interests in respect of all persons making decisions concerning the functions set out in the MOU.

Behaviours and Conduct

41. Members will be expected to behave and conduct business in accordance with:

    (a) The ICB's policies and procedures including its Constitution, Standing Orders and Standards of Business Conduct Policy; which sets out the expected behaviours that all members of the Board and its committees will uphold whilst undertaking ICB business;

    (b) The NHS Constitution;

    (c) The Nolan Principles;

42. Members must demonstrably consider equality diversity and inclusion implications of the decisions they make.

Accountability and Reporting

43. The sub-committee is accountable to the FIC and shall report to the FIC on how it discharges its responsibilities and its assurance role in relation to contract awards, including highlighting any significant risks, issues or themes arising from its work.

44. The sub-committee will submit copies of its minutes or a report to the FIC following each of its meetings.

45. The sub-committee will provide the FIC with an annual report. The report will summarise its conclusions from the work it has done during the year.

Review

46. The sub-committee will review its effectiveness at least annually or more frequently if required.

	47. These terms of reference will be reviewed at least annually and more frequently if required. Any proposed amendments to the terms of reference will be submitted to the FIC and the Board for approval.

Date of approval:	TBC
Date of review	TBA
Version:	1.0

Annex 1 – Key Documents

Delegation Agreement	https://northeastlondon.icb.nhs.uk/wp-content/uploads/2023/07/NEL-Primary-Care-and-Dental-Delegation-Agreement-FINAL.pdf
Memorandum of Understanding (PODS), including the Terms of Reference for the Commissioning Oversight Group	 MOU POD Services London.pdf  London POD Commissioning Ove
London Operating Model for PODS	[insert]
Link to the Operational Scheme of Delegation for Primary Care	https://intranet.northeastlondon.icb.nhs.uk/wp-content/uploads/2024/02/Appx-4.2-Operational-Scheme-of-Reservation-and-Delegation-for-Primary-Care.pdf

North East London
Integrated Care Board

NHS North East London ICB board

30 September 2026

Title of report	Audit and Risk Committee exception report
Author	Keeley Chaplin, Governance Lead
Presented by	Bidesh Sarkar, Audit and Risk Committee Chair
Contact for further information	keeley.chaplin@nhs.net
Executive summary	This report provides a summary of the discussion at an extraordinary Audit and Risk Committee meeting held on 12 August 2026.
Action required	The board is asked to note the report.
Previous reporting	A report was presented to the board at its meeting in July 2026.
Next steps/ onward reporting	An exception report will be presented to the board going forward.
Conflicts of interest	No conflicts of interest have been identified in relation to this report.
Strategic fit	The ICS aims this report aligns with are: • To improve outcomes in population health and healthcare • To tackle inequalities in outcomes, experience and access • To enhance productivity and value for money • To support broader social and economic development
Impact on local people, health inequalities and sustainability	The remit of the Committee is to contribute to the overall delivery of the ICB's objectives by providing oversight and assurance to the Board on the adequacy of governance, risk management, internal control processes and arrangements to manage conflicts of interest within the ICB.
Impact on finance, performance and quality	N/A
Risks	The Committee will be driven by the organisation's objectives and the associated risks, and its duties will be governed by the Terms of Reference. An annual programme of business is agreed before the start of each financial year; however, this will be flexible to new and emerging priorities and risks.

1.0 Purpose of the report

1.1. This report provides a summary of the extraordinary Audit and Risk Committee meeting held on 12 August 2026.

1.2. The board is asked to note this report.

2.0 Key messages

The Audit and Risk Committee held an extraordinary meeting to consider the draft External Auditor's Annual Report 2025/26 and Value for Money commentary ahead of its publication with the ICB's Annual Report and Accounts at the end of September 2026. The report includes commentary on the ICBs procurement arrangements, financial sustainability, governance and improving efficiency, required by national guidance and regulations.

2.1. Discussion focused mainly on procurement improvement arrangements, including the need for clear actions, ownership, milestones and oversight. The website publication will signpost the ongoing procurement improvement work and progress made to date, supporting transparency on the actions being taken to address the issues identified.

2.2. The Committee endorsed progression of the report towards publication, subject to ongoing consideration of procurement improvement work through the Finance and Investment Committee.

20 August 2026

North East London
Integrated Care Board

NHS North East London ICB board

30 September 2026

Title of report	Remuneration committee exception report
Author	Katie McDonald, Governance Lead
Presented by	Diane Herbert, Non-executive member
Contact for further information	katie.mcdonald3@nhs.net
Executive summary	This report provides a summary of the key items from the Remuneration Committee meeting held on 9 September 2026.
Action required	The board is asked to note the report.
Previous reporting	A report was presented to the board at its meeting in July 2026.
Next steps/ onward reporting	An exception report will be presented to the board going forward.
Conflicts of interest	No conflicts of interest have been identified in relation to this report.
Strategic fit	Employment and workforce – to work together to create meaningful work opportunities and employment for people in north east London now and in the future.
Impact on local people, health inequalities and sustainability	The Committee will receive assurance on the ICB's employment flagship priority, ensuring that we utilise the ICB's ability to provide meaningful and positive employment opportunities for local residents.
Has an Equalities Impact Assessment been carried out?	An Equalities Impact Assessment is not required for this report.
Impact on finance, performance and quality	The Committee and its members are bound by the ICB's constitution, standing orders, standing financial instructions, policies and procedures of the ICB.
Risks	The Committee will be driven by the organisation's objectives and the associated risks, and its duties will be governed by the Terms of Reference. An annual programme of business will be agreed before the start of the financial year; however, this will be flexible to new and emerging priorities and risks.

1.0 Purpose of the report

1.1 This report provides a summary of the key items from the Remuneration Committee meeting held on 9 September 2026.

1.2 The Board is asked to note this report.

2.0 Key messages

2.1 The Committee considered the proposed interim changes to the People Function, which are intended to strengthen workforce leadership, organisational resilience and

organisational development capacity while longer-term arrangements are considered. The Committee supported the proposed approach and recognised the importance of maintaining strong people leadership through organisational transition.

2.2 An interim update on the organisational culture review was considered. The review identified positive themes relating to staff commitment, engagement and optimism for the future, alongside opportunities to strengthen leadership visibility, staff development and alignment between organisational values and staff experience. The Committee noted that actions to address the findings are being incorporated into broader organisational recovery and development programmes.

2.3 The Committee reviewed the organisation's comprehensive Workforce Race Equality Standard (WRES) and Workforce Disability Equality Standard (WDES) analysis. Whilst acknowledging the diversity of the workforce, members noted the continued under-representation of Black and Asian colleagues in senior leadership positions and requested further analysis to better understand workforce progression and inform future action planning.

2.4 In relation to the organisational change programme, the Committee was concerned with the Equality, Diversity and Inclusion (EDI) analysis which highlighted impacts on some staff groups. Members emphasised the importance of understanding the underlying causes of these outcomes and requested a comprehensive lessons learned review.

2.5 The Committee reviewed the Freedom to Speak Up quarterly report and received assurance that no concerns relating to patient safety, safeguarding, bullying, harassment or discrimination had been raised during the reporting period. Members supported continued promotion of the service and requested enhanced trend reporting and organisational learning.

2.6 Members considered the appraisals and objectives of the executive management team.

2.7 The Committee also reviewed the workforce capacity and capability risk that is on the Board Assurance Framework. Members agreed that organisational capacity, culture, workforce recovery and capability remain important areas for monitoring and supported the development of a dedicated workforce risk reporting mechanism. A copy of the BAF risk as attached as Appendix 1.

2.8 Members discussed the future role of the Committee and supported proposals to broaden its remit to reflect the increasing focus on workforce, organisational development and culture, including the potential development of revised Terms of Reference and a new committee title.

11 September 2026

Appendix 1

BAF Risk 06	ICB capacity, capability and culture		
Risk description	If the ICB does not have the capacity and capability across its workforce, finances, data and infrastructure to operate effectively, particularly following organisational change, its ability to plan, commission and assure services may be compromised, resulting in failure to deliver strategic objectives.		
ICS aim	**Which of the three shifts?**	**Risk Owner**	**Responsible Committee**
• To enhance productivity and value for money	• Hospital to community - Medium • Analogue to digital - Medium • Sickness to prevention - Medium	• Chief executive officer	• Remuneration committee
Initial risk score – July 2026	**Current risk score – September 2026**	**Target risk score – March 2027**	**Risk appetite**
16 4x4	**16 4x4**	**8 2x4**	2: Cautious: We have limited tolerance of risk with a focus on safe delivery

Progress since last update

This is the first instance of the risk being reported to the Committee. Progress made to mitigate the risk will be reported at each meeting going forward.

Controls and assurances

- Organisational change process oversight and delivery through Remcom, Transition Committee and EMT part 2
- Stocktake of new structure proposed from December 2026
- Comprehensive Transition Process designed to highlight issues for handover, pausing, stopping or amending
- Reflect, Reset and Rebuild programme to build ICB from strong foundations
- Freedom to Speak Up Guardian process offers route outside organisational change process
- Strengthened Staff Networks, including establishment of new Inter-Faith Staff Network

Mitigations in place

- Clarify roles and accountabilities as part of developing the Operating Model and future structure – December 2025
- Strengthen leadership capacity through operating model and ICB structure – senior leaders in place from March 2026, clinical leadership in place from March 2026
- Embed new operating model in new structures through the organisational change process (started December 2025, due to conclude September 2026)
- Weekly all staff briefing moved to fortnightly from September 2026 to maintain two-way flow of information

Planned and outstanding mitigations, controls and assurances

- Continuation of Transition Committee until September 2026

North East London
Integrated Care Board

NHS North East London ICB board

30 September 2026

Title of report	Finance and Investment Committee exception report
Author	Keeley Chaplin, Governance Lead
Presented by	Henry Black, Chief Finance Officer Lakh Jemmett, Non Executive Member for Strategic Finance / Chair of the Finance and Investment Committee
Contact for further information	keeley.chaplin@nhs.net
Executive summary	This is the first report of the Finance and Investment Committee (FIC) which met on 2 September 2026. The Committee considered the following business: • The Chief Finance Officer's report • The Month 4, 2026/27 finance report • The finance risk register and the Committee assigned Board Assurance Framework (BAF) risk • An update on the ICB procurement pipeline and improvement arrangements • Proposals concerning prescribing rebate scheme governance and future contract governance arrangements • Update from the Primary Care Contracts Sub-Committee • The Committee approved a number of business cases including the signing of a number of 2026/27 contracts and contract variations as well as a project initiation document to redevelop a health centre in north east London. One business case was deferred until further detail is made available.
Action / recommendation	The Board is asked to note the report.
Previous reporting	None – this is an exception report from the first Finance and Investment Committee meeting held on 2 September 2026
Next steps/ onward reporting	The Committee's feedback on its assigned Board Assurance Framework risk will inform the overarching Board Assurance Framework presented to the Board. Proposals concerning governance and delegation arrangements will be considered through the appropriate Board processes.
Conflicts of interest	No conflicts of interest have been identified in relation to this report.
Strategic fit	The ICS aims this report aligns with are: • To enhance productivity and value for money • To support broader social and economic development
Impact on local people, health inequalities and sustainability	The Committee considered how financial planning, commissioning, procurement and investment decisions support population need, health inequalities, patient care, service continuity and the effective use of the estate.

Impact on finance, performance and quality	The Committee provides assurance and oversight to the Board on the ICB's financial strategy, financial management, investment and associated governance. At its September meeting, it reviewed the Month 4 financial position, finance risks, procurement arrangements and proposals affecting future contracting and decision-making.
Risks	The Committee highlighted continuing risks relating to delivery of efficiencies, medicines price pressures, procurement capacity and timescales, contract oversight, and the alignment of financial and commissioning risks with the Board Assurance Framework. It requested clearer, measurable mitigations and stronger visibility of patient, service and value-for-money impacts.

1.0 Introduction

1.1 The Finance and Investment Committee met on Wednesday 2 September 2026. This report highlights, by exception, the key matters requiring the Board's attention following that meeting.

1.2 The Board is asked to note this report.

2.0 Key messages

2.1 The Committee welcomed the inaugural Chief Finance Officer's report as a useful framework for discussion. Members made suggestions for future reports including broader commentary on system-wide financial sustainability, governance and strategic challenges, regulatory developments, emerging commissioning arrangements and relevant learning from other integrated care systems.

2.2 The Committee approved the revised Finance Directorate Risk Register reporting approach and requested clearer risk tolerances, trigger points, measurable mitigations and alignment with Board Assurance Framework (BAF).

2.3 The Committee noted its assigned BAF risk on aligning commissioning resources with population need and agreed that its feedback should inform the overarching BAF presented to the next Board meeting, with regular review as priorities and commissioning arrangements develop. A copy of the BAF risk is attached at Appendix 1.

2.4 The Committee noted the Month 4 2026/27 financial position, including a favourable year-to-date variance and a continued forecast breakeven position. However, delivery of efficiencies and several demand and price pressures remained risks. Members requested further analysis demonstrating the strategic shift of investment towards community, primary care and preventative services, together with greater consideration of emerging national policy and wider system risks.

2.5 The Committee noted progress in developing the procurement pipeline and improvement arrangements. It requested that future reports identify the potential effects of key contract risks and procurement delays on patient care, service continuity and benefit realisation, and emphasised that value for money should be considered alongside patient outcomes and quality.

2.6 The Committee deferred the proposal to transfer approval of primary care prescribing rebate schemes to the Commissioning Board until the ICB Board had considered the wider governance and delegation arrangements.

2.7 The Committee noted proposals to establish a Contracts Sub-Committee and disestablish the Primary Care Contracts Sub-Committee. Its feedback for Board consideration emphasised clear approval routes, delegated responsibilities, escalation arrangements and manageable workloads.

2.8 The Committee considered business cases and contractual matters for decision.

3.0 Risks and mitigations

3.1 The Committee highlighted ongoing financial, procurement, contracting and governance risks. It requested clearer risk tolerances and trigger points, measurable and outcome-focused mitigation actions, and stronger alignment with the BAF. Members also requested improved reporting on the effects of procurement and contract risks on patient care, service continuity, outcomes and value for money.

Author: Keeley Chaplin, Governance Lead
Date: 10/09/2026

Appendix 1

BAF Risk 05	Resource allocation to meet the needs of the population		
Risk description	If commissioning resources are not aligned to the needs of the population, service delivery may become unstable and quality may deteriorate, resulting in poorer health and wellbeing outcomes, failure to deliver system objectives, increased regulatory intervention and reduced ICB autonomy.		
ICS aim	**Which of the three shifts?**	**Risk Owner**	**Responsible Committee**
• To enhance productivity and value for money	• Hospital to community– High relevance • Analogue to digital – Medium • Sickness to prevention - Medium	• Chief finance officer	• Finance and investment committee
Initial risk score – July 2026	**Current risk score – September 2026**	**Target risk score – March 2027**	**Risk appetite**
20 5x4	**20 5x4**	**12 3x4**	2: Cautious: We have limited tolerance of risk with a focus on safe delivery / 3: Open: We are willing to take reasonable risks, balanced against reward potential

Progress since last update

This is the first instance of the risk being reported to the Committee. Progress made to mitigate the risk will be reported at each meeting going forward.

Controls and assurances

- Alignment of the Commissioning and Finance objectives though the annual operating plan process
- Finance and Investment Committee oversight and scrutiny of the MTFS process and financial position.
- Commissiong Board and Strategic commissioning committee processes
- System level strategic alignment through regular engagement at System CFO and Chief Strategy Officer forums, ensuring financial and strategic plans and risks are aligned
- Financial performance and planning, reported and reviewed by regional/national teams.
- Participation in NOF Oversight meetings of the ICB and with Provider Trusts in north east London

Mitigations in place

- ICB financial planning and resource allocation processes strengthened through restructure process, concluded July 2026
- Updated MTFS reviewed at June 2026 Finance and Investment Committee.
- Development and deployment of transformation fund through the operating plan process to help enable strategic change, formulated pre-April 2026
- Development of integrated needs assessment and spend analysis to provide key resource allocation intelligence. First version submitted to Strategic Commissioning Committee and to Board, July 2026
- Enhancement of commissioning process through the Commissioning Group / Board architecture. Revised governance processes agreed by ICB Board, March 2026
- Medium-Term Operating submission made to NHSE on 12th February 2026, covering 2026/27, 2027/28 and 2028/29 outlines a shift in resources to enable strategic delivery.

Planned and outstanding mitigations, controls and assurances

- Continue to develop and refresh the Medium term financial plan ensuring the principles are applied to the operating plan (December 26)
- Further embed dynamic data tools for strategic planning and resource allocation processes. (Ongoing)
- Strengthen the ICBs service evaluation and benefits realisation capabilities (Ongoing)
- Further develop contractual forms and payment mechanisms to support the shift of care. (March 27)
- Refresh the deconstruction of block contracts and Service Line reporting information to provide intelligence into the operating plan process (November 26)
- Continue to develop the use of population health data to prioritise investment and disinvestment decisions, supporting alignment of financial resources to areas of greatest population need and impact. (Ongoing)

NHS North East London ICB board

30 September 2026

Title of report	Strategic commissioning committee exception report
Author	Katie McDonald, Governance Lead
Presented by	Imelda Redmond, Non-executive member and committee chair
Contact for further information	katie.mcdonald3@nhs.net
Executive summary	This report provides a summary of the key items from the Strategic Commissioning Committee meeting held on 9 September 2026.
Action required	The board is asked to note the report.
Previous reporting	A report was last presented to the ICB Board in July 2026.
Next steps/ onward reporting	The committee meets again on 14 October, and a further report will be presented to the board.
Conflicts of interest	No conflicts of interest have been identified in relation to this report.
Strategic fit	The ICS aims this report aligns with are: • To improve outcomes in population health and healthcare • To tackle inequalities in outcomes, experience and access • To enhance productivity and value for money • To support broader social and economic development
Impact on local people, health inequalities and sustainability	The Committee is required to monitor the impact of commissioned services on health and equity outcomes for our local population, performance, quality, workforce and finances.
Has an Equalities Impact Assessment been carried out?	An equalities impact assessment is not required for this report.
Impact on finance, performance and quality	There are no direct impacts resulting from this paper.
Risks	The duties of the committee will be driven by the ICS and organisation's objectives and the associated risks. The committee's programme of work will be flexible to new and emerging priorities and risks.

1.0 Purpose of the report

1.1 The Strategic Commissioning Committee (the Committee) was held on 9 September, and this exception report outlines the key messages and actions taken by its members in accordance with its terms of reference.

1.2 The Board is asked to note this report.

2.0 Key messages

2.1 The Committee adopted its Terms of Reference, subject to any membership amendments. It also supported, in principle, formalising the Commissioning Board as a sub-committee. Further revisions to the sub-committee Terms of Reference were

requested to clarify delegated authority, escalation routes, and chairing arrangements.

2.2 The Committee received the first recurring Chief Officers' report and welcomed the proposed approach to providing oversight of strategic commissioning developments. Members highlighted the need to minimise duplication between strategic commissioning, quality and finance governance, particularly for cross-cutting areas such as neighbourhood health.

2.3 Members reviewed the five Board Assurance Framework risks within the Committee's remit. The Committee requested clearer and more outcome-focused mitigations and requested a consistent route for identifying emerging corporate risks. The wording of the digital infrastructure risk will also be refined to distinguish strategic digital enablement from technical cyber-security assurance. A copy of the BAF risks are attached at Appendix 1.

2.4 The Committee considered the ICB planning approach for year two of the three-year planning cycle. Members supported the direction of travel but requested stronger involvement of local authorities and wider partners, clearer alignment with place and neighbourhood planning, and earlier opportunities for the Board and Integrated Care Partnership to shape priorities. Future updates will also show more clearly how inequalities are tested through the Integrated Needs Assessment, Health Inequalities Statement and Outcomes and Equity Framework.

2.5 Members supported the direction of the Medium-Term Financial Strategy, including the intention to create headroom for moving activity and resources towards primary, community and neighbourhood care. Members requested that the principles give greater prominence to population health and quality and clearly articulate substantive risks.

2.6 The Committee reviewed progress on neighbourhood health and noted that the self-assessment provided a useful developmental baseline but required further validation with places. Future reporting will combine the national framework with the north east London frameworks and provide clearer assurance on delivery and impact.

2.7 Members welcomed the emerging proposal for clinically vulnerable people with overlapping complex needs. The Committee emphasised the importance of partnership with local authorities and housing organisations, avoidance of duplication and evaluation based on outcomes and quality of life as well as financial impact.

2.8 The Committee received an update on the care closer to home programme and supported the whole-system approach to aligning and scaling existing local services. Members requested a delivery-focused update setting out measurable outcomes, timescales, benefits and completed mapping of current provision. Benefits realisation should include quality, resident outcomes and social value alongside activity and financial measures.

17 September 2026

Appendix 1

BAF Risk 01	Population growth and meeting demand		
Risk description	If population growth, demographic change and increasingly complex need are not met through timely service transformation, robust commissioning, resource allocation and capacity planning, demand may exceed system capacity, resulting in reduced access, poorer outcomes, failure to meet constitutional standards and unsustainable financial pressure.		
ICS aim	**Which of the three shifts?**	**Risk Owner**	**Responsible Committee**
• To improve outcomes in population health and healthcare	• Hospital to community – **High relevance** • Analogue to digital – Medium • Sickness to prevention – **High relevance**	• Chief strategic commissioning officer	• Strategic commissioning committee
Initial risk score - July 2026	**Current risk score – September 2026**	**Target risk score – March 2028**	**Risk appetite**
20 **5x4**	**20** **5x4**	**12** **3x4**	2: Cautious: We have limited tolerance of risk with a focus on safe delivery

Progress since last update

This is the first instance of the risk being reported to the Committee. Progress made to mitigate the risk will be reported at each meeting going forward.

Controls and assurances

- Oversight via EMT and committees
- System Neighbourhoods and Community Steering Group – newly focused Group will meet from September 2026
- Implementation of National Neighbourhood Health Framework, including development of Place based Neighbourhood Health Plans developed through 2026/2027 with Health and Wellbeing Boards
- System Commissioning Plan informed by Integrated Needs Assessment with future modelling and impacts understood
- Commissioning infrastructure and governance through the Strategic Commissioning Committee and ICB Board
- Updated MTFS process, including focus on resource allocation to support strategic shifts

Mitigations in place

- Development and delivery of neighbourhood health models, Strategy signed off by the Board in March 2025
- Strengthen integrated demand planning with providers through Collaboratives, System Level Groups and operating place process – submission February 2026, reflect in Strategic Commissioning Framework, refreshed May 2026
- Align commissioning intentions to population growth projections as part of Strategic Commissioning Plan submitted to NHSE May 2026

Planned and outstanding mitigations, controls and assurances

- Refreshed focus on future modelling of impact and delivery
- Care Closer to Home programme to set out mitigations on demand of population growth for urgent and emergency care
- Elective Care programme to demonstrate future modelling against growth in population and demand
- Transformation Fund execution to reflect innovative means of delivering transformation and change

BAF Risk 02	Population health improvement and prevention		
Risk description	If commissioning and contracting arrangements do not align system partners around improving population health and shifting resources from treatment to prevention, avoidable ill health and demand pressures may increase, resulting in poorer population health outcomes and failure to deliver the long-term strategy.		
ICS aim	**Which of the three shifts?**	**Risk Owner**	**Responsible Committee**
• To improve outcomes in population health and healthcare	• Hospital to community - Medium • Analogue to digital - Medium • Sickness to prevention – High relevance	• Chief clinical and quality commissioning officer	• Strategic commissioning committee
Initial risk score – July 2026	**Current risk score – September 2026**	**Target risk score - March 2027**	**Risk appetite**
16 4x4	**16 4x4**	**9 3x3**	3: Open: We are willing to take reasonable risks, balanced against reward potential

Progress since last update

- Workshop between ICB and Directors of Public Health (July 2026) took place to define further the Public Health role in strategic commissioning, areas of joint work and ensuring public health clinical leadership. including

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Controls and assurances

- Population health management approach
- Prevention strategy development
- System partnerships at Place and across north east London
- NEL System Strategy, NEL Strategic Commissioning Plan and NEL Strategic Commissioning Intentions are all structured against our NEL Outcomes and Equity Framework
- Newly established Population Health and Prevention System Group being established (autumn 2026)

Mitigations in place

- Align incentives towards prevention and early intervention (June 2026)
- Improve use of population data and insight (March 2026)
- Development of Optum platform for Population Health management
- ICB Prevention Strategy is being socialised
- Work with local Directors of Public Health to align approach
- Workshop between ICB and Directors of Public Health (July 2026) took place to define further the Public Health role in strategic commissioning, areas of joint work and ensuring public health clinical leadership. including

Planned and outstanding mitigations, controls and assurances

- Further engagement with Directors of Public Health to develop data and use in commissioning services (December 2026)
- Use of Optum to be maximised (March 2027) through continued focus on bespoke support to roll out
- Use examples with improvement socialised (December 2026)
- Strengthen cross-system accountability for prevention outcomes (March 2027) through newly established Population Health and Prevention System Group working to the Outcomes and Equity Framework

BAF Risk 03	**Health inequalities**
Risk description	If health inequalities are not systematically addressed through commissioning, service redesign and delivery, disparities in access, experience and outcomes may widen, resulting in failure to meet statutory duties and system objectives, and increased inefficiency, waste and duplication.

ICS aim	Which of the three shifts?	Risk Owner	Responsible Committee
• To tackle inequalities in outcomes, experience and access	• Hospital to community – **High relevance** • Analogue to digital – **High relevance (digital exclusion)** • Sickness to prevention – **High relevance**	• Chief strategy officer	• Strategic commissioning committee
Initial risk score – July 2026	**Current risk score – September 2026**	**Target risk score - March 2030**	**Risk appetite**
20 **4x5**	**20** **5x4**	**10** **5x2**	3: Open: We are willing to take reasonable risks, balanced against reward potential

Progress since last update

This is the first instance of the risk being reported to the Committee. Progress made to mitigate the risk will be reported at each meeting going forward.

Controls and assurances

- Equality and health inequalities duties
- Equality Impact assessments
- Programme reporting / CORE20PLUS5
- Health Inequalities Annual Statement
- NEL Outcomes and Equity Framework

Mitigations in place

- Embed reducing inequalities outcomes in all commissioning plans using evidence and community driven insights – March 26
- Target resources to the reduction of health inequalities through the Health Inequalities funding and commissioning plans, ensuring Transformation Fund spend on shift from sickness to prevention is clear in our monitoring - ongoing.
- Develop capability for monitoring access and outcomes by population group through population health management tools – July 26
- Work with communities through ICB health Inequalities Programme – 2022- current
- Independent evaluation of ICB Health Inequalities Programme – June 26
- Development of Prevention ‘green paper’ in partnership with Directors of Public Health – Mar 26
- Range of capability building initiatives including PCN health inequalities leads network, Pride in Practice, VCSFE development offer – Mar 26
- Launch of our Homeless Health Strategy – Sep 25
- Review of our work supporting clinically vulnerable cohorts – June 26

Planned and outstanding mitigations, controls and assurances

- Develop and implement board reporting mechanism for NEL Outcomes & Equity Framework – Nov 26
- Develop proposal for next phase of our work on health inequalities informed by an independent evaluation and engagement activity – Sep 26
- Launch of new Health Inequalities Programme – April 27
- Undertake equity training needs assessment and capability building with commissioning leads aligned to strategic commissioning functions – Nov 26
- Establish analytics collaboration with public health – Mar 27
- Review of Translation Services – Mar 27

BAF Risk 07	**Community, clinical and stakeholder engagement**
Risk description	If local people, communities, clinicians and partners are not effectively engaged in co-production, co-design and decision-making, services may become misaligned to population needs, resulting in poorer outcomes, reduced quality, safety and patient experience, ineffective commissioning decisions, and failure to meet statutory engagement duties.

ICS aim	Which of the three shifts?	Risk Owner	Responsible Committee
• To support broader social and economic development	• Hospital to community – **High relevance** • Analogue to digital - Medium • Sickness to prevention – **High relevance**	• Chief strategic commissioning officer	• Strategic commissioning committee
Initial risk score – July 2026	**Current risk score – September 2026**	**Target risk score – March 2028**	**Risk appetite**
15 **5x3**	**15** **5x3**	**9** **3x3**	3: Open: We are willing to take reasonable risks, balanced against reward potential

Progress since last update

This is the first instance of the risk being reported to the Committee. Progress made to mitigate the risk will be reported at each meeting going forward.

Controls and assurances

- Working with People and Communities Strategy, signed off by ICB Board on 1st July 2022
- Strategic Commissioning Framework in place March 2025 identifies key roles of effective engagement and co-production across the whole system as part of the commissioning cycle
- Strategic Commissioning Plans in place annually aligned to national framework – latest version refreshed for submission to NHSE in May 2026
- NEL HealthWatch Group facilitates regular meetings of the ICB with all HealthWatches to ensure alignment and sharing of key insights
- HealthWatch led Community Insights System funded by ICB since 2022 now core part of INA
- Outcomes and Equity Framework includes Resident Success Measures developed through the Big Conversation across north east London
- Clinical Cabinet established June 2026
- Make up of ICB Board continues to reflect local residents and clinical and care voice

Mitigations in place

- Funded delivery of Working with People and Communities Strategy, signed off by ICB Board on 1st July 2022
- Delivery of Strategic Commissioning Framework in place March 2025 with key roles for the People's Panel, Resident led Success Measures, Community leadership, in engagement
- Strategic Commissioning Plans in place annually aligned to national framework – latest version refreshed for submission to NHSE in May 2026
- NEL HealthWatch Group facilitates regular meetings of the ICB with all HealthWatches to ensure alignment and sharing of key insights
- HealthWatch led Community Insights System funded by ICB since 2022

Planned and outstanding mitigations, controls and assurances

- Refresh of Working with People and Communities Strategy – by March 2027
- New approach to business case development to incorporate focus on engagement – currently in progress

BAF Risk 08	**Infrastructure (estates and digital)**
Risk description	If digital transformation is not strategically planned and implemented, with effective management of cyber, digital exclusion and emerging technology risks, digital solutions may not be adopted safely, effectively or equitably, resulting in operational inefficiencies, increased cyber risk, poorer access, widened inequalities, reduced organisational capability and missed productivity opportunities.

ICS aim	Which of the three shifts?	Risk Owner	Responsible Committee
• To enhance productivity and value for money	• Hospital to community - Medium • Analogue to digital – **High relevance** • Sickness to prevention - Medium	• Chief strategy officer	• Strategic commissioning committee
Initial risk score – July 2026	**Current risk score – September 2026**	**Target risk score - March 2027**	**Risk appetite**
15 **5x3**	**15** **5x3**	**9** **3x3**	2: Cautious: We have limited tolerance of risk with a focus on safe delivery

Progress since last update

This is the first instance of the risk being reported to the Committee. Progress made to mitigate the risk will be reported at each meeting going forward.

Controls and assurances

- Annual DSPT & CE+ accreditation inc. external audits
- Monthly Cyber reporting into IGSG – covering controls inc. MFA, encryption, vulnerability and patch management
- Established process for reporting escalations into A&RC
- Weekly ICT SMT reporting & monitoring of Cyber dashboard
- Twice yearly corporate ICT DR failover testing inc. backup/restore
- NHSE CSOC Cyber alerting compliance reporting

Mitigations in place

- Upgrade end of life client operating systems inc. W10 prior to October 25 – completed September 2025
- Deploy MFA to corporate externally hosted systems – completed December 2025
- Deploy Beyond Trust for secure remote-control access – Completed November 2025
- Improved patching process to ensure 90%+ monthly across ICB / Primary Care – Introduced January 2025
- Access to corporate ICB network locked down to UK only- Completed July 2025
- All corporate and primary care sites behind enterprise firewall – Completed November 2025
- OneLondon virtual firewalls upgraded with latest Forti appliances and more restrictive policies applied – Completed September 2025
- OneLondon network routing restricted to ensure all internal traffic could not bypass firewalls and is centrally managed increasing security – Completed December 2025
- Technical assessment group (TAG) established for improved assessment on new software and system requests – Established February 2025
- Embed agreed approaches to addressing digital exclusion and work with London Health Mission to address digital exclusion across the capital – March 2027

Planned and outstanding mitigations, controls and assurances

- Migration of all GP practices to NHSmail national tenant cloud, enabling MFA for device login – Target date March 2027
- Replacement of low-risk end of life switches in GP practices by March 2028
- Pilot NAC solution for greater control of physical network access in GP practises (subject to FPP Cyber funding) March 2027, full deployment target March 2028
- Deployment of Cyber SIEM solution linked to NHS CSOC for greater visibility (subject to FPP Cyber funding) March 2027
- Focused review on the OneLondon platform cyber posture in recognition of increased storage of PID data and growing risk profile, target date for external review with a costed remediation plan November 2027

North East London
Integrated Care Board

NHS North East London ICB board

30 September 2026

Title of report	Quality, Safety and Improvement (QSI) Committee exception report
Author	Keely Horton, Governance Officer
Presented by	Kevin Cleary, Non-Executive Member
Contact for further information	Keely.horton1@nhs.net
Executive summary	This report provides a summary of the key items from the meeting held on 3 September 2026.
Action / recommendation	The Board is asked to note the report.
Previous reporting	The topics covered in this report have previously been considered and scrutinised by the QSI Committee.
Next steps/ onward reporting	The Committee next meets on 13 October 2026 and a regular exception report will be presented to the Board.
Conflicts of interest	There are no conflicts of interests.
Strategic fit	The ICS aims that this report aligns with are: • To improve outcomes in population health and healthcare • To tackle inequalities in outcomes, experience and access • To enhance productivity and value for money • To support broader social and economic development
Impact on local people, health inequalities and sustainability	Each topic is an area of service delivery which aims to improve the quality of care for local people through recognising opportunities for quality improvement.
Has an Equalities Impact Assessment been carried out?	An Equalities Impact Assessment is not required for this report.
Impact on finance, performance and quality	There are no additional resource implications/revenue for this report.
Risks	The Committee will be driven by the organisation's objectives and the associated risks, and its duties will be governed by the Terms of Reference. An annual programme of business is agreed before the start of each financial year; however, this will be flexible to new and emerging priorities and risks.

1.0 Purpose of the report

1.1 This report provides the Board with a summary of the key items from the Quality, Safety and Improvement (QSI) Committee held on 3 September 2026.

1.2 The Board is asked to note this report.

2.0 Key messages

2.1 Members discussed a delayed internal referral during Barking, Havering and Redbridge University Hospitals Trust (BHRUT)'s electronic patient record implementation. Members sought assurance on incident learning, duty of candour, outstanding referral risks and

communication with partners.

2.2 The continuing healthcare review backlog was reported to be ahead of trajectory. Work continues to strengthen service specifications, contractual engagement and access to provider data.

2.3 The Committee received an update on the Standardised Hospital Mortality Index (SHMI). The Committee requested a report in Q4 after assessment of a proposed module and revised data source, with findings also to be presented to the Clinical Cabinet for wider clinical oversight.

2.4 The Committee received an update on the north east London maternity and neonatal demand and capacity review. Members supported the proposed model of care and emphasised the importance of engagement with women and families and alignment with neighbourhood services.

2.5 The Committee noted positive overall assurance on providers' progress against the Fuller recommendations, with further work continuing where arrangements can be strengthened.

2.6 The Committee received updates on safeguarding reviews relating to Goodmayes Hospital and a local child safeguarding practice review in a special school. Multi-agency action and assurance arrangements are in place, with the final Goodmayes report scheduled to return to the Committee in October. Members discussed risks arising from reduced ICB capacity for on-site quality and safety visits.

2.7 Members welcomed the new combined approach of the integrated performance and quality report. Members supported its continued development to provide a stronger strategic commissioning, population health, outcomes and equity perspective.

2.8 The Committee was provided with a summary of the quality accounts received from providers and discussed the national NHS Quality Strategy and the development of the ICB's quality approach. Members supported further Board discussion on the quality management system and the capacity required to meet its expectations.

2.9 The Committee reviewed its Board Assurance Framework risk relating to quality management and quality improvement and agreed that the risk should include further detail on the mitigations in place. A copy of the BAF risk is attached at Appendix 1.

Author: Keely Horton, Governance Officer
11 September 2026

Appendix 1

BAF Risk 04	Quality management and quality improvement		
Risk description	If quality management and continuous improvement are not embedded across the organisation through effective oversight, learning and regulatory responsiveness, access, experience and outcomes may deteriorate, resulting in poorer health and wellbeing, increased regulatory intervention and reduced organisational autonomy.		
ICS aim	**Which of the three shifts?**	**Risk Owner**	**Responsible Committee**
• To tackle inequalities in outcomes, experience and access	• Hospital to community – High relevance • Analogue to digital – High relevance • Sickness to prevention – High relevance	• Chief clinical and quality commissioning officer	• Quality, safety and improvement committee
Initial risk score – July 2026	**Current risk score – September2026**	**Target risk score - December 2026**	**Risk appetite**
15 **5x3**	**15** **5x3**	**9** **3x3**	3: Open: We are willing to take reasonable risks, balanced against reward potential

Progress since last update

Clinical Directors have been recruited who will provide a clinical voice at all levels of commissioning, including at neighbourhood health delivery level, to address quality and patient safety.
Induction of Clinical Directors to ensure they understand the function of the ICB as a Strategic Commissioning body and their role in ensuring high quality services for our local residents.

Controls and assurances

- Quality Management System Framework supported by the NEL System Quality Impact Group
- Role of quality metrics across all contracts and contract management approaches
- Outcomes and Equity Framework
- Development of a consistent approach to quality across the ICB
-

Mitigations in place

- To ensure evidence-based quality improvement is featured in all commissioning specifications and contracts (July 2026)
- Recruitment of Clinical Directors who will provide a clinical voice at all levels of commissioning, including at neighbourhood health delivery level, to address quality and patient safety. (July 2026)
- Training of Clinical Directors to ensure they understand the function of the ICB as a Strategic Commissioning body and their role in ensuring high-quality services for our local residents. (September 2026)

Planned and outstanding mitigations, controls and assurances

- Development of a quality strategy with evidence of quality improvement based on clear evidence of system gaps, e.g. maternity, primary care etc (September 2026)
- Train staff in approach to quality and Quality Management System. (December 2026)
- Training staff in quality improvement techniques (January 2027)
- Develop ICB approach to quality improvement including the reduction of unwarranted clinical variation work with system. Work collaboratively across the system to highlight and maintain quality improvements (December 2026)

NHS North East London ICB board

30 September 2026

Title of report	Transition committee exception report
Author	Katie McDonald, Governance Lead
Presented by	Dame Marie Gabriel, Chair
Contact for further information	katie.mcdonald3@nhs.net
Executive summary	This report provides a summary of the key items from the meeting held on 8 September 2026.
Action required	The Board is asked to note the report.
Previous reporting	A report was presented to the Board at its meeting in July 2026.
Next steps/ onward reporting	This was the final meeting of the Transition Committee. Ongoing oversight of residual transition activity, risks and assurance will be transferred to the relevant committees and executive governance arrangements.
Conflicts of interest	No conflicts of interest have been identified in relation to this report.
Strategic fit	The ICS aims this report aligns with are: • To improve outcomes in population health and healthcare • To tackle inequalities in outcomes, experience and access • To enhance productivity and value for money • To support broader social and economic development
Impact on local people, health inequalities and sustainability	The transition programme supports the development of an ICB operating model focused on strategic commissioning and population health improvement. The Committee continued to consider the implications of organisational change for staff, statutory functions, service continuity, equality and the communities served.
Has an Equalities Impact Assessment been carried out?	An equalities impact assessment is not required for this report. The Committee considered equality analysis relating to the organisational change programme and agreed that learning from the process should inform future recruitment and organisational change activity.
Impact on finance, performance and quality	There are no direct financial, performance or quality impacts arising from this report. The Committee reviewed assurance on the affordability of the future operating model, the management of transition costs and the continued safe delivery of statutory functions and services.
Risks	The Committee oversaw risks relating to the transition process and future ICB operating model. Residual risks and outstanding work will transfer to the appropriate standing committees and executive governance arrangements, with further assurance on statutory functions to be considered through the Audit and Risk Committee.

1.0 Purpose of the report

1.1 The Transition Committee (the Committee) held its final meeting on 8 September 2026. This exception report outlines the key messages, assurances and actions arising from the meeting in accordance with the Committee's terms of reference.

1.2 The Board is asked to note this report.

2.0 Key messages

2.1 The Committee reviewed progress with the organisational transition programme. It noted that 96% of staff appointments had been completed, and that most critical-path activity was expected to conclude by the end of November, with some transfers and staff support activity continuing into early 2027. The Committee was assured that statutory functions continued to be monitored through executive governance arrangements.

2.2 The Committee considered workforce equality analysis and was concerned by disproportionate compulsory redundancy outcomes for some staff groups. Members recognised the safeguards applied during the process but agreed that the outcomes required further examination. An after-action review will therefore consider the design and operation of the change process, qualitative and quantitative evidence, and any indicators of structural or unconscious bias, with learning used to inform future recruitment and organisational change activity.

2.3 The Committee reviewed the financial position associated with the transition and received assurance that the future structure could be managed within the planned running-cost allowance. The position retains headroom for redundancy, workforce overhang and legal costs, although delivery remains dependent on planned leaving dates and the successful redeployment of displaced staff. Members emphasised that future decisions must continue to prioritise the safe delivery of functions and services.

2.4 Members considered arrangements for closing the Committee and transferring its remaining responsibilities. It was agreed that residual risks and outstanding work should move to the relevant standing committees, supported by clear handover information. The Audit and Risk Committee will be asked to review the assurance approach for statutory functions and to consider a more graded assessment of assurance beyond confirmation that minimum statutory requirements have been mapped.

2.5 The Committee supported a broader after-action review of the transition, covering processes, outcomes and the experience of staff. Learning will be taken forward through executive arrangements and the relevant committees. The Committee also noted the continued work to support staff and rebuild organisational identity, leadership, engagement, culture and ways of working. The Chair thanked members and officers for their contribution and formally concluded the work of the Transition Committee.

14 September 2026

ICB Board forward plan

NHS
North East London
Integrated Care Board

Period / meeting date	Category	Agenda item	Lead	Notes	Status
21 October 2026	Corporate items	Anti-racist strategy	CEO		Planned
25 November 2026	Corporate items	Chair and chief executive report	Chair/ CEO		Planned
25 November 2026	Corporate items	Committee exception reports	NEMs		Planned
25 November 2026	Finance and performance	Quality and performance report	CSO/ CCQCO		Planned
25 November 2026	Corporate items	Current affairs in north east London	CSCO		Planned
25 November 2026	Corporate items	Governance update	CSCO		Planned
25 November 2026	Corporate items	Resident story - themed around a key agenda item	Neighbourhoods		Planned
25 November 2026	Finance and performance	Financial overview	CFO		Planned
25 November 2026	Strategic items	0-25 commissioning strategy	CSCO		Planned
25 November 2026	Strategic items	VCFSE strategy and delivery plan	CSCO		Planned
25 November 2026	Strategic items	Neighbourhood working update	CSCO		Planned
25 November 2026	Strategic items	Working with people and communities strategy refresh	CSCO		Planned
28 January 2027	Corporate items	Chair and chief executive report	Chair/ CEO		Planned
28 January 2027	Corporate items	Committee exception reports	NEMs		Planned
28 January 2027	Finance and performance	Quality and performance report	CSO/ CCQCO		Planned
28 January 2027	Corporate items	Current affairs in north east London	CSCO		Planned
28 January 2027	Corporate items	Governance update	CSCO		Planned
28 January 2027	Corporate items	Resident story - themed around a key agenda item	Neighbourhoods		Planned
28 January 2027	Finance and performance	Financial overview	CFO		Planned
28 January 2027	Strategic items	Anti-racist strategy review	CEO		Planned
28 January 2027	Strategic items	Board Assurance Framework	CSCO		Planned
28 January 2027	Strategic items	Neighbourhood working update	CSCO		Planned
31 March 2027	Corporate items	Governance improvement plan	CSCO	May be included in Chair / CEO report	Planned
31 March 2027	Strategic items	EPRR annual report	CCQCO	May be included in Chair / CEO report	Planned
31 March 2027	Corporate items	Chair and chief executive report	Chair/ CEO		Planned
31 March 2027	Corporate items	Committee exception reports	NEMs		Planned
31 March 2027	Finance and performance	Quality and performance report	CSO/ CCQCO		Planned
31 March 2027	Corporate items	Current affairs in north east London	CSCO		Planned
31 March 2027	Corporate items	Governance update	CSCO		Planned
31 March 2027	Corporate items	Resident story - themed around a key agenda item	Neighbourhoods		Planned
31 March 2027	Finance and performance	Financial overview	CFO		Planned
31 March 2027	Finance and performance	Approval of ICB budget / operating plan	CFO		Planned
31 March 2027	Strategic items	Neighbourhood working update	CSCO		Planned
31 March 2027	Strategic items	Review and approval of corporate objectives	Chair/ CEO		Planned
May 2027	General	CDC procurement	CSCO/ CFO		Planned